

Max Healthcare (MAXHEALTH)

Conference call transcripts, oldest-first. 16 call(s). Generated 2026-08-02.

Call: Aug 2022

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August 17 , 2022

Listing Department, Listing Department,
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Symbol: MAXHEALTH Scrip Code: 543220

Sub: Transcript of the e arnings call on financial results of Q1 FY23 of Max
Healthcare Institute Limited ("the Company") held on August 11, 2022

Dear Sir/Ma'am,

We refer to our intimation dated August 11 , 2022, w.r.t. uploading of audio recording on
the website of the Company , of the earnings call hosted on August 11 , 2022 to dis cuss
the financial results of the Company for Q1 FY23 .

In this connection, pursuant to Regulation 30 of the SEBI (Listing Obligations and
Disclosure Requirements) Regulations, 2015, please find enclosed the transcript of the
said earnings call. The said transcript is also being uploaded on the website of the
Company i.e. <https://www.maxhealthcare.in/investors/investor-resources> .

Kindly take this intimation on your record.

Thanking you ,

Yours sincerely,
For Max Healthcare Institute Limited

Ruchi Mahajan
SVP – Company Secretary & Compliance Officer

Encl. as above

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Max Healthcare Institute Ltd
Q1 FY23 Earnings Conference Call Transcript
August 11, 2022

Moderator: Ladies and gentlemen, good day and welcome to the Max Healthcare's Q1 FY23
Earnings Conference Call. Please note, that this conference is being recorded.

I now hand the conference over to Mr. Suraj Digawalekar from CDR India. Thank
you, and over to use, sir.

Suraj Digawalekar: Thank you. Good morning, everyone, and thank you for joining us on Max
Healthcare's Q1 FY23 earnings conference call. We have with us today, Mr. Abhay
Soi, Chairman and Managing Director, and Mr. Yogesh Sareen, Senior Director and
Chief Financial Officer of the company. We will begin the call with the opening
remarks from the management, following which, we will have the forum open for
interactive question -and-answer session.

Before we begin, I would like to point out that some statements made in today's discussion may be forward-looking in nature and a disclaimer to this effect has been included in the earnings presentation shared with you earlier.

I would now like to invite Abhay to make his opening remarks. Thank you, and over to you.

Abhay Soi: Good morning, everyone. It gives me immense pleasure to welcome you to Max Healthcare's earning call for the first quarter of the ensuing financial year. Let me provide you with the key highlights of the quarter's performance before opening up the forum for question and answers.

In continuance of our trend towards the end of the previous quarter, quarter one FY23 was largely a normalized quarter with average occupancies hovering around 74%, while less than 1% of beds were used for COVID patients. It was the highest ever revenue and EBITDA quarter and marked some movement in the direction already articulated earlier. We witnessed consistent performance throughout the quarter, and almost all the operating and financial parameters touched a new high.

Consequently, we registered highest revenue, ARPOB, EBITDA, EBITDA per bed, etc. This was driven by improvement in payor mix, surgical medical mix, annual price revision and normalization of patient footfalls, as we continued to focus on the growth levers set out by us post Omicron wave.

I would like to remind you all, in quarter one last year, we had separately reported a revenue of Rs.136 crore and an EBITDA of Rs.59 crore from COVID-19 vaccination. To that extent, the year-on-year growth numbers are being reported on a like-to-like basis, excluding this non-recurring item.

The key highlights for the quarter one performance were, the average occupancy for the quarter recovered to 74% from 68% in quarter four FY22, which was largely impacted due to Omicron COVID wave in the first half of Q4 FY22. The bed share of

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institutional patients, relatively a lower ARPOB channel has been brought down to 30% from 33% in Q4. This has added to growth in ARPOB, as well as EBITDA per bed. This is in line with our guidance and objectives. The ARPOB for this quarter rose to Rs. 66,000 implying a growth of 28% year-on-year and 4% quarter-on-quarter.

Network gross revenue stood at Rs. 1,473 crore of which, Rs.2 crore is contributed by revenue from vaccinations. Gross revenue excluding COVID-19 vaccination grew by 18% year-on-year and 14% quarter-on-quarter.

The Digital channel's share of revenue was the highest ever at 16% in quarter one FY23 as compared to 13% in quarter one FY22, with website traffic growing by 14% quarter-on-quarter to reach 33 lakh sessions. In absolute terms, the Digital revenue stood at Rs.232 crore in quarter one FY23, which is 40% of the Digital revenue of Rs.585 crore clocked in the last financial year, that's the whole year.

Also, our International Medical Tourism revenue reached pre-COVID levels in this quarter despite negligible revenue from Afghanistan, which was one of our key territories till FY20. We continue to expand our geographical presence and expect to have 20 plus offices operating from various countries by the end of this financial year. This coupled with the new Heal in India initiative should augur well for medical tourism to Max Healthcare as well.

We continue to do our bit to provide care to underprivileged. During this quarter, we served 38,500 indigent patients in IPDs and OPDs free of charge. The notional value of this treatment was Rs.49 crore.

Network operating EBITDA excluding COVID-19 vaccination for quarter one FY23 was Rs.370 crore compared to Rs.301 crore in Q1 FY22 and Rs.304 crore in the fourth quarter of FY22, reflecting a growth of more than 23% year-on-year and more than 22% quarter-on-quarter, respectively. EBITDA margin was 26.6% as compared to 24.8% in the fourth quarter FY22, leading to a PAT of Rs.229 crore. EBITDA per

bed, most importantly, for the quarter was Rs.62 lakh, showing an improvement of 10% quarter -on-quarter.

Cash generated from operations after interest tax and replacement CapEx was Rs.237 crore versus Rs.179 crore in the fourth quarter of FY22. Further, net debt reduced to a low of Rs.217 crore at the end of June '22, from Rs.441 crore at the end of the previous quarter. However, the present net debt includes a put option liability of Rs.141 crore. So, the actual net debt after that is less than Rs.100 crore.

Coming to the strategic business units, Max Lab, which is a non -captive pathology vertical reported gross revenue of Rs.26 crore. It added 90 channel partners during the first quarter of FY23, taking the overall active clients to 850 plus, spread across 32 cities. On a like -to-like basis, the revenue excluding COVID -19 related tests grew by 50% year -on-year and 24% quarter -on-quarter. We continue to invest in this business and expanded the team to more than 700 people working across functions.

Max@Home, our Home Healthcare vertical reported gross revenue of Rs.32 crore, a growth of 10% over fourth quarter levels, fourth quarter last year, and representing a growth of 18% year -on-year. The 650 plus strong team at Max@Home also manages a network of 62 medical outposts across corporates, of which 13 medical rooms were added in the first quarter of FY23.

Going forward, we continue to employ the following growth levers. One, International Medical Tourism growth from both existing and new geographies. Two, improvement in payor mix. Three, fast tracking brownfield expansion. 100 additional beds will be operational in this year at Max Shalimar Bagh, while additional 300 beds in Dwarka would be in early first half FY24. With large capacity addition in FY25, we are poised

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for growth in the foreseeable future. And finally, inorganic expansion in

both hospital

and perhaps, if we get some good opportunities in the diagnostic space as well.

We also made considerable progress on the digital front and are in the final stage of rolling out a proprietary app, which will not only help us engage with more patients and widen our range, reach, but also serve our customers better and provide improved experience.

On this note, I would like to open up the forum for question and answers. Thank you.

Moderator: The first question is from the line of Damayanti Kerai from HSBC.

Damayanti Kerai: My question is, how much price hikes you've taken for the hospital services and also as you can elaborate how price hikes flow across different payor groups in terms of change?

Abhay Soi: So, essentially, we have, normally a price impact of about 2%, 2.5% at best year -on-year at the end of this thing, okay? What it impacts is, essentially, the international patients, by that extent, it will impact your cash paying patients. Insurance are typically two- or three -year contracts. So, every couple of years, it's sort of retriggers. And then you come to the institutional business, which doesn't get impacted by it.

Damayanti Kerai: Okay. So this 2% to 2.5% kind of increment which you have taken for the current year, it's already visible for the cash and international patients and insurances, as you said, when contracts are renewed and the institution business it takes longer?

Abhay Soi: Yes. So, what we are retriggering right now is when the new contracts come, we'll be on the new. So, we got plethora of contracts, and all of them don't start at the same time. So if they are two -year contracts, you can pretty much evenly sort of spread them over two years or three years. You will have some impact of that. Whichever contracts are coming off, are coming on stream or coming up for renewal, they will renew at new prices.

Damayanti Kerai: Okay. So, does it mean the patient was paying cash, they have to bear the costs? Like higher costs compared to someone who is covered by insurance on an immediate basis?

Abhay Soi: That's right.

Dam ayanti Kerai: Okay. And in general, like what kind of impact we have seen on healthcare demand due to inflation? So, like price hikes have gone up. So, any impact on the demand?

Abhay Soi: You have seen the occupancy, right? From 68% we moved up to 74% quarter -on-quarter.

Dam ayanti Kerai: So, we'll say it's immune to price changes, because it's essential services?

Abhay Soi: I think it is a very marginal price change, isn't it? I mean it is not a very large price change.

Dam ayanti Kerai: No, in general, like 2% to 2.5% price hike for this year, but say, the inflation goes up meaningfully across the industry, then should we assume any tapering on the demand part?

Abhay Soi: The inflation in the industry doesn't impact the consumer. Does it? The price hike as a result of the inflation impacts the consumer. That's right? So, if the price hike is 2% to 2.5%, that is what is impacting the consumer. Now, if tomorrow, inflation is significantly, well, whatever the inflation has been, you've seen, has gotten absorbed,

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and you've seen the margin sort of expand in spite of the 2% to 2.5% increase in price. So, you're better off in terms of margins. That's one.

Secondly, and more importantly, if tomorrow, there is a further, let's say, there is a push up because of inflation and you have to reprice it, then perhaps in October, you look and relook at pricing and say, look, if I have to pass on the inflation, I have to improve the price or increase the price. Your question would become relevant at that stage.

Dam ayanti Kerai: Okay. And this kind of price revision, you do once in a year or?

Abhay Soi: Once in a year, typically, but we always have the option of relooking at it on October 1. I mean, you can even do it every quarter,

you can do it mid-year, anytime you want. But the fact is disruptive. So, I mean, if we have to look at it, we'll look at it at that stage, after six months. I mean every two months, three months, you can't keep changing your schedule of charges.

Dam ayanti Kerai: And on the International part, you mentioned, its broadly back to the pre-COVID level, and then you're stepping up your presence in the target market to get more footfalls. So which key markets were you looking to?

Abhay Soi: That's not what I said. I said we are back to pre-COVID levels, okay, in spite of one of our key markets, which is Afghanistan giving us negligible business. That was a key market because of travel restrictions from Afghanistan. So, when Afghanistan normalizes, that would improve it even further. We are already back to pre-COVID levels, because we made efforts in other markets as well. Right.

Dam ayanti Kerai: And finally, like in terms of contribution from international business, how much that can go up in next two to three years?

Abhay Soi: You know, sky's the limit. I think the Government is now really focused on this. You will probably hear on Independence Day, as a part of the Independence Day speech, at least what I'm reading in the press is, the Heal in India is going to get a big push which is India becoming a destination for healthcare tourism, and that is what the Government of India and the Prime Minister himself is going to include in his speech. So, that should all augur very well. Sky's the limit. If you are asking me for precise things, we don't give forward-looking guidance in terms of specific numbers like that. But look, I think you have a huge comparative advantage in India. With the right push and the right encouragement from the Government, literally sky's the limit as far as that.

Moderator: The next question is from the line of Nikhil Mathur from HDFC Mutual Fund.

Nikhil Mathur: My question is on the new beds and the associated costs that will have to be incurred for new beds expansion that the company is under taking. In FY22, the company had indirect overheads of Rs.1,700 crore odd, which was associated to 3,200 capacity beds, roughly Rs.54 lakh or Rs.53 lakh was cost of bed, is the indirect overhead that I'm able to calculate on a ballpark in FY22. With 100, 300 and 1,200 beds likely to be added next three years, we are looking at bed buildup of 1,570. So, when I look at the cost buildup for the new beds, should I multiply the 1,570 beds with the indirect overhead you have on the existing capacity bed or it could be lower? If you can give some sense, what kind of possible that should be associated with the new beds that are coming on stream?

Abhay Soi: See, likely the brownfield, if you see out of the 3,000 beds, 80% of the new capacity is brownfield, right. In a brownfield, you already have your management cost, as well as your senior clinician costs being incurred by the existing hospitals. So, I mean, I would look at it little differently rather than looking at the cost and start dividing it and say okay, look, my EBITDA per bed is Rs.62 lakh today, right. Now this Rs.62 lakh,

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what is it going to be three years down the line? And more I mean, with the payor mix distillation, any inflation increases in price and so on and so forth?

But that is what is going to be attributable to the old capacity. The new capacity which comes up, okay, will have some operating leverage in it simply because, like I said, you are not going to have a lot of the costs that have already been incurred within the Rs.62 lakh per bed. So that number for the additional beds should be higher theoretically. So, you would need EBITDA per bed and multiply it by the occupancy in both installed beds. That's how we will look at it. I don't see any major fixed costs coming on brownfields.

Nikhil Mathur: So, before these beds comes on

stage, whatever, EBITDA per bed the company kind of stabilizes and in the next two or three quarters, while the new beds will come on stage, there should not be any major dilution in EBITDA per bed. Would that be a fair assessment?

Abhay Soi: Correct. That's correct, which we've guided into also in brownfields, typically your breakeven is the first quarter or whatever. And we've seen that in Vaishali Hospital, we came up with 100 odd beds. I mean, there was no sort of --- I mean these are required. These hospitals are operating 80% occupancy. You put another 100 beds that get taken up very quickly. We don't see any abatement in EBITDA in absolute terms.

Nikhil Mathur: Got it. And two near-term bed addition in Shalimar Bagh and in Dwarka, in the last two investor presentations, the timeline has remained same. So, can you highlight some milestones which would have achieved in both these facilities just to give some comfort that these bed additions are on or on track?

Abhay Soi: See, Dwarka is in finishing stage and as far as Shalimar Bagh is concerned, it is reaching finishing stage. I mean, if you are going to open up one capacity in the current year, you can, you've already got a --- current year means that obviously, you're looking at the next six to nine months, okay. And then the new capacity in perhaps three to four months after that, you have to be pretty close to finishing. And that's what on the ground reality is. I'm not giving visibility something three years later. I mean, if you see something is coming up in six months or nine months, right, I mean, then obviously the structure, this thing, everything is ready, you're doing fit-outs and stuff like that.

Nikhil Mathur: And third question is on the international patient? Can you tell me the number of beds which are occupied by international patients in this particular quarter? And then what this number of beds can look like, let's say in 12 months?

Abhay Soi: First question, I think about 6% but Yogesh will just sort of lean in on this. But secondly, like I said, I'm not giving any guidance on where it can be. The sky's the limit, like I said, So that number can keep moving up.

Yogesh Sareen: Yes. 4.5% to 5% of the beds were occupied by international patients this quarter.

Nikhil Mathur: So, the revenue mix and the bed count mix is the same. So, my understanding might be wrong, but I thought that international patients come in with a much higher ARPOB, right.

Abhay Soi: Right.

Yogesh Sareen: Yes. That is right. That is why you see that the overall revenue share is 8% and the and the bed share is 4.5% to 5%, right. That signifies that.

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Moderator: The next question is from the line of Praveen Sahay from Edelweiss Wealth Management.

Praveen Sahay: My first question is just I'm repeating the earlier question that's related to the EBITDA per bed. As you said that down the line three years, with the new capacity coming in place, you trying to manage this EBITDA per bed from there or is there some deterioration we can see?

Abhay Soi: So, like I said, look 80% of the capacity is coming in the form of brownfields, right. What is a brownfield? You have existing hospital which is operating, let's say at 80%, 85% occupancy. You are waiting in the ER for a couple of days for beds, okay, the hospitals run out of space. Okay. Where do you go from there? You put up another building adjacent or right next to it. Now, okay, so you incur the CapEx of the building, okay. Now, your CEO, your management, your clinicians etc., already, all your sort of fixed, the fixed indirect cost has already been incurred by the existing hospital.

So, let's say in the new building, I want to open which is 100 beds, but I only have demand for 20 beds. I'll only open 20 beds over there. I will put nurses etc. of 20 beds. Although my building is ready. My fixed cost,

this thing is already there. So, also, the higher operating leverage. So, I don't see your EBITDA margins, or EBITDA per bed going down, because it should be accretive theoretically.

Praveen Sahay: And as you are in this year in the coming year, coming capacity in the Shalimar Bagh and Dwarka, so, with this, any change in the clinical mix also we can say, because?

Abhay Soi: No, this has nothing to do with the clinical mix.

Praveen Sahay: Shalimar Bagh and Dwarka has some different, on the company level, it's a different clinical mix. So, is there any impact you are expected to see in the next two years in your clinical mix on the overall company level?

Abhay Soi: Where do you see from a company level; it is a different clinical mix? Dwarka and Shalimar Bagh?

Praveen Sahay: It is similar, you're saying?

Abhay Soi: No, no, I'm asking you. I don't think the specific hospital clinical mix is something that we have ever guided to.

Praveen Sahay: Okay, so now the next question is related to the payor mix, and which you had guided earlier for Institution business to go down to 15%? So, what's the timeline for that where you want it?

Abhay Soi: Over the next year and a half. I mean, do keep in mind, this was 37%, previous quarter. It was brought down to 34%, 33%. Now it is down to sub 30%. And hopefully, that will be the trajectory, right. So, the next year and a half, this should be down to 15% or below.

Praveen Sahay: And last question on the occupancy side. The current occupancy level, you are

seeing this as a normal 75%, 76%, is a normal rate, which you will run on?

Abhay Soi: I mean pre-covid level, we used to be 71%, 72%. Okay. I mean, obviously, as the disease burden goes up. We've also operated and you've seen 80%, 81%, 82%. On a sustainable basis, I've said in the past, we can go up to 77%, 76%, 77% across the network, at the network level. And these have been the occupancies. Yes, so I mean, there could be a couple of percentage points sort of over a period of time, but that's about it. So, it's marginal. So, the real value will come from the distilling the payor mix.

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Praveen Sahay: Okay. And in the inorganic, are you also looking in the hospital business, any opportunity?

Abhay Soi: Will we are mostly looking at opportunities only in the hospital business to be honest.

Praveen Sahay: Not in diagnostics?

Abhay Soi: Well, not actively at present.

Moderator: The next question is from the line of Andrey Purushottam from Cogito. Please go ahead.

Andrey Purushottam: Just a little bit more light on your margin drivers. Specifically, if you were to look at a two or three -year term horizon, how do you see your international tourism shaping up in terms of percentage of revenue from 7.3% or so? And also, in terms of, I believe your CGHS proportion to revenue is already low as compared to other hospitals. Would you be seeing this coming down to zero in the near future and seeing it substituted by higher margin payors? And what other elements of the mix can you know, add further color to, which would give us an idea as to how your margin would expand in a two or three -year horizon?

Abhay Soi: So first, I think your presumption is incorrect. Compared to our peer group, our institutional business CGHS is the highest. More than twice of Apollo and maybe more than 50%, 70% of Fortis.

Secondly, we are not giving any guidance on margin like we have resisted from doing that in the past. Again, like in the previous one also I said, also said international business, I'm not going to give you. I can tell you what the opportunity set is, which is a great thing. I've been a big proponent of international medical tourism. And really, I think the big flip we're going to get now is because the Government of India, okay, is putting its weight behind it. And we are reading about Heal in

India initiative, okay, again, which the Prime Minister may take up on his Independence Day speech, which is a big, big thing. When the Prime Minister takes it up, the rest of the ministries and organizations will follow. So, I see a huge opportunity emanating from that. I've always been a proponent that we have a big comparative advantage, I think it's being recognized at present.

A little difficult for me to answer what percentage of their businesses because like I said, even if I have an internal assessment, it's something that we've resisted from giving forward-looking guidance on margins as well as this. What we have said is that the levers and you can perhaps make that out, okay, for our margin improvements going forward, besides medical tourism is going to be a key is distilling the payor mix, that means reducing the institutional business which we are from 30% odd, looking to bring down below 15% over the next year and a half. This business yields us 45% to 50% lower revenues, okay. And once this business is replaced by, as we are seeing, is trending also, by a non-institutional business, it will generate 45% to 50% more revenues and 85% of that comes straight to our EBITDA. So that can have a major impact on both, ARPOB, as well as EBITDA per bed.

Andrey Purushottam: So, this reduction in the share of Institutional business, what impact will it have on occupancy rates? And would you be able to keep the same occupancy rates that you're at right now? From what you said earlier in the call, you operate, let's say, on an average of maybe 77%, 78% in the span of sustainable occupancy rates, right?

So, how do you see the trade -off between that and...

Abhay Soi: There is no trade -off. I think you're looking at the wrong equation. Okay. See, today we do this business not because we want to --- not because we have to, because we want to. Rather than keeping a bed idle, we'd rather do institutional business,

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right? It's not as if you stop the institutional business, make the bed idle, okay, and then hope you're going to get business. Because if that was the case, then we would never start doing this business. The way to do it is okay, that today we have non-institutional business which is our preferred channel right, which has a certain rate of growth now, I won't even go down to all the aspirational things that international business, etc., etc. that we are doing. Forget all of that. See, that business has been growing at a particular rate. Now, we've already hit an overall ceiling almost as far as the occupancy is concerned, like you rightly alluded that we already at 74%, 75%. So, where do you go from here? Maybe a couple of percentage points.

But the preferred channel is growing, requires more beds, 200 or more beds per year. Where do you get them from? I have to displace my institutional business. Each hospital of ours has about 200 contracts with various public sector undertakings, so on and so forth. And these are hospital level contracts which are for a period of six to nine months and then you have a notice period of one month both sides, if you want to discontinue. So, you keep switching off one after the other on a particular hospital wherever you have no bed for preferred channels effectively. Case in point was Gurgaon facility. We used to do 20% business from the institutions. Okay. Now it's down to zero. But our EBITDA per bed over there Rs.90 lakh plus compared to Rs.60 lakh for the rest of the thing, because of that. So, I hope I've been able to sort of answer your question in some manner. So it's not either, or, I mean, we only, once we have the demand we move away. So, you don't ever see a reduction in occupancy because you are moving from institutional

Andrey Purushotta m: Okay. And one last clarification. A lower AL OS is good for you commercially? Lower the average length of stay, the better it is commercially for you?

Abhay Soi: That's right. That's a key indicator for all hospitals. For any hospital, you make money in the surgeries of the procedure. Most profitable thing is the daycare. Okay. The longer a patient stays in a hospital, it is a loss making. Every day a patient stays in a hospital, that is loss making. Most people believe that hospitals are holding them back and making money. That is the other way round. The hospital want you out.

Andrey Purushottam : Because all the expenditure really becomes upfront ed right on the first day itself?

Abhay Soi: That's right. That's right. So, your returns on daycare etc., if the room charge is separate it's Rs. 4,000, Rs.5,000 a room, with the nurses, with this, with that, etc. etc., you can imagine.

Moderator: The next question is from the line of Dheeresh Pathak from White Oak .

Dheeresh Pathak: So in your presentation you have a slide, which says that CapEx for the quarter was Rs.13 crore only, and there was another slide, which says that the total Capex for the year is supposed to be some Rs.600 crore plus. So why was this quarter low?

Yogesh Sareen : Yes. I think we haven't really made the payment to the contractors, because there were already advances issued in last quarter. We will be making some payments in this quarter. That 's also just cash outflow stuff, right. It doesn't have anything to do with the work which is happening at the site.

Dheeresh Pathak: Okay. So, work at the Saket site and the Nanavati site, how are they progressing?

Abhay Soi: So Nanavati site, the piling work is on more or less, more than 70% of the piling has been done, the D-wall has been done. We will start --- demolition has been done. the digging is going to start now. Most of all, almost all the approvals are in place. As far as the Saket is concerned, I think retaining wall has been done. Recently, we have to transplant trees, there was an order that you can't transplant trees in Delhi. Now the order has come that you can transplant trees, re-transplant those trees. So,

we're doing that and I think we're going to start working. I mean work has already started; I think constructions will start.

Dheeresh Pathak: Okay.

Abhay Soi: There is a process that you have to make a retaining wall, you have to make a D-wall, you have to do piling. You have to do foundation.

Dheeresh Pathak: Yes. More than that, I wanted that all the approvals that were, like the tree issue and all those Government approvals are in place, right?

Abhay Soi: Right.

Dheeresh Pathak: So, it is now in our control. Then the second question was on the Max Labs. So I'm just trying to understand that there's a gross revenue of Rs.40 crore, and then there is a net of Rs.25 crore. The balance is payment to the collection center as well as to the Max Hospital Lab, but that only represents payment to the collection center and payment to the lab gets charged later in the cost line item.

Yogesh Sareen: No. So, we recognize the revenue on a net basis in our financial. This is GMV, right. So, don't compare with the GMV. GMV is basically what is the franchise charging or the center charging to the patient? And also, we have some HLM's, right. There they charge a fee to the manage third party hospital labs also. Obviously, we get 60% of the revenue and 40% is the markup that the hospital is charging the patient. So, the 40% is not coming in the revenue line. We consider only 25% in the revenue line.

Dheeresh Pathak: Yes, that is understood. So, my question was that of the 40% and the 25%, the difference is 15%. This 15% includes both right? The payment to the collection center, which is third party and payment to our hospital labs?

Abhay Soi: No.

Dheeresh Pathak: 15% is only payment to collection center, payment 40% of the net revenue which is payment to the lab, that will be as an expense line item.

Yogesh Sareen: So, when you send the sample to the hospital lab, the hospital lab will charge you a cost, which is something, that comes in the cost line, not in the revenue line. So, If you pickup Rs.100 worth of sample from the market, which is charged to the patient, you probably give 25% discount to the franchise and 75% is what you charge the patient. That is your revenue line. INR100 is GMV. Then you send the sample to the hospital, the hospital charges you for testing the sample, that comes in your Cost line.

Dheeresh Pathak: Right. So, based on what we're showing here, it seems the customer is paying INR100, we are getting only one-third, right? 40% is going to the collection center and 25% or so is going to the lab?

Yogesh Sareen: No. I think you are confused. So what I am telling you is, 40 is getting charged to the patient, 25 is coming revenue in our books, right. We are collecting 25 and on that collection we are getting the sample tested in the hospital lab, there is a transfer pricing mechanism there. So, then they pay part of the money to hospital for testing the samples. So, 25 is the what we collect. So, 40 to 25 is what, the money which is paid to be third party hospital as well as third party franchise.

Moderator: The next question is from the line of Harith Ahamed from Spark Capital.

Harith Ahamed: My first question is on one of your closest peers who announced foray into Gurgaon, which is Apollo hospitals, and they've acquired a 650-bed hospital there. We also have a significant presence in the region. And then we've announced a 1,000 bed

expansion as well. Firstly, any thoughts on the incremental competitive intensity in this important micro market? And secondly, if you could give an update on the status

of the two projects that you have announced?

Abhay Soi: So, I think first and foremost, this particular target what they have acquired has been available in the market for six years. Okay. And it's been available, and there's a reason that they were the only bidders right now or the only people through a bilateral this thing. We have passed it on numerous times in the past, and we've passed it on off again. I mean, that's the extent I'd like to sort of go through. for not doing that. And Yes, so I mean, I don't see any threat or real challenge coming from that.

When you say significant in Delhi, you have to keep in mind I mean, today, Max hospitals is equal to the size of perhaps Apollo, Medanta, Fortis and maybe somebody else put together.

Harith Ahamed: Maxlab Currently the net revenues are tracking at a quarterly run rate of around Rs.25 crore. So, is there an aspirational revenue targets that we've set for this business, let's say from the next three years from the current Rs.100 crore annualized level. And then do we plan to explore any other adjacencies such as pharmacy, retail or primary care clinics to further leverage the brand?

Abhay Soi: No, we're not looking at doing anything to leverage the brand which is unviable. Primary clinics, etc., are not the most viable this thing. We've been in that and we rolled it back I think we've seen it with some of the other competitors also. They've done that, even paused on it or whatever else it is. As far as Max Lab is concerned, it's a business, we have aspirations. We had a three-year, five-year plan and so on. Of course, tactically we have to keep changing, and the outlook will keep changing, depending on what others are doing and what market is, right now. Market is disruptive in terms of discounts, etc. We don't participate in that, because, you know, our sort of client base is very different. So yes, we continue to build this business out. And we've seen, although on a small base, good growth, both quarter-on-quarter, year-on-year. So, 50%, year-on-year growth and a 24%, quarter-on-quarter growth.

with non-COVID business in Max Lab is very, very healthy.

And Yes, so we continue to invest in growth of this business. It has multiple benefits for us. A brand reach. B, it's an important part of our offering to be able to do home pickups for our patients, etc., and to reach out to more and more people. So, yes, I think we will continue to build this business in...slightly challenging time right now because of more of what is happening in the marketplace. But these things are known to settle. At some point of time, the discounting and there will be some shakedown in the industry and hopefully, we will get back to building this business to what our previous aspirations were.

As in before, I don't we avoid or desist from giving any forward-looking statements. So, I don't want to make a statement on what I want to see in three years.

Coming to Max@Home, I think that's a very, very interesting business. Okay, it's again something which is about Rs.30 odd crore in this quarter but it's been healthy 10% growth over last quarter and 18% growth year-on-year. It's business of the future in my belief. We've been doing it for many years. We've got more than --- it's a profitable business, most importantly. Perhaps the only profitable Home care business in the country. We've got more than 650 people doing this business. So, we are foreseeing great opportunities here as well. But again, I want to desist from giving you two-year or three-year statement.

Harith Ahamed: Yes. Is there a standalone profitability for this business that you can share, like you shared for Max Labs?

Abhay Soi: It has mid-teens sort of EBITDA margin.

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Moderator: The next question is from the line of Ashwin Agarwal from Akash Ganga Investments Private Limited.

Ashwin Agarwal: Congratulations to the Max team, and Abhay, in last three years, you have built a fantastic franchise. I think Max stacks well in all the operational parameters

compared to the peer group. I had one question, one of the co-promoters, KKR as we know for their own internal reasons is selling their stake. If there is a potential stake sale of the entire, would you be looking out for financial investors or what is the strategy on that front?

Abhay Soi: I think it's a private equity fund. Private equity funds are known to churn their portfolios and sell, because they have a particular holding period in mind. Their holding period for this investment was seven years. They have only done three years but doesn't mean --- but yet they are seeing great returns because of the performance of the company. And they have sold on about 20% odd percent in the last six, eight months. So, the flip side is a large amount of stock which is coming to the market, yet you do have to keep in mind that the trading in our stock is not very large. It is basically because whatever has been sold also has gone into a lot of the long hands. And these long hands don't sort of trade in the stocks. So, your trading volumes of two lakh, three lakh shares, two lakh, three lakh shares or whatever.

So, I'm not that sort of worried about you know, further stock coming into the market in the future. As a private equity fund, it's a matter of time rather than is not a question of if, it's a question of when. But yet, they've been very clear in stating and we put it out in the news item, as well as well as the part of a release to the stock exchanges that they are not in conversations with any strategic buyers or conglomerate etc. So, what that basically leaves it is, sale in the marketplace only.

Ashwin Agarwal: Okay. But in future whenever that term or their holding period, maybe next one, two, three years, would you in consultation with them, look at placing this large stock among two, three long-term investors rather than it coming to the market?

Abhay Soi: Even right now, the 20% which is sold has been go

ne to long-term investors only. I

mean, if you actually see I mean, right, now, if you look at the stock between KKR and me, we are 50% of the company. The balance 50% which is about INR18,000 crore of stock is out there in the market, which is about 48 crore shares. Yet the trading in shares is about two lakh or three lakh a day.

Ashwin Agarwal: So, you're not disturbed with having 23% stake and the other co-promoter selling off whenever they need to?

Abhay Soi: No. I don't have any concerns on that. I think there are immense examples of companies where the promoters have shareholding in this range. I mean, be it players like Cipla, be it players like UPL, be it players like, a lot of the Tata Group companies and other companies as well. So, I think, yes, I'm not concerned about shareholding and our balance shareholding going out in the marketplace.

Moderator: The next question is from the line of Prakash Agarwal from Axis Capital.

Prakash Agarwal: Just one question on the growth. So, we had a fantastic ARPOB growth, occupancy, increased substantially and business seems to be normalizing now. Now, the question is, the growth and the margin, EBITDA growth, both are mid-single digit. So, so you mentioned in your opening remarks, whatever the levers, but when do we see these levers playing out? Is it from next year onwards and this year would be still this mid-single digit? Or how do we see the business shaping out over the nine months period given that there are a lot of initiatives which are at play?

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Abhay Soi: I think where it's going to shape out and what the margin is going to be, something for you to work out. I can essentially tell you what the levers are. And you can see what the historic. The main levers will be International, will be the payor mix. There'll be other smaller sort of this thing.

Do keep in mind two things. Quarter one typically is not the strongest quarter because you have price increases, etc. in the first quarter, okay, yet, we don't have a bad quarter in the first quarter this year. That's one. Second is the payor mix. Like I said, from 33%, in one quarter, it came down to 30%. Over the next six quarters, we are expecting it to below 15%. That means 15% of your beds will be able to generate about 45% or 50% more revenue, and 85% of that will go down to your

EBITDA. This is on top of what your regular growth should be. So, if you do the math, we'll come to some sort of you know, that for you to analyze. I am going to desist this from giving any forward looking. And we haven't given in the past also guidance, as far as I know.

Prakash Agarwal: I understand. I'm not asking for, you know, the percentage or I'm saying when do you see this happening? I mean, this because we are sitting on a decent page, given the strong execution we have done over the last two years, just trying to understand when does it start playing out double-digit growth? Or with the addition of new beds, then only we start seeing that or it can happen?

Abhay Soi: You are talking about double-digit growth as far as?

Prakash Agarwal: Top line and EBITDA is concerned.

Abhay Soi: I have not guided you to double digit growth at top line or something. This is something that you're working out. That's one, okay. You're already seeing some sort of growth, which is, I think, quarter-on-quarter has been the double digits, you already see in the 33%, come down to 30%. So, your growth right now is embedded with some payor mixture as well. Okay, and when we are guiding it down to 15%, then, you know, your only question can be it is going to be bunched up in the last quarter of on the sixth quarter? Or is it going to happen sequentially? Or, you know, how is it going to sort of play out? That's why, you know, we can't do it on a quarter-on-quarter, month-on-month basis.

And I refuse to give that sort of guidance. Now, if you

want to draw a linear line, you're welcome to between, you know, where we are today, to from 30% beds being going towards institutional payor mix to 15% in sixth quarter.

Yogesh Sareen: 17% like-to-like basis and EBITDA growth is 23% on like-to-like basis. Right. So, if we take out vaccination related revenues, which we knew is non-recurring, and we reported a number separately, also in quarter one last year. If you take that out, I'm going to compare then it is 17% plus growth. EBITDA is also 23% growth. So, you are saying, double digit growth. I don't know what do you mean by that?

Prakash Agarwal: I'm talking about reported basis. So there was COVID and the non-COVID was down, now non-COVID is fully up, whereas COVID is down. So, I'm just saying COVID.

Yogesh Sareen: We have taken vaccination out. We have not taken the COVID out. What we have taken out is the vaccination. Vaccination is non-recurring and that it will be reported number separately for these vaccinations. So, I'm not taking the COVID out of taking only the vaccination out, which is actually OPD business and it does not impact us.

Prakash Agarwal: Okay, also I just wanted to understand the Shalimar which is getting added by end of you know this calendar or by fiscal end, how do you see the ramp up I mean, given that, you know, your turnarounds are pretty fast. So, what is the plan here and similarly it can guide for the Dwarka also?

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Abhay Soi: Like I mentioned our ramp up will be very quick right I mean and will not have any additional Shalimar Bagh brownfields will be very quickly. As far as Dwarka is concerned is not brownfield will be a little longer you know we expect the breakeven within the year but you know it's on a large base so 3,500 beds that you will get another 300 beds coming up. so it should not disturb your numbers too much.

Prakash Agarwal: Okay. And lastly if I look at the Devki Devi particularly you know sales have been soft but margin has improved significantly anything happening there you want to call out?

Abhay Soi: Not particularly.

Yogesh Sareen: Yes, so don't read too much into it because there'll be some payments which we which we people made, which is definitely as to donate some money, etc.

Prakash Agarwal: The slide where you have that revenue and EBITDA breakup?

Yogesh Sareen: Don't read into too much individual hospital slides.. I'm saying those movements happen from quarter on quarter, because they have to make some donations which they made . I would say, yes, there is improvement overall, in all the hospitals, compared to Q4. As you know that the EBITDA has gone from Rs 304 crore to Rs.370 crore So obviously, you will see that in the underlying hospital also. And, you know, so that's, that's, that's a better step. The point I'm making is that don't read too much into Individual Hospitals . Gross number is more important than the individual hospitals, especially at PHF level, right? Because here, the elements that provide is charged to the P&L and then that will come to the MHIL Side . Right. So that will happen, because of this year, something this will be a temporary phenomenon before the before the agreement revision takes place .

Prakash Agarwal: Okay, perfect. And lastly, on the, you know, further growth initiatives, so next five, six years, we all set with a stated plan or bed additions. But you also talked about we'd kind of structure asset like, you know, development, you know, tie up with developers, or you know, ready to move in hospital kind of setups. Is there anything on? You know, we're still looking at it, or our hands are very tied up with this thing? expansion plan?

Abhay Soi: Dwar ka is just that, isn't it? There is plenty of this thing and I think there's quite a few conversations on that . It is a big focus area for us.

Moderator: Next question is from the line of Yash Shah from Investec India.

Yash Shah: My question is, given that our revenue or our footfalls from international patients, it is back to pre-COVID levels and Afghanistan is not completely back as compared to FY20. Right. So, I just wanted to understand which country have you seen sudden inflow, like in footfall? And do you think it is sustainable? Or will we be able to sustain from the other from the same country? Yes, so we it is sustainable?

Abhay Soi: Yes. So it is sustainable , we have been making efforts to enter into new markets and increasing also our patient flow from existing markets, which is sort of what augmented and absorbed lack of business from Afghanistan. I am going to again desist from giving you specific names of new markets because you know, I don't want to do it in a public forum. Yes, but we do see stickiness about this business in fact more stickiness with this business because this is a result of a lot of direct to fly offices that we are setting up overseas.

Yash Shah:

And just one small clarification when we say international football this back to pre - COVID levels, we mean absolute revenue terms right?

Abhay Soi: Yes, absolute terms.

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Moderator: The next question is from the line of Amit Dhawani an individual investor.

Amit Dhawani: Can you discuss what levers -- are there any low-hanging fruits still there in ARPOB? And I mean if we look at ARPOB as something is there we can guide for certain Q - o-Q growth or do you think there's any seasonality in the ARPOBs? Just wanted to know, what can drive it higher?

Abhay Soi: First and the most primarily the low hanging fruit is going to be the payor mix, increasing the non-institutional, which we've been speaking about on this call. And second is the international business.

As far as the seasonality is concerned, you know, there is a slight seasonality, you're typically the fourth quarter is the best your first quarter in your third quarter typically not the strongest quarters etc. But Yes, so that's where we are.

Moderator: I would now like to hand the conference over to the management for closing comments.

Abhay Soi: Thank you so much for being on the call. The results are robust as you can see, but don't come as a surprise for us in some manner and you've been a part of the calls in the past you've seen us guide towards normalization and these sorts of results and we are quite certain that this path will continue going forward as well.

Call: Nov 2022

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MUMBAI - 400 051 MUMBAI - 400 001
Symbol: MAXHEALTH Scrip Code: 543220

Sub: Transcript of the earnings call on financial results of Q2 and H1 FY23 of Max
Healthcare Institute Limited ("the Company") held on November 02, 2022

Dear Sir / Ma'am,

We refer to our intimation dated November 02, 2022, w.r.t. uploading of audio recording on the website of the Company, of the earnings call hosted on November 02, 2022 to discuss the financial results of the Company for Q2 and H1 FY23.

In this connection, pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed the transcript of the said earnings call. The said transcript is also being uploaded on the website of the Company i.e.

<https://www.maxhealthcare.in/investors/investor-resources>

You are requested to take the above on records

Thanking you

For Max Healthcare Institute Limited

Rakesh Kumar Kaushik

Interim Compliance Officer

Encl: as above

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Max Healthcare Institute Ltd
Q2 and H1 FY23 Earnings Conference Call Transcript
November 02, 2022

Moderator: Ladies and gentlemen , good day and welcome to the Max Healthcare's Earnings Conference Call. Please note that this conference is being recorded. I now hand the conference over to Mr. Suraj Digawalekar from CDR India. Thank you, and over to you, sir.

Suraj Digawalekar: Than k you. Good morning everyone, and thank you for joining us on Max Healthcare's Q2 and H1 FY'23 earnings conference call. We have with us today, Mr. Abhay Soi, Chairman and Managing Director, and Mr. Yogesh Sareen, Senior Director and Chief Financial Office r. We will begin the call with opening remarks from the management , following which we will have the forum open for interactive question and answer session.

Before we begin, I would like to point out that some statements made in today's discussion may be forward looking in nature, and a disclaimer to this effect has been included in the earning s presentation shared with you earlier.

I would now like to invite Abhay to make his opening remark s. Thank you and over to you Abhay.

Abhay Soi: Very good morning to everyone. We are pleased to welcome you to Max Healthcare's Q2 earnings call. Let me start by giving you updates about the company performance for the quarter .

Like the previous quarter, this was a normalized quarter. We witnessed a steady growth in occupancies driven by improved patient footfalls from international and insurance segments. There were concerted efforts to unlock value through previously articulated growth levers. Consequently, we delivered our best ever performance for the second conse cutive

quarter this financial year.

A significant development this quarter is that we are now a Net Cash Surplus company. Compared to a Net Debt of Rs. 217 crore at the end of Q1 FY '23, we have a Net Cash Surplus of Rs. 42 crore at the end of this quarter.

We are also happy to share that our digital app, Max MyHealth, has been successfully launched at the end of September 2022. The app has enhanced the experience of both patients and clinicians in our ecosystem. It provides a gamut of services including pathology, radiology, ambulance, home care, and facilitates physical and virtual consults among other things. It also provides access to patients' health records and trends at the click of a button.

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Before I move on to the highlights of this quarter, please note that in Q2 last year we had a revenue of Rs. 91 crore and an EBITDA of Rs. 25 crore from COVID-19 vaccinations. Since this was a non-recurring revenue, the comparative numbers and percentages are thus being reported on a like-to-like basis.

Key highlights of our performance in Q2 are:

Occupancy for the quarter improved to 78% from 74% in Q1 FY '23 and 75% in Q2 FY '22. Institutional bed share has been brought down to 28% this quarter from 30% in Q1 FY '23 and 37% in Q2 FY '22, in line with our strategy. Consequently, the institutional revenue share dropped to 16% compared to 23% last year.

Revenue from international patients grew by 16% quarter-on-quarter and reflected 110% of pre-COVID average, despite negligible patient footfalls from Afghanistan, a key territory for us that contributed around 12% of the revenues previously.

Network gross revenue rose to Rs. 1,567 crore, our highest ever, reflecting a growth of 6% quarter-on-quarter and 17% year-on-year.

ARPOB for the quarter was Rs. 66,000, same as previous quarter, but grew 12% year-on-year. The mix of medical patients went up compared to last quarter due to seasonal infections, and this was reflected in the lower ARPOB. Increase in ARPOB over Q2 last year was led by improvement in the payor mix and case mix as well as annual price revisions.

Network operating EBITDA for Q2 FY '23 was Rs. 410 crore compared to Rs. 370 crore in the previous quarter and Rs. 337 crore in Q2 FY '22, reflecting a growth of 11% quarter-on-quarter and 22% year-on-year.

Indirect overheads were up during the quarter due to relatively higher provisioning for CGHS bills outstanding beyond 365 days in line with our tight provisioning policy, seasonal cost increase for power and marketing expenses related to international patients.

EBITDA margins improved to 27.7% versus 26.6% in Q1 FY '23 and 26.7% in Q2 FY '22. Annualized EBITDA per bed, most importantly, rose to Rs. 64.3 lacs, our highest ever, clocking a growth of 4% quarter-on-quarter and 17% year-on-year.

Q2 FY '23 PAT was Rs. 267 crore versus Rs. 229 crore in Q1 FY '23 and Rs. 207 crore in Q2 FY '22. This excludes gain of Rs. 244 crore in tax expenses due to reversal of deferred tax liability relating to intangible assets transferred to MHIL pursuant to voluntary liquidation of Saket City Hospital Limited.

During Q2, Rs. 28 crore was deployed towards ongoing capacity expansion projects. Construction of 100 beds at Shalimar Bagh and 300 beds at Dwarka is on track, and we expect them to be commissioned in last quarter of FY '23 and first half of FY '24 respectively, as indicated earlier. The outlay for some of the expansion projects has been deferred in view of ongoing discussions with some of the world's top contractors for faster build out.

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Digital revenue grew to Rs. 242 crore and accounted for 15% of overall revenue.

Continuing our effort to give back to the community, we treated ~39,700 OPD and ~1300 IPD patients from economically weaker sections of

society free of charge.

Both our strategic business units continued their growth momentum. Max Lab reported a gross revenue of Rs. 30 crore. This reflects a growth of 21% quarter -on-quarter and 65% year -on-year, on like-to-like basis. We added 65 + channel partners during this quarter, taking the overall active clients to 900 + and now offer our services across 34 cities. You may keep in mind that we have made huge investments towards Max Lab for the organic growth in the past quarters.

Max@Home reported a top line of Rs. 35 crore reflecting a growth of 9% quarter -on-quarter and 26% year -on-year. Supported by a team of 800 + people, Max@Home offers services across 13 service lines and enjoys a high degree of customer loyalty.

Now, coming to the overall overview of the company financial performance in

the first half of this financial year. Network gross revenue stood at Rs. 3,040 crore reflecting a growth of 17% year -on-year. Network operating EBITDA grew by 22% year -on-year to Rs. 780 crore. Increased OPD footfalls, improved case mix and reduction in institutional bed share resulted in margin expansion by 120 basis points to 27.2%, while EBITDA per bed grew by 26% to Rs. 63.2 lakhs per bed.

We continue to focus our efforts on the growth levers articulated earlier. That is, we are -- one, we are making significant investments to expand our bed capacity in the next three to four years, which is in line. Second, to complement Indian government's initiatives such as Heal in India, we continue to augment our international outreach initiatives and setting up offices across new geographies. We are also actively looking at inorganic expansion opportunities across existing and new markets, and we continue to improve our case mix through deployment of technology and hiring of new talent.

With this, we open the floor for Q&A. Thank you.

Moderator: Thank you very much. The first question is from the line of Kishan Amarchand Tosniwal from Polar Capital LLP.

Kishan Amarchand Tosniwal: Couple of questions. Firstly, how do you see the international business growth in the coming quarter?

Abhay Soi: We're very positive about it. It's been growing, like I mentioned, in the announcement that is already 110% of pre -COVID levels. This is in spite of the fact that 12% of our business, our key market was Afghanistan, where the Indian Government right now is not issuing visas. So once that opens up, you will see, and we hope it opens up shortly, because things seem to be normalizing over there, at least from Government to Government perspective. And once that happens, you know, this will auger even better for us. It's a big thing to say that we've been able to sort of compensate and overcompensate this through other growth levers that we've invested in, in the international business. So that's done well and we are quite positive about it going forward. I think, you know, if you

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couple this with the new Heal in India program of the Indian Government, you know, very similar to Make to India, which has been, sort of announced, by the Government, I think that will give us huge impetus, particularly because we have so much of our capacity in the metros.

Kishan Amarchand Tosniwal: Okay. The second part is that how is Nanavati doing after the VRS has been done?

Abhay Soi: It's been doing reasonably well. We are in mid -teens as far as EBITDA margins are concerned. Going forward, hopefully we can grow that to the rest of the portfolio as well. But like I mentioned in the past, the ROCE, it's the highest amongst the ROCEs. Although in terms of percentage, the EBITDA margins are lower than the rest of the pack.

Kishan Amarchand Tosniwal: The building that was coming up, which has been already dismantled and we started work, how is the progress on that?

Abhay Soi: Very well, I think we are well on schedule as far as that is concerned. The piling work has been completed. We are beginning excavation soon and you're going to see that online in the next two years.

Moderator: The next question is from the line of Nikhil Mathur from HDFC Mutual Fund.

Nikhil Mathur: My first question is on the CapEx plans of the company. If I look at first half, the CapEx incurred, I think is Rs. 41 crore, if I'm getting that number

right.

Abhay Soi: That's right.

Nikhil Mathur: And I think the budget in the most recent investor presentation was around Rs. 657 crore for FY '23.

Abhay Soi: That's right.

Nikhil Mathur: So, any reason why this gap?

Abhay Soi: Yes, so a large amount of it was to be bunched up, towards the second half. It's a little difficult to sort of break it up. You're going to see a majority of that investment happening

in the second half of the current year and first quarter of next financial year. Of course, you know, one is engaging contract engaging vendors and work starting, et cetera, but besides the mobilization advance, really the payments start happening, on delivery of certain milestones. I think as and when that happens, you will see that payout happen. It's a little -- when you're making five year plans for so many hospitals, a little difficult to sort of put it out quarter by quarter. So we put it out in a year. So you're going to see, significant investments will be complete. Invest the entire Rs. 640 crore in the second half of the year? Doubtful. But, you know, will it sort of bunch up in the first quarter, of the next year? Likely. But at this stage we are not seeing any delays on overall project schemes.

Nikhil Mathur: Right.

Abhay Soi: You know, I'll give you an example, because, when we were conceiving the projects you look at construction as usual, but then new technologies such as structural steel and hollow tubes, et cetera, which cuts down

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timelines as far as the entire construction is concerned. But, it sort of takes you back a little bit into the planning stage about how to technically go about it. So that's why we don't see delays because we see benefits coming out of using, you know, alternate techniques. They may be marginally more expensive, but that's the new sort of way of doing things faster these days.

Nikhil Mathur: So it can be safe to assume that, CapEx running behind budget will not have any impact, at least on, Shalimar Bagh and Dwarka. Is that the right assumption?

Abhay Soi: Absolutely. As far as Dwarka is concerned, you know, we are hoping within the first quarter of next financial year for it to come on stream. And as far as, Shalimar Bagh is concerned it will be on stream early on in the last quarter of the current year. So Shalimar Bagh expenditure has been as per budget, as per plan, so there's no sort of delay over there. And Dwarka, as you're aware, there's actually somebody else who's constructing it and that's well on stream.

Nikhil Mathur: And second question I had on Max Lab. Now, there seems to be some pretty strong traction building up, from Max's perspective. There's quarter on quarter growth. There's YoY substantial growth in non-COVID revenues, and I think you have added quite a few partners as well, in two quarters, I think 150 partners have been added. So two questions here, I mean, A) obviously it would be very helpful if you can share the Max Lab outlook from a three -to-five years' horizon, and B) when you talk about partners, is it some sort of a franchise model or who exactly are these partner, some revenue sharing happens, if you can help us understand, a bit of construct there and how you're going about growing your business?

Abhay Soi: Yes, sure. I think, you know, the two things, one is, you know, we've been investing heavily in this business. When I say investing, both in terms of, I mean there's been -- mild investment towards marketing, but investment towards creating partners and partnerships with franchisees and so on and so forth. I think in the first half of the current year we had about 400 odd franchisees, to this, and it's a significant sort of this thing. So your EBITDA sort of gets depressed because of the investments that you're making in it. So I don't want you to kind of get misguided by looking at lower EBITDA margins or somebody actually mentioned to me, that -- from a loss making in the past quarter, that you've gone to -- marginally profitable, et cetera. But, this is on account of the new sort of, investments that you're making. I think overall, this business has always been profitable. There's no reason for it to not to be cetera. And if you were to

sort of draw a line and not grow, immediately you'll see, this business coming to prof

itability. But that's not what the idea is. It's about, it's such an underpenetrated market and we have, such a strong sort of, brand and strategic advantage and comparative advantage compared to other players in this that we continue to invest in the business. Yogesh, do you want to sort of share light on what sort of arrangements that we have?

Yogesh Sareen: Yes, so we have obviously we have franchise model, we also have our own company owned centers. We then also have a model, which is phlebo at site (PAS) . This is where we station a phlebotomist at the nursing home of a doctor et cetera, or a smaller hospital. We also have HLMs right? We do third party hospital lab management (HLM) . There are more than 20 labs that we manage here, you know, big ones I would say. And then we also have pick up points (PUPs) where we -- where our

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phlebo goes and pick s up the sample s from. So there are obviously various models. I would say the HLM is the deep discount model where we -- where we get probably 55% of the amount billed to the patient, but others would be at discount of 25% to 30 %. So that's the model. And obviously company owned center, we bill at the retail price, so there's no discounting here.

Nikhil Mathur: Right. And in terms of the lab network, are only the hospital based labs being leveraged as of today, or, there are some, standalone labs as well.

And, what's the lab network outlook over the next three to five years?

Yogesh Sareen: So as of now, we are leveraging the hospital lab, and not only our own hospital s, but also these HLMs that we have i.e. the third party hospital lab that we manage. So we pick up samples and try and get it tested in the nearest lab. The nearest lab well may be the lab, which is managed by us, but not owned by us. So that's the way it is, in a way they have their own labs in terms of the HLMs, but they don't have a reference lab of their own. They will certainly be looking forward to one, once the business shapes up to that level .

Abhay Soi: I mean, right now we have spare capacity in our lab s, so we obviously use that, but as when there's a need for a centralized lab, I don't think there's a problem. In any case, the investment isn't too much , right? It's a very negligible sort of investment.

Nikhil Mathur : Right. So I mean, would you comment Abhay Sir, Max Lab now that it is crossing Rs. 100 crore kind of a revenue mark, if I annualize this quarter numbers, is as important a piece for the company as the hospitals? Can it be looked at that way?

Abhay Soi: Absolutely. It always has been. I think, you know, we started focusing more on it, and, in the past we also said we'd like to look a t inorganic growth, but obviously, you know, sort of, numbers and valuations and, you know, the dynamics of the industry sort of changed. So we decided to kind of look at, focus on more on the organic growth as far as this is concerned. But this is always going to be a key focus area, and, you know, otherwise we wouldn't be in it, otherwise we won't be showing it as a separate segment. And similarly, I mean, you're saying this about Max Lab and similarly about Max @Home as well . And you've seen, I mean, that's a high number and it's a profitable business. It does mid -teen sort of EBITDA margins, it's the largest home care business in the country, and actually the only profitable one. And we are very excited about that as well. I mean, the traction that we are getting is essentially giving you a hospital in your, is extending a hospital to your home.

Moderator: The next question is from the line of Prakash Agarwal from Axis Capital.

Prakash Agarwal: Yes, so first question, trying to understand this occu pancy run rate better. So we've been tracking very well and now touching 78%, I understand Saket and marque e ones would be one higher 85% plus. So what is the headroom here for us to -- y

ou know, max out on the existing hospitals?

Abhay Soi: So look, I think there are two or three things that you need to look at this , in perspective. Okay? Pre-COVID level, we were at 70% to 73% compared to about 75% - 76% post COVID as a base. Now, certain quarters, and particularly quarter two every year is, that's the time when you have the rains, et cetera. So we have more viral diseases, more

dengue and so on and so forth. This quarter is typically characterized by higher occupancy, okay? Because of more medical patients coming. But also it's characterized by, because these patients you know, your earnings from these beds are lower, okay? You have a downward pressure on ARPOB. So although you see the 78% occupancy, it's basically, this peak has a little bit happened because of the viral load and the medical business, which has come, the dengue business and so on and so forth, which is the seasonal business. Having said that, you also have a secular increase, okay, in your business as usual, which is your surgical business, and your regular business, which is the non-seasonal business, which is further augmented by your international business reverting to even higher levels and growing, your insurance business growing, and so on and so forth. So what you're going to see in the subsequent quarter is because, and this business, okay, A) it will bring you higher, little higher occupancy, but also brings you B) higher ARPOB. Overall, when you look at let's say a 75% to 78% sort of range, of occupancy, your question being where do you sort of go from here? You have to keep in mind that even now, 28% of the total beds, which means of the 78% occupancy, 28% of the 78, so almost like one third of it, okay, is being consumed by institutional business, which is very low ARPOB business, and the ARPOB basically is close to half of what the ARPOB of the business as usual is.

Prakash Agarwal: No, I understand that. That was my second question anyways, but occupancy wise, what is a max we can get over?

Abhay Soi: So let me put it this way. In month of September, we were operating at 81% occupancy, right? I mean, if you ask me the same question, we have hospitals which are operating at 90% plus occupancy. The question is, can all hospitals operate at 90%? Theoretically, yes. Okay. You know, one year back, if somebody asked me that, look, on a sustainable basis, can you operate at 77%, 78%, I would have said difficult. Today, I'll say, yes, next year I'll say, you'll probably eke out two more percentage points here, and so on and so forth. But you are pretty much, look in my mind, 77%, 78% occupancy on a sustainable basis is where you should be. And then, you know, your patient services, et cetera, on the subjective areas start getting compromised a little bit.

Prakash Agarwal: Understood. So, second one is similar on the ARPOB side, as you already touched that you already declined to 28% in terms of institutional business versus 37% a year back. So here, I mean, do we have minimum threshold where we have to give minimum institutional services to central government employees? Or this can go to say 10%, 0%? What is the view here over the next four to eight quarters?

Abhay Soi: So I can give a 30-day notice today, and at the end of 30 days, bring this business down to zero. Keep in mind, we don't--

Prakash Agarwal: Don't have any obligation.

Abhay Soi: None, none whatsoever.

Prakash Agarwal: So, what is our goal for the next four to eight quarters?

Abhay Soi: I do this business because I want to do this business, not because I have to do this business. We do this business because we don't want the bed idle. If I stop, bring this down to zero, today my occupancy comes down

to 55%, 57%. Right? The next question is, why aren't you filling those beds? Now, my goal, in the past also we have said, that for

five quarters later

or six quarters later, this will be below 15%. And that goes down to 15% because that's when majority of my capacity starts kicking in as well. Now, I would say not five to six, let's say four to five quarters. And we have always sort of guided down to that. Then we comfortably come down to 15% or below. And the reason is that the new capacity comes in. Suppose the new capacity wasn't to come in or get delayed, this goes down even further. That's the advantage within this thing, these beds are built, they're sub optimally used right now. And you have this entire brownfield, etcetera, kicking in.

Prakash Agarwal: Okay, perfect. Great. And one more question was on the CCI probe, which came in on, you know, a few of the hospital companies including yours. So if you could give some color, is it to do with the annual price

hike or is it due to that we came into limelight like because our ARPOB is highest in the industry today? And how are you tackling this? Because I heard in your opening comments that you've still taken price hike.

Abhay Soi: So I'm glad you asked this question. Okay. This is purely and simply relating to a case in 2015 where a person came to the hospital and said that, look, a syringe, you're delivering a medicine at, an injection at so and so price. Okay? Whereas I can get it from a pharmacy at so-and-so price, which is lower than that. So I should be able to bring my syringe in. And he went and complained regarding this. The sum total of this case is that they're saying that look, okay, I think firstly it's an investigation that they've done. Okay. They've asked us for a response on that investigation, okay? They haven't given us the basis of that investigation. So we went to Delhi High Court and we said please ask them to give us a basis for the investigation so we can give a response. Because as a hospital, what we do is we do not sell medicines and injections. We apply it, right? What we sell is at MRP. So, I mean, for the life of me, we can't understand where we are sort of, off track on this because we sell at MRP and we are not in the business of selling the medicines. And we sort of, when the nurse comes, gives the injection and so on and so forth. It's got nothing to do with any price hike. It's got nothing to do with pricing. It's got to do with, is a patient allowed to bring his medicine from outside? That's not even the jurisdiction of the CCI, frankly, because we can't have somebody bringing a spurious injection, okay, which may not be sterile or whatever from outside the hospital to inside the hospital.

Prakash Agarwal: Perfect. No, this is very helpful, thank you.

Abhay Soi: As narrow as that. And there's some comments made on the investigation. Okay. Which we don't know the basis of. Okay. Such as the hospital beds are, the rates for the hospital beds are more than four star hotels in the neighborhood. Firstly, you know, nothing stops us from selling our beds at any price. That's one. Secondly, and it's definitely not within the jurisdiction of the CCI. Secondly, you know, we don't sell the rooms. There's a nurse, there's a doctor, you know, and there all of those services which comes with the beds. You can't make a like to like comparison, right? I mean, right now it's an investigation. Then, you know, if we will give our response, you know, if it's a litigation it will be litigation or it'll be killed by them at this stage.

Yogesh Sareen: Also the question is that when we have some 70% of the patients being treated on cashless basis, how can we ask patients to bring medicine from outside? Right? So they're saying that you allow patients to bring from

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medicine from outside now, I mean, whole cashless falls off if that also happens. Right.

Prakash Agarwal: Okay. Got it. Okay. And my last question is on, your M&A and asset light strategy, given that, you know

, you have a, you know, six-seven years' plan, you're doubling from internal accretion largely, but still net cash balance sheet. But you know, to propel growth or maybe, you know, add on to the growth, we've seen some companies like KIMS, you know, buying out, doctor owned models, hospitals which are not run properly. What is your thought there, or what are the M&A and asset light acquisition plans you have?

Abhay Soi: So, look we've written the playbook on it, right? I think essentially, as far as, buying hospital and unlocking value, but you have to maintain a certain critical mass and be able to do it, et cetera. Because KIMS or anybody else finds an opportunity does it, is not the sort of this thing. We at any given point of time are diligencing companies. We are very, very focused on inorganic growth. And like you rightly pointed out, we have an unlevered balance sheet, and we have excess cash on the books. We can easily sort of do that, but at the same time, you have to maintain, we have a ROCE of 33%, and whatever we do needs to be accretive to that, in the long run. And, I'm fairly certain in the next, you know, sooner than later we will be able to conclude another transaction. But do keep in mind, over the last 10, 12 years, our entire platform has been based on acquiring assets, unlocking value. So it doesn't sort of stop us from doing it. We just more, we just -- we have a stronger balance sheet, stronger team, stronger abilities to do that, to execute even better on this.

Prakash Agarwal: So, you would be still looking at it, but you're not talking about it, which way you're going. I mean, is it asset light? Is it KIMS model? Is it M&A ?

Abhay Soi: No. So look, partnering with doctors, et cetera, doesn't excite us. If we like something, we want to own more of it than less of it, and we like to have control on it as well. And I think, sort of our EBITDA per bed, keep in mind, is 50% better than the next best player in the industry. So obviously our model is, sort of, works very well for us and for investors.

Moderator: The next question is from the line of Damayanti Kerai from HSBC.

Damayanti Kerai: Hi, my question is on, bed addition, new Bed addition, happening over next, nine to 12 months. So we have Shalimar Bagh and Dwarka adding around 400 new beds. So, currently you are operating at somewhere, 26% - 28% EBITDA margin. So after these new beds come in, should we expect some dilution in margin or, other way to ask you like how fast, you think these can achieve EBITDA breakeven after the launch?

Abhay Soi: I think the way to look at it is look at what our EBITDA per bed is. And then you say, look, if this our 400 beds coming, how long will it, get take me to get to let's say 75% occupancy. That's about, I guess, 300 beds and multiplied by EBITDA per bed. So yes, I think as far as Shalimar Bagh is concerned, it should be a matter of, you know, I mean, there's no dilution. In fact, day one, there should be accretion as far as this thing is concerned because it's a brownfield. There is no significant fixed cost or any fixed cost, which is being incurred. It is essentially variable costs as and when you open the bed, and, you know, my belief is that it's only a hundred beds in that location, which is, a hospital which is operating at 90% plus occupancy at present. So, you know, it's untapped demand. As
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far as Dwarka is concerned, again, you know, I think, it's not really going to be dilutive because it's a very sort of, 300 beds on top of, this thing. But, I think, the kind of response we've got, the ramp up and the break even and everything else should be very, very quick over there. So I'm not seeing any real dilution on overall basis.

Damayanti Kerai: So, broadly the current level of margins can be maintained?

Abhay Soi: Absolutely. But you know, again, like I said, please focus on E

BITDA per

bed rather than EBITDA margins. I would rather do a \$10,000 surgery with a 20% margin than a \$ 2,000 surgery with a 50% margin.

Damayanti Kerai: Okay. My second question is on Nanavati hospital. So you mentioned, this facility is currently operating at mid teen margins. So, how like how long it can take further to reach near to the corporate average or how should we look at margin for this particular unit?

Abhay Soi: Well, you know, Mumbai by and large has a higher sort of doctor payout. Okay. So you typically have lower margins, but yes, there is room to increase the margins over there. Do keep in mind, you know, if one was to sort of increase the margins from, by 5% or 6% also from here, it means on a Rs. 400 odd crore top line, Rs. 450 top line, you're talking about Rs. 20 crore – Rs. 22 crore, you know, on an overall base of Rs. 1600 odd crore. So it doesn't really move the needle from that standpoint, but I think the big, big sort of swing will come over there, when the new capacity comes in, which is, you know, the construction is on. What it also does is, it will flatten out higher doctor cost as well as personnel cost.

Damayanti Kerai: Okay. So, operating cost will be spread over a larger bed network, and that will.

Abhay Soi: So actually, you know, there is, and as far as Nanavati is concerned there is a legacy personnel cost. Okay? Which is the worker's cost. Okay. That's the only single line item which is off. It is 30% - 31% compared to 22% - 23% for the rest of the group. And that, either through expansion of capacity, or through VRS is the two ways of tackling it. We tackled it partially through VRS. We may be looking at another VRS going forward, but more importantly, I think when the new capacity comes in, it gets taken care of by itself.

Damayanti Kerai: Okay. And my last question is on, seasonality in a hospital business. So, 2Q as you said, due to rainy season, we have higher cases of infections, etcetera. So, 3Q should we assume it could be a lower quarter due to like major festivals falling in and again, fourth quarter should be a better one? Or how does this seasonality vary across different quarters.

Abhay Soi: So let me put it this way. Usually your Q1 and Q3, are the sort of weaker

quarters, right? But rather than timing it like this, because sometimes Diwali is here and there, so on and so forth, H2 is usually better than H1 historically for all hospital groups.

Damayanti Kerai: Okay. Very broadly second half performs better than the first half.

Abhay Soi: Always. I think if you see any hospital group typically, and historically our hospitals or any other hospitals, for them all H2 is better than H1.

Yogesh Sareen: Should see a 48:52 type in EBITDA. Revenue will be 49:51 types.

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Abhay Soi: I mean, historically I'm just giving you based on experience or whatever, but I'm not giving you guidance. Usually at 48:52, first half versus the second half.

Damayanti Kerai: And final clarification, CapEx you maintain whatever, budget we have done, or we have disclosed earlier that remains on track and this lower CapEx during first half of this fiscal is just a matter of timing issue and eventually, as and when payments etc. start happening it should be in the budgeted lines?

Abhay Soi: Yes, so I mean, if I was to look at things which are going to come up in the next one year, okay. There is obviously certainty because we know where we are, we are in the fit outs etc. over there. As far as, you know, anything which is coming up really is bunching up towards the end of '24 – '25, and '26, I think there is strong visibility that we should be able to meet timelines over there.

Moderator: The next question is from the line of Praveen Sahay from Edelweiss Wealth Management.

Praveen Sahay: Yes, one clarification related to the bed addition. Beyond Shalimar Bagh and Dwar

ka, you have a – bed addition planned for FY '25. So is there any deferment in that 1170 bed odd?

Abhay Soi: Not really, no. Like I mentioned, the visibility is there and, works have started. At the same time, I just want to sort of also, layer it up that we -- like I mentioned, the reason I was going down to about 15%, institutional share is because I have visibility of this coming out at that time. Let's say hypothetically speaking, some project gets delayed at the end, and let's say it's not adjacent to a place which is at zero institutional, you still have that lever, by the way. Having said that, just squarely answering your question, we are not, right now as far as our visibility is concerned, we are not foreseeing any delays.

Praveen Sahay: Okay, and the second question is related to the ARPOB. For a sequential basis, if I look at your ARPOB is around Rs.66K, even after improvement in the payor mix like institution gone down to a 28%, and the international patient mix also improved. There also sequential improvement we have seen. So, what exactly, on the QoQ basis, relates to, the flat ARPOB.

Abhay Soi: Can you just repeat that question? Sorry.

Yogesh Sareen: So basically, this is because of the fact that the medical mix of the patients have gone up during this quarter. Abhay mentioned this earlier also, that, you know, this Q2, we had some dengue and viral fever patients. So the internal medicine has jumped by 26%. You know, you seen that comment in the earnings update also. Basically because of the, so if it was not to happen, generally you'll find that the ARPOB would drop in quarter 2 compared to quarter one, because of the medical patients going up, right. Medical patients, typically, you know, the Dengue patients would be 50% of the normal ARPOB that we have, right? So it should have dropped, but by the fact that we have this institutional share going down and the international patients going up, so it's been maintained at the same level.

Abhay Soi: Yes. But purely you can't look at it like, look, the occupancy went up, so, you know, there's a secular increase in occupancy also of your business as usual, right?

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Praveen Sahay: Yes, I got the answer because of our internal medicine increased the contribution, maybe that is the reason why the ARPOB is maintained on the same level.

Moderator: The next question is from the line of Shaleen Kumar from UBS.

Shaleen Kumar: Thank you. So more of an understanding thing. See, I understand your institutional patients are coming down, but is it right way, to think that they generally take, general ward, right? So you will be replacing them with a patient in general ward, right? So probably my improvement in ARPOB

when I replace institutional patient will not be the same level of my average ARPOB.

Abhay Soi: See, that's not true. Okay. Let's say you are working in Northern Railways, right? Who are these people effectively, these are public sector undertakings? They're Delhi Jal board, they're various central government, etc. It could be anybody from income tax to let's say from irrigation department, right? All IAS officers, Rajya Sabha members, Lok Sabha members, former members, et cetera, et cetera. All judges, all Supreme Court judges, high court judges, and so on and so forth. Now, each one of them, if you're a lower tiered officer, or you are a let's say a class three employee or whatever, then your allocation or your entitlement, like in insurance, may be general ward, but if you're a judge or IAS officer or whatever else it is, it will be single room or deluxe room, well, single room, okay? So what you're replacing it by is not that. The other thing you need to keep in mind is my Rs. 66,000 ARPOB is a weighted average, which includes the Rs. 35,000 - Rs. 36,000 of CG HS as well. What actually replaces it is a higher ARPOB.

Shaleen Kumar : No

, it replaces, but will it replace by hospital average? And I think what you said is it's possible.

Abhay Soi: Right. But the other point I want to make is that yes, you know, although we are talking about reduction in institutional business, right? I think the right way to, and the best way to think about it is increase in the non-institutional business. Because if you can increase occupancy, all right, that's what I sit with the teams on. I'm saying, look, rather than pushing your best case scenarios where you can find ways increasing occupancy, retaining this, as well as increasing your, and finding ways to accommodate your, preferred channels.

Shaleen Kumar : But you said that beyond 80% like service compromise can happen, so difficult to, I mean, I don't know.

Abhay Soi: Today, yes. Like I said, you know, three years, two years back, somebody asked me a 75%, I would've said, no, you compromise. The fact is, and I'll give you example of Breach Candy hospital in Bombay. I mean, your service is not compromised. It operates a 90 plus percent occupancy. It's just that over a period of time, they found ways to do it. You become more efficient. If you look at Hinduja hospital over there in Bombay, I mean, they've got 27 ICU beds. They've got the lowest ALOS because they've got it down to a T. Because they've been living with this situation or saturation where they can't expand it even by a square inch for so many years. I mean, everything is down to just -in-time and so on and so forth. But yes, those are all incremental efficiencies. But I still want to sort of put that down.

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Shaleen Kumar : But that's important, right? Because as a modeling perspective when we model, we start doubting that whether the hospital can hit beyond 80%.

But if you, if you see there is a possibility and there are models and you can kind of build in, then it's very interesting and it's very important for us.

Abhay Soi: I have hospitals right now operating 90% plus, and without compromising anything. Because that's one place we put our foot down as far as you know, because what you don't want is, and immediately you'll see a sort of a pushback in the next couple of quarters. Your doctors will have a problem, your patients have a problem, and so on and so forth. Our PSAT scores, all of that, on a daily basis, which we look at has been increasing and improving.

Shaleen Kumar : So do you have a score like NPS score kind of thing as well here? Do patients satisfaction score, you track something like that?

Abhay Soi: Oh, absolutely. Multiple things, including, and we do it through, you know, even SMS et cetera, where it's voluntary for you to sort of, respond to it. So although you only have 4% or 5% people responding to it but those are very true sort of these things, right? -- It's not as if you are sitting in a hospital where the management or the nurse is coming up to you and saying, sir, please sign this.

Shaleen Kumar : You also mentioned, about, moving away from probably a traditional way of construction. Are you going with the hollow structured tubes for the construction instead of RCC?

Abhay Soi: No. So look, we evaluate hollow tubes versus structural steel frames. Or,

let's say composite. So you do your basements, we still have to do it in concrete. We are still finding ways of, doing that mostly in steel as well, but the rest of it you do on steel frames, which is, which is very, very promising. You see, unlike, you know, the cost of construction is maybe higher by 15% or 20%, but if you can save 20% of the time, okay, in our case, you can get to market that much sooner. Because for me, every day is a loss of profit, right? So unlike a residential real estate where the cost of construction matters because the delay is to, let's say, the consumer's account. Here, it's actually, I have a positive incentive for me to get it

up

and running sooner than later.

Shaleen Kumar: True, true. So, ballpark, have you looked at the IRR? Basically it's all about IRR at end of day, right? So it's positive, it's accretive?

Abhay Soi: I mean, each day, I mean, when you have a cost of construction or a brownfield of let's say Rs.130 - Rs.150 lakhs or whatever and your EBITDA per bed, okay, is Rs.60 odd lakhs, you may as well get that sooner. You have 50% ROCE. I mean, the question is how soon do you get to that 70%, 60%, whatever that occupancy is, and largely these are brownfields, right? And like I said, there is unsatiated demand at my doorstep. I have no fixed cost. I mean, there is absolutely no benefit on any Excel sheet for even a day's delay on this.

Yogesh Sareen: Shaleen, it is also about the patients' convenience because when you have the traditional construction, you have more disturbance to the patients who are in the hospital, right? So there is obviously noise around etc. So by doing this structure, you are also able to reduce the inconvenience to the ongoing operations.

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Shaleen Kumar: Very much agree.

Yogesh Sareen: Let's say you're doing it in Nanavati, right? So you already have a running hospital there, right? If you have all this digging out going there and a lot of construction activity going on there, this obviously disturbs people staying in the hospital. And you can't do 24x7 construction in that case. So I think this also allows us to, you know, fabricate this stuff outside and bring it in and, it's a faster construction and also lower disturbance on the site, especially where you're running the hospitals.

Shaleen Kumar: Fair enough. That's about from my side, thank you so much.

Moderator: The next question is from the line of Dheeresh Pathak from White Oak Capital Management.

Dheeresh Pathak: Yes, thank you for taking my question. Can you give the CapEx outlay for the Dwarka project and as well as for the Shalimar Bagh?

Abhay Soi: So, as far as Dwarka is concerned, we are not incurring the CapEx. We are incurring will be about Rs. 130 odd crore, Yogesh can give the precise figure as far as the medical equipment is concerned. CapEx is being -- entire CapEx is being incurred by the developer. We have a fixed rental that we are going to be paying him, which, -- which is Rs. 20 odd crore.

Yogesh Sareen: Shalimar would be roughly a cost of Rs. 150 crore, including the equipment and on the -- Dwarka one, that will be around Rs. 170 crore because we are expecting the LINAC also there. Now this Rs. 170 crore, in addition, we've given some deposits for these guys, to start with, as -- when we signed the contract.

Dheeresh Pathak: And what is the rental in Dwarka? Rs.20 crore?

Yogesh Sareen: The rental in Dwarka would be Rs. 22 crore a year.

Dheeresh Pathak: So on the labs business, can you give like the share of revenue from B2B and B2C?

Yogesh Sareen: Yes, so I would say it'll be a 50-50 type. So 50% will come via B2B. When I say B2B, this also includes franchisees, right? And balance will be B2C.

Dheeresh Pathak: But franchisee in true terms, it is B2C, right? But the HLM business would be typically B2B?

Yogesh Sareen: HLM will be around 20% of the business will be HLM. And 25% will be coming via the franchisees. So another 5% through the phlebotomists at site, etcetera. Balance will be all direct including pickup points, home pickups and wellness, etc.

Dheeresh Pathak: Okay. So one last question for Nanavati what would be the EBITDA per bed?

Yogesh Sareen: We don't share the hospital level EBITDA per bed.

Dheeresh Pathak: Okay. Because the number you said is double digit EBITDA margin, and

if I do the math , Rs.450 crore revenue you said, and 15% margin.

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Yogesh Sareen: Yes, it's around 15% margin. Yes. So I would say actually in quarter two it's

around 16% margin, right? And you have the -- you have the revenue from Maharashtra already in Earnings Update -- so you can compute it.

Dheeresh Pathak: Yes. So then also it'll look lower and primary reason, like Sir explained, is it because the legacy doctor cost, which is 10 percentage point?

Abhay Soi: No, person nel cost.

Dheeresh Pathak: There's a union there, is it?

Abhay Soi: But I mean, it's a benign union. That's not -- the issue is not that . The issue is that look, when you do a VRS, right, typically you pay let's say six months of salary for every, year of service left or nine months of salary for every year of service left. So we did the first --VRS at six months of salary for every year of service left. Now the ask is on e year of salary for every year of service left, you know, so rather than paying that out, I'm saying, look, the new capacity comes in, this gets defrayed over a larger this thing in any case, the excess manpower. We also have to do a cost benefit.

Dheeresh Pathak: Okay. So even outside of that, also the EBITDA per bed, I don't know, based on the numbers we are sharing, even adjusted for that would look lower. So is there something el se also in Nanavati ? Like is it lower occupancy, is it lower ARPOB apart from that?

Abhay Soi: Higher number of general ward beds at present. The configuration has fewer single rooms, fewer double rooms with attached bathrooms because these are older sort of buildings. So the big, big thing over there is, for single rooms. If you are from Bombay you'll know that you won't get a single room in these hospitals, right? I mean, you typically get admission in lower category than move up, move up.

Moderator: The next question is from the line of Tushar Manudhane from Motilal Oswal Financial Services.

Tushar Manudhane: So just firstly on the international patients, like historically if -- does the patient from Afghanistan have better realization than compared to what you are having from the current international patients? Or will that be just driving the volume, if at all the Afg hanistan patient starts coming into India.

Yogesh Sareen: No, Afghanistan won't have any better, you know, ARR et cetera. So it'll be all the same. It'll be in the same range.

Tushar Manudhane: Okay. So it will drive the volume basically not the realization.

Yogesh Sareen: Yes, yes. Nevertheless, with internatio nal patient, the ARR is generally double of the domestic, right? Because we get more acute patients there and that obviously helps us and it is more complex work that we get on international side. And so obviously that helps us in terms of the ARPOBs and the EBITDA per bed et cetera.

Abhay Soi: Your average bill is twice. It's not about the pric ing, the average bill is twice.

Tushar Manudhane: But resource as well as the service aspect also will be the typically much superior compared to?

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Abhay Soi: No, but understand, in spite of that, I mean, if you -- if today I have a patient for a fracture and I have a patient for a liver transplant, right? I make a lot more absolute margin as well as the overall billing on a liver transplant . Of course the resources are higher, but my margins are much higher. So when a person comes, nobody's going to have a fracture and come from Afghanistan to get it sort of sorted over here, but you come for a lifesavin g liver transplant or transplant or a lifesaving procedure or whatever, where the average billing is much higher.

Tushar Manudhane: So just to understand , approximately what would be the margin from the international patient s compared to the company leve l?

Abhay Soi: No. See, again, you're talking margins in percentage, which is the wrong sort of cadence to look at. Well, like I said, you'd rather do a 20% margin on a \$10,000 surgery than do a 50% margin on a \$2,000 surgery. So what I'm telling you is the aver

age billi ng is twice.

Tushar Manudhane: And secondly, on the case mix side, the cardiac sciences seem to be improving nicely over past couple of quarters. And oncology remaining pretty stable. So while the payor mix change can definitely drive the

ARPOB, but from a case mix perspective, typically cardiac is little lower compared to oncology. So will that have certain impact on the overall ARPOB?

Abhay Soi: No. And you've seen it, like you said, yourself, right? You've seen it increase yet you've seen overall increase in ARPOB, correct?

Tushar Manudhane: Okay. And just lastly, just to understand the mix of, so while this quarter had a viral load on account of dengue, so the typical mix of, surgery and the medical business for the quarter or for the first half, and how probably that can change in the coming when there is no viral infection per se. So any broad color on that?

Yogesh Sareen: So Tushar this is -- the medical mix was up by 2% this time. So let's say this is, you know, generally if 43:57 this time it was 45:55, 45 is the medical. So in quarter one it was 43:57. So I think as we get into quarter three, this will normalize to the old levels.

Tushar Manudhane: So effectively resulting into, let's say a ballpark, what kind of increase in ARPOB, is it like in the meaningful range of Rs. 1000, INR 2000 or much more than that because of change in this portion?

Yogesh Sareen: No, I can't obviously reveal the figure. But I think you can see that, you know, 2% change in the mix of the PSU patient and an increase in the international patient, the level which has come in the quarter two compared to quarter one, if that increase was not to happen, then obviously as I said, the ARPOB is only one and a half times for the international one and for the PSU it is double. So if you compute that, you'll get to a number.

Moderator: The next question is from the line of Bharat Sheth from Quest Investment Advisors Private Limited.

Bharat Sheth: You said typically in Mumbai, doctor payout is higher than the rest part of the world. Is that correct understanding and if that is there, what exactly are we doing to retain this kind of talent? I mean, for our hospital and in future then would you like to go for Mumbai?

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Abhay Soi: So my Mumbai hospital does not compete for doctors against hospitals in other parts of the country. They compete with hospitals in Mumbai. So it's a secular trend that the doctor payout in all hospitals across the Mumbai are similar, which is higher than compared to the rest of the country.

Bharat Sheth: But then ARPOB in Mumbai is much higher than the rest of the country, because the payout is higher?

Abhay Soi: Not necessarily. The doctor payout has nothing to do with ARPOB.

Doctor payout is a cost line. Your ARPOB is a revenue line.

Bharat Sheth: Can you share some kind of, I mean, broader statistics say apart from in these -- all these metro city, how these international tourists, which city attracts more international and which is less?

Abhay Soi: Number one place in India is Delhi NCR. 40% of all medical tourists come to Delhi NCR. The rest is distributed; Mumbai's share is the least.

Bharat Sheth: Second, now since we are -- currently next two years, we will be expanding brownfield. But once we go for our brownfield opportunities over and if we go for a greenfield, then it'll be taking a little hit on the margins. Is that understanding correct?

Abhay Soi: When you do greenfields, which are sizeable in nature compared to the rest of your portfolio, it will have, sort of temporary in towards your margins. It does not mean that you give up that opportunity, when you have -- that opportunity. So if I get opportunity to do, let's say three greenfields right in the middle of Mumbai, not that I'll get the land for it, but if I was to, I'd do it. But having said that, we have

a large base of, you

know, Rs. 1500 crore - Rs. 1600 crore of EBITDA, about Rs. 6,000 odd crore top line. So how much will be impacted by it is the question.

Bharat Sheth: Okay. Fair. Last question, sir. We have several levers for expanding the margin. So when we are talking currently, we have around annualized, Rs. 65,000 to Rs. 67,000 per bed EBITDA. So with all these levers, where do, what is our aspiration? And of course, occupancy is also increasing.

Abhay Soi: Like I said, you know, we cannot give you, forward looking guidance on EBITDA on this thing. Our EBITDA is Rs. 64 lakhs, not thousand per bed.

As far as the levers are concerned, all of them should auger better and improve this going forward. Some sort of calculations you can do, like I said, um, you know, I'm reducing institutional bed share by 13%, in

absolute terms. So that means that 13% should be able to generate at least 50% more revenue, 85% of that will flow to EBITDA. So that has, some impact on your EBITDA numbers, your international patient s increasing, your insurance patient s increasing, et cetera, et cetera. So I think overall we are in a good space, exactly where it will lead you to, which quarter is something for you to estimate. But, like I said, you know, I think, you know, as a sector, as a company, we are in a good space. We are generating significant amount of free cash flow, and we have very good land banks right in the middle of the metros. From 85% capacity, we move to 93% capacity post expansion. And we already have these land where work has started most importantly. And there's an opportunity set in the rest of the country, and I've always gone out to say that look, any location which is viable, where at least two of my competitors have proven viability, we'd be more than happy to sort of enter those places. And of course, you know, M&A is another big lever for us and you know, that will

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throw out further geographies and opportunities for us. And we certainly have the balance sheet and the cash flows to support those expansions.

Bharat Sheth: Okay. So what are the -- I mean, while criteria for M&A I mean, do we still, I mean, would like to have an evaluating any 33% kind of ROCE would like to generate?

Abhay Soi: Of course, in the long run, of course. I mean, it may not be immediately available that sort of ROCE, but yes, over a period of time, through those assets, through further expansions over there, brownfield, et cetera, we can unlock value. Do keep in mind, brownfield expansions allow you a significantly higher ROCE than your present set of operations because your EBITDA per bed is much higher in a brownfield and there is no stress on your -- even your short term sort of EBITDA margins.

Moderator: The next question is from the line of Harith Ahamed from Spark Capital.

Harith Ahamed: Hi, so looking at our bed addition plans, we have roughly 1,500 new beds getting commissioned in FY '25 and I believe a higher number, probably by FY '26. So how should we think of the payor mix specifically at these new beds? Will we stick to our targeted 15% share from institutional patients, or will we prioritize occupancies and probably accommodate a higher share at these new beds?

Abhay Soi: So look, 15% is a derived number, right? Essentially what we believe is that the reduction in institutional business will stop when this new sort of capacity comes in. It's beneficial that majority of this capacity or almost all of this capacity in '25, '26 is all brownfield. So it will not disturb the payor mix at that time, but at the same time, you know, any further acceleration will stop towards non-institutional. So I mean, I don't, and then again, coupled with the fact that look, you've got lot more operating levers for almost all of this capacity because it's brownfield. It will not disturb your payor mix or your margins at

that stage.

Harith Ahamed: And then in terms of M&A priorities, are we open to assets outside metros and tier one cities, which is our current market?

Abhay Soi: Absolutely.

Harith Ahamed: And also assets outside the north region of the country, which is again, our core market today.

Abhay Soi: Look, we have capabilities of executing anywhere in the country, you know, on similar sort of models that we understand. My only criterion has been that, in a new geography I don't want to do a greenfield and B, I don't want to go to uncharted territory where, you know, I only want to go to places where at least one or two of my -- two of my competitors at least have proven viability. We will do it better like we do in each and every micro market that we compete in, without exception. And you are witness to that over quarters. Right?

Moderator: Thank you. Ladies and gentlemen, this was the last question for today. I would now like to hand the conference over to the management for closing comments.

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Abhay Soi: So thank you so much for your time, for logging onto the call. And we look forward to connecting with you in the next quarter with good news again. Thank you.

Call: Feb 2023

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February 8, 2023

Listing Department, Listing Department,
National Stock Exchange of India Limited BSE Limited
Exchange Plaza, Plot C -1, Block G, Phiroze Jeejeebhoy Towers,
Bandra Kurla Complex, Bandra (E), Dalal Street,
Mumbai – 400 051 Mumbai – 400 001

Symbol: MAXHEALTH Scrip Code: 543220

Sub. : Transcript of the Earnings Call held on February 3, 2023

Dear Sir / Ma'am,
Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed copy of transcript of the earnings call, organized on February 3, 2023, on the financial results of the Company for the quarter and nine months ended December 31, 2022.
The said transcript is also available on the website of the Company at www.maxhealthcare.in/investors/investor-resources.
Kindly take the same on record.
Thanking you,
Yours Sincerely,
For Max Healthcare Institute Limited

Dhiraj Arora
SVP - Company Secretary and Compliance Officer

Encl.: As above

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Max Healthcare Institute Limited
Q3 FY23 Earnings Conference Call Transcript
February 03, 2023
Moderator Ladies and gentlemen, good day, and welcome to the Max Healthcare Institute Limited's Earnings Conference Call. Please note that this conference is being recorded.
I now hand the conference over to Mr. Anoop Poojari from CDR India. Thank you, and over to you, sir.
Anoop Poojari: Thank you. Good morning, everyone, and thank you for joining us on Max Healthcare's Q3 and 9M FY23 Earnings Conference Call. We have with us Mr. Abhay Soi, Chairman and Managing Director and Mr. Yogesh Sareen, Senior Director and Chief Financial Officer of the company.
We will begin the call with opening remarks from the management, following which we'll have the forum open for an interactive question-and-answer session. Before we start, I would like to point out that some statements made in today's call may be forward-looking in nature and a disclaimer to this effect has been included in the earnings presentation shared with you already.
I would now like to invite Abhay to make his opening remarks.

Abhay Soi : A very good morning to everyone. I am pleased to welcome you to Max Healthcare 's Q3 Earnings Call.

Our performance for this quarter was on expected lines and reflected the continued focus on execution by the hospital teams while maintaining high levels of medical quality and patient satisfaction. Compared to Q3 last year, the occupancies, revenues, EBITDA and other operating and financial parameters have improved considerably. While compared to Q2, this quarter expectedly witnessed a slight dip in occupancy due to festive season and revenue was relatively flat. However, we yet again reported our highest -ever EBITDA – both in terms of absolute value and margins, EBITDA per bed, ARPOB and ROCE for the third consecutive quarter this financial year.

Our digital app 'Max MyHealth', which was soft launched at the end of September 2022, has already witnessed approximately 90,000 downloads. The app is now ready for a formal launch in the fourth quarter of the current year. Before I move on to the highlights of this quarter, please note

that comparative numbers and percentages are

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being reported on a like -to-like basis, excluding COVID -19 vaccinations and one -time tax gain in the previous quarter.

Key highlights of our Q3 performance are :

1) Occupancy for the quarter improved to 77% from 74% in Q3 last year. However, it was marginally lower than 78% for the previous quarter due to festive season.

2) Institutional bed share fell to 29% compared to 31% in Q3 last year. The bed share was 1% higher than the previous quarter due to relaxation owing to lower occupancy in festive season.

3) Network gross revenue was Rs. 1,559 crore compared to Rs. 1,385 crore in Q3 last year and Rs. 1,567 crore in the previous quarter. This reflects a growth of 13% year -on-year, while remaining flat quarter -on-quarter due to seasonality.

4) Revenue from International Patients grew by 62% year -on-year and reflected 110% of pre -COVID average. This accounts for around 9% of the revenues now.

5) Digital revenue grew to Rs. 272 crore and accounted for 17% of overall revenue.

6) ARPOB for the quarter rose to approximately Rs. 66,800, reflecting a growth of 10% year -on-year and 1% quarter -on-quarter.

7) We reported our highest -ever Network Operating EBITDA of Rs. 419 crore compared to Rs. 364 crore in Q3 last year and Rs. 410 crore in the previous quarter, reflecting a growth of 15% year -on-year and 2% quarter -on-quarter.

8) We are actively managing the costs, and there is a reduction in overheads quarter -on-quarter due to better collections and reduction in power costs.

9) The Network Operating EBITDA margin improved to 28.3% versus 27.8% in Q3 last year and 27.7% in the previous quarter. Annualized EBITDA per bed, most importantly, rose to Rs. 66.9 lakhs, yet again our highest ever, clocking a growth of 12% year -on-year and 4% quarter -on-quarter.

10) Profit after tax was Rs. 269 crore versus Rs. 252 crore in Q3 last year and Rs. 267 crore in the previous quarter.

11) Net Cash position stood at Rs. 372 crore at the end of December 2022 compared to Net Debt of Rs. 296 crore last year. This is after deployment of Rs. 102 crore towards the ongoing capacity expansion projects.

12) Continuing our efforts to give back to the community, we treated 38,344 OPD and 1,264 IPD patients from economically weaker section free of charge. In addition, we provided nutritional support to around 2,300 TB patients during the quarter.

13) Both our SBUs continued to report robust numbers.

(a) Max@Home reported a top line of Rs. 36 crore, reflecting a growth of 30%

year-on-year and 4% quarter -on-quarter. It started Immunization -at-home services and now has 14 service line offerings.

(b) Max Lab reported a gross revenue of Rs. 28 crore. This reflects a growth of 46% year -on-year, while declining by 4% quarter -on-quarter due to

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seasonality. The active network partners stood at over 900 , spread across 34 cities and supported by a dedicated team of more than 700 personnel.

Now coming to the overview of the company's financial performance for the nine months ended December 31, 2022 :

14) Network gross revenue stood at Rs. 4,597 crore, reflecting a growth of 16% on a like-to-like basis.

15) Network operating EBITDA stood at Rs. 1,199 crore, registering a growth of 20% on a like -to-like basis. A RPOB improved by 16% due to price, payor mix, case mix, etc., and led to margin expansion by 92 basis points. EBITDA per bed grew by 21% to Rs. 64.4 lakhs.

16) COVID -19 bed occupancy was negligible throughout. On average, COVID -19 patients occupied merely 5 beds in Q3 and 20 beds during the nine months.

17) On expansion projects, the current status of capacity coming on stream by FY '25 is as follows:

(a) 100 beds at Shalimar Bagh have been more or less handed over to operations team, and we are on schedule for starting the operations in the current quarter.

(b) 300 beds at Dwarka – The st

structure is complete and interior work is

underway. We plan to file for occupancy certificate by May and operationalize in Q2 FY '24 as planned.

(c) 329 beds at Nanavati – The contract has been awarded to Larsen & Toubro on a turnkey basis in December for handing over the hospital on or before 24 months. L&T is fully mobilized and we expect to commission the facility by end of FY '25.

(d) 300 beds at Sector -56 Gurgaon (Phase 1) – Work has commenced. Excavation and D-wall work will be completed by Q1 FY '24 and by which time the civil contractor will also be mobilized.

(e) We would also like to point out that Sector -53 land cancellation has no impact on our bed additions plan until FY '28. It was not a part of our expansion rollout but will , if it all , impact bed potential post FY 2028 as we have put down in the investor presentation.

So, all in all, up to this stage, we are seeing no delays. We are focused on execution and it is going to be on the spot as planned.

(f) 350 beds at Max Smart – The project is delayed by around 3 months for lack of final tree cutting permission, which has been received in January this year now. The work will start in the current quarter, and we hope to recoup the time lost as we progress on the project.

18) While we continue our focus on the growth levers articulated in the past, we have also been evaluating avenues for catering to demand for quality health care in the near term. We expect to add over 100 beds in the next 2 quarters at some of our hospitals through internal reconfigurations. Moreover, with our net cash surplus and deleveraged balance sheet, we are extremely well positioned and actively

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evaluating inorganic growth opportunities. As of yesterday, our Board has given in-principle approval to raise finance of up to Rs. 4,200 crore of NCDs for any future M&A. However, we intend not to breach 2 to 2.5x Net debt to EBITDA, considering that any new acquisitions will also bring its own EBITDA in to play.

With this, we open the floor for Q&A.

Moderator : Thank you very much. The first question is from the line of Nikhil Mathur from HDFC Mutual Fund.

Nikhil Mathur : My first question was to be on Net debt to EBITDA, I think you have kind of cleared it. But just at a slightly higher level, I wanted to understand with so much of organic bed expansions planned over the next 4 - 5 years, is there a pressing need to do an M&A or whatever M&A that you are seeking is going to be very, I mean very valuation conscious and there has to be some clear rationale for you to be looking for an M&A because I believe the bed CAPEX plan itself will kind of take care of the medium to long-term growth of the company.

Abhay Soi: I think we are not putting a cap on the long-term growth. If you look at the opportunity set, which is out there, it is significantly higher than what we are looking to tap through our brownfield or whatever we are expanding. One is that. Secondly, also, given the fact that we have a completely unlevered balance sheet; we have got more than Rs. 1,000 crore of cash sitting; we have a net debt position of some Rs. 360 -370 crore. Our entire expansion, which is at a cost of about Rs. 4,000 -4,500 crore over the next 4 years, is going to be conducted entirely through about 50% of our free cash flows. So, we have a totally unlevered balance sheet and we have the rest of our free cash flows to deploy. And given the opportunities set out there and again, if you've seen our history, it's been about buying assets at values which are intrinsically below what we believe we can eke out over there in EBITDA, okay? So, these will all be massively value accretive.

We are very, very conscious of ROCE. We are already operating at very, very high ROCEs, as I have said before also in my past calls. So, I think anything that we will be acquiring is going to

be accretive. We're not acquiring it for the sake of it. I want to add one more thing, it also gives us presence in newer geographies.

Nikhil Mathur : And in terms of regional priorities, I think North looks pretty sorted for the company. There is obviously a media article of you venturing out in Kolkata, looking out for something there. But would it be safe to assume that, I mean, my understanding perhaps the Southern market is a bit more competitive. So, you might be looking more for West and East. Would that be a fair assumption?

Abhay Soi : Not really. I've in the past said, look, we won't go to uncharted territory. I will go to any territory where at least 2 or 3 of my competitors have proven viability, and we will do it better like we pretty much have done without exception, in every micro market that we are present. I mean, if you go by that list, I think the number of cities would be in the mid -20s where number of people have proven viability. I mean, we do operate a hospital in Mumbai, which is in Western India. It is the only asset that we have and it operates pretty well. And if we look at what we are doing in even outside the cluster of Delhi NCR in Tier-2 cities like Mohali or Dehradun, which are extremely high ROCE businesses for us.

So, I think even entry into the Southern markets, when you look at what their profitability, etc., are, I think so long as we have conviction that we can eke out and we can make it accretive, it becomes a sensible entry point for us.

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Nikhil Mathur : One more question I had on the operations for this particular year. Now I think your investor presentation gives a clinical update on liver transplants, kidney transplants and bone marrow transplants done till date. But can you give some sense as to what is the number for these 3 categories of transplants looking like in FY '23 and what growth are we envisaging in the number of transplants that we'll be doing in the coming 2 -3 years? I mean the clear question here is that these initiatives are ARPOB accretive. So, that could be an ARPOB driver for you in the coming 2 -3 years.

Abhay Soi : I mean it doesn't really move the needle. I mean you have to look at it collectively. I think overall, this transplant business will be 5% -6%.

Yogesh Sareen : Nikhil, basically, if you come to the liver transplants, we do around 40 to 45 of them every month. Kidney transplants would be a bit higher than that, it will be around 60 - 65 every month. And BMT would be another 25 to 30. So, these are the ranges that we have. Obviously, the endeavor of the team is to start the program in more and more hospitals. For example, that's what we did in liver transplant. We started a program in Vaishali, we started a program in BLK, we are starting the program in Mohali and Dehradun now and we are waiting for the license. So, one is, obviously growing acuity for work, that's always the effort. So, now to put a projected number, I mean that's very tough for us. There is no order book that we have.

Abhay Soi : But moreover, what I'm saying is, this is going to be incremental because there is already a particular pace at which we add. So, this is going to be incremental. Even if I double this number, you're not going to see exponential change in EBITDA or

margins because of it.

Moderator : The next question is from the line of Damayanti Kerai from HSBC.

Damayanti Kerai : First, one clarification. Abhay, did you mention you would be able to add another 100 beds through internal reconfiguration, apart from the planned ongoing CAPEX ?

Abhay Soi : Well, I said over 100 beds. So, it's a number which is higher than 100 beds, yes.

Damayanti Kerai : Okay. That's clear.

Abhay Soi : Let me make it clear. Through internal reconfigurations, okay, we will be adding this over the next few months.

Damayanti Kerai : Next few months, okay. My second question is how do you see your

operating cost

increasing up as your planned beds come online over the next 2 -3 years, both on a variable as well as fixed cost part ?

Abhay Soi : I think the majority of the CAPEX that we're doing with the rollout we are doing it towards Brownfields , right? I mean 80% -85% of the total rollout is towards

Brownfields . And Brownfields by the very virtue have higher operating leverage, because it doesn't necessarily have a fixed cost associated to it because the fixed cost, be it in terms of management or senior clinicians , etc., is already being incurred by the existing hospitals. So, when you add another tower next to it, it doesn't have that. So, technically you have a higher operating leverage. So, our operating cost overall should come down.

Damayanti Kerai : Because you would be able to expand existing resources to a higher number of beds , right?

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Abhay Soi : That's right. And what you are actually putting over there are nurses or resident doctors , etc., your senior clinicians are already in place. And the reason you're doing these Brownfields in the first place is because your capacity is out, right, and we've got unsatiated demand at your doorstep, your doctors don't get OT time, they don't get beds for the patients and so on and so forth.

Damayanti Kerai : In most of the brownfield expansion s, should we assume the breakeven should be achievable within 12 to 15 months of the commencement of the unit?

Abhay Soi : No. I've stated in the past, the brownfield should have a breakeven in the first quarter or two, if not the first quarter itself.

Damayanti Kerai : So, within 2 quarters, we can reasonably assume that?

Abhay Soi : Absolutely.

Yogesh Sareen : In a brownfield , you don't open the beds until such time you are able to fill it, right? So, there's no need for us to open all the beds.

Abhay Soi : I mean, not that we don't have the need. We have the need; I think the point that Yogesh is making is that you are holding the assets on your books, right? But there's no real fixed cost associated to it from the operating standpoint.

Damayanti Kerai : My last question is, can you talk a bit about your ESOP program s? And right now, what percentage of Max employees are covered by your ESOPs? And what kind of annual expense do you expect for the stock option programs?

Abhay Soi : So, I think they're about 81 lakh shares, 271 employees, okay, are covered by the ESOP plan. I think the important thing is that the cliff for the ESOP plan, okay, is 20% IRR, which comes to a price of, I think .

Yogesh Sareen : To vest 100% of the ESOPs , the price is Rs.1.260 after 5 years. Basically, the ESOP has 2 portions . So, one is the individual performance, the other is the company performance. So, the company performance part, which is mainly for the leadership team, will vest after 5 years provided there is a 25% CAGR in the share price from when we issued these ESOPs. So, at that point the rate, the price required for 100% to vest is Rs. 1,260 at the end of 5 years.

Moderator : The next question is from the line of Amit Kadam from Canara Robeco Mutual Fund.

Amit Kadam : Sir, can you dwell a little bit more on the international patient s, how that particular mix of segment is moving ? In one of the presentation, in one of the slides, you mentioned that the sales are already at like 10% higher than pre -COVID. Just wanted to know that on the footfalls, how the things are looking? Where the traction is coming? How do you see the year going forward? Because last time when we spoke, you had mentioned about Afghanistan not present, but you are trying to cope with that particular thing with some other geographies. Just maybe 2 -3 minutes brief on that particular segment.

Abhay Soi : We were not trying to cope up, we have coped up, right? So, in spite of Afghanistan which was 12% of our business being down to 0, we are at 110% of

pre -COVID levels.

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So, what that basically implies is that not only have we coped up for lack of

Afghanistan, but we've sort of overcompensated for it, right? That's one.

Secondly, I mean, to have a discussion on present footfalls, you'll have to have a point of reference from past footfalls. I mean the fact that this number has been moving up, I mean, we are getting massive traction on this, it's over 60% compared to last year, it's over 110% compared to pre-COVID levels and in spite of 12% of Afghanistan not being there. Hopefully, in the next quarter or 2 quarters, depending on the geopolitics as and when Afghanistan does open, this will give us further push. But in the meantime, we'll be looking at other geographies as well. We've been opening offices in other places, and I guess, with also the support of the Indian government through Heal in India and Heal by India, there is focus on global medical tourism. So, I think we can have a step change over here.

I haven't given any guidance in terms of footfalls in future or revenue guidance in terms of international patients or any other sort of revenue guidance, I'm going to avoid that even at this stage.

Amit Kadam : So, when we say like revenue is like 10% higher, than even the footfalls are, I assume that if realization has been same, then those footfalls are higher than the pre-COVID?

Yogesh Sareen : Yes, more or less. But may not be 10%, maybe probably 4% to 5%.

Amit Kadam : Second is, in the presentation footnote, you have mentioned that there was a recovery in terms of bad debts, which you have put under indirect overheads. So, can you just help me with that particular number, please?

Yogesh Sareen : So, that number would be around Rs. 7 crore during the quarter. So, this is a charge during the quarter. So, there is obviously a running system that we have that at the end of each quarter whatever bill is not cleared within 365 days, they will get provisioned for.

Abhay Soi : We have a very stringent policy that anything which is over 365 days, okay, we provide for.

Amit Kadam : Because you had mentioned that why that OpEx is sequentially down. One of the line items you mentioned that one of this as the reason.

Yogesh Sareen : Yes, we did mention. That's right. So, there's impact. And there's always the impact. This obviously depends on when the collection comes. We have around 16%-17% of business, which is PSU revenue share and the payments from CGHS are tardy, they come in blocks.

Abhay Soi : Sometimes they come over 365 days, then we write it off at the end of 365 days, provide for it and then when it comes back, we have to write it back.

Amit Kadam : And just like I want to know further like because you had said that another 100 beds is what you are expecting in the next few months through the internal reconfiguration. Is it across the various hospitals? Or is it from one particular thing? Second is that, is it safe to build this particular increase in terms of like somewhere in quarter 1 or quarter 2 of FY '24.

Abhay Soi : Yes. I mean it should come in through by quarter 1 or quarter 2 of FY '24, right. These are in 4 or 5 separate hospitals, maybe 20 beds somewhere, 30 beds somewhere

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and so on. So, collectively, it's over 100 beds. And clearly, you eke out this capacity in places where you're hitting thresholds. When you get to that stage, you find elasticity, you find reconfigurations and so on to be able to get by. I mean you don't do it in a place where you already have even a little bit of leeway as far as idle capacity is concerned.

You have to understand, as we are coming close to threshold capacities in places, what we earlier thought was threshold capacity, both in terms of operations and number of beds, we find there is elasticity at the end, right? It's like a manufacturing process, your rated capacity may be 75%. When you

come to the 75%, you realize

that you can operate at 80%. When you get to 80%, you'll realize you can get to 82%-83% and so on.

Moderator : The next question is from the line of Naman Bhansali from Perpetuity Ventures LLP.

Naman Bhansali : So, my first question relates to international patients. And I think it's around 8.5% contribution. But if you want to view it as a percentage of only Delhi Hospitals, so how much would that percentage be? And relating to the international patients, like what is the structure that we follow to attract these international patients, like do we pay some medical consultancy fees to any agencies? Or what sort of cost do we pay to attract these patients? And how is it versus the industry and Max? So this is my first question.

And my second question relates to the margin structure of brownfield expansion, which we are doing. So, with every incremental bed additions, how do we see the incremental margins which we get on the brownfield bed. Lastly, on the ARPOB difference, like what is the difference majorly in our business between the

international and domestic ARPOB? And how would it boil down in terms of EBITDA per bed? So, these are my questions.

Abhay Soi : Let me start by margins on brownfields. So, as far as margins on Brownfields are concerned, our typical brownfield will cost us about Rs. 1.30 -1.40 lakh per bed. Our present EBITDA per bed is about Rs. 66 lakh last quarter. If you apply even a 75% occupancy rate to it, so that comes to about Rs. 66 lakhs into 75%, so maybe about Rs. 45 -50 lakh I presume. So, you're looking at Rs. 50 lakh on top of Rs. 130 lakh. This is if you were doing business as usual. But like I mentioned, there is operating leverage in Brownfields, so EBITDA per bed should be northwards of that. And more importantly, over the next couple of years, by the time all of this comes on stream, you'll have some real and inflationary growth on top line as well.

So, it is massively accretive. Look, the most attractive thing we'll ever do, okay, more than M&A, more than greenfield, etc., is Brownfields, okay? Because A, you're building in a proven area where you are tapping into unsatiated demand, and then you have a huge amount of operating leverage coming out. So, that is exactly what we are doing. So, that's one. I hope that answers your question as far as margins are concerned.

Yogesh Sareen : So on the ARPOB question that you asked on the international side, typically, the ARR, the ticket size for the international patient is double of the domestic because of the fact that there are more acute patients. But the ALOS is also higher, right? So, the average length of stay of this patient is also 1.3x of the normal. So, typically, that means that the ARPOB will be one and a half times of the domestic patients. When I say domestic patients, they will be cash domestic patients. That's on the ARPOB. Yes, the EBITDA per bed is higher because of the fact that these are higher ARPOB. So, even if you maintain the same margin percentage, after payout on these HCFs or

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facilitators, your EBITDA per bed would be around 20% higher than the domestic patients.

Abhay Soi : But having said that, our marketing is done through multiple channels. At the very least, there are walk-in patients, where there are no facilitators involved. Then you have patients and these people come through our digital platform, come through our office in overseas and so on and so forth. Then you have patients who come through international medical tourism companies. So, in those situations, we make a payment of facilitation fees to those international medical tourism companies.

The third is we have tie-ups with various ministries of health in various countries. We have tie-ups in hospitals. At any given point of time, a number of our doctors are traveling overseas conducting OPD screenings. So, it's a fai

ly organic process. We

also have doctors from these countries who come to our hospitals and work as observers, they go back and they campaign for us. But we conduct OPDs, we screen the patients, we do pre-consults, post consults over there and so on and that's the backbone of how we do medical tourism.

Yogesh Sareen : Also, on your question about the share of international patients in the NCR hospitals or Delhi hospitals, it ranges from 4% to 18%, right? So, the maximum is in BLK-Max Hospital and then followed with Saket and then you have other hospitals like Vaishali and Patparganj, etc. So, on an average, it will be probably 11% to 12% in the NCR hospitals.

Naman Bhansali : And what is the maximum on a consolidated basis, like we are currently at 8.5% in international contribution, and this can go up towards 10% -11%?

Abhay Soi : I mean it depends how much we are doing, right? Like I said, I'm going to avoid giving any sort of guidance in terms of how much of this business we are going to be shooting for over the next 4 to 5 years. But like I said, the potential is exponential, not incremental.

Moderator : The next question is from the line of Ashish Thavkar from IIFL AMC.

Ashish Thavkar : Sir, if you could spell out, is there a certain seasonality in our business because if we try to have a look at the occupancy rate, quarter 4 seems to be much lower than the earlier quarters. So, if you could just help us understand the nature of the business?

Abhay Soi : Absolutely. So, there is a seasonality in the business. So, even if you go back to our last quarter's investor call or the presentation, when we had the higher occupancy, we said in Q2 that the occupancy is higher because of seasonality, right? In the rainy season, you have waterborne diseases, airborne diseases, etc. So, in Q2, typically, all hospitals have higher occupancies, right?

I mean this is like I mentioned due to the seasonality and the flu season and so on and so forth. So, you'll have a lot of internal medicine patients, you have a lot of pediatric patients, etc. They come to the hospital for medical reasons, not for surgical reasons.

So, this quarter 2 is usually characterized by higher occupancy, but lower ARPOB. And if you look at Q3, it is typically a weaker quarter. Year-on-year, you will find it to be a weaker quarter, which is this present quarter, because it is characterized by the festive season. A lot of the New Year, Christmas holidays, etc., I think this year Diwali

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was in October, so you will see patients postponing their surgeries or doctors postponing the surgeries, etc., by a few days or a few weeks or whatever. So, typically, Q3 is a weaker quarter, which is characterized by lower sort of occupancies, but you have higher ARPOBs in this season because whoever is coming for surgery is somebody who can't really put it off. So, therefore, he is coming for more sort of acute and more Quaternary care surgeries, etc., which typically will have a higher amount of billing. Also, because Christmas and New Year, a lot of international patients don't sort of come in, they postpone their visits because look Christmas, New Year's also, etc. So, the way to compare health care this thing is quarter compared to the previous year quarter rather than sequentially the previous quarter because of the seasonality. Q4 is typically a stronger season, right, is the strongest sort of quarter in the year.

Ashish Thavkar : So, last one on the ARPOB, sir, we are already at 77%, 78%. Obviously, the concern remains as last time you had highlighted at peak, you can go to 80% -82% and the bed additions like you're trying to reconfigure or internalize. So, 100 plus beds over the next 2 quarters, where do you find comfort and do you feel that before we materially add a higher number of beds, can we still manage to do 10% to 12% top line growth which is 77% or say 80% kind

of occupancy levels?

Abhay Soi : No. So, look, I think there are 2 or 3 separate things over here. Firstly, the number of beds we are coming out with over the next 6 -7 months, I presume, is going to be 300 in Dwarka, 100 in Shalimar Bagh and 100 plus that we are doing. So, that's over 500 beds. That's one.

Secondly, last quarter, which is quarter 2, we were at I think 78% occupancy. We've done months of 81 %-82% also and we've done quarters of 81 %-82% as well. Some of our hospitals are operating at higher occupancies. And that's where, when you start, in those hospitals when you start hitting occupancy thresholds, we also have the lever of payor mix, right? I mean you start accommodating your preferred channel at the cost of the unpreferred channel. And that's why we had guided the unpreferred channel or the institutional business would come down accordingly. So, there's enough leeway in the system to accommodate any sort of growth that we have, while the new capacity comes in. And I have only given you 500 beds, which is estimated to come in over the next 6 to 7 months. And then in 2025, we have more beds coming and so on.

Ashish Thavkar : Sir, lastly, versus the volume growth. So, typically, the industry does around 8% to 10% volume growth. Any color on how the pricing trends are currently in the industry?

Abhay Soi : Year-on-year, you normally have a 2% -2.5% impact on revenue as far as pricing is concerned. And that's pretty much across the industry right?

Moderator : The next question is from the line of Lavanya Tottala from UBS.

Lavanya Tottala : So, I just wanted to understand how you look for breakeven timelines for the greenfield project? I understand for the brownfield, it's around 2 quarters, so for the greenfield project in Gurugram, how do you look at the breakeven timelines?

Abhay Soi : Yes. So, look, I think historically, greenfield used to be about a 2-year sort of breakeven. My belief is now it will be a 12 to 15 months sort of a breakeven in greenfield.

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Lavanya Tottala : And the brownfield where the current occupancies are in the range of 75%, that should see somewhere like 3 quarters, high occupancy, something like 2 quarters and greenfield 2 years? Is it the right way of looking, sir?

Yogesh Sareen : No. we're not saying that. So, you take the example of Shalimar Bagh, now quarter 3, the occupancy was 85% in that hospital, right? So, at 85% occupancy, that means, obviously, you are not admitting all the patients. So, at that point of time when we open 100 beds, we think there should be EBITDA accretion in the first quarter itself, right? So, we are not going to wait for 3 quarters for us to get some EBITDA from those additional 100 beds. So, that should be immediate. Now it depends on where the occupancy is, but I would say, probably not more than 3 to 4 months for EBITDA accretion in a brownfield situation.

Lavanya Tottala : I just got disconnected in the earlier commentary time. So, if I understand right, like 100 beds with the existing capacity on the Shalimar Bagh, so these 200 -250 beds should be available for the full year FY '24? And 300 beds from Dwarka should come in at what time line, sir?

Abhay Soi : We said Q2 FY '24.

Lavanya Tottala : So, around like for 2H it will be available, like the 300 beds of Dwarka.

Abhay Soi : That's right.

Lavanya Tottala : And so on the acquisitions, which you have highlighted, so what kind of efforts you will be looking at like the stand-alone hospitals or a chain? I just wanted to understand your view on what kind of assets would you be looking if you are trying to enter a new region or at least certain number of beds is the thing which you look at I mean consider an asset for this opportunity. So, how you will be looking at it?

Abhay Soi: So, the bigger the better. I mean that if you have a chain versus a single hospital, we prefer a larger sort of asset.

et. But it doesn't mean that we don't look at single hospitals.

We look at that as well. What is important for us is that it has to be accretive to ROCE, there is intrinsic value for us to unlock over there. And like I said, ultimately over the medium term, it needs to be value accretive and needs to be ROCE accretive.

I mean if I have to go to South India, I won't go with 1 hospital, but if I have to do it in adjacent to a cluster that we are, then we may do 1 hospital over there. So, I mean I just won't buy 1 random hospital, let's say, of 200 beds in Chennai. If I have to do Chennai it has to be a larger sort of this thing. If I have to do, Bangalore, it has to be a larger format. It just can't be a hospital for the sake of it because we are acquiring some EBITDA or something like that.

Lavanya Tottala : Yes. So, this is helpful, sir. So, that's what I wanted to check if you would be looking for a certain one asset in Chennai or a place like where other things are present, so that's what I wanted to check.

Moderator : The next question is from the line of Krishnendu Saha from Quantum Mutual Fund.

Krishnendu Saha : Sir, Just trying to understand the business as to why we have a higher ARPOB because of a lot of foreign clients or because of the mix ratio. Just trying to get a feel of the business.

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Abhay Soi : We have a higher clinical mix. Firstly, I think if you look at the 2 things, which drive ARPOB, one is the payor mix, other is clinical mix, right? As far as payor mix is concerned, 7%-7.5% of our beds are totally free for the poor compared to 1% or 2% for most of our competitors. If you look at 29% of our beds are catering to institutional business compared to maybe 20% and 13% for some of our competitors, right?

So, the payor mix is clearly inferior, but our ARPOB is maybe 20%-25% better than the next best player in the industry. But more importantly, our EBITDA per bed is 55% better than the next best player in industry. Now EBITDA is better because on each and every, I think, line item, we outperform each of the hospitals where we compete in pretty much every micro market.

But as far as the revenue is concerned, we operate at a higher sort of more Quaternary care, higher end of clinical mix. So, it's also indicated in the fact that we have more beds, which are critical care beds, 35% of our beds are critical care beds. Most other people are between 25%-30%. So, it basically portrays that we are doing more high-end work as a proportion of the total work that we do. So it overcompensates for the lack of payor mix.

Krishnendu Saha : And a bit of high occupancy also compared to the peers?

Abhay Soi : Yes. There is no role in the ARPOB but the fact that we are present in the NCR that also helps us, right? And the stronger brand will be higher occupancy. The occupancy is not a function of ARPOB or this thing. Occupancy is a function of your brand strength.

Krishnendu Saha : So, just the expansion, which we will have, do you expect to maintain that 35% of the specialty types of the revenue stream, going ahead, because we'll be expanding at a faster clip in the next 2-3 years? Just trying to get a feel that we will be able to maintain the 35% mix going ahead?

Abhay Soi : It would be higher. I mean there is no reason for it not to be. We're looking at the same mix of business going into the future as well.

Krishnendu Saha : All right. In the sense, more on understanding. Yes, there's an available market which you can definitely address to.

Abhay Soi : Available market is, it's where the crunch is, right? I mean 35% of our beds are critical care beds, yet where we don't have any beds available or the highest occupancy if we look at in our system is not in the ward bed, not in the rooms, etc., it is typically in the single room that more importantly in the ICUs, the critical care. So, as we are going forward, we are in fact

intending to build more critical care beds because that's where the bottleneck is in our current system.

Moderator : The next question is from the line of Sachin Kasera from Svan Investments.

Sachin Kasera : I had just one question. As we bring this capacity to brownfield and some of these efficiencies, what is your thought in terms of being able to sustain or maybe improve the current payor mix and as well as sustain the current occupancy levels?

Abhay Soi : Like I said, the majority of the expansion is coming in our brownfields, right? The reason we are doing it, we're not doing it in order to tap the market, we are doing it because we've got unsatiated demand at our doorstep. So, I mean, the payor mix that we've got waiting of 6 hours to 2 days in a ER for the sort of bed that you may want.

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If you want an ICU bed, even a single room is not available. I mean, if you go to Nanavati Hospital, if you go to Max Saket, if you go to Mohali, if you go to any of our hospitals that's where the challenge is. If you go to places like Gurugram and all, you don't have beds, right? So, there's no reason for that payor mix not to be sort of to be any differentiation over there.

Of course, we've guided in the past that our payor mix should be improving to a certain extent and then it will be plateauing out. It will be plateauing out because this new capacity is coming in by then.

Sachin Kasera : So, do you think the current payor mix is already plateaued out or you think there is still some improvement in the payor mix after which it will plateau out?

Abhay Soi : So, we have guided that it should be down to 15% in the next 4-5 quarters.

Sachin Kasera : And you remain confident on that number, sir?

Abhay Soi : That's right. You have to keep in mind, if you look at sequentially, in a weak quarter, you're going to be accommodative, right? I mean a weak quarter in the sense that in a festive quarter where you have lower occupancy, okay, why would you want to have idle beds? You're going to take more institutional or whatever else is at that point of time. And this was a weak quarter. I mean this is a seasonally weak quarter, not for us, but from an occupancy standpoint.

Sachin Kasera : And sir, any thoughts in terms of the revenue per operating bed.

Abhay Soi : I think what needs to be looked at, okay, is the fact that in spite of it being a seasonally weak quarter on an overall standpoint, right? We've navigated it to be the highest EBITDA quarter in margins, EBITDA per bed or absolute EBITDA in the history of the company. So, what happens in the stronger quarter?

Sachin Kasera : So, which would mean that even with this additional capacity coming in and with the leverage that we are talking about, actually, we could see improvement in ARPOB, EBITDA per bed and EBITDA margins over the next few quarters?

Abhay Soi : No doubt on that front. I mean I've repeatedly said that in the past that EBITDA per bed, it will be accretive.

Moderator : The next question is from the line of Ranvir Singh from Nuvama.

Ranvir Singh : My question was more about the macro situation here in healthcare or hospital industry. I see in this budget, the Government has allocated some, I think, Rs. 6,000 crore to Rs. 7,000 crore for setting up AIIMS in different locations. Just I wanted to understand going forward because the Government thrust has been to improve infrastructure, mostly in public sector. So, do you see that the public sector is emerging strongly here and that could give the private sector a bit of competition in the next 3-4 years. So, how is your view on it?

Abhay Soi : I think anybody who's been to a public sector hospital should be able to answer that question. And even during COVID when health care was in focus and everybody was looking at it, you saw while there was no beds in private hospitals, there were idle capacities, okay, in Government

hospitals across the board. And I think if this public sector supersedes, then probably in my memory, it will be the first time and first
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country where the public sector sort of outperformed the private sector. I don't think it's ever happened anywhere else. So, I really don't see that coming as a threat. I think public sector hospitals are free hospitals. They are free to the poor and to the rich and everybody else. When it is free, your idea is to provide health care coverage to more and more people, not necessarily to provide creature comforts. It's not as if your room may have a TV over there or you have AC in the room and so on and so forth, I mean it'd be a very basic sort of thing. And you may not be aware, no government hospital even has a stream, which is called 'critical care'.

I mean there is nightingale wards and so on and so forth. I think the whole push of the government is very, very different from that standpoint. I mean after that the next stage will be hospitals which are doing PMJAY, we are not even doing PMJAY. We don't even cater to that sort of subject.

Yogesh Sareen : For us, a hospital like AIIMS is basically a source of medical talent, right? So, typically, doctors working in AIIMS after 20 years, they take voluntary retirement and they try and work in the private sectors. I think that to us is a good thing, right?

Abhay Soi : I mean, that's massive because the sort of range of skill sets that they develop in public hospitals is immense simply because the volume and the complexities they see over there. So, typically, you'll see a doctor having worked in any of the Government hospital after a while leaves and joins the private sector.

Ranvir Singh : Thanks for your view. And specific to the company, I guess I missed a number, that what was the contribution of international patients during this quarter in revenue?

Abhay Soi : 9%.

Ranvir Singh : And what was it in Q2?

Abhay Soi : Same.

Ranvir Singh : Same. Okay. And ARPOB is normally much, much higher in international. So, what would be the average ARPOB in international patients?

Abhay Soi : I think Yogesh mentioned in the previous this thing, it's 50% higher ARPOB.

Moderator : The next question is from the line of Prakash Agarwal from Axis Capital.

Prakash Agarwal : Just trying to understand the margin outlook better for the next 4 to 6 quarters. I understand the bed additions of Shalimar Bagh, Dwarka and others you mentioned, they could have a little lower EBITDA per bed and the ARPOBs than the average.

Would that understanding be correct? And would it have any impact on the margins?

Yogesh Sareen : Not in the brownfield hospitals, but Dwarka, yes. It will take time for Dwarka to start generating EBITDA. So, as I think, as Abhay mentioned, 12 to 15 months is when we see the breakeven in terms of EBITDA in Dwarka, that's the greenfield site. But other than that, it should be all better EBITDA per bed, etc., and better margin because these are all brownfields. And as I mentioned, Shalimar Bagh had 85% occupancy in quarter 3, right? So, in that hospital when I add 100 beds, you can understand how soon the uptake in the occupancy will be.

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Abhay Soi : And the 100-plus beds actually which we are eking out of the present capacity, right?

I mean their EBITDA is again, we'll be swinging for the fences over there, literally with the EBITDA margins. So, that 200-plus bed, 200 to 250 beds should give you a higher EBITDA on day 1 sort of thing. And Dwarka will take 12 months to sort of get there and thereafter.

Prakash Agarwal : But the 200 addition is higher than the average that the company is clocking?

Abhay Soi : Very much. No doubt about that.

Prakash Agarwal : So, on a blended basis, you are okay with the margin trajectory or maybe see an improvement also?

Abhay Soi : That's right.

Prakash Agarwal : And how about M&A? I mean,

clearly, with the fundraise plans and the cash. I mean

we are sitting in Delhi NCR, which is the best micro market as such. But if we expand in the gaps maybe in Mumbai or even other Tier-1 metros, this kind of ARPOBs are unheard of. So, when we look into this M&A, would it be fair to say that we would be okay with the lower ARPOB to start with and with case mix, etc., we would be improving? Or how should we think about it in terms of M&A going into that particular micro market selection with respect to our margin stability and ARPOB stability?

Abhay Soi : So, I think Prakash, firstly, our highest ROCE businesses are not Delhi, it's the Tier-2, Tier-3 cities, okay? Secondly, my ARPOB in Mumbai is comparable, if not, I mean, higher than any of the Delhi hospitals.

Prakash Agarwal : Yes. Barring Mumbai and Delhi, we would be looking at M&A outside also, right?

Abhay Soi : So, I'm saying outside Delhi and Mumbai, like I said, even if I look at Tier-2, Tier-3 cities, my ROCE over there is much higher than Delhi.

Prakash Agarwal : Yes, understand ROCE concept. But from the ARPOB and margin perspective, would we be open for lower ARPOB and lower margin business?

Abhay Soi : Of course, I'll be very open to lower. The only thing I'm concerned about is ROCE. I am okay with lower ARPOB and I am okay with lower margins so long as whatever in absolute terms is EBITDA per bed vis-a-vis what we put in over there is accretive to us on a return on capital basis.

Prakash Agarwal : And just one more clarification. So, during COVID times, there was a higher cash patient mix. So, has that normalized now post COVID? And how is cash versus insurance? I mean insurance would have gone up, right?

Yogesh Sareen : Prakash, so yes, insurance has gone up once the COVID set in and there was more policy being sold and that trend is continuing, right? So, it's not that it was one-off and the share of the insurance business has come down. It's been maintained at the same level now.

Prakash Agarwal : And cash would have gone down.

Yogesh Sareen : Yes, it did. Yes.

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Prakash Agarwal : And typically as per you would be about, what, 20% in pricing, 20%-25%?

Prakash Agarwal : So, in terms of insurance are obviously contracted rates. I mean, that is about 20% - 25% lower than the cash patient billing.

Yogesh Sareen : 7% to 8%.

Prakash Agarwal : Oh, that's it ?

Abhay Soi : And it actually works out better because when you have insurance policy, and yes, there's been a step change in the frequency of people buying insurance, health insurance being more. But the trend has been on for the last 10 odd years in some manner or the other. It works out well for the quaternary hospitals because as and when you have insurance policy, you're paying the same amount of premium, whether you go to Max or you go to ABC Nursing Home. And what we see is people don't window shop at that stage. You're less price sensitive, you go to the better brand, better hospital because eventually you pay the same premium.

Moderator : The next question is from the line of Kunal Dhamesha from Macquarie.

Kunal Dhamesha : So, the first one was on the 100 beds that we are adding in our existing capacity, would that be more kind of the deluxe beds or the private rooms, et cetera? Or would it be a mix of everything?

Abhay Soi : No. So, it will be more critical care beds than more single rooms , etc., which is more of what we need.

Kunal Dhamesha : So, probably more ARPOB basically if those gets filled, right, because of a single room or deluxe room, critical care beds, et cetera.

Abhay Soi : We make more out of doing a liver transplant in a general ward than we do from doing a delivery in the suite.

Kunal Dhamesha : But over a longer period of time, that's not in your hand, right? Like, over a longer period of time, the private room s would obviously be better assuming that the mix o

ver a long period across beds remains same.

Abhay Soi : That's right. So, I would actually take the current mix and project it. It won't be lower than that because it's a general ward or whatever else it is. Like I said, most of our hospitals are suffering from a bottleneck at the critical care.

Kunal Dhamesha : And as far, I believe that those critical care bed addition , etc., requires permission or something like. So, all those are in place for us ?

Abhay Soi : Yes, I mean no permission required within the existing capacities .

Kunal Dhamesha : In terms of government being able to operationalize AIIMS , etc., which they are kind of opening up now or investing in now because their operational cost or operational cost structure is very different from private players, right? Maybe around the nursing expenses or nursing salaries around maybe 2 x-2.5x than what they get in the private hospitals. So, at any point, is there any talks between Government and a player like you to hand over the operations and management of such apex institute to a company like Max, given we are also kind of providing similar kind of care?

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Abhay Soi : It's a different profile. But what we are seeing is a lot of state Governments , be it Haryana, be it UP and so on and so forth, are inviting private players from and they 've been some RFPs for public-private partnerships, okay, on creating newer facilities. I mean the government has its own way of working. You would have seen even the total outlay has been increased by about 13% towards the health care sector. And whatever the economics of the government are the economics of the Government facilities.

Their viability, I'm not going to sit and question. The fact is they are trying to cater to perhaps an audience, which is an unaffordable audience, right, which can't even afford PMJAY sort of , etc. That's for free health care effectively. And irrespective of viability, I don't think a private player is going to be catering to anybody who can't afford to pay at all.

Moderator : The next question is from the line of Sumit Gupta from Motilal Oswal.

Sumit Gupta : I just have few questions regarding international patients. So, I just want get a gist on the overall pricing. So, I presume that international ARPOB is more than Rs. 1 lakh. So, it is gross or net of discounts and every other thing that you pay to the overall medical tourism agents and all?

Yogesh Sareen : ARPOB is always Gross , right? It's based on the revenue that you book. There may be some expense linked to that revenue. So, we don't net that. That comes on the expense side.

Abhay Soi : No matter how you look at it, EBITDA is 20% higher like what Yogesh said, per bed.

Sumit Gupta : And one more thing regarding, like sequentially, if I see that payor mix for international patient it is nearly like 9%, so consistently. So, going forward, do you see it at 9% or like moving it to 10 %-11% also with like new geographies opening up?

Abhay Soi : Our focus is always on the occupied bed days increasing, right? I mean if it remains 9% and other business is also increasing, so be it, right? So, as long as the operating

bed days on an absolute or a stand-alone basis for medical is increasing. So, we don't necessarily look at it as a percentage of the overall pie. We look at it as a separate segment altogether. So, we look at, look, is international number of bed days occupancy increasing or not? Is the total revenue in absolute terms increasing or not? And then separately we look at the same way for upcountry business, we look at the same way for cash business, we look at the same way for other business, right? My focus or our focus is on this thing. The 9% being flat on 2 quarters is also because, like I said, it's seasonality. People don't travel during the festive season. Doctors themselves put off surgeries, etc., because they are on holidays. I mean the be

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way to look at it is at the same time last year. Unfortunately, last year being a little bit of COVID year, you have a kind of an uneven comparison, and we're all forced to look at pre-COVID levels. But sequentially, one should avoid. You'll typically see the first quarter and third quarter are weak quarters. And for these very reasons they'll be.

Moderator : The next question is from the line of Amit Kadam from Canara Robeco Mutual Fund.

Amit Kadam : Just one small question, can you help me with the tax rate for maybe how do we look at it going forward, maybe FY '24?

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Yogesh Sareen : I think for tax rate. I said this earlier also, the ETR should be in the range of 18% to 20%, right? So, this quarter was 18% plus, right? But I would say it will peak at 20%, on the Network basis. But you will note that this quarter, the CTR has come down, i.e. the current tax rate has come down. It's by virtue of the fact that last quarter, we did some voluntary liquidation and there are some benefits of that liquidation in terms of depreciation on intangibles. So, I would say 2 things. One, the rate will be 18%-20%. Secondly, I would say watch out the CTR rate, that's better parameter because that's where the cash outflows are linked to, right? And that's improved over quarter 1 and quarter 2.

Amit Kadam : So, 18% would be a fair assumption to consider somewhere between 18% to 20%?

Yogesh Sareen : I would say go towards the higher range of 18% to 20%.

Moderator : The next question is a follow-up question from the line of Ashish Thavkar from IIFL AMC.

Ashish Thavkar : Yes. So, if you get an opportunity in future, would you also be willing to consider stand-alone hospitals, which are specialized in oncology, pure play oncology or say something like pure play child care?

Abhay Soi : We will evaluate everything; we have no issues evaluating anything. When the opportunity arises, we will evaluate.

Ashish Thavkar : Sir, lastly on this digital app-based model, if you could just give some color on the potential of this model. And would there also be a cash burn?

Abhay Soi : On the app, there is no cash burn. It's not a diagnostic platform. It is a platform for our patients to interact with our doctors and our doctors to interact with the patients and for the hospital to provide services to the patients, right? Be it diagnostics at home or home care business or video consultations and so on and so forth. I mean, I encourage you to actually look at the app, please download and look at it. So, I think all your reports, everything is sort of stored over there. So, it's effectively a platform between the hospital and the patients to interact. And for us to deliver those services to the patients.

Ashish Thavkar : Okay. Perfect. That was it, sir.

Abhay Soi : We are not a platform for third-party players, etc. So, I mean it's very, very different. I mean it's not 24/7 or anything like that. I mean it is 24/7, but it's for us, so we're not marketing it as a third-party platform. I mean the cash burn itself essentially what it is for technology and a little bit of marketing that we will be doing within the Delhi NCR or whatever reason through the hospitals, etc. I'm not looking at any significant cash burn or anything which will even move the needle over here.

Ashish Thavkar : Okay. And your diagnostic offerings would also be clubbed with this app?

Abhay Soi : Absolutely. So, we will be providing diagnostics through the app as well.

Ashish Thavkar : Okay. Sounds good.

Abhay Soi : So, on our app at the top, okay, there's a red button, which can get you an ambulance in Delhi anywhere, anytime within 20 minutes. That's one, which will take you to a

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Max Hospital. Then diagnostics, then nurses, then all your medical records, then any video consultations that you want, any scheduling of appointment that you may want to do and so on. Ph

ysiotherapy at home, anything, so anything that of. So, the entire suite of services that the hospital provides can be provided to you at your doorstep effectively or without you entering the hospital premises. That's what the app does.

Yogesh Sareen : It will also review trends of your diagnostics , reports. So, in a way, it's there to improve the experience. It's there to improve the, I would say, the interaction of the patient with the hospital and vice versa.

Abhay Soi : But check it out, it's out there. I think that's the most important thing .

Ashish Thavkar : Yes, sure. I will.

Moderator : The next question is the follow -up question from Krishnendu Saha from Quantum Mutual Fund.

Krishnendu Saha : Just clarifying our average length of stay at 4.2 days, is there higher or it probably could go down a little bit more. Just trying to understand it is high because of the critical care being a larger portion is it? And can it go down further?

Abhay Soi : And higher services. And eventually, we need to look at average revenue per occupied bed because your inventory in hand effectively is number of beds days that you have. So, it's higher ALOS but which leads to higher ARPOB growth for you, that's because you're doing more higher end. I mean, so look, if a place has more medical patients, your ALOS is lower but if you have more transplants patients it is longer. But transplant patients will pay you more. I mean the more BMT transplants , lung transplant s, heart transplant s you do, people tend to stay longer. And billing is more, ARPOB is more, the length of stay is more. You can't really compare A particular chain with B and C.

Krishnendu Saha : Yes. So, some metrics if you look at ALOS in all these days, occupancy and all, so just trying to figure out as to why because we are a little bit higher as of that sir.

Abhay Soi : You can't compare. That's what I'm saying. Within my chain, I can't compare two hospitals because they may be leading 2 different medical programs. I have to really compare program to program or I have to compare with the same hospital to its absolute in the previous year.

Yogesh Sareen : It will actually be procedure -to-procedure , right? So, that will compare the ALOS . And average revenue per occupied bed is the better metric, but that will have ALOS play in to it already, right? If that metric is growing and it's better than others, that to my mind , means ALOS should not be even looked at.

Moderator : Ladies and gentlemen, that would be our last question for today. I now hand the conference over to the management for their closing comments . Thank you, and over to you.

Abhay Soi : So, thank you. I appreciate everybody coming online. And I just want to reiterate that the 2 pillars of the foundation that our organization rests on is firstly, execution. So, we are very, very focused on execution, not only in operations but on projects as well to see that they are rolled out in time. And second is fiscal discipline, so you would see that any leverage , if at all , we use for inorganic growth, we would be withi n what
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our declared norms or prudent norm s for leveraging the company are, which is not more than 2 -2.5x Net debt to EBITDA. So, thank you for the opportunity. Good bye.

Call: May 2023

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May 24 , 2023

Listing Department ,
National Stock Exchange of India Limited
Exchange Plaza, Plot C -1, Block G,
Bandra Kurla Complex, Bandra (E),
Mumbai – 400 051

Symbol: MAXHEALTH Listing Department,
BSE Limited
Phiroze Jeejeebhoy Towers,
Dalal Street,
Mumbai – 400 001

Scrip Code: 543220

Sub.: Transcript of Earnings Call held on May 17, 2023

Ref.: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements)
Regulations, 2015

Dear Sir / Madam,

Please find enclosed copy of transcript of earnings call , organized on May 17, 2023, on financial results of the Company for the quarter and financial year ended March 31, 2023.

The said transcript is also available on the website of the Company at
www.maxhealthcare.in/investors/investor-resources .

Kindly take the same on record.

Thanking you

Yours truly,
For Max Healthcare Institute Limited

Dhiraj Arora
SVP - Company Secretary and Compliance Officer

Encl.: As above

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Max Healthcare Institute Limited
Q4 & FY23 Earnings Conference Call Transcript
May 17 , 2023

Moderator Ladies and gentlemen, good day and welcome to the Max Healthcare Institute Limited Earnings Conference Call.
Please note that this conference is being recorded.

I now hand the conference over to Mr. Suraj from CDR India. Thank you, and over to you.

Suraj : Good morning , everyone and thank you for joining us on Max Healthcare Q 4 FY'23 Earnings Conference Call. We have with us today Mr. Abhay Soi – Chairman and Managing Director; and Mr. Yogesh Sareen – Senior Director and Chief Financial Officer of the Company . We will begin the call with opening remarks from the Management , following which we will have the forum open for an interactive question and answer session.

Before we start , I would like to point out that some statements made in today's call may be forward -looking in nature and a disclaimer to this effect has been included in the Earnings Presentation shared with you already.

I would now like to invite Abhay to make his opening remarks.

Abhay Soi : A very good morning to everyone. I am pleased to welcome you to Max Healthcare 's 4th Quarter Earnings Call.

At the outset, I would like to state that this has been a seminal year for Max Healthcare on many accounts and an important one of them being the overall translation of Rs. 1,636 crore of EBITDA to Rs. 1,281 crore of free cash flows. Consequently, in spite of a decadal opportunity for investment in the hospital sector, we have been encouraged to declare our maiden dividend .

Q4 was a robust quarter for us and exhibited the results of commendable execution of our strategy by teams on the ground . This quarter once again reflected our best ever performance across nearly all the financial and operating parameters recording significant growth year -on-year.

We commissioned and operationalized a 92-bed oncology block at Max Shalimar Bagh from 1st March . This contributed positively to both revenue and EBITDA in its very first month of launch. EBITDA margin on the incremental revenue was in the range of 35 % to 40% due to operating leverage and overall occupancy at the hospital

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was 83% in Q4. We expect this to contribute to improvement in EBITDA , both in absolute and margin terms in the ensuing quarter.

Key Highlights of Q4 Performance:

Occupancy for the quarter improved to 77% from 68% in Q4 last year and remained at the same level as in the previous quarter. However, it is pertinent to note that operating capacity moved up by 100 beds in March 2023 compared to December 2022 .

Institutional bed share fell to 29% compared to 33% in Q4 last year and remained flat compared to the previous quarter. Again , there was higher capacity in the current quarter as well.

PSU tariffs for room rent and consults have been revised in mid -April. Further discussions are ongoing to increase tariffs for packages , diagnostics , etc. In view of these developments, we have not taken a hard call yet for some of the major accounts. At the same time, all our hospitals have been tasked with improving their operating occupancy thresholds to accommodate growth in the preferred channels. Network gross revenue was Rs. 1,637 crore compared to Rs. 1,298 crore in Q4 last year and Rs. 1,559 crore in the previous quarter. This reflects a growth of 26% year-on-year and 5% quarter -on-quarter . The year-on-year increase was driven by growth in AR POB and occupied bed days.

Revenue from international patients grew by 43% year-on-year and 10% quarter -on-quarter . This now accounts for around 9.1% of the revenue from hospitals and amounts to 120% of pre -COVID levels.

Digital revenue grew to Rs. 292 crore and accounted for 18% of overall revenue .

Led by improvement in channel mix and specialty mix , ARPOB for the quarter rose to approximately Rs. 70,700, reflecting a growth of 11% year-on-year and 6% quarter -on-quarter .

We reported our highest -ever network operating EBITDA of Rs. 437 crore compared to Rs. 304 crore in Q4 last year and Rs. 419 crore in the previous quarter, reflecting a growth of 44% year-on-year and 4% quarter -on-quarter .

The Network operating EBITDA margin stood at 28.2% versus 24.8% in Q4 last year and 28.3% in the previous quarter . Annualized EBITDA per bed, most importantly , rose to Rs. 70.3 lakhs, yet again our highest ever, clocking a growth of 25% year-on-year and 5% quarter -on-quarter .

Profit after tax was Rs. 320 crore versus Rs. 172 crore in Q4 last year and Rs. 269 crore in the previous quarter . The year-on-year growth of 85% was primarily attributable to improvement in operating metrics of all the hospitals and lower finance costs.

Free cash flow from operations stood at Rs. 425 crore , of which Rs. 65 crore was

deployed towards ongoing capacity expansion projects. The Net Cash position improved to Rs. 733 crore at the end of March 2023 compared to Net Debt of Rs. 441 crore last year.

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Continuing our efforts to give back to the community, we treated approximately 36,600 OPD and 1200 IPD patients from economically weaker sections of society free of charge.

Both our strategic business units continued to maintain their growth momentum.

1) Max@Home reported a top line of Rs. 37 crore, reflecting a growth of 26% year-on-year and 2% quarter-on-quarter.

2) Max Lab reported a gross revenue of Rs. 31 crore, reflecting a like-to-like growth of 57% year-on-year and 10% quarter-on-quarter.

Performance for the Full Year:

Network gross revenue stood at Rs. 6,234 crore, reflecting a growth of 18% on a like-to-like basis.

Network operating EBITDA stood at Rs. 1,636 crore, registering a growth of 25% on a like-to-like basis while ARPOB improved by 15% due to price and improvements in payor mix and case mix leading to margin expansion by 152 basis points.

EBITDA per bed grew by 22% year-on-year and touched a new high of Rs. 65.9 lakh.

Status of Expansion projects coming on stream by FY 2025 is as follows:

We have recently signed an ATS (Agreement to Sell) for purchase of land to enable

expansion of Max Vashi, which is consistently operating at more than 80% occupancy. If and when the deal is consummated, it has the potential to add 100 Brownfield beds to our network.

For 300 beds at Dwarka, interior work is in progress, lifts are under installation and external development has started. Medical equipment has been ordered. And as communicated earlier we expect to commission the hospital by end of Q2 FY24, subject to the developer obtaining occupation certificate by that time, of which we are quite certain.

For 329 beds at Nanavati the work is in full swing at the site. Foundation and column work have already begun. As you may be aware, Larsen & Toubro (L&T) is handling the project and we expect to commission the facility by the end of FY25.

For 300 beds at Sector 56 Gurgaon in Phase -I, D-wall work is complete and excavation is underway, while the civil contractor mobilizes for starting construction by end of June.

In order to obviate the delays at Max Smart caused due to tree transplantation issues, we are fast tracking the construction at Vikrant site, which is part of the same complex.

We have received in-principle environmental clearance (EC) approval and are expecting other approvals over the next 6 to 8 weeks.

Lastly, we continue to actively, but prudently, evaluate inorganic growth opportunities for strategic deployment of our cash surplus from operations. With this, we open the floor for Q&A.

Moderator: Thank you very much. We have our first question from the line of Ashwin Agarwal from Akash Ganga Investments.

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Ashwin Agarwal: Abhay, could you highlight what is your vision going beyond three to five years beyond what have you announced in terms of organic opportunities? And whether you would be looking at the South and the other regions in the country?

Secondly, you own around 24% stake now so would you like to increase your stake so that you can use the stock as an option in terms of acquisition, else you get diluted?

Abhay Soi: So, let me start with the last one first. Firstly, thank you for the compliment. The consequence of this performance has been that three years back when we were listing the company, one of the biggest critiques of the Indian Healthcare or the Hospital sector was that there are no free cash flows and you have to keep redeploying. I think with Rs. 1,281 crore of free cash flows translating from Rs. 1,636 crore of EBITDA which shows ~80% translation of EBITDA to free cash flows, I think we have answered that question. So, that is, as far as I am concerned, right up there as far as our achievements for the current year.

Now with respect to my vision over the next three to five years, I think there is a multi-decadal opportunity in the hospital sector. The kind of infrastructure, which is required to be created simply because you need to address that demand and we are seeing this every day. So all the cash flows that we are generating, we would be

redeploying it into the sector and we have been looking at pretty much every part of India.

We have been very clear about two things that we will only look at places where at least a few of our competitors have proven viability. We will go there and we will do it better, like we do in each one of the micro-markets that we currently operate in. We have not been the first anywhere, but we are the top performer wherever we operate. So, I think that is the strategy that we want to employ. So, that doesn't preclude any place, but of course we like a 'cluster approach'. If I have to just do one hospital in Kerala or just one hospital in Chennai or something I would not do it, we prefer clusters so we will look at chains and we will look at clusters.

Ashwin Agarwal: Anything on your stake in terms of using the stock as an opportunity in terms of M&A?

Abhay

Soi: So, look I think my stock, one of the consequences of our success is that it reflects on the market cap and the stock price. To buy anything meaningful, I would effectively have to leverage my current stock, which I am not interested in doing because I don't leverage my stock at all. You know 1% of the stock will cost me Rs. 500 crore, so pre-tax that money is about Rs. 800 crore. As my only form of employment is Max Healthcare, my salary does not permit me to buy more stock. But I would not be afraid of diluting if there is a great opportunity, let me just put it this way. When I say diluting, if there is an opportunity for a merger or an acquisition for the growth of the company, I would not shy away from the opportunity.

Ashwin Agarwal: Yes, that is what I was wanting to know.

Abhay Soi: There is no, and there has never been an issue as far as that is concerned.

Ashwin Agarwal: Would you be only city-centric in next three to five years in terms of opportunities or you would also look into Tier-II opportunities? And do you have opportunities like Nanavati which give you a gateway to any big city?

Abhay Soi: No, absolutely. I think, firstly, we are very happy doing Tier-II and Tier-III cities also. Our highest ROCE business is Mohali, second highest is Dehradun in Uttarakhand,

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it is not Delhi and Bombay. Like I said, we will go to any city where two or three of our competitors have proven viability. We have a list of 21 such cities.

Moderator: Thank you. We have our next question from the line of Damayanti Kerai from HSBC.

Damayanti Kerai: Abhay, my first question is on your payor mix change plan. So, you mentioned we have seen upward revision in PSU tariffs etc., because of that is there any rethinking from your side to bring the institutional bed share to 15%-16%, which you highlighted earlier from 29% currently?

Abhay Soi: So, look, I think the trajectory is changing on account of two or three things. One is that we are finding marginally more capacity, we are operating at higher levels than what we were thinking of previously, so we are able to keep that business and be able to do more and you are seeing that in your margins month-on-month. The best thing is that if you can, we were pushing down that business because we wanted to accommodate our preferred channel of CTI. Now necessarily you are always going to find some elasticity towards the end, so you see the operating levels becoming higher.

Secondly, we have added 100 more beds, which is about 3% more capacity that has been added. Yet you see that the PSU number has still come down marginally.

The third thing is that there has been an increase in rates as far as PSU is concerned, an increase in tariffs, which will work well for us in the current year. That is about 20% or 30% of the total tariffs that they have revised. They have revised it quite significantly by about 70%. So, only four line items, but that amounts to about 20% of the total, 20%? --

Yogesh Sareen: The overall increase would be around 4% to 5% on the billing to the PSUs.

Abhay Soi: But the rest of the tariffs are looking to be revised in, they are saying by July. So, I think we have a little bit of a wait and watch here. Because if we have the same sort of increase by July, it has been proposed that rest of the packages and everything else is being revised. This is what we have been informed.

Damayanti Kerai: Just to clarify, this 4% to 5% tariff hike is for the PSU contracts so far and you are expecting ...

Yogesh Sareen: So, what I am saying is that for the line items that they have raised prices on, the average price increase is around 70%, but in a way the contribution of those line items is around 10% to 12%, so that means the overall increase would be 4% to 5%.

Damayanti Kerai: Okay, and in July, another round of price hike?

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Yogesh Sareen : Yes, we are expecting another round of price hike.

Abhay Soi : For the rest of the items.

Yogesh Sareen : They have not touched the diagnostics, they have not touched the packages, they have not touched the other elements of the blood bank etc. They have not touched that. They just touched four items : one is room rent for ICU beds, room rent for single room, and IP consultations and OP consultations. Only these four elements have been changed.

Abhay Soi : And whatever they have touched has gone up by 70% on average.

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So, let us say there are 100 things and the revenue is 100, only 12% of the revenue has been touched by them. They have increased that 12% by about 70%, and for the balance 88%, we are expecting something to be done by July.

Damayanti Kerai : So, my second question is on your average revenue per occupied bed. A gain, I think you have surprised positively quarter-after-quarter. In the last two years, we have seen around 15% increase. And I understand specialty mix change is the big driver of it. So, can you explain what has changed so significantly in last two-three years that you continue to see better and better specialty mix and how should we see this part moving ahead?

Abhay Soi : I think your ARPOB is being generated by payor mix and specialty mix. It is not only purely payor mix. We have also seen a massive increase in international business, which plays on the ARPOB. It is a combination of factors. During the last three years, you are looking at COVID in the middle, right. So I think your current ARPOB vis-à-vis a COVID year is going to be higher, and you need to compare it to pre-COVID years. Then you take up, perhaps, the cost increases over this latent period of COVID, because the disease burden has come back after COVID. In the middle, you are looking at COVID business, which was high occupancy but lower ARPOB. Now you have business, which is essentially high ARPOB, but your occupancy has moved up because there was not any capacity creation during the COVID years. So, I think you are going to see ARPOB move up, but obviously you can't do a comparison with the COVID year.

Damayanti Serai : Not even with COVID year, so if I look at the period of FY'19 before COVID, you were somewhere at Rs.46,000 to Rs.47,000 ARPOB, and now it has moved up to say, Rs.70,000 plus. So, I am asking from that perspective.

Abhay Soi : I will just let you know what the FY19 ARPOB was. But as you move up the occupancy curve, you are going to distill your payor mix. Now you are talking about a time when my PSU payor mix was 40% plus and now it's 29%, right. I think as you go up the curve, you start getting that operating leverage when you start getting that better quality of business. We still have hospitals in the portfolio that are doing Rs. 90,000 + ARPOB.

Yogesh Sareen : So, our FY'20 ARPOB was Rs. 50,300.

Abhay Soi : So Rs. 50,300 has gone up to Rs. 70,700 in three years now.

Yogesh Sareen : The average is Rs. 67,400 for FY'23. So, if you take year-to-year average, it will be 10% to 11% higher.

Abhay Soi : So, it's 10% to 11% increase right, from Rs. 50,300 to Rs. 67,400 you are saying.

Yogesh Sareen : Yes

Damayanti Serai : So, going ahead also say since we continue to see better mix both in payor and specialty part --

Abhay Soi : I think from distillation of payor mix itself, you should see an increase in ARPOB. For our clinical mix, of course like I said, there is always innovation going on. It's not only at Max Healthcare, but we have plenty of single hospitals in Mumbai, for example, which have been there for 20 odd years, which have no payor mix lever as it has sort

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of saturated. They are not adding an extra bed, not a square inch that they have been able to add, yet the clinical mix sort of drives ARPOB up. I mean, there are lovely exam

ples

of that -- Hinduja Hospital, Breach Candy Hospital and so on and so forth.

Damayanti Kerai : So, comfortably, we should be seeing say high single digit to say double-digit growth in the ARPOB going ahead also?

Abhay Soi : I avoid giving any forward-looking guidance.

Damayanti Serai : And my last question is on your view on the competitive landscape in Delhi NCR market given we have seen many of your competitors trying to step up their presence. So, is there any possibility of bed oversupply in say foreseeable future, if not now?

Abhay Soi : I have not seen any new beds come up in Delhi NCR or even under construction.

Damayanti Serai : Actually , I think Apollo has a facility coming up in Gurugram so –

Yogesh Sareen : This facility was already there by the way; the first floor is already constructed. I think Apollo has just done an agreement ; I mean it was not operating but I think this building has been there for quite some time. So, let us see when it starts to operate, it is not yet operational, right?

Damayanti Serai : Yes.

Moderator : Thank you. We have a next question from the line of Nikhil Mathur from HDFC Mutual Fund.

Nikhil Mathur : My first question is a kind of a clarification , when I look at the audited cash flow statement , the CAPEX incurred under the line item of purchase of property is around Rs. 335 crore whereas in the proforma numbers that we talked about in the executive summary, the company has given a CAPEX number of Rs. 208 crore. I guess there might be some technicality here. Can you please explain me why this difference in audited and executive summary?

Yogesh Sareen : So, just to clarify that point. When we report the Rs. 208 crore spend, that spend is on the capacity expansions, on the projects. So, whatever is the routine CAPEX in the cash flow , it comes in the property line, but eventually when we report numbers to investors we take it out from the free cash flows. When Abhay says that we have Rs. 1,281 crore of free cash flows against Rs. 1,636 crore of EBITDA , in that Rs. 1,281 crore whatever amount that is spent , the total network spend is around Rs. 211 crore on the routine CAPEX and that amount is taken out of the free cash flows . We only report the number of Rs. 208 crore , which is on the capacity expansion, basically on our ongoing projects. There are two type of CAPEX spends, one is the routine CAPEX , which is replacements happening in the running hospitals . And the other one is CAPEX for the capacity expansions . Rs. 208 crore is the number for the capacity expansion and whatever is the number for the routine replacement in the running hospitals , it is taken out from the operating cash flows.

When you say cash from operations , this number is already netted there, but in the audited financials this number , by virtue of the fact that if cash flow is as per a particular Ind -AS requirement, both numbers flow into that number.

Nikhil Mathur : So, Rs. 335 crore is the right number to look at, when we are looking at on a proforma basis?

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Yogesh Sareen : No, that number will be around Rs. 419 crore vis-à-vis what you see is only consolidated financials, right. These consolidated financials don't have the partner healthcare facilities (PHFs) number. The overall number , if I take the network cash flow, will be Rs. 419 crores of which Rs. 211 crore would be routine CAPEX and Rs. 208 crore will be capacity expansion.

Nikhil Mathur : In the cash balance, there is a difference of Rs. 100 crore again in the audited and the proforma balance sheet that you have given , some technicality here as well --?

Yogesh Sareen : So, if you read the very first page that we have put out there in our investor or earnings update , you will find what the difference is for . We have the PHFs where we control the medical operations through the Hospital Management Committee; we

have a 3 :2 ratio so we control the operations. Since it is a medical services agreement , we are not able to consolidate the PHF financials with the Group financials. So, what we do is we do a memorandum consolidation for the investors, get it certified and that's how we report the numbers to the investors. So, if you read the very first slide that would clarify this doubt that you have.

Nikhil Mathur : On the bed expansion plans, I think in the last investor presentation there was the page that you share regularly now that Shalimar Bagh has come on stream, we should refer to that bed expansion plan in the previous investor presentation , that still holds or how should we look at the expansion plan ?

Abhay Soi : That is right. Except for the Max Saket (Smart) site – as we said there is a little bit of delay there , everything else is going to be in line .

Nikhil Mathur : And what's the CAPEX plan for FY24 both routine as well as ongoing projects?

Abhay Soi: Routine would be Rs. 170 crore and ongoing projects would be Rs. 900 crore.

Nikhil Mathur : So, Rs. 900 crore plus Rs. 170 crore ?

Yogesh Sareen : Rs. 900 crore would be for the capacity expansion , which is on the ongoing projects , and around Rs. 170 crore will be on the routine.

Nikhil Mathur : On the international foot falls, how should we look at growth in this business in the coming 2 to 3 years? Now FY 23 has been pretty strong at 43% kind of growth. Do we expect to grow in FY24 as well on this particular base? And not just short term, what are your thoughts on the 2 -to-3-year horizon as well in the growth of international business ?

Abhay Soi : I think I have said on numerous occasions before that there is an exponential opportunity, and what we are seeing is only incremental . And now the Government has put its weight behind it. Hopefully , we will be able to tap it in the medium run, but in the short run we will still see incremental gain. I am going to avoid giving you any sort of guidance on the number , like I avoid on every other financial parameter as well. But we believe this is a space where we have not even hit the tip of the iceberg.

Nikhil Mathur : I also read somewhere , I don't know how true this is, that Delhi NCR accounts for almost 70 % to 80% of international foot fall that come into the country.

Abhay Soi : 40%.

Nikhil Mathur : And one final question, the tariff hikes whether on the PSU front or whatever you are contemplating on the packages side, is it a reflection of supply demand balance

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being in favor of private hospitals at this point of time in Delhi NCR and do you think we foresee that this is likely to be an industry phenomenon not just specific to Max?

Abhay Soi : No, this is an industry phenomenon . CGHS rates are for the whole country.

Nikhil Mathur : This is in response to the news that had come in that government has allowed some price hikes on the CGHS side .

Yogesh Sareen : Yes, and by the way, the last price was fixed in 2014 . It is a revision after 9 years, in a way .

Nikhil Mathur : But you are also considering revising prices for the packages, I mean, which is --?

Abhay Soi : No, that is also government.

Nikhil Mathur : That was on the government side.

Yogesh Sareen : Yes, so they just revised the prices for four elements . Other billing items for example, pathology, radiology, blood bank, packages, surgeon charges etc. all those things are under consideration –

Abhay Soi: Lot of things we do under package right? It is not itemized billing , say in case of CABG or C-section . They have not increased the C-section or the CABG package rates . They have increased the room rents, the doctor visit charges, etc.

Moderator : Thank you. We have our next question from the line of Lavanya Tottala from UBS.

Lavanya Tottala : I just wanted to get a clarification on these packages , so is the tariff hike only related

to CGHS packages or is it something related to diagnostic business of Max also?

Abhay Soi : No, so right now they have not increased the diagnostics . Like Yogesh mentioned, out of only 12% of the revenues or line items that account for 12% of the revenues , price increase has happened over there and it is up to 70% increase . For balance 88%, which includes diagnostics , includes packages , and includes a lot of other things , has not happened . It is under consideration by the government and we have been told by July there may be some visibility on this.

Yogesh Sareen : Ma'am, do not mix the CGHS price increase with the other price increase . There is 29% bed share by PSU where the tariff is given to us by the state , Central Government actually. So, we are talking of that element of the business. For the other business, we do increase prices. We have increased pricing from 1st April. We also increase the prices with insurance companies as and when their contracts become due . So, just do not mix the two .

Lavanya Tottala : So, that's the reason I am just asking for clarification that this is everything the discussion is related to CGHS and nothing related to the Max --?

Yogesh Sareen : That's right.

Lavanya Tottala : So, Max Diagnostics are we planning to put any price increases for just diagnostic business -- in general Max diagnostic business ?

Yogesh Sareen : There are two elements to the diagnostic business: one is in-hospital diagnostics and the other is outside retail business . For the in-hospital diagnostics , we do increase prices every April and we have done some bit of it this year as well . For the non -

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captive pathology business or the retail business , we have not increased any prices there.

Lavanya Tottala : And on the ARPOB I understand that increase in ARPOB is supported by higher international contribution this quarter. So, do you expect an increase in international business in the coming year FY24 or do you think that we have reached the optimal level in Q4?

Abhay Soi : So, I think I have answered this question earlier. Like I said , there is an immense opportunity , exponential opportunity in the medium run . In the short run, it is still incremental. Nevertheless, I think it is clearly going to be incremental. It is nowhere near the peak . I mean with the comparative advantage that we have , there is no reason for it to slow down . It should only snowball and become much larger.

Lavanya Tottala : So, I just wanted to check because we have already crossed the pre -COVID level, so it should be incremental from here, but not a jump or significant jump that we have seen over last few quarters. So, I just wanted to check that.

Abhay Soi : It is a 10% growth quarter -on-quarter in international . Even pre-COVID you had a very good clip of growth. If you look at even pre -COVID levels , the growth rate was not slow. I mean actually 2 years later to go up 20% over pre -COVID levels means you still haven't caught up to the rate of growth.

Moderator : Thank you. We have our next question from the line of Andrey Purushottam from Cogito Advisors .

Andrey Purushottam : I just had a question on the 87% increase in PAT on Q4 '23 compared to '22. I can make out that there has been an increase in international business , I can make out that there has been a decrease in institutional business , I can make out that there is an increase in occupancy. Could you give some more granular insights as to what are the elements of the channel or payor mix that have contributed to this 87% increase ?

And if you think that many of these profit drivers will remain in the future.

Abhay Soi : You want me to describe this 87% increase impact?

Andrey Purushottam : Yes.

Abhay Soi : Every line item before that?

Andrey Purushottam : No, not every line. I am just saying I have identified , as I said , 3 of those factors.

Could you just give us a broad sense of which are the other specific factors when you channel mix and payor mix that have additionally contributed to this 87 %?

Abhay Soi : It is payor mix, it is clinical mix , it is better cost management, lower consumption ratios , lower finance costs. I mean it is pretty much everything, but cost management other than payor mix and channel mix .

Andrey Purushottam : Anything specific in payor mix, apart from what I have outlined --?

Abhay Soi : In payor mix, there is a reduction in PSUs , while the cash, insurance and international business has grown . Effectively , it is not about reducing PSU, but it is about increasing these three other elements.

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Andrey Purushottam : And you think that the operating margins of 28.2% or so are maintainable in the future ?

Abhay Soi : I focus more on EBITDA per bed, not operating margin , because you would rather do a \$10,000 surgery and have a 20% margin than do a \$2 ,000 surgery and have a 50% margin, right. I think the question really is that the EBITDA per bed, even if it means lower margins tomorrow , going to increase tomorrow or not, because that is really what matters from EBITDA overall growth standpoint as well as ROCE standpoint . That, in my mind, will grow. We will continue growing .

Yogesh Sareen : We have the numbers Quarter 1 was 26.6%, Quarter 2 was 27.7% , Quarter 3 was 28.3%, and this quarter it is 28.2%. So, obviously that means it has been consistent , right?

Abhay Soi : Over last quarter, there has been an increase in EBITDA per bed, but if you see the margin , it is marginally lower . But we are happier about the fact that EBITDA per bed has increased. That is why your overall EBITDA has increased .

Moderator : Thank you. We have our next question from the line of Prakash Agarwal from Axis Capital.

Prakash Agarwal : Just trying to understand this 4% to 5% price increase is limited to the 17 % to 20% of the total mix, is that right?

Yogesh Sareen : 29% of the beds are occupied by the PSUs , which constitute around 17 .5% of the revenue right . Now out of this 17.5 % --

Prakash Agarwal : 17.5% in value terms , which is about 4% to 5% in terms of pricing .

Yogesh Sareen : Yes, so I would say that if nothing else happens , then there is a 4 % to 5% increase in the price. Now , we do expect price increases in the other elements because you cannot increase the room rent only for open surgery . You have to increase the packages as well . The moment you admit that the room rent has to go up that means it has to go up for the room rent component under packages as well .

Prakash Agarwal : So, the current increase is 4.5% on 17.5%?

Yogesh Sareen : Yes, that is what I am saying.

Prakash Agarwal : And you also mentioned that every year you take some price hike around April. So, that is more standard across the board in the hospital business ?

Abhay Soi : That is right, yes .

Prakash Agarwal : So, that is 2 % to 3% or it's like a high single digit kind of increase ?

Abhay Soi : No, it is 2% to 2.5%.

Prakash Agarwal : So, what I am trying to understand, we had fairly good ARPOB increase last year

and couple of years , just in terms of headroom of growth we do have international lever , we do have case mix , payor mix lever s. So, then 10% to 12% is not way to off to model for this year as well , would that be right?

Abhay Soi : I am going to avoid giving you guidance on that Prakash , like I always have. But there is no reason for it to not climb.

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Prakash Agarwal : And secondly on the volume side , occupancy side on a blended basis is 77% and we would be having few hospitals , which are 80 % to 85%. So, I am just trying to understand the headroom in terms of , you mentioned that you have focus on growth in the preferred channels A, which is more on the pricing, but immediately how can

we improve the occupancy or we are already operating at an optimal level of occupancy?

Abhay Soi : No, look I think that at certain places where you have absolutely hit the capacity , over there you will automatically start distilling. The levers that we have are the following : one is that at the very sort of end when you start pushing the envelope, there is some elasticity in terms of operations . We were earlier at 75% to 76% and now at 77% to 78%, but we also operate at 80% to 81%. So, each one of these facilities will have more efficient system of discharging and so on and so forth when it starts coming to the edge.

Second is, we have mentioned that about close to 100 odd beds internally that we are unlocking : 10 beds here, 20 beds here, 30 beds here, so on and so forth. 100 beds is about 3% of the total capacity. A couple of percentage points through occupancy and that gives you about 4 % to 5%. On top of that, we also have the payor mix distillation that we can do as and where necessary.

Third , finally – but what you want to do is first you want to maximize on the first two, then you want to move to the payor mix distillation and – then finally you have another 300 beds coming in. Then by next year, nearly every hospital will have its own beds coming.

Prakash Agarwal : And this CARE Hyderabad one is finally not happening right?

Abhay Soi : Well, I will not say that because we have sued for specific performance and the matter is in arbitration , it is sub-judice so I will not delve into it . But , we have asked for specific Performance over there .

Prakash Agarwal : Okay, because I read Blackstone is getting into that so I was just checking on that, okay. Lastly, on the regulatory side off late there has been news flows on having reservations for the government as well as the poor and the needy. I mean there is too much unverified information. So, is it happening somewhere ? Is it already happened? It had already happened, you know, in the past and it's just a rehash or if you could just --?

Abhay Soi : I think you rightly put it . Firstly, I think there is way too much being made out of something way too little. First and foremost, please understand that there is already a Right of Health across the country, through a Supreme Court direction . If a patient comes dying or a trauma patient comes into hospital you can not say show me your wallet first. You have to take him in and stabilize him . That is Right to Health even as per Supreme Court . No hospital would ever even think of not doing that . I mean, really, if somebody was to do that you will read about it in the papers . That is how rare it is. And you have seen those situations before in the papers, etc. No proper hospital will you know deny a patient.

Let us say somebody comes to a hospital , it is an accident patient and a number of these patients come year on year . If we were to actually reject that patient – firstly, it is against our own policy . It is against the Supreme Court strictures . More importantly , if you do something like that , the media will string you, it will hang you. It is inconceivable to do any of these things.

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Secondly, most states like Delhi NCR, Delhi , particularly, already for the last 10 years has the Delhi Arogya Kosh (DAK), which basically means that any accident victim comes over to your ER , DAK will pay you some money for it. Now the fact is in any case we were doing it free.

Now in Rajasthan, they pretty much rehash ed the same thing and they said no, no Right to Health. Right to Health in emergency , which already exists.

Prakash Agarwal : So, this is limited to emergency and not routine surgery etc.?

Abhay Soi : No, only emergency . Even in Rajasthan, it is purely emergency. In fact , they have diluted it down by saying it is not applicable to the private sector, it is only applicable to hosp

itals , which have been funded or subsidize d by the state government. That is the clarification given by the Rajasthan Government , which itself doesn' t make sense because it is not as if at a private hospital when somebody goes , then you are going to reject the patient . You would still do it free.

Prakash Agarwal : Just one last one on the trust hospitals we also do a set reservation for the poor and the needy. Is that right?

Abhay Soi : That is right. Every quarter when you see our results , you will see those numbers being announced .

Yogesh Sareen: So, not only trust hospital s but also some of the ho spitals where we have bought this land in auction, in the listed company , there we have some obligation to have 10% beds free and w e do provide for them.

Prakash Agarwal : Lastly, congratulations for Shal imar Bagh – turning it EBITDA positive in month one itself in March .

Abhay Soi : Yes, EBITDA positive in the first month and giving a 35 % to 40% EBITDA margin on the incremental revenue . This is something that we have always spoken about – Brownfield s do not suppress your EBITDA , in fact, they are accretive . We have always said that they will break even in the first quarter or two and obviously, we want to give you a more conservative estimate . Now you have seen the operating leverage com e in.

Moderator : Thank you. We have our next question from the line of Tushar Manu dhane from Motilal Oswal.

Tushar Manudhane : On PSU t ariff revision , how much of a spread would get reduced in terms of EBIT DA per bed with this price hike compared to say non -PSU or non -institutional patient EBITDA per bed?

Abhay Soi : It will actually rise per bed, because of the price hike.

Tushar Manudhane : No, the perspective here was that in the hospital s where we are already running up at a very high occupancy. That way we have a choice in terms of selecting the patient or distilling th e patient as you pointed in the earlier commentary. So, just trying to understand is this s ufficient enough to reduce that spread for EBITDA per bed through the patient payor mix?

Abhay Soi : No, it w ill not. But what happens is that if tomorrow I want to switch off 100 beds, yesterday I only needed to fill 40 out of those 100 PSU be ds to breakeven and to make back the money that I am losing by switching off those 100 beds . So, I would

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get 60 more beds to work with . Now that number from 40 has gone up to say 55, tomorrow that number will go up to 70 or 80. So what happens is that it becomes marginal , but does it get to the same amount? No. So yes, I think what you are going to see is that your trajectory sort of changes, but directionally you are going in the same dir ection . Albeit, you are doing it without impacting EBITDA , and well it's actually a more preferred approach since you are taking a fiscal call on the basis that it is goi ng to be more accretive with these new numbers rather than less.

Tushar Manudhane : And if you could also clarify this tariff, the proportion of PSU patient pool is higher in which hospital or rather which cluster of hospitals of Max healthcare?

Yogesh Sareen : We do not give hospital wise data. You have to understand that the hospital s, which are fuller , there the proportion would be lower and where it is less full, they will be having more --

Abhay Soi : For h igher occupancy , we have obviously moved away from th at right. The l ower occupancy places is, like Yogesh said, where you have higher amount of PSU. So, anywhere where your occupancy is getting higher , say to 80% to 85%, you start moving away from this business, you start distilling it over there . You do the business where you have idle beds.

Tushar Manudhane: Lastly on this Gurgaon thing is no more land coming up for auction.

Abhay Soi: No.

Tushar Manudhane: Because we have seen other hospitals also having some land pa rcel in this area.

So, is there more area available for hospital expansion as such or this is more or less done from the government side ?

Abhay Soi : Look, I think it is like Mumbai , right . I think you can have Central Bombay and you can have some place in Mulund . If you look at Gurgaon , the question is where in Gurgaon , Gurgaon extension , northern extension , etc., b ut where exactly. O n the main Golf Course Road , there is no land available.

Tushar Manudhane : So, the intention to understand was within that particular area where the hospital space is supposed to be sort of premium so in that space, th ere is no land parcel available?

Yogesh Sareen : Not that we know of , yes .

Moderator : Thank you. We have a next question from the line of Raj Rishi from DCPL.

Raj Rishi : Just wanted to find out if presently obviously the situation is great for you guys. How long do you think this supply won't come in looking at the present fantastic scenario? Like, right now the demand supply gap is huge, which is why your occupancy is what it is. So, whenever the situation is so robust, obviously new supply is going to come in – in various geographies.

Abhay Soi : Not necessarily , I mean no new hospital has come up in Mumbai in 20 years, because there is no land available. No new hospitals have come up in Delhi for 12 years, maybe one, because no land is available. How do you get land in these metros and land at viable cost, right ? If you want to buy land, you have to understand there is a height limit of 45 meters when you set up a hospital. If you want to set up a 400 - bed hospital, you need 4 to 5 acres of land, contiguous land. Where do you get it in Delhi and Mumbai ?

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Raj Rishi : That's right.

Abhay Soi : One of the biggest plus points that we have is that we have this land in our network, in our system.

Raj Rishi : And Abhay you had mentioned medical tourism going up many times by 2030. So, how do you see it, like what is the size presently and where do you see it in 2030 for the sector I am asking?

Abhay Soi : You know, if you are saying 2030, there is such a massive comparative advantage that we have in India. I mean we are less than 5% of the cost of the U.S. We are less than 30% of Singapore. We are less than half of Thailand. I mean, look, it is not like rich people from the U.S. will start coming to India. But in my case that's the largest part of the world pyramid , where the bottom is all self-paid. And for them, affordability is an issue. This has the potential of surpassing the IT sector. That is the amount of comparative advantage that we have and no other country has such advantage . I think because the comparative advantage that you hold in this space , no other country even comes close. While in IT other countries may come close, here there is nothing. The sheer volumes and complexities our doctors handle , what they do in a year, the Western counterparts do not do in a decade. Our skill sets are right up there and our cost is fraction of the global cost . Aging population, rising costs, most government treasuries going bankrupt , this is going to be something else. This is not even the tip of the iceberg right now.

Raj Rishi : And Abhay another thing, just a thought , like generally when private equity guys are so heavy on sector it seems that it's sort of topping out when smart guys like Emami or Manipal dilute and private equity guys buy. It's just a thought, I mean what are your comments on that?

Abhay Soi : No, I do not disagree , I think you had hospitals and hospital chains change hands many times. For Manipal, I think this is the 6th or 7th private equity round that they have done in Manipal Hospitals over the last 10 -12 years. If I look at, it is not as if Max was not up for sale, we bought Max. It is not as if other hospitals were not up for sale. Those transactions also happened in the

past.

I do not think it is a question of tapping out. I think there is very clearly a multi -decadal opportunity to invest in this sector in India, which is much more true today , but it's not something which was lost in the past either. The only difference was the biggest critique and I remember when we were listing the company in 2020 , now most of you will bear this out, right. The biggest critique of this sector was that the Indian Healthcare sector is a great investment but it is a mirage because there are no free cash flows. How do you invest? You have to keep raising money and keep reinvesting.

We have Rs. 1,281 crore free cash flows coming with Rs. 1,636 crore EBITDA. That is almost 80% throughput of the free cash flows. And you are seeing this , we are not raising any capital. My entire 2,900 beds is at a cost of Rs. 4,500 crore and that should not even be 50% of my free cash flows over next 4-5 years. And that's on a debt free balance sheet. Today this is the compounding. There is a massive multi -decadal opportunity to invest , I think, and to create infrastructure, so PE and all sorts of funds are going to be coming in.

Raj Rishi : Just one last question , how do you see this asset -light model? How heavy can it be for you guys ?

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Abhay Soi : Sorry, just understand one thing . A PE puts in money at valuations and they need to underwrite 20% to 25% IRR in dollar terms. So, only because they are seeing this opportunity and I believe Manipal for example was probably done at 29 to 30 times

2024 EBITDA. Those guys are expecting to do 2 5% CAGR from there on dollar. On the contrary , I think when PE s come in they see opportunity.

Raj Rishi : One last question about the asset -light model, how heavy can it be for you guys ? Like are you thinking of increasing that part of the business?

Abhay Soi: Well, I think quite heavy from that standpoint. I think over the next couple of quarters you will be coming across some announcements from us on this front. This is really the way for us to grow particularly where we are looking at any form of Greenfi elds. We do not like to build on our own . We like to collaborate because it decreases the construction and development risk , particularly in geographies that we are not currently present in. So, I think you are going to see some of those come through now, because that's going to be a big focus area for us. We have a 36% cash -on-cash ROCE , so we can afford 8% yield to developers.

Moderator : Thank you. We have a next question from the line of Neha Manpuria from Bank of America.

Neha Manpuria : Abhay, you m entioned improvement in specialty mix quite a few times. And if I were to look at the chart maybe through the profil e obviously there is improvement in oncology, a little bit in Neuro, Cardiac. So, are the beds that we are adding essentially focusing on do ing more higher ARPOB work like adding more Onco beds etc. or are we seeing the kind of work that we do within Cardiac, within Neuro itself increase that is improving this specia lty mix. So, how do I differentiate between the two?

Abhay Soi : I think it is a combination of the two. First and foremost, if you look at capacity spread and you look at overall ARPOB , the ARPOB that you see of Rs. 70,700 is a summation of ARPOBs. When you see our occupancy is 77% , it is a weighted average. It means there may be some hospital , which is operating at 70% occupancy. There may be another one , which is operating at 90% occupancy. Similarly, in ARPOB you will have certain departments that are low ARPOB. You will have a certain payor mix that is lower ARPOB. Yet, you are hitting the capacity thresholds. Firstly, if you are hitting capacity thresholds, then you are going to look at expanding over there. What you would seek is that if today I am creating more capacity and I was to take my lowest pay or mix and my lowest ARPOB sp

ecialty

there , that is really what I am creating it for. Any expansion that you are doing is serving the lowest common factor because that is what you are doing it fo r. Otherwise, if you were not to do that br ownfield field expansion , what you are going to do is you are going to distil your pay or mix and you are going to distil your clinical mix.

Neha Manpuria : But it could also be I am asking this one is let's say I am adding more Onco beds or I am adding more OT or ICU beds because of the improvement in the clinical --

Abhay Soi : What is an Onco bed? You know a lot of our beds are fungible , except for some daycare beds etc., but the beds are all similar. I mean in Cardiac , for example, there may be a Cath lab, but the OTs are similar. It's sort of quite fungible within the hospital , you have to understand that . We may segregate it and we may do some clinical program s there. B ut the way I will value it , please understand this , is that say I have 100 beds , of which say 50% beds are very high ARPOB and 50% beds are low ARPOB , and they are fully occupied . Then when I look at creating another 50 beds , the way I will calculate it is to take my lowest ARPOB of 50 beds and to put it

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over there. Alternatively, my opportunity is to just pla y the lower ARPOB out and occupy my current facility.

Neha Manpuria : So, essentially that would mean you either add a new specialty or add doctors , which will allow you to remove that lower ARPOB work, right?

Abhay Soi : No, even if you don' do that and a lot of that stuff could be aspirational, right. Today when you have a waiting list of two or three days in your ER for patients, you know there is unsatiated demand at your doorstep. I mean take Shalimar Bagh 's case -in-point . We put up a n Onco block over there , so what happens immediately is that the oncology patients from our current facility or the pre -existing facility move into the new Onco block. So, what happens to the old block? Our high ARPOB business may have gone out but the lower ARPOB business has started climbing up over there. When I evaluate setting up 100 beds in the new block , I have to evaluate vis -à-vis the low ARPOB business and not the high ARPOB business , even though I may be doing it for the high ARPOB business . Yet what you see is that even without adding much doctors, etc. , within the first month we broke even and hit a 40% EBITDA margin. We have overall an occupancy , now this is what matters , of this incremental facility as well as the existing facility , of the entire hospital of 83%. I mean, I am not saying it is 83% of the new 100 beds, I am saying it is 83% of 300 beds.

Neha Manpuria : The reason why I am asking this is for example we added the new Onco block, so that's not necessarily increasing my entire Onco contribution quarter -on-quarter. It could just be moving patients to the new block to unlock sort of free capacity to move on.

Abhay Soi : There is some increase, but really it is not a jump increase that happens because you have opened a new block. Of course, the new Onco block will have a disproportionate increase in Oncology, but it is not as if other services are not increasing.

Let me put it this way, let's say a hospital of mine has 50% PSU business and 50% other business and is running at full capacity. When the question comes about expanding over there, when a note is put up to expand, my first question is whom are you expanding for? Are you expanding for PSU? Because alternatively you could be distilling the PSU. So, for that expansion, whatever specialty that goes there is irrelevant. You obviously put up the specialty with the highest ARPOB, which will be more accretive etc., but it has to be evaluated looking at the lowest opportunity.

Yogesh Sareen: Neha, the hospitals are passed on both fronts: one is that within this specialty they have to work on

increasing the quantum of procedures, which are high teen procedures. Then they are also supposed to allocate more beds and fill up more beds for high ARPOB specialty. So both things play into it and that's what the hospital's job is.

Neha Manpuria : And is there any way for us to sort of segregate as to how much of our ARPOB increases come, let's say from an improvement in specialty --?

Yogesh Sareen : So, I can tell you that say the Quarter 4 increase is 5%. Around 2% of this is because of the procedures within the specialty, which means that with the same number of beds for each specialty, the specialty ARPOB has gone up. Around 2% of this is because of relative change in the Occupied Bed Days (OBDs) to the specialty. The Occupied Bed Days of Oncology and Cardiology, etc. have gone up, and they are high ARPOB specialty. The mix has changed of the beds - that is 2% impact and 2%

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impact is that within the specialty the ARPOB has gone up. I am talking about overall increase.

Neha Manpuria : The entire quarter -on-quarter increase.

Abhay Soi : Yes, I am saying 5% increase, 2% is from here, 2% from there.

Moderator : Thank you. We have a next question from the line of Amit Thawani, an individual investor.

Amit Thawani : My first question is can you tell me what the price increase that we have got in by the PSUs?

Yogesh Sareen : So, Amit there are four elements where the prices have increased : one is on the ICU beds charge ; second is the normal ward bed charge ; third is the I P consult s and fourth is the OPD consult s. OPD consult has gone up from Rs.150 to Rs.350, IPD from Rs.300 to Rs.350. These are the prices change s. So, overall average if I take , it's a 70% price hike we have got. This is after nine years; the last one was in 2014 . Now the point is, this is only a small quantum of the line items that get billed to the patients in the PSU. So, we do expect that there will be further price increases by the government because if the room rent prices increase, that means that the room rent, which is part of packages will also go up, right . Let us say cardiac surgery, there is 12 days stay and there is a package charge that we charge to the PSU patient, that should also go up. The CGHS authorities have put up a note to Finance Ministry, etc. so we expect that there should be a decision on that by July.

Amit Thawani : So, just trying to understand the institutional business is 20% of our sales. So, how much of that could

Abhay Soi : In bed share, understand that bed share is 29% and revenue is 17% . Now for 17% of revenue, 12% of that 17% will go up by 70% . That is a net impact of 4% to 5% on the institutional part.

Amit Thawani : My second question is the digital revenue is about 18% of our overall sales. Just trying to get a sense of what the margins on that digital revenue are. Is it higher than the overall margin of the company or is it lower?

Abhay Soi : No, it is just a channel. People used to call up earlier. People would come to hospital as walk-ins. Then they used to call up call centers. Now they will do it through digital platform . They are booking appointments, so your margin does not change. You can go into customer acquisition costs, advertising, etc. but that is meaningless here.

Amit Thawani : So, the press release said digital revenue from online marketing activities.

Abhay Soi : That is right, digital revenues means it is digital booking. If you book a liver transplant through our website, then that is digital revenue.

Yogesh Sareen : If you come on the website , we track the patient footfalls on the website and then we see conversion from there. So, if there is conversion from the website that is accounted as digital revenue.

Amit Thawani : My last question is on the contribution of ALOS to ARPOB, we saw the ALOS go up this quarter.

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Abhay Soi : Look,

ALOS is meaningless, right . A hospital that does more complex surgeries will have a longer ALOS. If I do a bone marrow transplant, the ALOS is 20 days. If I have a liver transplant , and this is the business you want actually , the ALOS is longer than doing non-surgical work. When you do more surgeries, you have more ALOS, but all of it plays out in ARPOB. Eventually what is it that you have , you have inventory and you want to earn the maximum in a day from there. So, it is all based on the ARPOB . No point having a very short ALOS and having a lower ARPOB . You would rather have a high ARPOB. You have beds into days in a year , so what you want is higher amount of revenue from each of the days.

Moderator: Thank you. We have a next question from the line of Alankar Garude from Kotak.

Alankar Garude : Abhay assuming the CARE acquisition does not go through, can you help us understand whether there are any meaningful acquisition opportunities of hospital chains left in India which fit your criteria?

Abhay Soi : Yes, plenty.

Alankar Garude : And we do not have any preference on geography, right. It is just the criteria , which you have of two players being successful and you preferring a cluster approach.

Abhay Soi : That is right. I mean our approach is a little more less casual than just that. We have worked out a list over 20 -21 cities , but that is an important criteria. What we find is that at the umbrella level , these are the places that typically will have the demand, supply, doctors , etc. and so we will be comfortable over there. I am putting it very simplistically by saying that at least two or three of my competitors have proven viability . In each one of these cases , this is what I find common. You read the profile s that we want to take . We are not pioneers; we do not like to go to uncharted territories.

Alankar Garude: And the other thing is, would it be fair to assume that most of these would be standalone assets now or if at all just two, three hospitals per chain or there are still say chains operating five to 10 facilities across India, which are on the block right now.

Abhay Soi : I think there are plenty of conversations happening and you are going to see consolidation. Firstly, consolidation happens because there is a requirement for those conversations , right. I mean in the case of Max Healthcare, you have seen Rs. 1,636 crore of EBITDA translating into Rs. 1,281 crore of free cash flows. Now typically for some companies and most mid -sized players that you will see , will have a Rs. 500 crore to Rs. 600 crore EBITDA and their EBITDA translates to a Rs. 200 crore to Rs. 300 crore free cash flow at best. This is pre-Ind AS, post maintenance CAPEX, post any increase in working capital and taxes. Now this is a capital -intensive sector , so if you have Rs . 200 crore to Rs. 250 crore, you can build one 500-bed hospital every four years . That is all you can do, one hospital every four years. If you are a listed company or unlisted, where do you go from there?

Alankar Garude: So, my question was more from some of our peers especially the unlisted peer s being quite aggressive on the M&A front and also private equity interest in the space continues to remain quite high.

Abhay Soi : Look , I will tell you something that there always be some people trying to outbid you . What is it that we look at? Our criteria is very simple , I want to look at a 20% ROCE three or four years down the line or four or five years down the line. If we have a clear line of sight and visibility of that ROCE, we will come in. Now a PE fund . Typically , will be getting excited at the multiples that they are paying . But our ability

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to underwrite an EBITDA or a business case, four -five years down the line is much more pronounced than a PE. I mean, for us to underwrite , let us say, in situations where we believe where 2x o

r 3x or 3.5x of EBITDA, is very possible three or four years down the line. For a PE to underwrite that gets very difficult . So with the amount we are willing to pay at the outset is a derivative of that , of where I believe I can take the business four years down the line. Do keep in mind, our EBITDA per bed is the highest in the industry , you know that , by 50% to 55%. A PE cannot underwrite my

case. He has to underwrite the industry case.

Alankar Garude: And the second one is more of a quick clarification. So, is it now fair to assume that we may not reach that 15% institutional volume mix number by March '24? But given that we are operating at high occupancies, the institutional volume mix will still gradually keep on coming down.

Abhay Soi : No, it will come down. We have done a bit of a pause , like I said , because we are expecting some increment, although it is still coming down. We have also found more capacity. We have more elasticity in operations. The price is moving up. It is not going to reach 15% immediately but your trend is going to keep reducing over there because eventually, it is finite, right . I can suffer that higher price for some time, but it is still not the same as my CTI business.

Moderator: Thank you. Ladies and gentlemen, that was the last question for today. I would now like to hand the conference over to management for closing comments. Over to you, Sir.

Abhay Soi : Thank you so much for being on the call. I think this has been a seminal year for us on many counts and we are happy to have most of you as our investors and the rest of you, we look forward to them joining our cap table. So, thank you very much.

Disclaimer: This is a transcription and may contain transcription errors. The transcript has been edited for clarity. The Company takes no responsibility for such errors, although an effort has been made to ensure a high level of accuracy.

Call: Aug 2023

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August 14, 2023

Listing Department ,
National Stock Exchange of India Limited
Exchange Plaza, Plot C -1, Block G,
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Mumbai – 400 051

Symbol: MAXHEALTH Listing Department,
BSE Limited
Phiroze Jeejeebhoy Towers,
Dalal Street,
Mumbai – 400 001

Scrip Code: 543220

Sub.: Transcript of Earnings Call held on August 8, 2023

Ref.: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements)
Regulations, 2015

Dear Sir / Madam,

Please find enclosed copy of transcript of earnings call , organized on August 8, 2023, on financial results of the Company for the quarter ended June 30 , 2023.

The said transcript is also available on the website of the Company at
www.maxhealthcare.in/investors/investor-resources .

Kindly take the same on record.

Thanking you

Yours truly,
For Max Healthcare Institute Limited

Dhiraj Aroraa
SVP - Company Secretary and Compliance Officer

Encl.: As above

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Max Healthcare Institute Limited
Q1 FY24 Earnings Conference Call Transcript
August 08 , 2023

Moderator Ladies and gentlemen , good day and welcome to the Max Healthcare Institute Limited's Earnings Conference Call .

Please note that this conference is being recorded.

I now hand the conference over to Mr. Anoop Poojari from CDR India. Thank you, and over to you sir.

Anoop Poojari : Good morning everyone and thank you for joining us on Max Healthcare's Q1 FY'24 Earnings Conference Call.

We have with us Mr. Abhay Soi – Chairman and Managing Director; Mr. Yogesh Sareen – Senior Director and Chief Financial Officer; and Mr. Keshav Gupta – Senior Director Growth, M&A and Business Planning of the Company.

We will begin the call with opening remarks from the Management , following which we will have the forum open for an interactive question and answer session.

Before we start, I would like to point out that some statements made in today's call may be forward-looking in nature and a disclaimer to this effect has been included in the earnings presentation shared with you earlier.

I would now like to invite Abhay to make his opening remarks.

Abhay Soi : A very good morning to everyone. I am pleased to welcome you again to Max Healthcare 's First Quarter Earnings Call for fiscal 2024.

Before we delve into the Quarter 1 highlights , I am happy to share that this is our 11th consecutive quarter of year -on-year growth in revenue and operating EBITDA.

Second, we have added 44 beds to our capacity through internal reconfiguration during this quarter. This is in addition to the 92 -bed oncology block we commissioned at Shalimar Bagh in March this year. Notably, Max Shalimar Bagh hospital has reported an average occupancy of 77% in Q1, with a year-on-year growth in revenue and EBITDA of 37% and 43% , respectively.

Third, we have released the in-patient module on our proprietary app, 'Max MyHealth', which allows IPD patients to track the entire process right from the admission to their discharge. It also provides them access to the reports and up-to-

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date billing data , with the ability to make payments online. This will not only enhance patient satisfaction on one hand , but also reduce administrative burden on the other.

Highlights of our Q1 performance

1) Occupied Bed Days (OBDs) went up by 3% year -on-year and the average occupancy for the quarter stood at 74% , do keep in mind this is on a higher capacity, remaining flat compared to Q1 last year. This is on expected lines coming off from seasonally the best quarter to the worst quarter. With increase in occupied bed days (OBDs) and marginal drop in ALOS, the admissions were up by 4%.

2) Institutional bed share fell to 29.7% compared to 30.3% in Q1 last year.

However, it is pertinent to note that excluding Max Shalimar Bagh , where we strategically decided not to optimize pay or mix at the cost of occupancy in light of the newly added 122 beds on a base of 280 beds , the share dropped by 270 basis points year -on-year to 27.4% of institutional business on overall basis. Moreover, as you may be aware , CGHS has revised its tariff in April and June this year for certain segments after a gap of nine years. And we expect further revisions for the balance segments in the coming quarters. Therefore, we have not taken any aggressive calls on disempanelments .

3) The average revenue per occupied bed (ARPOB) for the quarter touched a new high of Rs. 74,800 , reflecting a growth of 13% year -on-year and 6% quarter -on-quarter. This was mainly led by improvement in specialty mix, particularly Oncology, Orthopedics, Cardiac Sciences, and doubling of robotic procedure volumes. This was further aided by price revisions across almost all channels, which usually happens on the 1st of April.

4) Network gross revenue was Rs. 1,719 crore compared to Rs. 1,473 crore in Q1 last year and Rs. 1,637 crore in the previous quarter . This reflects a growth of 17% year -on-year and 5% quarter -on-quarter. The year -on-year increase was largely driven by growth in ARPOB and occupied bed days (OBDs) .

5) Revenue from international patients grew by 31% year -on-year and 3% quarter -on-quarter. This now accounts for around 9% of the revenues from our hospitals.

6) Digital revenue grew to Rs. 356 crore and accounted for 21% of our overall revenue.

7) Direct costs were up year -on-year, due to growth in surgical mix, medical oncology and doubling of robotic procedure volumes. On the indirect costs side,

while the overall percentage is lower, there is an increase in absolute costs due to commissioning of 4% additional capacity, creation of around 10 new operation theatres (OTs) and 80 ICU beds by cannibalization of existing ward beds over the last 12 months. In addition, we have strengthened our projects, digital and homecare teams, while also increasing efforts on ESG activities and marketing for the international channel.

8) Network operating EBITDA stood at Rs. 436 crore, reflecting a growth of 18% year-on-year, while remaining relatively flat quarter-on-quarter, in spite of the fact that we are coming off perhaps Quarter 4, which is the best quarter, onto Quarter 1, which is usually the worst quarter.

9) The operating margin stood at 26.8% versus 26.5% in Q1 last year and 28.2% in the previous quarter. This is largely because we have been focusing on higher

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payor mix, which is international patients, higher clinical mix which is Oncology, Cardiac Sciences and robotics, etc., which in percentage terms may provide lower margins, but in absolute terms provides higher value.

10) Most importantly, annualized EBITDA per bed, rose to Rs. 70.4 lakhs, yet again our highest ever, clocking a growth of 14% year-on-year and a nominal growth quarter-on-quarter.

11) Profit after Tax was Rs. 291 crore versus Rs. 229 crore in Q1 last year and Rs. 320 crore in the previous quarter. The year-on-year growth

of 27% was primarily attributable to the flow through of improved EBITDA.

12) Free cash flow from operations stood at Rs. 261 crore, of which Rs. 38 crore was deployed towards the ongoing capacity expansion projects. Net cash position improved to Rs. 957 crore at the end of June 2023 compared to net debt of Rs. 217 crore same time last year. Expediting our routine CAPEX, coupled with buildup of account receivables, led to higher working capital during the quarter.

13) Continuing our efforts to give back to the community, we treated approximately 37,500 OPD and 1,260 in-patients from economically weaker sections free of charge.

14) Both our strategic business units (SBUs) continue to report robust performance:

■ Max@Home reported a top line of Rs. 40 crore, reflecting a growth of 24% year-on-year and 7% quarter-on-quarter, with 'Critical Care' and 'Medical Rooms' service lines being the major contributors to this growth.

■ MaxLab expanded its geographic footprint to 36 cities and reported a gross revenue of Rs. 34 crore. This reflected like-to-like growth of 39% year-on-year and 10% quarter-on-quarter.

15) During the months of June and July, there was a slowdown in the projects activity due to heavy rain in Mumbai, Punjab and Delhi NCR. The current status of our expansion projects coming on stream over the next three years is as follows:

■ For 300 beds at Dwarka – The majority of the MEP and interior work are complete, while application for power, water and sewage connections are in progress. The developer is expected to apply for occupancy certificate (OC) in the latter half of this quarter. We expect to commission the hospital in Q3, subject to the developer obtaining the OC.

■ For 329 beds at Nanavati Hospital in Phase I – Excavation and Raft work are nearly complete. Steel fabrication and wall casting for LINAC and Brachytherapy are in progress. The project is largely on time and we expect to cast the ground floor slab by the end of October.

■ For 300 beds at Sector 56 Gurgaon in Phase I – 50% of site excavation is complete and the EPC contract is in the final stages of execution. All statutory approvals have been received.

■ For 350 beds at Max Smart in Phase I – As stated during the Q4 update in

May this year, this project was facing delays due to forest approval for tree transplantation. Happy to report that the final forest approval has now been received and we are initiating the process of transplanting the trees shortly after subsiding of rains.

■ For 300 beds at Vikrant (Saket) – Environmental clearance has been approved and is under processing with the State-level Environment Impact Assessment Authority (SEIAA). DDA has reconstituted our file and we plan to formally upload the drawings for approval in September. So, this should be well on its way as well.

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■ For 190 beds at Mohali – Demolition of existing building and shifting of services have been done. D-wall is completed and excavation work is underway. EPC contract is in the final stage of negotiations. The project is largely on time.

16) Lastly, I would just like to state the three months already into this fiscal, we continue to strive to improve operational metrics across all our current facilities and businesses, keenly monitor the execution of all our projects underway, while also prudently evaluating inorganic opportunities in our existing geographies as well as new, promising geographies.

With this I would like to open the floor for Q&A.

Moderator: Thank you very much. The first question is from the line of Tushar Manudhane from Motilal Oswal Financial Services.

Tushar Manudhane: Just on the kind of growth what we have seen on ARPOBs and how sustainable that is let's say for next 12 to 24 months? And secondly, while the ARPOB growth is there, the EBITDA per bed has been pretty stable may

be on a quarter-on-quarter

basis. So, if you could share your comments on that?

Abhay Soi: We will continue to keep doing what we are doing – focusing on higher end specialities and better payor mix, etc., so your ARPOB growth should continue. And when it comes to EBITDA per bed being almost flat quarter-on-quarter, like I said Q4 is the most solid quarter in the year. It's the strongest quarter and Q1 is the weakest quarter. I am very encouraged by the fact that in spite of this our Q1 results are flat with our Q4 because if you look at it historically, and I am taking away that one COVID year, Q1 is usually lower than Q4 because on April 1 you have increase in costs, particularly your salary costs, for the whole year. Therefore, Quarter 1 normally is seasonally the weakest quarter with high indirect costs, and thus has lower absolute EBITDA and margins emanating from it. But that is not the case this year. It has been flat compared to Q4, so it's actually a very encouraging sign.

Tushar Manudhane: So, what kind of growth one can think of on a sustainable basis given the kind of bed additions that are happening over next 12 to 24 months, on EBITDA per bed?

Abhay Soi: I am not going to give you any guidance on the exact EBITDA per bed, but I think this is our 11th quarter of year-on-year growth like I mentioned. Also, the fact that recently when we opened the Max Shalimar Bagh, i.e. 122 beds on a base of 280 beds, you can see that there is 77% overall occupancy within the first month or two itself. And it is giving us a revenue and EBITDA growth of 37% and 43%, respectively. That should hopefully continue into the future as well and it should be positive from all of these standpoints.

Tushar Manudhane: And just while you have already highlighted hospital wise the kind of work underway in terms of bed expansion, but just on the CAPEX number if you could also sum up for FY'24 and for the quarter?

Abhay Soi: We are looking at around Rs. 900 crore.

Yogesh Sareen: Tushar, the cash outflow expected is around Rs. 900 crore. That's obviously subject to change and we are just trying to give you a block number. But the outflow could shift from one month to another.

Moderator: The next question is from the line of Damayanti Kerai from HSBC.

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Damayanti Kerai: My first question is on your Shalimar Bagh, clarity on your comment that you decided not to optimize payor mix. So, does it mean like you have taken up more institutional patients there?

Abhay Soi: Yes, basically we do not disempanel over there and we don't filter business because the extra capacity is, firstly, at a significantly lower cost due to operating leverage. So, even at the institutional rates, we still generate EBITDA, as we have already seen. But the first thing to do is that you bring in the occupancy and then start filtering

down at a later stage.

Damayanti Kerai : Okay, so institutional bed share at around 29.7% for first quarter. So, earlier you guided like by end of '24 or so, you will bring it down to 15% to 16%, does it still hold or you have some different thoughts now?

Abhay Soi : Excluding Shalimar Bagh , it has come down to 27.4% and clearly we have had some increases in pricing for institutional businesses recently . Although, it doesn't have a big impact on ARPOB, it's about 0.6% to 0.7% so far. But we are expecting further revisions of balance rates in the next month or two as far as institutional is concerned. That makes us go slow on the churn , but again the gap is still too much between institutional and cash . It just pushes the can down by a quarter or two , not more than that. We are still going to face capacity constraints and going to have to distill because our CTI business on the preferred channels still need those beds.

Damayanti Kerai : So, you said like gap between CGHS state and CTI patient reduces so you might not

be chasing that 15% to 16% bed share very strictly, right?

Abhay Soi : We are not per se chasing a 15% to 16% bed share . Let me put it in perspective.

There is a certain rate at which our CTI business is growing. If the new capacity doesn't come in , we have no place to accommodate the growing CTI business. The best case for us is if we can keep the institutional business and we can accommodate the increased CTI business. As and when we can't , we will distill that capacity and bring institutional down. Today , let's say if I had a magic wand and we could put up these extra beds to accommodate the increase in CTI, we would not let the institutional business go either. We are having to do that so the trajectory that we have set, it will come down to about 15% by that particular time . It may go and up down a couple of quarters because of these changes, but that's about it. But that's still our trajectory . Let's put it this way, if I can today acquire big unoccupied capacities , theoretically , adjacent to my hospital s, then I will do both the businesses.

Damayanti : My second question is on EBITDA , so healthy ARPOB but occupancy down sequentially and then EBITDA down a round 260 basis point quarter -on-quarter. So, does it mean like you did more high-ticket surgeries where ARPOB is high but there you might be paying higher like higher payout to doctors , etc., like higher cost and that's why your margins declined sequentially, it should be read in that way?

Abhay Soi : You need to complete that story . A higher end surgery , let's say you do a LVA T surgery , it costs you Rs. 85 lakhs . The implant itself will cost you about Rs. 56 lakhs.

Now a patient stays in hospital for a few days , say 4-5 days , the hospital will make about Rs. 15 lakhs . The percentage margin is much lower. When you do robotics, higher end Oncology, Cardiac Sciences, Orthopedics , or any of the higher end surgeries , the percentage margins are lower. But the absolute bill is higher and in spite of the lower percentage margins, the absolute profitability is higher per patient and per bed.

When you do international business, the international people come for more severe diseases to India and so the billing is higher. ARPOB for the patient is higher. The

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percentage revenue is lower , but the absolute EBITDA per bed is higher. So, I have always said, I would rather do a \$10,000 surgery with a 20% margin than do a \$2,000 surgery with a 50% margin. As you move up the payor mix and you move up the clinical mix, you are going to see margins , in percentage terms , come down but in absolute terms ARPOB as well as absolute EBITDA per bed will move up. And that is what you have seen.

So, our overall EBITDA , in spite of a lower occupancy and cost increasing on 1st of April, has been flat over Q4. And in spite of Q4 being the strongest quarter and Q1 the weakest quarter . I mean that's a big positive isn't it.

Damayanti Kerai : My last question is on your utilized bed so as per your presentation in last quarter we have 2,523 utilized beds of the total operating beds and this quarter it came down to 2,474 . So, how should we read into it?

Yogesh Sareen : As we said that generally Quarter 1 is a weak quarter on occupancies . You will see the same trend even last year . Last year also the occupancy was 74% in Quarter 1, 77% in Quarter 4. This is the seasonality in the business . During the Quarter 1 when you have summers set in, doctors do go on leave . Generally, occupancy in the months of May and June come down.

Abhay Soi: Not just us, for any other hospital or hospital group , you will always see that Quarter 1 is always lower on occupancy, EBITDA and everything else compared to Quarter 4.

Yogesh Sareen : But, at the same time , even if the occupancy is same, the OBDs have gone up . If we were not to add beds in Shalimar Bagh and other places

, then the occupancy would

have been 2% higher. So, you have to also consider and that factor into account .

Abhay Soi : Our occupied bed days have moved up.

Moderator : The next question is from the line of Nikhil Mathur from HDFC Mutual Fund.

Nikhil Mathur : Just one question I have. Now you have reported pretty strong occupancy on the incremental beds in Shalimar Bagh. We are able to track numbers for one of your peer Fortis as well as in Shalimar Bagh they have reported decent numbers on a Q-o-Q basis, despite 1Q basically is generally weak. So, it kind of seems that in that particular micro market the demand is very strong. Would you say that this is reflective of many of your other markets as well where any concerns, I mean there is some bit of divided opinion whether the demand factor is likely to be weak or not in this particular year. Would you say that a similar trend is kind of playing out in other micro market as well especially in which you are present?

Abhay Soi : I don't see this playing out anywhere , in fact for anybody . I have read other people's numbers and our numbers, year-on-year we have an increase in occupied bed days .

If our total capacity goes up by let's say 4-5% and our occupied bed days go up by 3-4%, while our percentage occupancy remains the same, it is bound to happen .

When you increase capacity , in percentage terms your occupancy may be flat, but in the actual terms your occupancy is more.

Yogesh Sareen : Also, I think we have not seen any dwindling of demand in any of the hospitals. If that's what you are looking at, then there is no impact on demand . While in June quarter, the occupancy is generally lower , but we have seen that occupancy coming back in this quarter.

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Abhay Soi : I don't cater to this point at all . You have to look at the business year -on-year rather than quarter -on-quarter. Unless you have a fourth quarter , which is supposed to be a good quarter and is running below the third quarter or something like that , then I will be worried.

Nikhil Mathur : No, I am looking at it on a year-on-year basis only, but I mean in this particular micro market the numbers are looking strong on Q -on-Q basis as well, so that's why this question I had. But I mean I completely understand what your thoughts are on the demand and how its panning out.

Moderator : We have the next question from the line of Lavanya Tottala from UBS.

Lavanya Tottala : I just have one question, so I understood that there is an increase in employee cost , but if I look at Max consolidated level, there is also increase in raw material as percentage of sales. So, is there any increase in the pharma medicines that you are seeing ?

Abhay Soi : As far as the pharma cost is concerned , there is an increase because , like I mentioned, we are doing more higher -end work . Like I said earlier, we doubled your robotic procedures ; when you do all of these the consumables are more expensive in that and is higher as a percentage of the procedure. But the overall EBITDA per procedure, overall value that we get out of the billing is higher. So, in terms of percentage it is lower, but you will see that the ARPOB is higher . Cost of goods goes up, but your absolute EBITDA per bed and EBITDA also moves up. So, that is what we have seen .

Your second question was with respect to the employee cost. The employee cost moves up by usually about 6.5 -7% per year and it happens on April 1st. This year you will see a larger year -on-year increase because we also have 144 new beds that we put up, we have created 80 more ICU beds and we have created 10 more OTs. When we are doing that , the manning of all of these is more . Theoretically if you double capacity , then you will almost be doubling your employee strength, and therefore your employee cost. But then you have to have a resultant increase in revenue. So, if you look at i

ts percentage of revenue , there is not much of a change there.

When you look at it in absolute terms, it is bound to increase because we are putting up more capacity. If we put 4 -5% more capacity, that itself should increase our employee cost by about at least a few percentage points. And then you add a few percentage points of higher ICU beds . A regular ward , for example , requires 5 :1 nursing, while ICU requires 1:1 nursing. It's almost five times the amount of people that are required for ICU versus ward . But it generates you more revenue as well. So, you want to do all of those things.

Moderator : The next question is from the line of Kunal Dhamesha from Macquarie.

Kunal Dhamesha : I think there is some confusion regarding the seasonality at least on my end. So, I think there is seasonality with specialty mix that plays out in different quarters and then there is what you cited as in terms of volume also kind of there is some seasonality and that impacts our revenue on a quarter -on-quarter, year -on-year

basis. So, could you help us explain how this seasonality because as far as -- Quarter 2 is more like internal medicine patient because of infectious season in the country, right. So, how that impacts our revenue and EBITDA on a quarter, let's say starting from Quarter 1 to Quarter 4, how you see specialty seasonality and then how it impacts revenue and EBITDA?

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Abhay Soi : Orthopedics happen less in winter, Cardiac Sciences happen s more in winter . More insured patients come in for annual checkups, etc. in Q4 because it is lapsing and so on . For a ny hospital or hospital chain over the last 10 years, whether it's a mature hospital or otherwise , when you speak to their management or yo u look at the ir history, Q4 is the strongest, while Q1 is the weakest as far as occupanc y is concerned . As far as Q2 is concerned, occupancy is higher , like you rightly said , because of dengue and more internal medicine admissions, etc. So, you have the highest occupancy in Q2 , but you have lower ARPOBs in Q2 . That's a very standard thing .

Yogesh Sareen : Our Q1 is also down because doctors were on leave.

Abhay Soi : They go on summer holidays; they postpone their surgeries , their children 's schools are on holidays. In fact, people postpone their own procedures because its examination time for kids , etc. All of these things matter and not only this year, but they have been a factor every year since I have been in healthcare or even much before .

Kunal Dhamesha : And let's say to that extent the trends in July and August how are those panning out, because we are anyway one and half months into the next quarter, right.

Abhay Soi : I am not going to give you any forward -looking guidance , but it's secular.

Kunal Dhamesha : And secondly on the 13% ARPOB growth I am not sure if I have missed your initial comment , because I joined a little late. But on the 13% year -on-year growth in ARPOB, if you can highlight the major moving pieces , because we have quite a few right CGHS rates revision, international --

Abhay Soi : Revision in CGHS rates ha ve not impacted ARPOB by more than 0.6 -0.7%. Let's look at year -on-year to start with. Our international business has grown by 31%, our robotics ha ve doubled , our Oncology business is growing at a much faster pace and so is our Cardiac Sciences, Orthopedic s. These are all high end specialties . When you have that, your ARPOB increases . There are less medical admissions during this time , so that also kind of helps you r overall ARPOB to increas e.

The best part is that at least I can't remember the last time Q1 was almost equal to Q4 as far as overall EBITDA is concerned. I won't comment on the COVID year . Now this is not due to revision CGHS rates . Every year we have an increase in pri cing, which impacts our revenue by 2 -2.5%. But in spite of the increase in sa

laries and

other indirect cost s on the 1st of April, we not only absorbed it, but our Q1 overall EBITDA is equal to our Q4 EBITDA .

Kunal Dhamesha : And then if I may just t wo very housekeeping kind of question, what was the international bed share, we have share d the payor mix , but bed share if you can share?

Yogesh Sareen : It's around 5 .5%.

Abhay Soi : And i t gives you 9% revenue.

Kunal Dhamesha : And just a clarification, so the way I understood is any procedure which uses more devices, which is where our EBITDA per bed would be higher in absolute term , but in terms of margin, it might not be accretive, is the correct way to underst and.

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Abhay Soi : Actually, any high-end procedure has lower percentage margins and high value margin s. If you look at Oncology, Orthopedics, Cardiac Sciences , etc. all of these have lower margins percentage wise, but higher ARPOBs, higher billing, higher EBITDA per patient and higher EBITDA per bed.

Yogesh Sareen : Typically , you will also pay a doctor more for a surgical procedure compare d to medical specialties , so there obviously would be a higher impact of that also on margins .

Abhay Soi : Actually, for medical specialt ies, we are only giving the patient s medicine , doctor visits , etc. Your margins maybe more , but your absolute billing is low er.

Moderator : We have the next question from the line of Bino Pathiparampil from Elara Capital.

Bino Pathiparampi l: Just wanted to know earlier you used to give a schedule of new capacity coming online all the way up to FY'28, I haven't seen it for the last couple of quarters . So, just wondering if all those timelines given earlier two, three quarter back still stay or is there any change in that?

Abhay Soi : We just gave you an update so there is no necessary change in that. And I think other than the update that has been given in last couple of quarters.

Yogesh Sareen : It's there in our investor presentation.

Abhay Soi: It's there in our investor presentation, on our website.

Yogesh Sareen : But I think Abhay has given you the update . Firstly, everything else is on time except for Max Smart , where we are delayed because of the forest approvals and those approvals have also been received now, and we have to transplant the trees . That's the only project where we find some delays . And we said it last time , that because of those delays we are fast -tracking the Vikrant Foundation capacity addition .

Abhay Soi : Usually any data which is static , which doesn't change, we don't give out every quarter . But you can see it on our website , it is part of our investor presentation.

Moderator : The next question is from the line of Kunal Dhamesha from Macquarie.

Kunal Dhamesha : So, to the fact that we have added this 144 beds in Shalimar Bagh and we have also said that we have added some of the doctors , etc., so would it be fair to say that the addition of let 's say manpower would be largely in line with what our average is at network level would be.

Abhay Soi : Are you talking about for the incremental b eds at Shalimar Bagh and others?

Kunal Dhamesha : Yes.

Abhay Soi : No, it's lower . Because you don't have to add the same amount of manpower, and especially not the same cost of manpower . I have mentioned earlier that there is operating leverage in it. But that's not all, we added 144 beds , 80 ICU beds, 10 OTs .

All of this generates a lot more than what it was previ ously being used for , both in terms of revenue and EBITDA , but has higher manpower requirement.

And then we have , over the last year, strengthen ed our projects team because of all the projects we are doing, the digital team for the app that we have come out with , increased efforts on ESG , etc., as well. All of these things play out, but I would

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encourage you to look at it as a percentage of sales rather than an increase
necessa rily year -o

n-year. Our salary increase has been the same, 6.5-7%.

Kunal Dhamesha : And one of the strategy side our earlier comment was that we would be mainly focus ed on the metro cities like Delhi and Mumbai etc ., but some of the competitions have been now opening up into Tier II cities and they are seeing good kind of ramp up in terms of both top -line and the profitability . So, would we also look at these markets going forward and probably broaden our horizon as to where we are going to expand?

Abhay Soi : Any city , where at least one or two of our competitors have proven viability , we will enter into those cities. Presently the expansions we are doing, we are not doing because we only want to be in metros, we are doing because we have a business need . We have run out of capacities and there is demand which is s urning on my doorstep . I have waiting of a couple of days , if not more , sometimes in ICUs . We are doing brownfield capacity in order to tap that demand . First and foremost I need to alleviate the business need that I have. Thereafter we are looking at other cities we continue t o look at . As we have said in the past, there are 21 cities that we have identified that we are looking at. These are cities where at least one or two of our peers have proven viability. So, we think we can go there and do it better.

Kunal Dhamesha : And in terms of inorganic activity, obviously probably my sense is valuation could be a factor, but do you see that intensity, I mean in terms of the ask price versus what we are willing to pay is reducing, increasing over the last let's say six months?

Abhay Soi : For that you have to know how much we are willing to pay . We are usually guided by ROCE, we seek a 20 % to 25% ROCE in a business case . For us, i t's less important what we are paying today, of course nobody likes to pay more . But what is important to us is what is the business case we are willing to underwrite four or five years down the line. Do keep in mind that our EBITDA per bed is at least 50% to 55% better than the next best player in the industry. So, our ability to under write a stronger business case is more than others and that's what we are willing to back .

Kunal Dhamesha : So, that is why I ask about the spread and not if you are willing to pay right . So, spread whether that ha s increased, decreased you know, can give a sense --?

Abhay Soi : That is irrelevant . The point is this that it does not matter what somebody is willing to pay. I f we are shooting for a 20 % to 25% ROCE pre -tax, four or five years down the line , with a particular ability to underwrite a business case so that's the maximum number we are willing to pay today. Now somebody else will be willing to pay a higher number, but the fact is the other person th en has to under write a business case which is at least equivalent to mine .

Kunal Dhamesha : And the last again a housekeeping question, let's say the consumption of drugs if we

can quantify as maybe percentage of revenue in our hospital business as a percentage of revenue or as a percentage of COGs would be helpful, a broad range would also be fine not --?

Abhay Soi : That's not a static number . Let's say if we were to do more higher -end surgeries, this number would increase. What you need to do, we have beds and we have days in a year, right EBITDA per bed --

Kunal Dhamesha : Quarterly basis maybe two to three years o f average would also be fine or a range would also be fine.

Abhay Soi : We don't give forward -looking statement s and guidance .

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Kunal Dhamesha : I am asking about the historical what would be the drugs as a percentage of probably revenue.

Yogesh Sareen : That is in the range of 23% to 25% , depending on which month or quarter you are talking . That's the range I would say historically. And we don't call it material cost, we call it direct cost and this includes F&B (food &

beverages), drugs , and other direct costs.

Kunal Dhamesha : This includes what?

Abhay Soi : F&B and so on and so forth . We don't give a precise number as far as drugs or consumables are concerned . It includes F&B and some of the other cost s as well.

Moderator : The next question is from the line of Dheeresh Pathak from WhiteOak Capital.

Dheeresh Pathak : So, you gave a commentary on the projects that are underway, but I am just trying to get a better understanding of the commissioning timelines. So, Dwarka, which is 300 beds Q3 of this fiscal year, FY'23 -- end of Q3, right. In FY'25 as per the earlier Investor presentation bit there were four assets Smart is delayed so now there are three ; Nanavati is 329, Mohali 190 and Gurgaon Sector 56 , 300. So, of these three if you can individually call out their commissioning timelines, a s per your current understanding based on the current project progress.

Yogesh Sareen : What we given is the timeline s for the completion of the construction of these projects. You can presume that in the next quarter of that will be the commissioning . For example, if the slide says Q4 FY'25 that means Q1 FY'26 will be the commissioning . Once the beds are constructed , we need to obtain the OC, so there is a time lag of a 1.5-2 months, but as Abhay had mentioned already , we are largely on time for these projects.

Abhay Soi : Sometimes it comes sooner also . More or less it's the timeline .

Yogesh Sareen : Yes. All projects are on time except for the Max Smart , which is part of the Saket Complex . We are delayed by six to seven months there because of the forest approval and that's the only project where we feel th ere will be a slip in timeline. B ut we are fast tracking the Vikrant one which is also in Saket Complex.

Dheeresh Pathak : So, individually if you go, Nanavati 329 you expect in Q4 of '25?

Abhay Soi : Yes, as schedule d.

Yogesh Sareen : So, this will be finished in Q4 FY'25 . That means March '25 is when we finish and we will be starting this in the first quarter of FY'26, between February and April.

Dheeresh Pathak: Mohali?

Yogesh Sareen : Similar.

Abhay Soi : Similar.

Dheeresh Pathak: So, most of the assets Q4 of FY'25 commissioning and Q1 of FY'26 is where the commercial --

Abhay Soi : We had mentioned in the end of '25.

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Yogesh Sareen : Yes, end of '25.

Dheeresh Pathak : Now just to understand for these assets whatever is the estimated CAPEX outlay, I don't know if that number is handy or not, how much have you already spent?

Yogesh Sareen : So, I think as Abhay has given you the projects update, for example , in Nanavati we would have spent probably around 26% of the total spent. But other than Nanavati, Mohali we just finished the excavation, we have done the D wall . The contracts have been given to the EPC vendors now . So, I would say a large amount of CAPEX is yet to be spent. On an overall basi s, it will be probably at 12 -13% spent, but Nanavati is the only one where we have more than 25% spent.

Yogesh Sareen : Also, the CAPEX spent , because it also includes equipment, is largely back ended.

Abhay Soi : Yes, the super structure doesn't cost too much, around Rs. 2,000 square foot . So, if you are building 1 million square feet you will spen d Rs. 200 crore for the super structure. That's not the expensive part of the project , which we do upfront.

Moderator : We have the next question from the line of Harith Ahamed from Avendus Spark.

Harith Ahamed : Couple of questions on the CAPEX numbers that you talked about. Firstly, the operating cash flows you mentioned is lower because there was higher routine CAPEX spends this quarter. Will you be able to quantify the same, the routine CAPEX that you spent this quarter and the budgeted spend for the year? Just trying to understand what is our ongoing regular maintenance CAPEX.

Yogesh Sareen : Yes,

so typically we spend around Rs. 170 -180 crore in a year . This quarter alone we have spent Rs. 70 crore. That means we advanced and fast tracked the spend to get the benefit of it during the course of the year .

Harith Ahamed : And on the project CAPEX, the Rs. 38 crore spend I am assuming it was lower because of the weather situation that we had in the Delhi region and then we should expect it to pick up. So, for the year is there a number that you can share?

Yogesh Sareen : Yes, we shared earlier that it's around Rs. 900 crore spend for this year.

Harith Ahamed : So, there will be a significant pick up as we go into the next year?

Yogesh Sareen : Yes.

Harith Ahamed: So, on the M&A front are there any opportunities that we are exploring currently, our name keeps coming up in, as per various media articles in various M&A situations.

And on the litigations that we had initiated against Care is there any update and what are the next steps on that particular situation?

Abhay Soi : CARE matter is sub-judice , so we can't really speak about it, but we have filed an appeal in the Bombay High Court. As far as inorganic expansions are concerned, yes our name is everywhere , whether that is true or not is beside the point. But a lot of places where we are looking at opportunities, our name doesn't come up or those matters are not in the public space . But, coming to your point , we are looking at it quite actively . We have fortified our teams as well . Keshav has been a new addition to head M&A from our standpoint. So, we are quite focused on this.

Moderator : The next question is from the line of Pallavi Deshpande from Sameeksha Capital.

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Pallavi Deshpande : Just wanted to understand how was the operating cash flow and if you could also highlight on the debtor days. And secondly what would be the debtor days separately for the government mix?

Yogesh Sareen : DSO at the end of the quarter was 66 days . It has gone up compared to 55 days at the end of March. There is a build up of accounts receivables (AR) in the PSU and insurance segments. But, we are seeing that in July this has started to unwind a bit.

Pallavi Deshpande : So, just continuing on this I didn't follow the first part. There you said there were two parts to the increase, so the first part was due to ...

Yogesh Sareen : Yes, I said that the overall DSO went up by 11 days , from 55 days at the end of March to 66 days at the end of June. This is basically because of the build-up of AR, for the PSU segment (CGHS and ECHS) and also some build up in the insurance segment . The insurance segment had a 24 -days DSO at the end of March , which has gone up to 34 days at the end of June, so there is some build up there. But as we speak today , there is some unwinding which has happened in July . We have got some of those over dues in July and things are getting normalized.

Pallavi Deshpande : And overall I, on the strength of the PSUs and the CGHS are we going to see better years than last year in terms of government making the payments faster?

Yogesh Sareen : Yes, I mean so far the story looks bright, but we don't know what's going to happen. When the ir budget gets finalized, how much time they take for new allocation , etc., will determine how fast they will pay.

Moderator : The next question is from the line of Mayur Patel from 360 ONE Asset Management.

Mayur Patel : Just in your business planning about the new assets which you mentioned would be commissioned by end of FY25, what would be the peak occupancy which you are targeting in the new assets, is it possible to share some thoughts around that?

Abhay Soi : Peak capacity would be similar to our Network levels , could go up to about 80%.

Mayur Patel : In line with whatever are your matured assets currently 77% to 80% is fair to assume?

Abhay Soi : That's right.

Moderator : The next question is from the line

of Amit Kadam from Canara Robeco Mutual Fund .

Amit Kadam : So, just on the timelines that you provided for the commissioning of those biggest beds, so from that what I can understand is that from here on 300 beds of Dwarka is expected to get commissioned in start of the Quarter 4 of this year and then we have something coming in the quarter one was FY26.

Abhay Soi : Yes, last quarter FY'25, first quarter FY'26.

Amit Kadam : But then I am just focusing on the commissioning or revenue generation --

Abhay Soi : It's a little early to project exactly in which month it will happen . I mean when we said end of FY'25, we meant February/March . Now whether it continues to be February/March or its March/April , is a little difficult to project at this point . But assume it comes on stream next year for calculation purposes.

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Amit Kadam : So, the thing which I want to understand so during this particular period, so I understand we have a 300 bed to ramp it up , what are other levers in that particular year which could still make sure that our growth momentum continues?

Abhay Soi : We fall back on pay or mix, that's one. We have another 20 to 30 beds on the margins , which will come on stream through the year. This is in terms of occupancy, the quality of occupancy and occupied bed days. And of course, we have our case mix , which has increased as you have seen . That continues to hold strong. If you look at Q4 or Q1 last year versus Q1 this year , other than occupancy , and take out the 2 -2.5% increase in revenues, we have seen an increase in ARPO B because of increase in clinical mix largely , and in payor mix more international.

Amit Kadam : Just wanted to understand that when that Dwarka comes on stream what are the changes this thing will have on our overall number, because I think this will be a kind of a new complete standalone asset even though in the same region, but it would have its own individual standalone case where you have to do all the recruitments start from top to bottom.

Abhay Soi : Right, yes so typically there is breakeven period , but it shouldn't be consuming more than Rs. 30-40 crore .

Yogesh Sareen : There is some time taken to breakeven so there may be some loss in the initial phase. But that is the story with any green field .

Abhay Soi : But the loss should not exceed Rs. 30 -40 crore .

Yogesh Sareen : Yes, it's not a big sum of money that we are going to lose . Dwarka is a green field so we are targeting a breakeven in the 10th month of operations.

Abhay Soi: We intend to do better but our projection is about 10th month odd breakeven, but it should not consume more than Rs. 30 -40 crore .

Amit Kadam : And the third and final thing is on the international thing, so we have been doing it quite well in that particular segment, how do I look at this particular thing at least on an annualized basis how this piece should further do for us like moving towards double digit is like a percentage away, but then how do I look at this particular piece beyond that particular percentage point increase?

Abhay Soi : It's going well . I have always maintained that this is a big area of growth for us . Again I will stay away from making forward looking statements, but Afghanistan , which was 12% of our business , is still down to zero . Recently they have started some flights from Afghanistan , so that's a very big positive. Hopefully , in the next few quarters that should begin and should give us something as well. This business is growing, and I am going to avoid giving any forward -looking statements . But do keep in mind that we have had a 13% year -on-year growth , of which 3% has been through occupied bed days (OBDs) or occupancies as you may call it. Out of the 10% , if you take out a couple of percentage points for price increase , I think the balance has really been through clinical mix, a little bit has been through

rough international and so on.

Amit Kadam : Why I was asking, so one thing is that the volume or the inflow or the patient count may go up because there would be certain like some geographies would open up which were restricted or maybe there was some embargos. And second is that some efforts what you guys have taken by going into various geographies setting up front -end offices there that also all led to your inflow. And second is that this inflow could also have some kind of delayed surgeries because they were not able to have that particular --

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Abhay Soi : Well not so far. I don't think we have got business from any place , which had those embargos , but hopefully in the future some of it may be through that. We don't see any sporadic demand right now . It has been fairly kind of secular and should continue, I mean that's the plan. We have not seen any one-time business , if that is what you are saying , like bottled up demand which has suddenly come in and the rest is going to come later. We have seen this secular growth through the year.

Moderator : The next question is from the line of Alankar Garude from Kotak.

Alankar Garude : Firstly, Abhay when are we planning to commission the 100 Brownfield beds at Vaishali?

Abhay Soi : Vaishali was commissioned two years ago.

Alankar Garude : Brownfield, we had signed an agreement right for 100 beds?

Yogesh Sareen : We have signed an agreement to sell (agreement to buy for us) , so there are some conditions precedent (CPs) for that . Basically there are some ongoing litigation s at the High Court and district courts, and we have to settle those litigations first before we can take the handover of that piece of land.

Abhay Soi : Right now it's early stage . I mean right now firstly we have to procure the land . That's why I was wondering which Vaishali you are talking about. One, the agreement to purchase the land has to be sorted and then we will purchase the land and then will be building over there so that's –

Alankar Garude : And the same question on Gurgaon the seco nd piece of land I think there as well there is a litigation ongoing right?

Abhay Soi : Yes.

Yogesh Sareen : We have received a s tatus quo order from the Punjab High Court . There is a litigation ongoing since they cancelled the lease deed after one year of giving us the possession .

Abhay Soi : And taking full money.

Yogesh Sareen : And taking the full money . I think we have a good case there . The next date is somewhere in December.

Abhay Soi : But that w as in any case post 2028 plan , it was never part of the plan till 2028. Even if you see the CAPEX plans in the i nvestor presentation, it was something which was never even kind of accounted , neither the f unds nor commissioning nor start date for it was ever accounted for.

Moderator : The n ext question is from the line of Senthilkumar Natarajan from Joindre Capital Services Ltd.

Senthilkumar N.: Can you share the revenue and EBITDA growth for exclusively March, Shalimar in Q1FY24 on a year -on-year basis?

Yogesh Sareen : So, we already said that the revenue has grown by 3 7% YoY in Shalimar Ba gh and 43% EBITDA growth.

Senthilkumar N.: So, what is the contribution in topline from Shalimar --?

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Yogesh Sareen : So, I don't have the ready number .

Senthilkumar N.: And my second question is what is the gross debt as on June 2023?

Abhay Soi : There is no net debt, there is net cash.

Yogesh Sareen : On a gross basis , it will be Rs. 64 1 crore , which includes the CC limits .

Abhay Soi : But we have got cash against it of Rs. 1,681 crore .

Yogesh Sareen : Yes so there is a net surplus of Rs. 957 crore .

Moderator : The next question is from the line of Amit Thawani, an individual investor.

Amit Thawani : I saw the presentation and I saw the performance in the Oncology segment and even some of the peer hospitals have reported some r

eally good numbers in the Oncology

segment. So, I was just wondering what's happening, is there some kind of change in the health insurance policies that are allowing more coverage of Oncology or can you elaborate little on what's happening there?

Abhay Soi : Nothing in particular . The incidence of cancer is increasing, awareness is increasing , more people are acquiring health insurance and are able to afford to come to private sector hos pitals, which obvious ly for more high -end diseases are preferred compared to perhaps smaller nursing homes. So, we have seen a drive towards that.

Yogesh Sareen : Also, we have seen that in Oncology , there is more usage of ORC drugs, which improves the revenue / ARPOB because these ORC drugs are generally expensive .

Amit Thawani : And how does Oncology growth impact our in -patient, out -patient revenue, because how is really radiation and chemotherapy really accounted in terms of bed occupancy?

Abhay Soi : It's not , its day care. Any day care procedure is not part of occupancy or IPD. It comes through into the ARPOB because you take all the revenues and you divide it by in -patient beds . But any day care procedure is not part of occupancy.

Amit Thawani : So, the ARPO Bs will go up?

Abhay Soi : It's not a part of the ~3,500 beds since these are no t census beds. When you compute occupancy , you take a denominator o f total number of beds . We have ~3500 beds , which does not include the non -census beds, which is a chemo beds, dialysis beds, day care beds, ER beds , etc.

Moderator : The next question is from the line of Damayanti Kerai from HSBC.

Damayanti Kerai : I just have one question so Abhay you mentioned you added 44 beds though internal restructuring , etc., during the 1stst Quarter. How many more such beds can come say in 24?

Abhay Soi : We expect another 20 to 30 beds.

Damayanti Kerai : For this fiscal right, 20 to 30 beds?

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Yogesh Sareen : So, plan is to add another 40 beds . Some of them are getting added in July and some will happen in October .

Damayanti Kerai : 20 to 30 beds?

Yogesh Sareen : 20 to 40 beds.

Damayanti Kerai : And it will be an ongoing exercise right in coming years also you can choose to add such bed?

Abhay Soi : No, we can't . We are now pressing and at the final stages of capacity , we require that capacity so somehow you will find it. There is always some elasticity at the end. We move the laboratories out, we move the offices out, we move the kitchen out , etc. So, that's what we are doing to add these beds.

Abhay Soi : This is not annual; you can't do it every year let me just put it this way.

Moderator : The next question is from the line of Lavanya Tottala from UBS .

Lavanya Tottala : I just wanted a small confirmation, so the CGHS increase in rate is related to the radiology test that has not come in, in the Q1 any impact right. And what kind of impact that can have on our part?

Abhay Soi : No, some of it came in June and some of it came in May , but all of the impact so far has been about 0.6-0.7% on ARPOB.

Yogesh Sareen : For CGHS business , 60% of what we bill is material costs , which includes drugs, consumables, etc., and balance 40% are the items which are tariff items . 26% out of the 40% has been touched by the price revisions so far and on that 26% , we have around 50% price increase. In a nutshell , overall 5% price increase in the total billing for the PSU patients. And so that's the story today . The absolute amount for us would be around , if I take the Q4 volumes and the price increase which has happened till June, Rs. 49-50 crore number on an annualized basis.

Lavanya Tottala : All of that is still not there in Q1 number right then?

Yogesh Sareen : Yes, it won't be there fully. Q1 would be probably Rs. 10 -11 crore out of the Rs. 50 crore full year annualized number that we see in terms of price revisions.

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Moderator : The next question for the line of Alankar Garude from Kotak .

Alankar Garude : Just one follow up, historically in the run up the general elections has Max seen any increase on the receivables front given the higher institutional mix?

Abhay Soi : Not really .

Yogesh Sareen : No. There was an increase in the receivables by end of June, but in July we have seen some of it unwinding. So, we don't see any impact of that.

Alankar Garude : Yes but even in the prior years be it 2014, 2019 no impact per se?

Abhay Soi : No.

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Moderator : Thank you. Ladies and gentlemen that was our last question. I would now like to hand the conference over to the management for closing comments. Over to you sir.

Abhay Soi : So, I would like to thank each one of you for taking time the time out and being on the call . Appreciate your time, thank you and see you next quarter.

Moderator : Thank you. On behalf of Max Healthcare Institute Limited that concludes this conference. Thank you for joining us and you may now disconnect your lines.

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Call: Nov 2023

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November 13, 2023

Listing Department ,
National Stock Exchange of India Limited
Exchange Plaza, Plot C -1, Block G,
Bandra Kurla Complex, Bandra (E),
Mumbai – 400 051

Symbol: MAXHEALTH Listing Department,
BSE Limited
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Dear Sir / Madam,

Please find enclosed copy of transcript of earnings conference call, organized on November 7, 2023,
on financial results of the Company for the quarter and half year ended September 30, 2023.

The said transcript is also available on the website of the Company at
www.maxhealthcare.in/investors/investor-resources.

Kindly take the same on record.

Thanking you

Yours truly,
For Max Healthcare Institute Limited

Dhiraj Arora
SVP - Company Secretary and Compliance Officer

Encl.: As above

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Max Healthcare Institute Limited
Q2 & H1 FY24 Earnings Conference Call
November 07, 2023

Moderator: Good day and welcome to the Max Healthcare Institute Limited Earnings
Conference Call. Please note that this conference is being recorded.
I now hand the conference over to Mr. Suraj Digawalekar from CDR India. Thank
you and over to you, sir.

Suraj Diga walekar: Thank you, Aman . Good morning, everyone, and thank you for joining us on Max Health care Q2 and H1 FY24 Earnings Conference Call. We have with us Mr. Abhay Soi, Chairman and Managing Director, Mr. Yogesh Sareen, Senior Director and Chief Financial Officer, and Mr. Keshav Gupta, Senior Director – Growth, M&A and Business Planning. We will begin the call with opening remarks from the management, following which we will have the forum open for an interactive Q&A session. Before we start, I would like to point out that some statements made in today's call may be forward -looking in nature and a disclaimer to this effect has been included in the earnings presentation shared with you earlier.

I would now like to invite Abhay to make the opening remarks.

Abhay Soi: A very good morning to everyone. We are pleased to welcome you to Max Health care's earnings call for the second quarter and first half of fiscal year 2024. Let me start by stating that our performance in the first half of this fiscal year has set a commendable precedent for us to follow in the latter half. We recorded a year -on-year increase of 17% in network revenue and 20% in EBITDA in H1, while Q2 turned out to be the 12th consecutive quarter of year -on-year growth. Our Q2 performance this year largely mirrored our quarter -on-quarter performance last year ,

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alluding to the steady state of our operations as well as secular demand for quality healthcare services.

Further, with a granular focus on execution and capital allocation, as is evident from our pre -tax ROC E of 38.3% in Q2, we are well poised for the next leg of growth that is set to come from planned capacity expansion as well as inorganic opportunities. On that note, we are happy to share that the developer of an upcoming hospital in Dwarka has applied for the occupancy certificate, which is a significant milestone. It is the final milestone. We expect to commission the same in the fourth quarter of the current year.

Moreover, our most recent brownfield expansion, Max Shalimar Bagh, has reported an overall average occupancy of 78% and a year-on-year revenue and EBITDA growth of 41% and 48% , respectively in the second quarter.

On the clinical front, we have signed a memorandum of understanding with Intuitive Surgical, the US -based pioneers of robotic surgical systems, to establish Southeast Asia's first Total Program Observation Centre located at our Max Saket facility. This centre is expected to have a positive impact on both India and Southeast Asia's surgical healthcare ecosystem by enabling healthcare professionals to drive advancements in patient care using robotic -assisted surgery and elevate surgical healthcare standards in the region.

Moving on to the highlights of our Q2 performance :

- 1) Occupied bed days grew by 3% year -on-year and 5% quarter -on-quarter, reflecting an average occupancy of 77% for the quarter. 93% of the year -on-year and 118% of the quarter -on-quarter increase in occupied bed days was driven by preferred channels, which is cash, insurance, TPA, and international (CTI) . With the increase in occupied bed days and marginal drop in A LOS , the inpatient discharges were up by 7% year -on-year.
- 2) Even OP volumes exhibited a strong growth of 14% year -on-year and 4% quarter -on-quarter.
- 3) Institutional bed shares fell to 27.3% compared to 27.9% last year and 29.7% in Q1 this year. However, after excluding Max Shalimar Bagh, our most recent

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expansion, overall institutional bed share stood at 25.4% during the second quarter.

- 4) Average Revenue Per Occupied Bed (ARPOB) for the quarter stood at INR 74,600, growing by 13% year -on-year, and remaining flat quarter -on-quarter due to seasonality. Year -on-year improvement was witnessed across all specialties, with oncology being the key driver.
- 5) Network gross revenue was INR 1,827 crore compared to INR 1,567 crore in the Q2 last year and INR 1,719 crore in the previous quarter. This reflects an increase of 17% year -on-year, led by growth in ARPOB and occupied bed days (OBDs) . Quarter -on-quarter growth of 6% was mainly driven by increase in OBDs .
- 6) Revenue from international business grew significantly by 25% year -on-year and 11% quarter -on-quarter, now accounting for around 9% of the total revenue from our hospitals. During the quarter, we have operationalized company -owned patient assistance centre (PAC) in Nepal, while all formalities for the Bangladesh centre have been completed. We expect to operationalize this centre shortly. This is despite the Afghanistan business, which was 12% of our total international

business, still down to zero.

7) Direct costs were lower quarter -on-quarter due to increase in medical patients , attributable to seasonal vector -borne diseases. On the indirect costs side, while the overall percentage was lower, there was an increase in absolute costs primarily due to marketing costs for international channels and seasonal increase in power consumption.

8) Network operating EBITDA stood at INR 497 crore, just below the magic mark of INR 500 crore, reflecting growth of 21% year -on-year and 14% quarter -on-quarter. Accordingly, the operating margin increased to 28.7% versus 27.7% in the Q2 I

ast year and 26.8% in the previous quarter.

9) Most importantly, annualized EBITDA per bed rose to INR 75 lakhs, yet again our highest ever, clocking a growth of 17% year -on-year and 7% quarter -on-quarter.

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10) Profit after tax was INR 338 crore versus INR 267 crore in Q2 last year and INR 291 crore in the previous quarter on a like -to-like basis. The year -on-year improvement of 26% was primarily attributable to flow through of improved EBITDA and lower finance costs.

11) Free cash flow from operations was significantly higher this quarter at INR 436 crore, of which INR 90 crore was deployed towards ongoing capacity expansion projects. Net cash position improved to INR 1,303 crore at the end of September 2023 compared to net cash of INR 42 crore same time last year.

12) Continuing efforts to support the local communities, we treated approximately 39,000 patients in OPD and 1,300 patients in IPD from economical weaker section s free of charge .

13) Both our strategic business units continued to tr ead strongly on the ir growth trajectory :

- Max@Home reported a top line of INR 42 crore , reflecting a growth of 23% year-on-year and 8% quarter -on-quarter . We continue to receive good feedback for our services and the same is reflected in the SBU 's revenue growth.

- Max Lab, the non -captive pathology vertical , offers its services in 36 cities and now has an operational network of over 1,000 collection centres and active partners. This SBU reported a gross revenue of INR 39 crore, reflecting a like -for-like growth of 32% year -on-year and 15% quarter -on-quarter.

Now , coming to the status on our upcoming expansion projects :

- As most of you know, 122 beds at Shalimar Bagh have been operationalized at the start of this financial year. And as mentioned earlier, the hospital reported an annual occupancy of 78% for the quarter.

- For 300 beds at Dwarka – Application for occupancy certificate (OC) has been submitted in October and majority of the interior works have been completed. And some of it is just being finished as we speak. We expect to

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commission the hospital in later half of Q4 , subject to receipt of OC by the developer.

- For 329 beds at Nanavati – Excavation and raft work is complete. Steel fabrication up to the ground level and slab work have also been completed. Ground level structure is expected to be completed in the current quarter and the project continues to be on schedule.

- For 300 beds at Sector 56, Gurgaon in Phase I – D-wall has been completed and the site excavation is almost done. EPC contractor is already on board and design development is under process. TDR approval for additional 0.5 FAR has been received and the project is on schedule.

- For 190 beds at Mohali – D-wall is complete and excavation work is underway . All statutory approvals to start the construction have been received and the project is almost entirely on time. EPC contractor has been mobilized and the design development is in progress.

- 350 beds at Max Smart at Saket Complex , which had seen some delays initially – We have now initiated the process of transplanting the trees as permissions had taken some time to come . This has been on the critical part, but now the project is back on schedule and work should start by December 2023 .

- For 300 beds at Vikrant at Saket complex – Environmental clearance (EC) has been received and the submission of drawings in the Municipal Corporation of Delhi is in process .
 - For 250 beds at Patparganj – Drawings have been submitted to the Municipal Corporation of Delhi and the application for environment clearance has been submitted.
- So, all the other projects are on schedule and there is no delay as such. And finally, coming to the overview of the company's performance in the first half of the financial year:

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1) Network gross revenue stood at

INR 3,546 crore, reflecting a growth of 17% year-on-year.

2) Network operating EBITDA grew by 20% year -on-year to INR 933 crore . Increased ARPOB , improved case mix and augmentation of network bed capacity by 130 beds resulted in margin expansion to 27.8%, while EBITDA per bed grew by 15% to INR 72.8 lakhs per bed.

3) In the first half, we generated INR 697 crore of free cash flow from operations after interest, tax, working capital changes and routine capex, of which INR 128 crore was deployed towards ongoing expansion projects.

With this, we open the floor for Q&A.

Moderator: Thank you very much. The first question is from the line of Damayanti Kerai from HSBC.

Damayanti Kerai: My question is you continue to see progress in reducing bed share to institutional patients. So , a few quarters back, you had given an indication that we would like to bring it down to the industry level. But with Shalimar Bagh, I guess, you have taken on more institutional bed to ramp up occupancy, etc. So, do you still target to bring it down to, say, industry average and when it will likely happen?

Abhay Soi: There is no classified industry level , so to say . I think it highly sort of changes between metros and non -metros. You have more PSUs, headquarters, etc., out of Delhi NCR, so you have a larger share of institutional businesses coming through. Now , two-three things have happened.

One is apples -to-apples from 29.7%, we have come down to about 25% in the current quarter. Second is that it's on increased capacity, including Shalimar Bagh . Thirdly , certain rates have moved up in CGHS and we're expecting certain other rates for institutional business to move up in the current quarter , because of which we have taken the foot o ff the accelerator a little bit .

And finally, even within the institutional business, there's been a churn in the specialties we are catering to and the ones we are not catering to. And all of it largely comes down and plays out in your higher EBITDA. So , what we're seeing is, although because of the overall capacity constraints that we have, our occupancy has

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moved up by only 3% year -on-year, but there's also been a churn of about 3% to 4% year-on-year within the payor mix. And all of that tends to translate into a higher percentage margin as well as EBITDA per bed.

I mean, apples for apples, the same inventory going forward will be coming down as far as institutional business is concerned, but to your point, as you rightly mentioned, as and when we have new capacities coming in, those capacities initially will see a n increase in institutional business. So , in percentage terms, it will move up, or remain stagnant just when those capacities are coming. But I think overall, it still translates to better EBITDA margins.

Damayanti Kerai: Okay, so you mentioned like...

Abhay Soi: Just to complete that point, you may have seen because of new capacity coming in Shalimar Bagh, the institutional business has moved up. But if you see the EBITDA coming from those incremental beds, basis this increase in institutional business , it is yet at a 40% margin.

Damayanti Kerai: Okay, so that means obviously you are getting much better realization from these set of patients also. As you mentioned, rate hike in CG HS could be one of the reasons which might be contributing and then obviously specialty mix?

Abhay Soi: Not that, this is largely because of the operational efficiencies. We have a huge operating leverage for the new beds. We don't have the fixed costs, etc. As a result, what happens is that even the lower payor mix becomes more viable and even more viable on the new beds. Because our operating cost is very low on the incremental beds.

Damayanti Kerai: Okay, so it's primarily driven by efficiency as you mentioned. Like you have better absorbed overheads there that is resulting in these kinds of numbers .

Abhay Soi: That's right.

Damayanti Kerai: Okay, and just a clarification, you mentioned there has been a bit of hike on the CGHS patient s also. So right now, like what is the difference between that price channel and then the normal cash and others, like very broadly .

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Yogesh Sareen: If you take the ARPOB of these two channels, the CTI channel and the preferred channel , CTI is 85% higher than the CGHS / PSU channel.

Damayanti Kerai: So the preferred channel is 85% better ARPOB number?

Abhay Soi: If your PSU ARPOB is 100, CTI ARPOB will be 185.

Damayanti Kerai: Okay, 100. Okay, my last question is your difference between gross revenues and net revenues which you go for pro forma financials, that's primarily driven by what you pay for EWS patients, right? That's right.

Yogesh Sareen : Largely that number, yes.

Damayanti Kerai: Yes, and I'm seeing that number has broadly remained somewhere like 5% of gross revenues. So, should we assume similar numbers to trend, even if like say we are commissioning new facilities ahead and then according to government rules, we have to allocate some beds for the EWS?

Yogesh Sareen: Dwarka does not have any EWS obligation .

Abhay Soi: Dwarka does not have, Mumbai will have, and others will have. I mean, Gurgaon also won't have.

Damayanti Kerai: Okay, so Dwarka and Gurgaon does not have.

Abhay Soi : Other than Gurgaon, Dwarka and Mohali , all others will have EWS obligations .

Damayanti Kerai: Okay, okay. Thank you.

Abhay Soi: Out of around 2,600 additional beds, I think approx. 1,000 beds will not have EWS obligations , balance will have.

Damayanti Kerai: Approximately 1,200 beds will be utilized and then others don't have such requirement ?

Abhay Soi: Yes, so let's say we have approx. 2,600 beds further coming up, out of which around 1,000 beds will not have any EWS obligation s. Now the balance 1, 600 will have the 10% obligation , so let's say about 160 beds out of 2, 600 beds.

Damayanti Kerai: Okay, got it. Thank you. And all the best.

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Abhay Soi: Thank you.

Moderator: The next question is from the line of Kunal Dhamesha from Macquarie.

Kunal Dhamesha: Thank you for the opportunity, sir. First on the housekeeping question on the international patient bed share. What was that for the quarter?

Yogesh Sareen : Yes, that will be around 5%.

Kunal Dhamesha: 5% only?

Yogesh Sareen : Yes, the ARPOB is higher, so that's how the revenue share goes up. It's 5.5% to be exact .

Kunal Dhamesha: 5.5%, okay. Okay, so then probably the pricing, etc., kind of more or less has remained the same. Okay, thanks for that. And secondly, when I look at our specialty mix, oncology therapy, if I see, has been growing at almost 2x the overall revenue growth, at least for the last two quarters. And even if I loo k at a longer -term trend, for the last nine quarters in a row, it has grown faster than our overall revenue growth. So what are we doing differently there? And is it a market growth or?

Abhay Soi : Yes, I think the overall growth is 17%. And oncology growth is about 26% -28%.

Not exactly double. But there's been a focus on oncology, and we've also seen a larger disease burden playing out. We've also had a focus on robotics. And as we are moving up the value chain, what we are seeing is more people are choosing better technology.

As you're seeing higher percolation of insurance, then people tend to do less of window shopping for higher end procedures . They go to more established corporate hospitals and brands. We are seeing more -and-more people for oncology and some of the other specialities , whereas they would have gone to smaller places earlier and now coming to larger hospitals . So, insurance plays an important part.

Yogesh Sareen : Also, oncology tends to have higher entry barriers . There's a lot of investment required in equipment, bunkers, infrastructure, et

c. To that extent , the patients do navigate towards bigger players.

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Kunal Dhamesha: Sure. And is it possible to share the split of this 25% of revenue mix between, let's say, surgical and non-surgical? Because we have also have chemotherapy and radiotherapy included in this 25%, right?

Yogesh Sareen : Well, that's not a public number. But obviously, a large part of this will be chemotherapy . But I would say, it's a fair share of all three – radiation, surgical and medical. We haven't publicly disclosed that number , so we won't publicly disclose the exact split between the three.

Kunal Dhamesha: Sure. And is it fair to say that oncology would be highly accretive to profitability for us?

Yogesh Sareen : Yes. The oncology happens to have a higher ARPOB. To that extent, yes, it'll be higher profitability also.

Abhay Soi: It depends on how you're looking at it. I mean, it also occupies more space. So, there's also a return on capital and return on real estate over there.

Kunal Dhamesha: Okay. So return on capital is also higher in your view? Or does it require more space so it would more or less...

Abhay Soi: Return on capital is even with the rest.

Kunal Dhamesha: Okay. Great. Thanks for that clarity. And secondly, on d rivers of our strong ARPOB growth of around 13% year -on-year for the first half, if I kind of, do some back calculation using the bed share and revenue share, it seems that CGHS ARPOB or institutional ARPOB would have at least gone up by around 20%, 25%. Is that a fair number or I'm overestimating, underestimating?

Yogesh Sareen : No, that's right. The PSU ARPOB has gone up by 28%.

Kunal Dhamesha: 48% for the first half ?

Abhay Soi: 28% year-on-year. And that's not because of price increase. It's also because of the mix change, which I was mentioning earlier. It's not a change in price. The change in price will not even have INR 14 crore impact on your overall revenue.

Kunal Dhamesha: Okay. Perfect. Thank you. I have more questions. I'll get back in the queue.

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Moderator: The next question is from the line of Bino Pathiparampil from Elara Capital.

Bino Pathiparampil : Hi. Good morning and congrats on a great set of numbers. Just one question on the expansion plans. For all the facilities, the greenfield facilities that are coming up over the next couple of years, what's your internal target for EBITDA breakeven, in how many months or quarters?

Abhay Soi: Almost 90% of our expansion is brownfield. And normally, we see EBITDA breakeven in a quarter or two if not the first quarter itself. Our last brownfield experience was that we had EBITDA breakeven , and we were hitting 40% margin s within 40 days.

Yogesh Sareen : On Greenfields , we do see breakeven within the first 12 months. That means 11th or 12th month should be the EBITDA breakeven month.

Abhay Soi: Greenfields are only 10% of the total expansion.

Bino Pathiparampil : Okay, understood. So, within a year for Greenfields and within a quarter for brownfields?

Abhay Soi : Yes.

Bino Pathiparampil : Great. Thank you.

Moderator: Thank you. The next question is from the line of Nitin Agarwal from Dam Capital.

Nitin Agarwal: Hi, can you give us some numbers on what has been the increase in discharges, outpatient discharges of Q -o-Q and Y -o-Y?

Abhay Soi: 7%.

Nitin Agarwal: This is Y -o-Y?

Abhay Soi: Yes. Q-o-Q doesn't matter because you have more medical patients in Q2 . So, when you have more medical patients, your ALOS is lower, and your discharges tend to be higher . Look at everything on a year -on-year basis for better comparison .

Nitin Agarwal: And secondly, on the seasonality part of it, typically, how should one think about seasonality in our business? Q2 obviously is bigger than Q1. And how should we think about the rest of the year with respect to Q2?

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Abhay Soi: See, Q4 is the best quarter

in a year. Q2 is the second -best quarter. Q1 and Q3 are weak quarters. And that seasonality in the business that happens every year.

Typically, your H2 is better than H1.

Nitin Agarwal: And I guess given the way things are, that's a trend we should follow even this year for us.

Abhay Soi: That's right unless something disruptive happens. Typically, the secular trend in

healthcare is that H2 is better than H1. And Q4 is peak because of the burden of the diseases, etc. Also, Q2 is the second best simply because you have seasonality due to dengue and vector season. Q1 is weak because you have just had an increase in salaries and fixed costs on the 1st of April. So, your margins are kind of squeezed. And Q3 is the festival season, which is Diwali, Christmas, etc.

Nitin Agarwal: And secondly, on your expansion plans, barring beyond what we've already outlined so far, how are we thinking about expansion? We've got enough cash reserves on books now in terms of the inorganic growth opportunities which are there. I mean, how do you see the landscape playing out? There's been a lot of private interest in the space, which probably, I don't know, would have had its own challenges for value buying. So, how are you looking at the inorganic growth opportunities outside of NCR?

Abhay Soi: I think there are quite a few. There are 21 cities that we're looking at. And we've been busy at it. And hopefully in the near future, very shortly, we should come up with some surprising stuff. But yes, we want to maintain some fiscal discipline. There are quite a few opportunities, both on the build side, partner side, on the asset-light model, as well as certain acquisitions as well. So, yes, we intend to deploy this capital.

Nitin Agarwal: And last one, on the Shalimar bag expansion that we did in Brownfield, what is the capacity utilization on that?

Abhay Soi: 78% on the overall, new and old beds combined.

Nitin Agarwal: And when you would have put the new capacities, the older one would have been closer to 80% plus.

Abhay Soi: 82%-83%. Yes.

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Nitin Agarwal: And just sort of reconfirming, on the incremental beds, we are making 40% incremental margins.

Abhay Soi: Yes, that's right. And within 40 days of opening those beds.

Nitin Agarwal: Right, right. And in your assessment, incremental Brownfields that we're going to be putting out, how should we think about -- We did talk about the first quarter break - even. But in terms of... Is Shalimar Bagh an exception in the way it's played out, or this is going to be a template that's broadly going to get replicated across the new Brownfields?

Abhay Soi: Look, honestly, our Shalimar Bagh experience was the same as our experience in Vaishali before that as well. Because, theoretically, you're tapping into untapped demand on your doorstep to start with. And then you have operating leverage -- you don't have any real fixed costs with those incremental decisions. Theoretically, this should be the template going forward as well. I don't see that changing.

Moderator: The next question is from the line of Ankur, an individual investor.

Ankur: Hi. I think my question is partly answered in one of the previous questions that was raised by one of the participants. Really, it was about the last two years. There's a bit of a concern that we haven't acquired any project and added anything on to our already announced development pipeline. And obviously, we've been running an underleveraged balance sheet for a while now. And then now, we've got all this cash accumulating.

And you've talked about acquisitions, M&A, and all of that. But I mean, it's two years since we added anything. And, on the Greenfield side of things, are we looking at any Greenfield projects that we want to add? And I know you keep saying imminently that there should be some announcements. But it also takes about, I think, if you add a new pr

ject, a Greenfield project, about three or four years before it's operationalized. So, if you can throw some light on all these things, please?

Abhay Soi: Yes, I think there's always a tug of war between the desire to expand and the fiscal discipline. And one has to maintain that. It's not as if we haven't been looking. And we are quite certain that shortly we should be able to deploy the cash. Do keep in mind that it's not a huge amount of cash. Because even to construct a 500-bed

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hospital, you require about INR 1,000 crores. To acquire a 500-bed hospital will cost you another maybe INR 1,000-1,500 crores.

So, one or two acquisitions and you're done. On one side, we are excited about the fact that we are accumulating cash. But we are also conscious of the fact that this amount of cash and even the ability to leverage the balance sheet is not going to take us very far. Today, transactions are available at 15-16X EV/EBITDA. What that means is, even at entry, if I was to go and spend INR 5,000 crores, we'll be able to

buy INR 300 crore EBITDA? That's about 15% of my total EBITDA.

So, I can increase my EBITDA by 15% by deploying INR 5,000 crore. And that would pretty much use up all my cash and my leverage ability. So, it's important to, while we notice that there's cash accumulating, do keep in mind that one, this is a capital-intensive sector. And secondly, there are a massive number of opportunities in the sector yet.

So, if we want to sort of participate in that, we need to accumulate the cash and spend it with the right amount of fiscal discipline at the right time. Because there is money, but it's not that much money also.

Ankur: Yes. And also, like, you know, like as where we've been going and it's almost, let's say, 20% sort of EBITDA growth, cash flow growth over the last couple of years.

And going forward also, it seems we're going to continue that trajectory over the next four, five years. So, then beyond that, to continue growing at the 20% sort of rate, we'll also need to keep adding the bed capacity at that sort of rate. Right?

So, we need to have like this continuous development pipeline, which keeps, you know, every year keeps adding projects year on year so that, you know, the growth continues for long duration. So, I'm sure you guys are working on it, but just -- and you mentioned, like, we're looking at 20 cities. So, but there has been no, like, actual project acquisition. So, that was my only question. But, yes.

Abhay Soi: No, you're absolutely right. Just keep in mind two aspects. Right? In the last two years, there's not been any significant capacity expansion. Yet, we're seeing a round 20% increase in EBITDA. So, in the next three to four years, we're going to have around 2,600 beds coming up, increasing your capacity, 85% of which is through

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brownfields. That is almost like doubling your capacity over the next three to four years.

Those are coming on stream. So, shouldn't that be giving you expansion for the next five or seven or ten years itself? And given the fact that your breakeven is so short in these brownfields, it'll add a bunch of more cash flows, all of which again gets deployed and gives you further returns. So, there are three streams of growth over here.

One is your current bed capacity, which has been growing in terms of EBITDA. Then all the expansions that are already announced, which are already underway, which we said are largely online. And the third is what you want to do with this cash. Right? It's an exponential 3X strategy. It's not a strategy of 15% -20% growth. If I was not to sort of deploy this cash, give it all back as dividends, yet you'll be doubling your capacity over the next three years to four years.

Ankur: Yes. I think that's it, from my side. I understand where you're coming from. And also, it's clear, like, as the cash flow keeps accruing

over the next three, four years

and they keep growing, you continue to add on to your development pipeline and continue this growth for long term. So, that's it from my side and thank you and all the best.

Moderator: The next question is from the line of Tushar Manudhane from Motilal Oswal Financial Services.

Tushar Manudhane: So, just on the organic basis, on EBITDA per bed, while we've already, I presume you've optimized in terms of efficiency at the overall level. So, how do you think about the levers for improving EBITDA per bed for the next two to three years?

Abhay Soi: I think EBITDA per bed, now the first half has already happened, right? And second half is usually marginally better than the first half. At least for the rest of the year, some sort of trajectory has been already articulated.

Tushar Manudhane: Beyond FY24, how do you think about it? In the sense, the case mix or the payor mix is also, we have already taken good price hike on account of institutional patients, the insurance penetration is...

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Abhay Soi: No, it's not the price hike. In fact, the price hike impact is only INR 14 crore. There's a 28% increase in PSU ARPOB and that's due to the clinical mix, because we're moving into higher end procedures. We're distilling the procedures.

Tushar Manudhane: Right. Okay.

Abhay Soi: Price hike has been negligible, in fact.

Yogesh Sareen: In fact, of the 28% increase in PSU ARPOB, only 5% is due the price hike in the PSU segment.

Abhay Soi: So, like Yogesh rightly pointed out, out of the 28% increase in PSU ARPOB, only 5% increase is due to price hike.

Tushar Manudhane: Right? No, I meant to ask that how much more can further be optimized, if not on price hike, but other levers so as to drive the EBITDA per bed, maybe in mid-teens or more or less growth over the next two to three years?

Abhay Soi: In a similar fashion, as our payor mix starts moving up on a particular trajectory and our clinical mix moves up on a particular trajectory, all of it goes to our EBITDA.

Our indirect cost is increasing by around 6-7% every year. The difference between our revenue increase, now you have to make an assumption of our revenue increase, and the indirect cost increase is all going down to the EBITDA effectively.

Tushar Manudhane: Got it. Understood. Thank you.

Moderator: Thank you. The next question is from the line of Banshi Desai from JP Morgan. Please go ahead.

Banshi Desai : Hi. Thanks for the opportunity. So, I have one question and this is on the advancement of robotics that we have seen in the overall healthcare space. We have seen more and more specialties using robotics and even the non-complex ones are making use of robots. In general, where are we today in terms of surgeries which are getting done on robots and what is the scope here, where it can go to? Also, I am assuming this will be lucrative enough. What is it in terms of ARPOB and margins, how different they are compared to our traditional surgery work?

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Abhay Soi: First and foremost, although robotics has been around for some time, over the last couple of years, there has been a sudden uptake of that and acceptability between both doctors and patients has been quite dramatic as far as robotics is concerned. To be honest, it surprised us also on the upside. As I mentioned earlier, our total number of robotic procedures have more than doubled in the last one year. And, it has been a pleasant surprise. For most specialties now, you get into this flywheel concept as their acceptance goes up. We are being pushed by many of our hospitals and many of the clinicians now, to set up robots because, frankly, it has become more and more viable.

What happens is, of course, it's at a higher cost compared to laparoscopic, or even for that matter, a general surgery for the same procedure. So, it leads to higher ARPOBs. But

the contribution levels from robotics are lower. And EBITDA in terms of margins is lower, in percentage terms, although in value terms it is higher. And this is something that we have previously mentioned with respect to both higher-end payor mix and clinical mix, that you get lower percentage margins but higher value in terms of absolute EBITDA coming from this.

Yogesh Sareen: It also helps us in terms of reduction in ALOS, right? EBITDA per bed is obviously better. But in terms of margins, that may probably be lower than the overall average.

Abhay Soi: Right now, it's a little difficult to present a trajectory of where do we see growth happening and will it continue to be 100% growth, or will it taper down to 70% or 50%? Or will it increase from here?

Banshi Desai : Got it. But the adoption has increased. And in general, it also improves your throughput, right, within a particular specialty?

Abhay Soi: That's right. And, I mean, overall, it's got all the positives with it. And, as public acceptability increases, as technology advances, there are entry barriers in this, smaller places can't adopt it increasingly and the awareness increases, then you see this move towards larger hospitals and more sophisticated healthcare systems. It also results in better medical outcomes for the patients.

Banshi Desai : Got it. Thank you. That's it from my side.

Yogesh Sareen: Thank you.

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Moderator: The next question is a follow-up question from the line of Kunal Dhamisha from Macquarie.

Kunal Dhamisha: So on Dwarka. So as we are kind of getting closer to the commissioning of the facility, would we like started hiring in terms of doctors or key specialties, nurses, paramedical staff, or would it be more closer to the commissioning?

Abhay Soi: We have started all of that. And at least all the head of the programs and functions are already in place and in the system. As we speak, they are working in some of our other facilities. So, as far as the soft power is concerned, the people are concerned, all of it is in line and as per schedule. There is no lull on that. And there's enough availability and excitement around this facility from a clinical standpoint.

Kunal Dhamisha: And would it be more like a staggered hiring in terms of specialties, or we would go with the full-fledged 300-bed operationalization on day one?

Abhay Soi: No. We will do about 164 beds, like Yogesh pointed out. And as we require more

beds, we will open up more and more.

Kunal D hamesha: And one follow-up on the robotic surgery, while you have alluded that it's good from the ARPOB perspective and the absolute EBITDA perspective, but in a longer term, if let's say even the other hospitals also kind of start affording it, does that bring the market to the equal level in terms of surgical outcome, etc., and then the importance of brand or surgeon's skill kind of get reduced? Do you see that happening?

Abhay Soi: Not particularly. I think the market is growing. There's only so many players that can adopt robotics. Your people also have to be trained on it. You have to have availability of the talent to be able to do this. All of that will take time. But before that, your market would have already expanded. And we've seen this, right? I mean, why robotics? As far as any technology is concerned, the larger players adopt it first. The smaller players adopt it thereafter. The market only continues to expand. But I've never seen the larger players' market share go down because the smaller players have adopted the technology thereafter. You actually end up increasing the size of the market. All the smaller players also have a hand in increasing awareness for that product.

Kunal D hamesha: And for us, is it more like a capex model or is it more a pay-per-use model?

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Abhay Soi: Actually, we started robotics as pay-per-use because

we were quite unsure. But we've bought back or bought more than 50% of the robots recently because it kind of surprised us in any case. So, yes, at this point in time, we have a hybrid model simply because we started off with pay-per-use. But now we've got into buying it back. So, we've bought more than 50% of the robots that we have.

Kunal D hamesha: Okay. And return on capital is higher on...

Abhay Soi: Of course. That's why we are buying.

Kunal D hamesha: Okay. Thank you, sir.

Abhay Soi: Thank you.

Moderator: Thank you. The next question is from the line of Naysar Parikh from Native Capital.

Naysar Parikh: Yes, hi. Thanks for the opportunity. My first question was on the overall industry trend. Are you seeing still there is a gap between supply and demand growth? And do we see for the next, couple of years, there will be leverage to continuously grow price and ARPOB? How are you seeing that?

Abhay Soi: I'm not seeing it over the next couple of years, but I'm seeing it over the next few decades. I think this is a multi-decadal opportunity. There is a huge amount of under penetration. There's a massive gap between supply of quality healthcare and demand for quality healthcare, which is only increasing as we go by.

So, that's the reason -- and India perhaps offers this opportunity which nowhere else, no other country, no other health system in the world does. I mean, at one side, you operate almost like a utility because it's inflation-free, it's an insulated business. But on the other hand, you also have this massive growth opportunity because of just to sheer lack of penetration or availability of quality facilities.

Naysar Parikh: Got it. And on the international patient side, you said around 5-5.5% of your beds. Is there scope for that to significantly go up to, say, 10% or higher? And how are we seeing the international traffic on the hospital side?

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Abhay Soi: Traffic has grown by 25% in revenue terms at least over last year. Even in terms of total number of beds, that is in terms of volume of patients, also it has grown by 25% over the same quarter last year. And 11% over the last quarter itself.

And do keep in mind, that this is without 12% of the total business which came from Afghanistan earlier, now being down to zero. If you assume that coming back to normalcy, it will mean an increase of around 30% or more over last year. So, I mean, where does this train stop? I think we haven't even scratched the tip of the iceberg. You know, this should continue, in my mind, at a significantly higher pace than the rest of the hospital growth.

Naysar Parikh: Got it. And the 2,600-bed expansion that we have, can you give us some idea in terms of how much can come in the next six months and how much in FY25?

Abhay Soi: 300 beds should come in by end of FY24, current fiscal year. FY25, towards the end, you will have another 329 beds of Nanavati coming. Then Mohali, another 190 beds again, same time next year. And 300 beds at Gurgaon, same time next year. So, you have about 819 beds coming in the next one year.

Naysar Parikh: So, 300 by the end of this year and another 800 to 900 by the end of next year.

Abhay Soi: Yes, it's available in the presentation on our website. You can see year-by-year capex and date of completion.

Naysar Parikh: Got it. And just one last data point. You said the institutional ARPOB obviously has improved significantly. So, now, where does the gap between institutional and non-institutional ARPOB lie? What would be the gap?

Abhay Soi: 85%. If ARPOB for institutional is INR 100, for non-institutional, it is INR 185.

Naysar Parikh: Okay. Thank you.

Moderator: The next question is from the line of Alankar Garude from Kotak Institutional Equities.

Alankar Garude: So, you mentioned about expecting to make some announcements on the expansion bed shortly. So, just wanted to check whether

it comes to different expansion models,

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like say between partner, built-to-suit, O&M, and acquisitions, do we have any specific preference?

Abhay Soi: No. I mean, acquisitions are, at the right price for the existing, but otherwise built-to-suit, in the sense being asset-light, is very good. We don't particularly like Greenfields.

Alankar Garude: Understood. Okay. And on that point, on this CARE acquisition, you have been providing regular updates, including one yesterday night. Now, on one hand, the appeal is reserved for orders and on the other hand, Blackstone seems to have announced the acquisition, at least as per media articles. So, not sure what to make out of this. Can you please help elaborate on the current situation?

Abhay Soi: The situation is what it is. We have made an appeal to the High Court, now it's for the High Court to decide.

Alankar Garude: Understood. Okay. And one final question.

Abhay Soi: I can't give you any opinion on that. It's for the judge to decide.

Alankar Garude: True. And one final question. Now, when it comes to some of these allied services, we are into diagnostics, then at home, but we have seen some of the other hospital chains doing far more as far as some of these allied healthcare services is concerned, getting into pharmacies, then insurance, diagnostics in a maybe a bigger way. So, maybe in future, not immediately, but in future, are we open to being more aggressive on some of these allied services?

Abhay Soi: I am open to anything and everything in the healthcare business, which others have succeeded in. Philosophically, we don't like to do pioneering things. When they succeed, we will study, we will learn from their mistakes and we will gain confidence from what they got right, and then we will do it better, like we already do. Anybody who does it, I'm very open to doing those things, but let somebody else do it successfully first. There are more than enough examples in front of you where people have jumped into a situation and got it wrong. That's not a game we play. That's not what we're good at, to be honest.

Alankar Garude: Fair enough. Okay. That's it from my side. Thank you.

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Moderator: The next question is from the line of Amit Kavani, as an individual investor.

Amit Kavani: Hi, my first question is that I don't know if it's already been asked or have you answered it, but the revision impact on the institutional business, can you tell us what is expected to be in the December quarter and March quarter?

Abhay Soi: I have no idea what it is expected to be because they haven't taken us into confidence on that. So far, the impact has been 5% of the ARPOB of the PSU business, but we have absolutely no clue how the government is thinking about it.

Amit Kavani: So, no further institutional revision has been announced?

Abhay Soi: No. We were expecting it this quarter and now hopefully we are expecting it next quarter, but we don't know when it will come through and how much will it be.

Amit Kavani: Okay. The second question is actually, when I speak to other hospital companies who are not really in metros, they say that the institutional business does not have lower margins than the overall business. So, just trying to understand the kind of, the difference between them and us. Is it just because that we are in metros that our -- we are -- non-institutional business is higher paying? Is that the conclusion to reach?

Abhay Soi: No. The institutional business, let's say, has a n ARPOB of, let's say, INR 40,000. If the rest of your business, for whatever reasons, has a n ARPOB of INR 40,000 or lower, then it doesn't impact you much, does it? My ARPOB is the highest in the industry. Now, why is that? A lot of players have a n ARPOB of INR 40,000. Now, that should be a function of two or three things. One is that they're not doing high-end clinical pr

ograms like transplants, high -end oncology, etc. They have more medical patients. Their pay or mix is not very diverse . They don't have international patients. They don't have cash -paying patients, insurance patients to that extent, and whatever else it is. Substitution of institutional business isn't there. So, obviously, for them, there is no difference in the two.

Amit Kavani: But the question actually is that suppose someone has a hospital, let's say, in Ranchi, which is like a Tier 2 city. So, will the CGHS compensation to them be the same as another hospital in, Saket?

Yogesh Sareen : Almost the same. There are differences in the NCR price and non -NCR price, but I would say not a material difference.

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Abhay Soi: Negligible difference , basically the same. The only difference is, Ranchi, there won't be too many CGHS patients, right?

Amit Kavani: Got it. Got it. Got it. Thank you. Thank you, Abhay. Thank you.

Moderator: Thank you. The next question is a follow -up line from Kunal D hamesha from Macquarie.

Kunal Dhamesha: Thank you, sir. So, on the CGHS and self -pay ARPOB, we have said the difference of around 85%. Can you also quantify what would be the difference for the international patient s? So, let 's say CGHS is INR 100...

Yogesh Sareen : Well, international patients typically is 1.5x of the cash and insurance.

Kunal Dhamesha: So, could it be roughly around INR 250? Like, if CGHS is INR 100...

Abhay Soi: If CGHS is INR 100, cash and insurance is INR 185. This is 50% more than INR 185.

Kunal Dhamesha: Okay, okay. Perfect.. And these are the numbers for H -1, I would say? Or, like, more or less, this remains...

Abhay Soi : They're current numbers , running numbers .

Kunal Dhamesha: Current numbers. Okay, okay. And secondly, on CGHS, on institutional, you said that we are now taking higher complex procedures, etc. So, do we have that flexibility, you know, to choose on the specialty on CGHS or some of the institutional business?

Abhay Soi: Well, we inherently do, because some of our hospitals are now disengaged, and other hospitals which are engaged may not sometimes have those kinds of facilities .

Kunal Dhamesha: Okay.

Abhay Soi: There's a churn which happens.

Kunal Dhamesha: Okay, so, basically, some word of mouth, something, you know, more people...

Abhay Soi: Not word of mouth. Like for some of our hospitals, we 've stepped out of CGHS contracts . So, you start moving away from that.

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Kunal Dhamesha: So, we have the flexibility of, you know, saying no to other specialties, basically?

Abhay Soi: It's not a question of flexibility. It's a matter of contract. We've gone and told them that we can't treat patients, right? Our contract has been amended to that effect . It's not a flexibility that we have. If we have a flexibility for all of it, then you do all of it. You can't start anything. You can start for whatever arrangement you have.

Kunal Dhamesha: Okay. So, our contract is only for few specialties where we have, you know, strong base and more complex. Okay. Perfect. Thank you.

Moderator: Thank you. Ladies and gentlemen, that was the last question for the day. I would now like to hand the conference back to the management for the closing remarks.

Thank you, and over to you.

Abhay Soi: Thank you all for coming on to Max Network 's Q2 FY24 results. We look forward to seeing you for our next results as well. Thank you very much. Goodbye.

Call: Feb 2024

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February 7, 2024

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Sub.: Transcript of Earnings Call held on February 1 , 202 4
Ref.: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Sir / Madam,

Please find enclosed copy of transcript of earnings conference call, organized on February 1 , 202 4, on financial results of the Company for the quarter and nine months ended December 31, 2023.

The said transcript is also available on the website of the Company at [www.maxhealthcare.in/investors/investor -resources](http://www.maxhealthcare.in/investors/investor-resources) .

Kindly take the same on record.

Thanking you

Yours truly,
For Max Healthcare Institute Limited

Dhiraj Arora
SVP - Company Secretary and Compliance Officer

Encl.: As above

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Max Healthcare Institute Limited
Q3 & 9M FY24 Earnings Conference Call
February 01 , 2024

Moderator Ladies and gentlemen, good day and welcome to Max Healthcare Institute Limited Earnings Conference Call. Please note that this conference is being recorded.

I now hand the conference over to Mr. Suraj Digawalekar from CDR India . Thank you and over to you Suraj .

Suraj Digawalekar: Thank you, Michelle. Good morning , everyone and thank you for joining us on Max Healthcare 's Q3 and 9 M FY24 Earnings Conference Call.

We have with us Mr. Abhay Soi – Chairman and Managing Director, Mr. Yogesh Sareen – Senior Director and Chief Financial Officer, and Mr. Keshav Gupta – Senior Director – Growth , M&A and Business Planning.

We will begin the call with opening remarks from the management , following which we will have the forum open for an interactive Q&A session.

Before we start, I would like to point out that some statement made i n today's call may be forward -looking in nature and a disclaimer to this effect has been included in the earnings presentation shared with you earlier.

I would now like to invite Abhay to make his opening remarks.

Abhay Soi: A very good morning to everyone and a warm welcome to Max Healthcare 's Earnings Call for the 3rd Quarter .

This quarter has been exceptional for us , primarily for two reasons. First, it represents our 1 3th consecutive quarter of year -on-year growth. We recorde d a healthy increase in network revenue and EBITDA by 14% and 12% , respectively. And more importantly, we managed to grow ARPOB and EBITDA per bed sequentially, despite Q3 being a seasonally lean quarter.

Second, t his quarter marks our foray into central UP, which is one of the most populous and fast-growing states in the country , through the acquisition of 550 bed Sahara Hospital in Lucknow. This acquisition will fortify our presence in Northern India and is in line with our articulated strategy of entering new markets where peers have demonstrated success. We expect to consummate the transaction in Q4 and are ready with a plan of action to improve the infrastructure , augment the operational bed

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capacity , add to the existing medical tale nt as well as integrate the hospital with our IT systems.

On the clinical front , we have launched the CAR -T cell therapy at Max Vaishali in collaboration with ImmunoACT . Also known as a 'living drug ' , this groundbreaking therapy is expected to enhance the quality of life for many patients and reflects our continuous endeavor to provide our patients with the latest therapeutic options for cancer treatment.

Now, coming to the highlights of Q3 Performance :

1. Occupied bed days dipped marginally by around 1% year-on-year due to lower prevalence and spill -over of vector borne diseases in this quarter compared to last year. Average occupancy for the network was 73%.

2. Institutional bed share stood at 29 .5% compared to 29.4% last year, and 27.3% in Q2 this year. However, after excluding Max Shalimar Bagh , the overall institutional bed share stood at 27.4% during Q3 and occupied bed days were down by 8% for th is segment. Sequential increase institutional bed share was a conscious call taken owing to Q 3 being a seasonally weak quarter on occupancy.

3. Average revenue per occupied bed for the quarter improved to Rs.76,800 , growing by 15% year-on-year and 3% quarter -on-quarter . Year -on-year improvement was witnessed across all specialties with oncology, neurology and renal sciences being the key drivers.

4. Network gross revenue was around Rs. 1,779 crore compared to Rs. 1,559 crore in Q3 last year, and Rs. 1,827 crore in the previous quarter. This reflects an increase of 14% year-on-year, driven mainly by growth in ARPOB . Revenues declined by 3% quarter -on-quarter due to festive season in Q3 and vector -borne diseases led jump in admissio ns during Q2.

5. Revenue from international business grew significantly by 25% year-on-year. This pa yor channel now accounts for around 9.4% of the total revenue from our hospitals.

6. Network operating EBITDA stood at Rs. 471 crore reflecting a growth of 12% year-on-year and a decline of 5% quarter -on-quarter. Most importantly,

annualized EBITDA per bed rose to our highest ever of Rs.75.6 lakhs , clocking a growth of 13% year-on-year and 1% quarter -on-quarter . Operating EBITDA margin stood at 27.9% for the quarter .

7. Year-on-year growth in EBITDA was impacted due to additional charge of Rs.25 crore , driven by movement in provision for doubtful debts , reversal of provision for old phantom stock option plan in Q3 last year after launch of the new ESOP scheme in November 2022 , GST impact o f variable management fees and one - time litigation cost .

8. Max Shalimar Bagh, where we added 122 beds , recorded year-on-year growth of 36% and 42% in its revenue and EBITDA respectively , with an average occupancy of 74% .

9. Profit after tax was Rs.338 crore versus Rs. 269 crore in Q3 last year and at the same level as in the previous quarter . The year -on-year improvement o f 26% was primarily attributable to the flow through of improved EBITDA and lower finance costs.

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10. Free cash flow from operations generated this quarter amounted to Rs. 226 crore , after additional outlay of Rs. 40 crore for purchase of robotic systems at four of our hospitals . Of this , Rs. 137 crore was deployed towards the ongoing capacity expansion projects and Rs. 97 crore was distributed as dividend.

11. Net cash position improved to Rs. 1,295 crore at the end of December 2023 compared to Rs. 372 crore same time last year.

12. Continuing our effort to support the local communities , we treated approximately 36,700 patients in OPD and 1 ,250 patients in IPD from economically weaker sections of society absolutely and entirely free of charge.

13. Both our strategic business units continue to maintain growth momentum .

Max@Home reported a top line of Rs. 44 crore reflecting a strong growth of 24% year-on-year and 5% quarte

r -on-quarter. We continue to witness good demand for this SBU's services as d emonstrated by over 50% repeat transactions by patients in the last one year.

Max Lab, the non -captive pathology vertical , now offers its services in 41 cities and has a network of over 1 ,000 collection centers and active partners. This SBU reported a gross revenue of Rs. 34 crore , reflecting a growth of 20% year-on-year.

14. On the status of our expansion projects :

For 300 beds at Dwarka – As informed previously, application for occupancy certificate was submitted in October . Finishing is under progress . In the meanwhile, we already have the unit head and all other key functional heads in place. We have commenced hiring for middle -level staff as well.

For the 329 b eds at Nanavati – Basement and ground level structures have just been completed , while steel fabrication above ground floor has begun. The project continues to be largely on schedule .

For 300 beds at Sector -56 Gurgaon – Approval for structural drawing has been received in the first week of Jan and RCC works have already commenced.

For 190 beds at Mohali – Design development is under finalization . In the meanwhile , the base raft concrete activities have started.

Both of these are largely on schedule.

For 350 beds at Max Smart (Saket C omplex) – Tree transplantation work is underway , with 159 trees transplanted and balance 316 trees in process for transplantation . As per plan , existing structures have been demolished and shoring work for sewage and water treatment plants is going on. Barring the initial delay of 6-7 months th e project is on schedule now.

For 300 beds at Max Vikrant (Saket Complex) – The building plans have been resubmitted to the Municipal Corporation of Delhi post their initial review and the dues for water and sew er infrastructure charges have been paid. Tender documents are under review for floating to the contractors. Application for NOC

by Forest department has also been filed .

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For 250 beds at Patparganj – The fire department issued NOC for the building plans and municipal corporation approval is in progress.

All projects continue to be largely on schedule despite some disturbances linked to enforcement of GRAP (Graded Response Action Plan) in NCR to combat air quality issues.

Finally, moving on to the overview of the company performance for the nine months ended December 31, 2023 :

15. Network gross revenue stood at Rs. 5,325 crore , reflecting a growth of 16% year-on-year.

16. Network operating EBITDA grew by 17% year-on-year to Rs. 1,404 crore . Increased ARPOB , improved case mix and augmentation of network bed capacity by 146 beds translated into improvement in EBITDA per bed by 15% to Rs. 73.8 lakhs per bed .

17. During the nine months we generated Rs. 924 crore of free cash flow from operations , after interest , working capital changes and routine CAPEX , of which Rs. 265 crore has been deployed towards ongoing expansion projects and Rs. 97 crore was distributed as dividend.

With this will open the floor for questions and answers .

Moderator: Thank you. The first question is from the line of Damayanti Kerai from HSBC Securities and Capital Markets .

Damayanti Kerai: My first question is on this institutional tariff revision , which we saw in the month of December. So, you mentioned there will be net loss of around Rs. 6 crore annually , is this like ongoing and just want to understand if this wouldn't have happened the net gain which you mentioned for 3rd Quarter Rs. 14 crore should we have added this Rs. 6 crore back, Rs. 14 + Rs. 6 if the prices were not declined?

Yogesh Sareen : No, Rs. 14 crore is for the quarter. That is the run rate , which also has some impact of this price revision that happened in the month of December. So, around one week 's impact is already reflecting there. Rs. 6 crore number is an annualized number , while

Rs. 14 crore is a quarterly number.

Abhay Soi: Coming to your question , if you didn't have the Rs. 6 crore then you would have added this to the EBITDA for this quarter, which would be around Rs. 1.5 crore .

Damayanti Kerai: Okay. And do you expect any further revision in tariffs for this channel or do you think like now government has broadly done and we might not see any additional ?

Abhay Soi: We are expecting revisions in tariff. We think that even the PET CT revision has been erroneously done and we are in talks with them to revise the same .

Damayanti Kerai: Okay, so further revisions can come right and then , that should be like maybe beneficial to our institutional channel.

Abhay Soi : We expect that even this PET CT tariff should get revised the other way. We believe it was an error.

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Damayanti Kerai: Okay, Rs. 10,000 went to some Rs. 11,500 or something, right?

Yogesh Sareen : No, the tariff for PET CT has come down by around 50%, from Rs. 22,500 to around Rs. 11,000. So, it is 50% reduction and that has a major impact because oncology happens to be a big share of revenue for us. There is Rs.10 crore downward impact for the PET CT , while the cardiac tariff has gone up by around Rs. 4 crore . The overall net impact is Rs. 6 crore and these are all annualized numbers .

Damayanti Kerai: Okay, understood . And Abhay , a few quarters back you talked about this Heal in India initiative, which should be giving good boost to your international business. So, any update on that front or any discussions ongoing around that initiative ?

Abhay Soi: What they have done is that there has been a lot of movement in the embassies , etc.

They're fast-tracking medical visas, they've taken off those policies on reporting to police stations, they have brought down the price of the medical visas and are generally promoting it globally . The government of India is participating in various medical tourism forums. And this is leading to a trickle effect right now . Trickle effect meaning that it's a growth of 25% Y-o-Y for us . In some manner , just given the size of the opportunity , it's not exponential yet but it's still incremental at 25% CAGR on this channel .

Damayanti Kerai: Okay. And from March perspective right now it's around 9.4% revenue contributed by international patients, how much it can go up in say next three years, even if we don't

hear any big changes by the government and these kinds of small regular improvements happen?

Abhay Soi: It's growing at 2.5% CAGR. And it's not just this quarter but last couple of quarters it has been growing at this pace, so I don't really see it abating.

Yogesh Sareen: If the overall revenue growth is 14% and this channel is doing 25%, then obviously this share should go up going forward.

Damayanti Kerai: Got it. And my last question is, as you mentioned, oncology is a big focus for you and you have been investing in this segment. So, which are, I will say, key growth opportunities as per Max focus, and then what kind of expectation you have from this segment, in terms of addition to your hospital revenues, over the next few years?

Abhay Soi: There are three streams of oncology – surgical, medical and radiation. There are various sort of innovations happening within these three. If you look at surgical, with more robotics happening, it translates to better ALOS and better outcomes. At the same time, while the percentage margins are lower, but absolute value to hospitals is more because in terms of the EBITDA per bed that you are able to derive from that is higher than what you do for normal oncology surgery. And robotics number has sort of doubled for us over last year and we are seeing major growth in this. Now, similarly if you look at radiation, from a typical LINAC you have gone into various kind of innovations. And there have

been other kind of innovations within medical oncology

as well. But now, we are also looking at immunotherapy, like we have mentioned that we started CAR-T cell therapy recently. This is another stream altogether, which is a sub segment of chemo therapy. Globally, it's a little expensive right now and we are seeing prices come down so adaptability of that is going to be a lot more going forward. But that's what the world is moving towards is oncology, its immunotherapy now.

Moderator: Thank you. The next question is from the line of Harith Ahamed from Avendus Spark.

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Harith Ahamed: My first question is on the recently acquired Lucknow hospital. So, currently, we have around 250 operational beds there and you talked about the capacity of 550 beds.

So, can you share your thought process in terms of making the operational bed count higher more towards the capacity's headcount?

Abhay Soi: The amount of area, which has already been constructed, is close to about 1 million square feet and the number of beds operational are 250 so, there's obviously an opportunity to ramp up to 550 as intended originally within this area. The facility has been ignored for a while, so we are going to be changing the equipment and redo some of the infrastructure. When I say redo the infrastructure, it is basically internal renovation that needs to be done. It's a brand-new building made by L & T and designed by Hafeez Contractor, started in 2008-2009. It's not an old structure but we need to renovate it to some extent. Within a couple of years, we should be at that 550-mark, perhaps a little before that. Please do keep in mind that we in the process of closing the transaction. We are not in the saddle as yet. Majority of the conditions precedent (CPs) as per schedule have been done and the rest are also on schedule. We will be able to share more details about this facility once all the CPs are completed.

Harith Ahamed: Okay. My second question is on the timelines of some of the upcoming hospitals. So, apart from Dwarka, you have guided for three other hospitals, getting commissioned in the next 12 to 15 months. Mohali, Nanavati and Gurgaon but when I look at the project CAPEX specifically, we are tracking a bit lower versus our guidance of Rs.900 crore for FY24, we are at around Rs.300 crore nine months of FY24 so, are we still expecting these three hospitals other than Dwarka to get commissioned in FY 25?

Abhay Soi: Absolutely. So, Dwarka which is getting commissioned, do keep in mind it is off our books, because this was the asset light model. The developer is investing the money, whereas we are bringing the medical equipment in place. The hospital is complete in all respects and we are right now in the phase where we are doing the finishes to the hospital. And, sort of whatever medical equipment that we brought in, we are implementing it. It's not going to be a major CAPEX for us going forward either as far as Dwarka is concerned. It was never meant to be because it is asset light, where we are essentially paying a lease amount for that at a yield of 6.5-7% p.a. It is a sixty-year contract for finished land and building of 300 beds. As far as Nanavati is concerned, we've already built our three basements and we've come to ground level. Now, this is where things start speeding up. Please do keep in mind that the first part of construction is the superstructure, which doesn't really cost that much money. If you were to build, it typically costs Rs.1500 to Rs.1800 per square foot. So, if you are building even a million square feet, then you would have spent like Rs.120 crore just

doing the construction of the superstructure. Real money is spent towards the latter part of it when you go into finishing , interiors, equipment , lifts, air conditioning, etc. So, we are on schedule and as far as the payments is co

ncerned, I would not look at

that since the payments are always lumpy towards the end . So, I would not look at that as a surrogate for completion of projects, at least at this stage.

Harith Ahamed: Okay, last one with your permission . Slightly broader question , when I look at the Saket C omplex currently. And if I am correct, we have around 800 beds there. And in the next three years we are looking to add 800 , 900 beds and we have plans to add maybe 500, 600 beds beyond FY 28. So, that's almost 2000 beds in almost a single location. So, it's kind of unheard of so can you share your thought process behind such a large number of beds at a single location . How are we going to differentiate and how do we plan to whatever strategy to ensure that there is no overcapacity situation in that ?

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Abhay Soi: Firstly, Saket C omplex is the main flagship as you are aware . We are pretty much tearing at the seams as far as Saket is concerned. We've added 40-50% more capacity in a place like Shalimar Bagh , and within the first month or so, our occupancy went up by about 80 -85%. These 800-1000 beds that you are speaking of is not coming all together, as you may appreciate. First, the 300 odd beds come into play. My belief is , when we add 40% capacity on top of the current capacity of 770 beds that we have, those will be more or less instantly taken up. We could have a very quick occupancy of the first 300 beds. And very soon thereafter we are going to be ready for additional capacity . Like in the case of Shalimar Bagh , we have already run out of capacity. So, even over here we expect to have the same situation and will require the next 300 beds. So, w hat we can't do is start constructing 300 beds then and take another three years since we will need those 300 beds once these first 300 beds are occu pied. And that really speaks for the first year. Now then comes the last phase of let's say another 300 beds thereafter. Now that only when these 300 beds , the second 300 beds get occupied, we will look at the last piece . We will make the superstructure, which I again mentioned is not a major cost . This is if things are going as per plan , but we can always stall . We can, at the last 300 beds, always say look, we are completing the superstructure and the external façade but we are not fitting it out if we see any sort of softness of demand . Our belief is that this is the trajectory that we have over the next five to six years, and we will be able to occupy all those beds . But it does not mean that after phase one, if, let's say for some reason, the first 300 beds don't get occupied, we are not going to fit out the next year 300 beds. We will never build the next 300 beds.

Harith Ahamed: Okay.

Abhay Soi: This is the base case . At various stages and at various places, these are the default options . It's a running hospital , so we can't have a disruptive process . The superstructure cost isn't a lot . Again, i f you're looking at a million square feet , it is only Rs. 150-180 crore for that matter .

Yogesh Sareen : Also, FY 28 onwards is a placeholder . That's the potential we have , but we are not investing anything for the FY 28 capacity now. We don't even have the approval s from the Board to spend on that. It only reflects that we can build those many more beds on those land parcels .

Abhay Soi: Theoretically , in Lucknow, it is 27 acres of land so you can build another 3500 beds . That's the potential. So, you see the potential post 2028 up to infinity , but it does not mean we will be building 3,500 beds there .

Moderator: Thank you. The next question is from the line of Neha Man puria from Bank of America .

Neha Manpuria: Abhay , on the Lucknow acquisition, how should we think about narrowing the operating performance gap that is there between , let's say , what Max is doing now . In Lucknow particularly since we wil

I continue to ramp up the beds, is it fair to assume that we need to get to that full 550 to narrow that gap meaningfully and it would happen over a period of time ?

Abhay Soi: We are quite confident that within a couple of years we will be able to get all of these beds in the condition or perhaps better than what we have in any other hospital and be able to catch up with our peers as well in terms of every financial parameter, if not exceed .

Neha Manpuria: Okay, but this would take a few years in your view ?

Abhay Soi: It should be done within two years.

Neha Manpuria : Got it. And second, just extending the Lucknow question to a broader strategy , now that we have this, we will have 500 beds, you said you have land for adding more capacity. But do you see that as a market where you can have let's say, multiple facilities and is that something that you're looking at this could be a third hub outside of let's say the current two hubs that you have?

Abhay Soi: Absolutely. So, we are looking at Lucknow just to find certainly many more healthcare facilities. UP's population is the size of Europe and literally it's got only one and a half corporate hospitals over there right now as we speak. Besides, hopefully, when we get in , we will become two and a half. But I don't know if you've been there recently, but the level of development, connectivity, roads , infrastructure , the kind of focus that city is getting , I think it's going to be a metro very soon . And just the scale of the infrastructure, the airport , connectivity with other areas through rail, air, road, every which means . On top of that, the government itself has a very strong program . The Chief Minister 's Relief Fund over there actually pays hospital tariffs for patients. So, you can imagine there is an entire population where the Chief Minister 's Relief Fund is paying hospital tariffs for patients at the hospitals . Not the CGHS rates or some other low rates. It certainly justifies more than one hospital on multiple fronts .

Neha Manpuria: And this would be through M&A, do you think to just make it happen faster or would you choose a more organic approach of expanding into that market?

Abhay Soi: Unfortunately, I don't think there are too many M&A opportunities or existing hospitals in Lucknow. They are far and few, if we are able to get lucky again like we have got in this case . Of course , we will look at an acquisition first because that's faster, but if not, then we will look at organic growth as well.

Neha Manpuria: Right. And my second question is on Dwarka, you mentioned that the unit head and key function heads have already been appointed , is that cost already sitting in the quarter because if I look at our direct , hasn't increased as much in the last three quarters ?

Abhay Soi: That's right, that cost is sitting on our books. Also, certain doctors have been on boarded and that cost may also be sitting in this quarter.

Neha Manpuria: Okay. So, as we commission this and phase it up, you should start seeing that contribute EBITDA pretty quickly ?

Abhay Soi: That's right , because that cost is already incurred. The biggest cost is always the non - medical personnel , like we have always mentioned , around 23% or 24% of the overall revenue . In fact, for a new hospital it's higher because revenues are lower. It's basically the significant functional heads that you have under that cost . And that comes in closer to the hospital commissioning, which is now . Very shortly we should have that hospital up and running.

Neha Manpuria: And for breakeven in Dwarka is 12 months, 18 months a good timeframe to assume ?

Abhay Soi: Well, that's the textbook. We are hoping to do it faster .

Moderator: Thank you. The next question is on the line of Kunal Dhamesha from Macquarie .

Kunal Dhamesha: So, first one on the institutional rates there has been

some revision upward, some

revision downward, but when we sum it up, let's say for the first nine months what

could have been the ARPOB impact in that channel . For the first half we had suggested the institutional ARPOB has grown at 28% year-on-year, if you could provide that number for the first nine months ?

Abhay Soi: In my knowledge , it's been revised upwards on everything except for one item, which went down and we believe it is a mistake , so we have made appropriate representation.

Yogesh Sareen : Kunal, you have the number with you . The price increase is Rs. 14 crore per quarter and there is a Rs. 6 crore annualized impact of the downward revision . So, that means the net impact is Rs.50 crore p.a. on the institutional business in terms of price . So, if you divide it with the numbers, you will get the increase in the ARPOB rate, which will be around 3 -4%.

Yogesh Sareen : In the overall ARPOB , the impact is less than 1% .

Kunal Dhamesha: No, but last quarter we had said that institutional ARPOB increased by 28% ?

Yogesh Sareen : We are talking about the overall ARPOB impact , which is less than 1%.

Kunal Dhamesha: No, but if our institutional revenue is around 18% and that revenue is growing at 30%, then the weighted average impact should be around 5% , right?

Yogesh Sareen : Let's not confuse ourselves. First of all, the total impact is Rs. 50 crore as things stand today. On the overall top line, it has an impact of less than 1% . Our top line last year was around Rs. 6,200 crore, so if we take Rs. 50 crore on this, it will be less than 1%.

That's on the overall ARPOB . Now, if we talk about the ARPOB of the institutional al channel itself – on around Rs.1200 crore of business in a year from institutional , there is a Rs. 50 crore impact , so it will be a round 4%.

Kunal Dhamesha: Yes, this is clear but last quarter ...

Yogesh Sareen : The last quarter growth in ARPOB is not only price , we never said that it is all because of price.

Abhay Soi: There has been a growth in institutional ARPOB , but we haven't said it's driven only by price .

Yogesh Sareen Even this quarter if you see , the price impact is only 4% and rest is the business mix. Also, you should understand that when onco logy goes up , the chemo therapy goes up, the ARPOB improves .

Abhay Soi: We have been focusing even with institutional on the higher end specialty . That's the outcome you see .

Kunal Dhamesha: Sure. And the 15% ARPOB growth that we have seen, first half was also 13%, 14%.

Going into next year what is , is that what your kind of would be thinking about in terms of ARPOB growth, because clearly these numbers are in my view at least is above the historical trend line. And we have been doing high end surgeries , robotic surgeries . Do you think that this can sustain this run rate of ARPOB growth for the next two to three years?

Abhay Soi: We keep asking the same question of whether there any one-time price change that has led to this ? In my mind , there are a few factors which have led to higher ARPOB .

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One is more higher end surgeries , second is international patients , third is proliferation of insurance, etc. These are things which are leading to higher ARPOB and if any of these is non-secular , then your guess is as good as mine. My belief has been that , they seem pretty secular and that should read into next year as well. None of this is based on one time price changes, etc.

Yogesh Sareen : It's not only us. You can see the competition , ARPOB has been improving for all other hospital chains also.

Abhay Soi: Unless there's a non -secular trend which has been propelling it. But , I'm at a loss of what it could be.

Kunal Dhamesha: And sir one logistic question on the international patient bed share this quarter , if you can share it?

Yogesh Sareen : So, it w

ill be around 5.8%.

Moderator: Thank you. The next question is from the line of Ankit Shah from Canara Robeco AMC .

Ankit Shah: So, my question pertains to the Dwarka additions. So, you mentioned that some of the cost s have started comin g, but the major portion of the non -medical cost s will come going forward. So, can you give some sens e of like, how much of that non -medical cost would be of the total cost . And also, you had mentioned earlier that the total losses initially in this should not exceed Rs. 30-40 crore so can you give some sense on how this, would this be completely front ended or cost come in a stepwise fashion so it will be more divided , some granularity on that front?

Abhay Soi: I feel it wil l grade down , so it will be in a decreasing fashion . Of course, the first month is more , the second month should be similar and then it sort of starts peter ing down till you hit breakeven. So, the textbook would say that this size of the hospital should have around Rs.40-50 crore loss, but we hope to perform better and maybe break even hopefully sooner. This is hard to project , because as and when the doctors come in and those particular programs kick in , certain doctors will be brough t in with certain minimum guarantees, and others with certain other minimum guarantees, etc. So, in order to give you that loss figure and what is the trajectory of the loss, we need to figure out what are the exact revenue figures going to be in that particular month.

Ankit Shah: So, on the hiring front, does the hiring happen in one shot for the entire capacity or does it happen in stepwise fashion ?

Abhay Soi: No, we are going to be coming up with around 160 b eds first, and of which , let's say, we want to operate around 100 beds , then in the first go we will be hiring at least for 100 beds . Unless we get enough of those kinds of clinical programs to kick in where we start off with 160 beds . Right now, we are only doing this much but let's say if there are 20 letters of intent (LOIs) , which we have issued to 20 various doctors, and 12-15 of them have joined while others are in the process , it's in the works , right? So, as and when we have it in hand, we know this doctor is joining , we need to get him enough nurses and resident doctors to support him.

Moderator: Thank you. The next question is from the line of Tushar Manudhane from Motilal Oswal Financial Services .

Tushar Manudhane: Sir just on the international patient s would like to understand the growth maybe for the quarter and as well as for the nine months , if you could break down into volume and maybe the realization growth ?

Abhay Soi: The growth has been 25% both for nine months as well as the quarter .

Yogesh Sareen : Yes, so the ARPOB has grown by 9% in the quarter, but the bed share is 5.8% and it is flat overall Q -o-Q. Revenue from this channel was Rs. 158 crore last quarter and Rs. 157 crore this quarter.

Tushar Manudhane: Right. So, the 9% increase in realization is to do with case mix or it's like the similar treatment ?

Yogesh Sareen : There is no price element in this , so it is basically case mix . We haven't increased the prices . Typically , the price of international patients is linked to the self-pay patients , which gets revised only in the month of April. To that extent, there is no price element in this, it is only the case mix which is increas ing the ARPOB by 9%.

Tushar Manudhane: Understood. And secondly, just on the overall operational cost, in terms of let's say employee cost and other expenses for FY 25 in particular were considering the different new hospitals coming up. So, what kind of cost increase one should think about?

Yogesh Sareen : We can tell you about the cost increase on the existing hospitals, which is typically an increment of around 7%. And then if any additional bed is getting created

, there will

be manning required . So, typical manning would be six people to one bed. If we are adding 100 beds to the operational capacity , you should assume that the re will be six hundred more peop le. I can't give you exact number of the growth, etc., but I'm giving you the math to get to that number.

Tushar Manudhane: Got it. So, effectively trying to understand considering the scope of ARPOB growth as well, but at the same time addition of cost on account of , at the existing sites, as well as new hospital getting added. So, is there a scope for f urther margin improvement in FY 25, or we would be let's say having bed related revenue growth, but margin would be pretty stable, is that the way to understand or there is a scope for margi n improvement even in FY 25?

Abhay Soi: No, you are mixing two things. One is our current capacity , which is about 3,500 beds .

There is a certain cost growth and certain revenue growth. Assuming the revenue growth continues on the same trajectory , and then there is cost growth , which would be similar to what happened last year. So, EBITDA growth in that fashion should be currently what is there and plus, because you get some operating leverage as well. So, whatever EBITDA growth that you for existing hospitals , you will continue having that. Then comes the new hospital s. The new hospital will be, let's say 300 beds, which is about 7% to 8% further c apacity . Those 300 new beds on day one will start with the lower ARPOB and negative EBITDA . Then the re is the question of when do you break even – six months, eight months or 12 months ? If, say, you breakeven even in 12 months , then it will have a negative impact on the EBITDA for the year . If you breakeven in six months, then it may have a neutral impact on your EBITDA one year later. Even a neutral impac t will still pull the margin down because if it's zero, you are taking the positive EBITDA and you are taking the negligible EBITDA on an increased revenue base . So, you have got two conflating kinds of wins happening over here. But my belief is because it's only 8% bed addition on the overall bed base , it doesn't move the needle too much.

Moderator: Thank you. The next question is from the line of Karan Gupta from Varanium Capital .

Karan Gupta: So, basically my question is related to international business. So, what kinds of attraction we are seeing in the television for medical tourism and what systems we are taking to increase the traction , or maybe increase or improve the quality of our treatments . I am as king this question, because in India the treatment cost is 1/5 of what the US and Europe is. So, what's the traction coming from US and Europe , because basically you said in your early con-calls that the major traction is coming from Bangladesh . So, what's the position of these two US and Europe?

Abhay Soi: We don't get many patients from US and UK because even though India is much cheaper, but in US and UK, medical treatment is sponsored . For instance, NHS pays for it in UK . And in the US also , it's the government . So, it doesn't matter if it's expensive since somebody else is paying for it. It has never been our case that people from developed countries will come to India . You will only see people come to India from countries where you have self-paying patient s.

Karan Gupta: Okay. So, when you talk about medical tourism, where are the patients coming from ?

Abhay Soi: We're getting patients from around 145 countries.

Moderator: Thank you. The next question is from line of Amit Kadam from Canara Robeco Mutual Fund .

Amit Kadam: Just couple of things , the swing what we saw in the inpatient volumes this quarter, largely you attributed to the vector borne disease , just wanted to know because we have a very limited understanding like in last three years or so , we haven't see

n such

kind of large swing because the bed days for this quarter is -1%. So, can you just throw from your past experience we had seen such kind of large swing which could have a material impact on the inpatient volumes, and which could result in bed days ?

Abhay Soi: What we are seeing is that last year same quarter , we had a big influx of the dengue patients . We haven't had the same level of influx this year. Therefore, there is a reduction of 1% on occupied bed days (OBDs).

Yogesh Sareen : If you see the historical trends, you will find that from Q2 to Q3, there is always a decline . If you pick up any competitor's previous reports of Q2 and Q3 of previous years, you'll find that Q3 is always softer . And therefore -1% OBD is very normal. If you compare Apollo , Fortis, Narayana , and you pick up their last year results , you will find that there's a decline in Q3 over Q2. There is no exception here. This is the secular trend, and you will always find Q3 is down compared to Q2 because of the festivities – Diwali, Dussehra , etc. Now, in our case one peculiar feature is that in last year, especially in Delhi, we had an influx of dengue patients in October . Typically , dengue season finishes by September, but last year, we had patients even in October , which was a Diwali month . We had dengue patients and this Diwali month's occupancy was 76%, which is unheard of. In a Diwali month, you will usually have an occupancy of less than 70%. So, last year October was exceptionally high on occupancy because of dengue , and that is the reason why you see a sharper drop in OBDs . Otherwise, it would have been probably flat , if we were to take out the drop in internal medicine and pediatrics , where we get these dengue patients admitted . The drop in these specialities is 12% , while you see that Y-o-Y the drop in OBDs is only by 1%. But these particular specialities, which is 22% of the overall bed share dropped by 12% . So, what we are trying to say is that there is nothing operationally wrong in this quarter. It's only when you compare Y-o-Y because of this October influx of patients , there looks to be a sharp drop, rather than what it should actually look like.

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So, if you adjusted for this influx, then you'll find that the OBDs actually went up Y-o-Y.

Abhay Soi: It is a seasonal business . Q2 and Q4 are the strongest quarters in the hospital business, while Q1 and Q3 are weak quarters. Q1 because the cost structure moves up and Q3 because the revenue comes down.

Yogesh Sareen : Is that clear Amit?

Amit Kadam: Yes. No, I was just not contesting your that particular point. I just wanted to know because we have a very limited history because we got , so from the data point we did not have , I did not have the insight that whether such kind of some seasonality , when I'm talking seasonality , I'm trying to compare Y -o-Y because of some like vector borne disease, we have a material impact on the volume growth Y-o-Y. I can understand how Q2 to Q3 seasonality works. But I was just saying in the past history whether such kind of events like vector borne is not there, could have a material impact on the volume growth just wanted to.

Abhay Soi : 100% , it does . And it's not an anomaly .

Yogesh Sareen: And it depends on when the rains happen, when this epidemic type stuff happens .

Abhay Soi: You are not comparing 9-months to 9-months , you are only comparing this quarter to last quarter Y-o-Y. On an overall annual basis, it will be not a significant impact on the occupancy .

Amit Kadam: Right that I get it .

Abhay Soi: But yes, say if you don't have a dengue season , the drop can be much sharper than this, in fact much sharper than the 1%. Between 1% to 3% fluctuation on both sides, positive as well as negative.

Amit Kadam: Okay, got it sir. And this on the outpatient thing is it just a n outcome of the

similar

effect that because there were lesser patients visiting because of the there was no dengue disease ?

Abhay Soi: That's right.

Amit Kadam: So, nothing much to read in this particular thing based on this particular like because I've earlier seen this outpatient at least growing at mind -teens or kind of thing, but

now it showed 3% in this particular quarter there is nothing much to read .

Abhay Soi: One thing you are rightly doing is, you are looking at it Y-o-Y rather than Q-o-Q, because we have seasonality in this. So, Q3 in any case is weaker than Q2. But if you look at it on Y-o-Y basis , you can attribute it to the number of dengue patients .

Amit Kadam: Okay. And just one more final point is on the Lucknow thing. So, as per the current timelines we should be able to integrate that particular business financials to ours by quarter one next year FY25?

Yogesh Sareen: Yes. It depends on when does the transaction closing happens . Say, even if the closing happens in this month , then we'll have to consolidate it from the date of closing . We don't have a choice .

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Moderator: Thank you. Ladies and gentlemen, this will be the last question for today, which is from the line of Alankar Garude from Kotak Institutional Equities.

Alankar Garude: Sir on that question on ARPOB , we have reported a 15% ARPOB CAGR over the last three years. And you alluded various factors – case mix , international , institutional insurance, et cetera . Now, would it be fair to say that a larger chunk of this improvement can be attributed to an improved case mix?

Yogesh Sareen: So, Alankar you know the elements of ARPOB increase – one is the increase in the prices, which we've been saying that it can be up to 3%. Then there is an increase in OPDs – you know that OPD revenue increase will improve the ARPOB because you don't consume any beds for those OPD revenues. So, that typically contributes around 2%. So, this makes $3 + 2 = 5\%$. Then we have growth in oncology . In our case , you have seen that the oncology growth is outpacing the overall growth. Oncology happens to be a high-end specialty , so that typically adds another 2-3% to the ARPOB . And then comes this whole channel mix and the case mix . If you take oncology out of the case mix , then there is improvement in balance specialties as well and the channel mix . This typically has been contributing around 6-7% increase , and we don't see any reason why it should not continue going forward. From our perspective , if there's a 9-10% growth in ARPOB , that should be reasonable.

Abhay Soi: But, having said that , you have to keep in mind one thing that we are talking about a healthcare system where there is capacity constraint in some manner . You have got occupancy levels that are pretty much at the threshold. When you have this situation, then, typically, you will see a higher uptick on ARPOB vis-à-vis occupancy because you can't move up on occupancy as there aren't so many beds available . So, more and more patients get pushed to daycare, all smaller surgeries compared to larger surgeries. So, you would schedule the larger surgeries first . The smaller surgeries would be scheduled at or the appointments are given at later dates, etc., so some of the later date patients may evaporate. So, what you see is that at the quality of work starts moving up. But at the same time, if I had a magic wand in my hand and I could put up beds adjacent to each one of the hospitals, then what you would see is the higher uptick on occupancy and lower on ARPOB . There is always an interplay between the two.

Alankar Garude: That's helpful . Maybe a question linked to that is , as we double our bed count over the next four , five years would the case mix for the newer beds be as good as our flagship hospitals right from year one or the ramp up in our case mix would be ?

Abhay Soi :

You can look at expansion akin to Shalimar Bagh , where we did a brownfield, since 85-90% of the expansion is brownfield essentially. If you look at Shalimar Bagh where we added 40% or more beds, and you see our experience over there, the new beds got taken up more by the institutional business , which we are bringing down . So, therefore, if you see our institutional business , apples to apples, year-on-year, it is the same but if we look at it excl. Shalimar Bagh , then we are far better in the institutional, because overall institutional business has come down . But what it means is that even if your ARPOB is lower but your operating leverage is so significant , that your EBITDA per bed that comes out of from the incremental beds is significantly better. And in fact, the operating margin even from these beds is higher than the existing beds.

Alankar Garude: Understood. My second question is, you spoke about the potential of Lucknow and Uttar Pradesh for us in general earlier. Now, when it comes to expansion beyond our already announced plans, you said earlier that we are looking at some 20, 21 cities for potential expansion. Will we continue to aggressively look out for acquisition

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opportunities in these 20 odd cities now, or do our priorities get aligned more towards Uttar Pradesh now with the Lucknow entry ?

Abhay Soi: No. Our priorities don't get aligned towards UP from that standpoint, because it's not as if we are going to get two or three acquisitions in a place like Lucknow . Even if we have a strategy of looking at building it out, it's going to still take some time . It takes years , while we are looking at deploying our balance sheet today. And we have the wherewithal to do it, for us to add 3 to 4 hospitals in a space of one to two years is not a big deal because rest of the expansion that we are doing is essentially brownfield in any case . It doesn't tax us more . It takes us as much effort to do expansion on Shalimar Bagh as operating Shalimar Bagh , and similarly, wherever we are doing brownfields . Both from financial and human resources standpoint, we are quite comfortable with what we are doing and we are prepared to go to the other cities as well. We are actively looking at it, but not aggressively . We don't like to do things aggressively. But we are actively pursuing opportunities in other cities as well.

Moderator: Thank you. As that was the last question for today. I would now like to hand the conference over to the management for closing comments . Over to you.

Abhay Soi: Thank you all for being on the call , yet again for Q3 and we are looking forward to a robust Q4, as usually is the case because of the seasonality. Thank you so much .

Call: May 2024

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(CIN: L72200MH2001PLC322854)

May 29, 2024

Listing Department ,
National Stock Exchange of India Limited
Exchange Plaza, Plot C -1, Block G,
Bandra Kurla Complex, Bandra (E),
Mumbai – 400 051

Symbol: MAXHEALTH Listing Department,
BSE Limited
Phiroze Jeejeebhoy Towers,
Dalal Street,
Mumbai – 400 001

Scrip Code: 543220

Sub.: Transcript of Earnings Call held on May 23 , 202 4
Ref.: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Sir / Madam,

Please find enclosed copy of transcript of earnings conference call, organized on May 23 , 202 4, on financial results of the Company for the quarter and financial year ended March 31, 202 4.

The said transcript is also available on the website of the Company at [www.maxhealthcare.in/investors/investor -resources](http://www.maxhealthcare.in/investors/investor-resources) .

Kindly take the same on record.

Thanking you

Yours truly,
For Max Healthcare Institute Limited

Dhiraj Arora
SVP - Company Secretary and Compliance Officer

Encl.: As above

Moderator: Ladies and gentlemen, good day, and welcome to Max Healthcare Institute Limited Earnings Conference Call. Please note that this conference is being recorded. I now hand the conference over to Mr. Anoop Poojari from CDR India. Thank you, and over to you.

Anoop Poojari: Thank you. Good evening, everyone, and thank you for joining us on Max Healthcare's Q4 and FY24 Earnings Conference Call. We have with us Mr. Abhay Soi, Chairman and Managing Director; Mr. Yogesh Sareen, Senior Director & Chief Financial Officer; and Mr. Keshav Gupta, Senior Director – Growth, M&A and Business Planning of the Company. We will begin the call with opening remarks from the management, following which we'll have the forum open for an interactive question-and-answer session. Before we start, I would like to point out that some statements made in today's call may be forward-looking in nature, and a disclaimer to this effect has been included in the earnings presentation shared with you earlier.

I would now like to invite Abhay to make his opening remarks.

Abhay Soi: A very good afternoon to everyone, and a warm welcome to Max Healthcare's earnings call for the last quarter of fiscal 2024. This has been yet another noteworthy year for us. We witnessed a robust growth in revenue and profitability, with both gross revenue and operating EBITDA registering 16% to 17% growth year-on-year. We touched all-time highs for all our key operating and financial metrics including ARPOB, margin and EBITDA per bed. This year also marked our foray into 2 new cities, namely Nagpur and Lucknow, adding ~750 beds to our existing bed capacity with further expansion potential. Both these cities represent key markets in the fast-growing and populous states of Maharashtra and Uttar Pradesh. We consummated the Nagpur transaction on February 9 for a net consideration of Rs. 395 crore, financed through internal accruals and QIP funds. While the Lucknow transaction was consummated on March 7 for a net consideration of Rs. 993 crore, with Rs. 600 crore being

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financed through external debt. We have already embarked upon the performance improvement journey for these hospitals, which include revamping of existing infrastructure, expansion of bed capacity, strengthening of clinical talent and establishing a robust sales & marketing programme.

These hospitals have been renamed as Max Super Specialty Hospital, Lucknow and Max Super Specialty Hospital, Nagpur. We expect them to be the key drivers for future growth in these regions.

In addition, we have acquired ~5.4 acres of prime land at Shaheed path in Lucknow with the potential to build ~550 beds. The plans for this hospital will be firmed up once we turn around the operations of MSSH Lucknow.

Now coming to the Q4 performance – this is our 14th consecutive quarter of year-on-year growth.

The new hospitals have added Rs. 42 crore of revenue, Rs. 3 crore of EBITDA and a net loss of Rs. 11 crore during Q4, including one-time transaction expenses. Consequently, overall Network gross revenue stood at Rs. 1,890 crore, registering a growth of 15% year-on-year and 6% quarter-on-quarter. While Network operating EBITDA was Rs. 503 crore, a growth of 15% year-on-year and 7% quarter-on-quarter. PAT dipped marginally to Rs. 311 crore compared to Rs. 320 crore in Q4 last year, due to increase in effective tax rate having an impact of Rs. 31 crore, net loss from the new hospitals of Rs. 11 crore and movement in non-cash item of fair value of contingent consideration by Rs. 25 crore, which reflects improved projected profitability for the managed hospitals.

Growth numbers from here on are being shared on a like-for-like basis, excluding new hospitals.

1) Average occupancy for the Network was 75%. Occupied Bed Days (OBDs) rose marginally by around 1% year-on-year and 3% quarter-on-quarter, driven largely by increased admissions under preferred channels (Cash, TPA / Insurance and International). Growth in occupancy and footfall was impacted due to capacity constraints.

2) Average revenue per occupied bed (ARPOB) for the quarter improved to Rs. 78,100, growing by 10% year-on-year and 2% quarter-on-quarter. Year-on-year growth was driven by increased share of super specialties (oncology, neurology and cardiac sciences) and increase in tariff, including that for the institutional segment.

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3) Institutional bed share was 29.1% compared to 29.2% last year and 29.5% in the previous quarter. However, after excluding Max Shalimar Bagh, the overall institutional bed share stood at 27.1% during Q4 and OBDs were down by 6% year-on-year for this segment.

4) Network gross revenue was Rs. 1,847 crore compared to Rs. 1,637 crore in Q4 last year and Rs. 1,779 crore in the previous quarter. This reflects an increase of 13% year-on-year and 4% quarter-on-quarter.

5) The international patient revenue grew by 14% year-on-year, slightly subdued due to

credit risk -related actions taken by us and visa -related issues in two markets.

6) Network operating EBITDA touched the magic mark of Rs. 500 crore , reflecting a growth of 14% year -on-year and 6% quarter -on-quarter. Most importantly, annualized EBITDA per bed rose to our highest ever of Rs. 78.5 lakhs , clocking a growth of 12% year-on-year and 4% quarter -on-quarter . While operating EBITDA margin stood at 28.4% for the quarter.

7) Year -on-year growth in EBITDA was impacted due to cost of people hired for Dwarka hospital , GST on variable management fees and higher provisioning due to build -up of accounts receivable.

8) Max Shalimar Bagh, where we added 122 beds , recorded year -on-year growth of 33% and 48% in its revenue and EBITDA, respectively, with an average occupancy of 78%.

9) Profit after tax was Rs. 322 crore versus similar level in Q4 last year and Rs. 338 crore in the previous quarter.

10) Free cash flow from operations generated this quart

er amounted to Rs. 412 crore . Of this,

Rs. 176 crore was deployed towards the ongoing capacity expansion projects and Rs. 1,509 crore was spent for recent acquisitions.

11) Consequently, Net Cash position stood at Rs. 22 crore at the end of March 2024 compared to Rs. 733 crore same time last year.

12) Continuing ou r efforts to support the local communities, we treated approximately 35,200 patients in OPD and 1,200 patients in IPD from economically weaker section s of society entirely free of cost.

13) Both our strategic business units (SBUs) continued to report significant growth in the revenue and profitability :

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- Max@Home reported a top line of Rs. 46 crore , reflecting a strong growth of 25% year-on-year and 3% quarter -on-quarter. This SBU continues to garner customer loyalty and has now expanded its footprint to over 10 cities.

- Max Lab, the non -captive pathology vertical , now offers its services in 41 cities and has a network of over 1,100 collection centres and active partne rs. This SBU reported a gross revenue of Rs. 39 crore , reflecting a growth of 26% year -on-year and 15% quarter -on-quarter.

14) On the status of expansion projects :

- For 303 beds at Dwarka – We are awaiting last few licenses and will be ready to launch the hospital by early June. We have already onboarded over 280 people, including senior docto rs.

- For 241 beds at Na navati (Phase I) – The hospital structure should be up by mid - July, and the proj ect is on schedule , with expected completion by Q4 FY25.

- For 375 beds at Max Smart (Saket Complex) – Post the initial delay due to tree transplantation issues, this project has now been fast tracked and is expected to be completed by Q1 FY26 , 9 months ahead of the previously communicated timelines .

- For 155 beds at Mohali – Slab work for the 3 basements is underway and base raft has been completed in May. The number of beds has reduced due to change in configuration and requirement to build a fire ramp, which is a new requirement as per Punjab Fire Authorities. Project completion is expected by Q1 FY26.

- For 300 beds at Sector -56 Gurgaon (Phase I) – Slab work for the 3 basements is in progress. The pace of work at the project has slowed down due to restricted working hours in view of adjoining residential complexes. This will likel y impact the project timeline by a maximum of 6 months.

- For 250 beds at Patparganj (Phase I) – Fire and Water Departments have issued NOCs for the building plan, while the Environmental Clearance (EC) and Municipal Corporation approvals are in process . Tendering work has been initiated.

- For 415 beds at Max Vikrant (Saket Complex) (Phase I) – EC and Consent -to-Establish (CTE) have been received, while tendering work has been initiated.

- For Nagpur Hospital – Work has been initiated to add 25 beds through internal reconfiguration by Q3 FY25 , while we are simultaneously firming up plans to augment infrastructure by another 140 beds.

- For Lucknow Hospital – We have commenced work s for installing additio nal 140 beds and refurbishing existing 250 beds by December 2024. Further 50 beds will be added through internal configuration in FY26. In addition, we plan to put up a new tower with 300 beds in the first phase by Q1 FY27.

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Finally, moving on to overall p erformance, including new hospitals , for the 12 months ended March 31, 2024.

15) Network gross revenue stood at Rs. 7,215 crore , including new hospitals, reflecting a growth of 16% year -on-year.

16) Network operating EBITDA grew by 17% year -on-year to Rs. 1,907 crore , including Rs. 3 crore from new hospitals. Increased ARPOB, improved case mix and augmentation of network bed capacity translated into a 13% improvement in EBITDA per bed to Rs. 74 lakhs.

17) During the fiscal year, we generated Rs. 1,336 crore of free cash flow from operations after interest, tax,

working capital changes and routine capex , of which Rs. 441 crore has been deployed towards ongoing expansion projects, Rs. 97 crore was distributed as dividend and Rs. 1,509 crore was spent for recent acquisitions.

With this, I would like to open the floor for Q&A .

Moderator: We take the first question from the line of Tushar Manudhane from Motilal Oswal Financial Services .

Tushar Manudhane: Sir, firstly on Lucknow, where we already have acquired Sahara, where you also plan to add 140 beds and then another tower of ~350 beds. So , if you could just explain conceptually the need for the buying land at Shaheed Path, Lucknow, that is my first question.

Abhay Soi: Firstly, the size of UP itself is the size of Europe, and it is an extremely underserved market. It has a major supply of clinical talent because traditionally they've had many government hospitals and medical colleges, etc . It drains a lot of patients from various places, right up from Bihar to Nepal and so on. We believe that there is an immense need for quality infrastructure over there. And furthering our presence in the Lucknow market, in the UP market, will add to further volumes.

I mean, we've barely been there since the 7th of March and in the last couple of months itself, we have moved up the occupancy by 8%. Just our presence right now has taken it up from 54% to 62%. That's the extent of ramp -up that we expect . Having said that, I would also like to tell you that our infrastructure is AA A+ over there. It is right in the heart of Lucknow , if you're familiar with Lucknow.

This is in the heart of Gomti Nagar, 27 acres of land and with access like no other. In terms of infrastructure , it is superior. In terms of location, it is superior to anything else over there. And we believe that there will be a lot of traction. In the state of UP itself , the kind of development, which is happening that you see -- 5 airports have been launched, another 4 are

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being added. The development which is happening is huge , the state is aiming to be a \$1 trillion economy by 2027.

It's not a matter whether they do it by 2027 or 2028 or 2029, but the rate of development over there is second to none -- I mean not even second to none, but it's right up there. And for development of any state, any economy, quality healthcare is a prerequisite. There is a lot more demand over there.

Tushar Manudhane: Understood, sir. Sir, but just that when we could have spread across maybe other cities of UP because peers are also trying to at least build -up 500 bedded hospital if not in Lucknow but in other smaller cities so that can sort of reduce the patient flow to Lucknow, and they are also coming up with super specialty hospital like that.

Abhay Soi: The demand is there across the country, right? The supply of clinicians is where the problem is. Lucknow is where the clinicians reside. That is where the government hospitals, the medical colleges have traditionally been. Now our belief is that as far as tertiary care is concerned, the entire tertiary care of the region will be serviced through Lucknow. You cannot set up tertiary care hospitals in tier -3 cities, in smaller cities , because doctors aren't willing to go there and stay there because the social infrastructure isn't there, the school isn't there and so on.

And given the kind of roads which are being made, the connectivity which is there, it will act like what Delhi is in NCR. You get people from various places in Delhi . Similarly, in Lucknow , you'll have many more hospitals. I mean for the entire state of ~240 million people, there are 2 corporate hospitals and both of them are in Lucknow. We haven't heard of a third corporate hospital in all of UP, right?

I believe that you can have 5 more super specialty hospitals, why 1 or 2 more. The only question that I needed to ask at that point in time when

we were even bidding for this piece

of land is that does it deserve another super specialty hospital , what if somebody else gets it , etc.? Similar to in Delhi, our approach is a cluster approach. We believe that Lucknow could be the next Delhi NCR. And frankly, it can do with many more hospitals.

Tushar Manudhane: Got it, sir. That's helpful. And secondly, with these beds sort of coming up and just to refresh per se in terms of the EBITDA growth for, say, '25, '26, the kind of expenses that will be coming on board, particularly for FY '26. So , if you could share in terms of whether we'll be

able to grow on the EBITDA front even in '26?

Abhay Soi: You're talking about FY26 or FY25 now?

Tushar Manudhane: FY26 , particularly.

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Abhay Soi: Let's imagine what's happening in FY26 . You would have had Dwarka, which starts now in the month of June 2024 and we breakeven by the end of the fiscal year. And next year, we're definitely in the positive and making money. We put all the building blocks of Lucknow and Nagpur in place . Both of them are profitable in any case , and they become more profitable in FY25.

But look, it will have a tremendous growth in FY26 simply because in this year, we'll be fixing everything . Then we have three capacities coming in, in beginning of FY26 effectively, for which we will get the benefit of the whole year – 155 beds in Mohali, 241 beds in Nanavati and 375 beds in Max Saket.

All three additions are brownfields , adjacent to properties, which are already fully occupied.

We should see the same sort of results that we've seen in any of our brownfields . In fact, FY26 will be a very exciting year for us . This year, if you look at FY24, was always meant to be a year of incremental growth. We're very fortunate we were able to add new hospitals . In fact, FY25 would have also been incremental growth if we hadn't added Lucknow and Nagpur. Now, we'll have the EBITDA and revenue kicking in from both of these. Then we add Dwarka, where we will have some losses up to breakeven in the first year, which will get more than absorbed by Lucknow and Nagpur, but next year all three will be exploding.

And in relation to that, we have three brownfields, which don't take time to breakeven and you have seen that in the past.

Tushar Manudhane: Understood, sir. That's helpful. And just lastly, what may push or in fact, prepone the Max Smart by almost nine months. If you could just elaborate on that.

Abhay Soi: As you are aware, we were coming up with a much larger capacity in two phases in Max Smart, and we were also coming up with the first phase of Max Vikrant capacity. Now , the fact is that we got delayed on Max Smart. So , what we've done is instead of doing one phase of Max Vikrant, which is contiguous, on the adjoining land, and doing two phases of Max Smart, we are doing one phase of Max Smart and two phases of Vikrant.

So, what we've done is essentially we have preponed. We are not going to be making as many basements and so it will take less time and we have managed to prepone the entire project. I mean, we'd rather have 375 beds sooner than have 600 beds later and then another 300 beds coming through Vikrant at an even later stage. Right now, we have sort of caught up with Vikrant timelines , which was supposed to be later. And that's how we've done this.

I'd like to add that it's a pleasant surprise that we've been able to prepone Max Smart. A lot of people were expecting delays, but let alone delays, we have preponed this entire project.

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Moderator: We'll take our next question from the line of Damayanti Kerai from HSBC Securities and Capital Markets India Private Limited.

Damayanti Kerai: Abhay, you mentioned in your opening comment, there were some visa issues in two markets, which slightly impacted the international segment -- international patient segment. Can you please

elaborate on it?

Yogesh Sareen: Damayanti, basically there are markets of Iraq, etc., where we have accounts receivable build - up. We took some calls on credit control and we decided that we will not take more patients till such time we recover our money . And that's what Abhay was alluding to. There are also some markets , where there were some disturbances on the visa side . Because of the elections in these markets , visas were not being issued.

These are the 2-3 markets that got impacted . You will notice that overall, we've been growing the international market revenue by around 23%. This quarter, it is 14% growth , which is a bit subdued because of these two reasons.

Damayanti Kerai: Okay. And these are more temporary in nature, right?

Yogesh Sareen: Yes, very much.

Abhay Soi: We continue to see great traction with new offices that we are opening overseas . It is very positive and we intend to sort of double down on that further.

Damayanti Kerai: Okay. In all focus markets, you will be doubling down your effort to get more footfall.

Abhay Soi: We started these direct -to-fly offices , as you recall last year, and we've seen great traction and great throughput through those. We're seeing a lot of success through the new markets where we're setting up our own offices. We will continue with this strategy.

Damayanti Kerai: Okay. Abhay, is the way you mentioned attractiveness of Lucknow market in terms of like demand and then availability of clinical talent, etc. Can you talk a bit about how do you see the Nagpur market? And do we have similar supply of clinical talent in that market, too?

Abhay Soi: Yes, you do have supply of clinical talent. But if I look at India, Lucknow that way is unique,

as in perhaps it has the most amount of supply -- just to give you an example, at least 20% or so of our own doctors in Delhi, which work for Max, have come from Lucknow. So, it has traditionally been the repository for talent. Of course, Nagpur is also a repository for clinical talent, but not to the same extent as Lucknow. You don't have other places with the same sort of supply. I would compare Nagpur more to Dehradun, where we have a hospital which is doing quite well. So, I would compare it to that market rather than Lucknow.

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Having said that, it is emerging as the transplant capital of the country. Because there is a huge amount of awareness of organ donation and it seems to be a cultural thing there, with a lot of people donating organs. Also, it has been a big centre for mouth cancer because of much higher consumption of gutka, etc. We're seeing a lot of oncology patients in Nagpur. And with oncology being our mainstay, I have no doubt that we will do well there. Let me also clarify that Nagpur also has much fewer quality health care assets.

I don't think there is anybody even compared to Alexis Hospital over there. Competition is also significantly lesser. In a way, we will have first dibs on most of the clinical talent over there. We are quite comfortable that even though the supply may be less than Lucknow, but nevertheless, we'll get the best. We don't have a challenge in getting the clinical talent. And the market itself is quite big. It also draws a lot from Madhya Pradesh – Chhindwara, etc. And of course, it also has great connectivity and infrastructure.

Damayanti Kerai: Okay. And my next question is a quick clarification on occupancy. So now you're giving it on like-for-like basis, 75% for fourth quarter and then you have given separate numbers for the new units. Can you just for understanding purpose, on a blended basis, what is the number occupancy number for fourth quarter?

Yogesh Sareen: Around 74%.

Abhay Soi: Please do keep in mind that Nagpur has only been there since 9th of February and Lucknow since 7th of March. So, you don't get the full impact of the fourth quarter,

right?

Damayanti Kerai: Got it. And my last question is like your upcoming hospitals, most of which are located in very attractive locations, brownfield location, etc. For both, we are assuming breakeven within a year of start of the operation. So very broadly, like what kind of loss you generally incur when hospitals are ramping up maybe like loss per month or any indication. For your upcoming hospitals, most of which are brownfield. So, I just want to understand while hospitals are ramping up, what kind of loss you generally incur.

Abhay Soi: I'll give you the example of the last brownfield we did. We added about 122 beds on a base of 280 beds. That would have broken even in 10 or 15 days. And by the 40th day, the additional beds were generating 40% EBITDA margin. And in the whole year, we have 78% average occupancy on the new and old beds. So, brownfields don't take time to breakeven. They don't take years; they take a month. We normally say a quarter at best, but we have kind of beaten that every time.

Damayanti Kerai: Yes. Just like in NCR, I can understand this kind of performance we may expect. But is it true for, say, Nanavati or Mohali, etc, also, we should see similar ramp-up?

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Abhay Soi: No doubt. I mean Nanavati is in Mumbai, right? In the heart of Mumbai, we have another 375 quality beds coming in. And in a place where no new hospital has come up in 20 years, I don't see a challenge in breaking even. Mohali again -- it's the highest ROCE, the highest occupancy that we have in the Network right now. It's not Delhi or Mumbai.

Moderator: We'll take the next question from the line of Neha Manpuria from Bank of America.

Neha Manpuria: Given the background you gave on Lucknow is it fair to assume that Lucknow can get to what we have margins for our corporate average much faster probably, let's say, in FY26 itself. Would that be a fair assumption? Or do you think because of the addition, etc, that could take a little bit longer?

Abhay Soi: See, addition is separate altogether. Addition is a capital expenditure, right?

Neha Manpuria: Sir I mean -- associated with the addition, etc?

Abhay Soi: I didn't get the question. Can you repeat the question?

Yogesh Sareen: Neha, basically, in terms of margin, we can probably get to the overall similar level of margin very soon; not probably in the initial part of the year, but maybe by the end of the year. But the EBITDA per bed will have to wait because the ARPOB there is lower. ARPOB of Sahara Hospital when we acquired it was Rs. 47K.

First, the ARPOB needs to go up. Higher margin on lower ARPOB doesn't mean much because we generally look at EBITDA per bed metric. So, EBITDA per bed will have to wait. I would say it will take some time to get to that level. But in terms of margin, probably by the end of the year, we should be closer to what the margins are on overall basis.

Abhay Soi: On pretty much all, including ARPOB and other metrics, we will be able to catch up in the next two years. And when I say catch up, I mean we know the numbers of some of our peers

over there. So, we should be able to get there, if not further.

Neha Manpuria: Okay. Understood. And what about Nagpur, given the margins there at least the numbers that you gave seem close to 14%, 15%. And it's probably not as flourishing as UP, would that take a little bit longer, a much slower pace to Lucknow.

Abhay Soi: Nagpur or Lucknow?

Neha Manpuria: Lucknow.

Abhay Soi: So, Nagpur is a different ARPOB, and different ARPOB means different ROC E as well. You can compare it similar to a Mohali or Dehradun rather than any other location. You can't

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compare ARPOB of Nagpur to Delhi or Mumbai for that matter. You'll be looking at obviously more subdued ARPOBs, but given the price you pay, you'd look at ROC E.

Neha Manpuria: So, if I were to look at the

Nagpur ARPOB, it is very similar to where Lucknow is, right? So

that's why I was asking. Will it take the same trajectory as Lucknow?

Abhay Soi: Today it is, but Lucknow has potential for a lot more, right?

Like I said, for Lucknow – even if we look at some of our peers who are operating there, they operate at a significantly higher ARPOB.

Neha Manpuria: Yes.

Abhay Soi: Nagpur as a market, well -- you can use maybe Dehradun as a surrogate.

Neha Manpuria: Okay. Understood. Got it. And in terms of incremental M&A, now that you've announced this in land in Lucknow, you've always said we'd rather be the second or third player in the market. Any markets that you think are interesting, particularly given you've done two of these Nagpur and Lucknow now, which could interest you?

Abhay Soi: In the 19-20 cities, where at least two or three of our peers have proven viability and are doing fairly well. We intend to go there if we find the right targets. And we are quite confident we'll be able to perform better than them.

Neha Manpuria: And Abhay, what would be your target leverage? I mean, let's say, if you were to get multiple assets, what leverage are you comfortable with in terms of when you're looking at M&A, particularly given the strong cash flow generation?

Abhay Soi: Yes. So, we are quite comfortable going up to 2.5x debt-to-EBITDA. If you see our EBITDA at present, we will be circa Rs. 2,000 crore. We are at Rs. 1,907 crore presently, and if we add up 11 months of EBITDA for both Nagpur and Lucknow, we'll be closer to Rs. 2,000 crore plus. And then we have growth in the current year.

But even if we take 2.5x that, we get to around Rs. 5,000 - 6,000 crore of debt that we can use. Please do keep in mind, anything bigger than that would mean that we're acquiring something that is listed. And if it's a listed entity in any case, you'll have to look at a merger because you can't have a step-down subsidiary of a listed company, being a listed company.

Moderator: We have a next question from the line of Andrey Purushottam from Cogito Advisors LLP.

Andrey Purushottam: Okay. I have two questions. One is, could you comment on the valuation parameters in acquiring these two hospitals, they are at two different EBITDA multiples. Are they meant

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to be ROC E accretive? Or what is the philosophy in -- financial philosophy apart from growth, obviously? That is my first question.

And my second question is going forward in the next 12 months, how do you see the leveraged for profit working in your favour, whether it is the increase in mix of international patients, whether it's a better mix, whether it is a lower proportion of government cases, etc. I just wanted to gain your general outlook on the trend that you see in the next 12 months that will lead to profitability growth.

Abhay Soi: Two aspects here. One is on the financial aspect; we seek 20% to 25% pre-tax ROCE within 4-5 years from acquisition. In both these cases, our belief is that we will be able to hit that number far sooner than that. And we do these transactions by seeing what is the value we are paying versus what we believe with adequate amount of intellectual integrity, what is the business plan that we are able to or capable of underwriting. And then we do a goal seek for that number at 20%-25% pre-tax ROCE, and that's how we come up with the maximum that we are willing to pay for an asset. Of course, we wouldn't pay that maximum. We try to negotiate, but that's where we are on the financial side. I hope that answers your first question. Now as far as drivers for the year is concerned, it is all of the things that you -- sorry?

Andrey Purushottam: For the next 12 months.

Abhay Soi: Next 12 months is this financial year, effectively. Well, instead of 12 months I will draw a line at 10 months because that's the number of months left for the

current financial year. All

of these factors that you mentioned -- higher international patients, which means better payor mix, better clinical mix as we are seeing more robotics happening, we are seeing more growth

in oncology , higher -end surgeries , etc. So, all of these things should be in play. We're also hoping and believe that CGHS rates will get reworked. So even on the institutional side, we're expecting better rates. So , all of these are levers for us in the current year.

Other than that, of course, compared to last year, we would be adding incremental EBITDA emanating from both Lucknow and Nagpur, whilst Dwarka will have some level of initial losses at start-up. But those initial losses at start-up, we will be able to more than absorb it with our profits that we're generating through Lucknow and Nagpur.

Andrey Purushottam: Right. And do you see a drag on profitability?

Abhay Soi: No. Like I said, of course, Dwarka will take time to breakeven, but we've added two new capacities, with ~500 beds in total , which are already generating profits and free cash flows. We will be adding to that. That number is only likely to go up under our watch.

Moderator: We'll take a next question from the line of Bino Pathiparampil from Elara Securities (India) Pvt Ltd .

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Bino Pathiparampil: Just two clarifications. One, did you say you're adding -- planning to add more beds in Nagpur over there over and above between 200 beds there?

Abhay Soi: That's right. We're going to add another 140 beds on top of the 200 beds that we have.

Bino Pathiparampil: By when would that be?

Abhay Soi: We are working that out at present, but definitely within the next 24 -30 months .

Bino Pathiparampil: Okay. And I didn't quite understand the Saket Smart and Saket Vikrant. So , both put together, how many beds are now getting added in FY26 ?

Abhay Soi: No, Vikrant is not getting added by FY26. Smart is getting added in FY26 now. Vikrant wasn't meant to be commercialized in FY26 . Whereas Smart , which was going to be added in FY27 (after initial delay), has now been preponed by nine months and is going to come through in Q1 FY26 .

Bino Pathiparampil: That is 350 beds?

Yogesh Sareen: 375 beds.

Bino Pathiparampil: Sorry, 375, okay. I was looking at your Feb investor presentation, that shows Vikrant 300 beds in FY '26. Is that old one?

Abhay Soi: Maybe you're looking at the older presentation.

Yogesh Sareen: No. I think in the investor presentation , we are yet to change to 375 beds as well as other changes . That will be updated now with whatever we have done in terms of preponing the beds. There are some beds being preponed and some being postponed. I would say the new presentation will get updated in another week's time . And that is the one that should be referred to. As Abhay mentioned, we've done some re-setting in terms of some beds coming in advance and some beds getting delayed for that reason.

Bino Pathiparampil: Okay. Understood. I will refer to it.

Abhay Soi: Your total number of beds remain the same overall . But like I said, this gets preponed. The total number of beds don't change.

Moderator: We have the next question from the line of Rishabh Tiwari from Allegro Capital Advisors.

Rishabh Tiwari: So, I have two questions. Firstly, what would your EBITDA look like now post Ind-AS for this quarter and for the year?

Abhay Soi: We don't provide forward -looking projections.

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Rishabh Tiwari: No, I'm asking for Q4 FY24 and FY24 .

Yogesh Sareen: For Q4 , EBITDA was Rs. 500 crore from the existing hospitals and another Rs. 3 crore of EBITDA came from these two hospitals that we added in Q4 – one in February and one in March . And the Rs. 3 crore is actually net of Rs. 5 crore of one-time transaction expenses . The overall EBITDA for the year is Rs. 1,907 crore .

Rishabh Tiwari: T

his would be post-Ind AS right?

Abhay Soi: Yes.

Rishabh Tiwari: Yes, I'm asking post Ind-AS and pre Ind-AS.

Yogesh Sareen: For pre-Ind AS, you will have to reduce the number by around Rs. 70 crore . So, if you want to take the leases up, that's the only difference in our case. The lease cost would be around Rs. 70 crore .

Rishabh Tiwari: For the year or for the quarter?

Yogesh Sareen: For the year.

Rishabh Tiwari: And next question is Abhay you mentioned that we took the occupancy up from 54 % to 62 % in Sahara Hospital. I just wanted to know more about it – was this through operational levers?

Or did we see some improvement in seasonal mix ? How do we take this up within Q1?

Abhay Soi: No, it is the result of changing some basic processes. You always have some very early and easy wins when you walk in, because you would have seen those things earlier in your existing hospitals . So those changes kick in. These are very cheap , early wins.

Rishabh Tiwari: Okay. And are we looking to add any beds here as well in Sahara?

Abhay Soi: We will be adding beds. Like I said, we will be adding 140 and then another 450.

Moderator: We move on to our next question from the line of Amey Chalke from JM Financial Institutional Securities Ltd .

Amey Chalke: Congratulations on a good set of numbers. The first question I have is on the volume growth.

So IPD volume seems to be around low to mid-single digits. Anything to read there? Or do you expect the volume growth to remain at this kind of level? And second question I have is on the safety selection. I understand we generally map 20-odd cities across country. What would be the cities you would not like to enter into? And what are the reasons for the same?

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Abhay Soi: First and foremost, you have to keep in mind that our occupancy growth till we come up with new capacity, brownfield or otherwise, will be fairly muted because we have capacity constraints. We don't have beds available to put more patients. That's why we are doing the brownfield.

Having said that, because of constraints on capacity, our occupancy ramp-up will be limited, whilst it will have a play, a positive impact on ARP OBs. What tends to happen is that the lower-end surgeries get posted at a later date, whilst the major ones get priority, as they should. Therefore, the lower-end ones sometimes have a tendency of evaporating, because the patients then decide to go to some other facility or some other brand, because they may not be very important sort of surgeries as well.

What happens is that -- while the occupancy does not ramp up, you see a significant growth in ARPOB. But when you come up with brownfield capacity, the interplay which happens is that the occupancy ramps up immediately, but your ARPOB growth becomes more muted. Having said that, the impact on overall EBITDA per bed is positive simply because you have huge operating leverage emanating in the new capacities that you add, since the management cost, clinical costs, etc., are already being incurred by the existing capacity.

I just wanted to answer that question as far as occupancy is concerned and assure you that there's nothing to read into it. It is simply that we don't have capacity. So, our occupancy will go up at a slower pace.

Your second question was with respect to which cities we would not go to -- so conversely, we would not go to cities where our peers have not proven viability or to uncharted territories because we don't see ourselves as pioneers going into new places and taking chances. We would rather have somebody else define the market, demonstrate success and then we would go there and try to do it better. We've been very comfortable in doing that. And we've been successful in doing that.

Amey Chalke: So, is there any city in which you think is oversupplied with the quality of the beds, etc.? Or is just

that the -- because it's uncharted territory, that's why you don't want it.

Abhay Soi: Let me give you an example. The maximum number of beds, in any geography, is in Delhi NCR. We have 11 hospitals over there with ~2,500 beds. We are the largest player by far. We are equal to next three of our peers put together and multiplied by two in terms of number of locations, and we are equal to all three of them put together, as far as number of beds are concerned.

And the next 15 players have similar number of locations and a similar number of bed capacity. Yet we only have ~2,500 beds -- 10% of which are free, 10% of which are catering

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to international business, 40% of which are catering to upcountry. Essentially, if the largest player has ~800 beds catering to the NCR, and the next three peers of ours also have 800 beds and then the next 15 people have 800 beds, that is like 2,400-2,500 beds for a population of ~48 million people, which is ~85% of the U.K.'s population.

And this is a place where you have the maximum amount of supply. Look at the places we're going to -- if we are going to Lucknow, literally there are 2 corporate players, then we've come up with a hospital and in a state with a population of 24 crore people. In Dehradun, we are pretty much the only players over there. In Mohali, it is us and one more player. If some of our peers in Patna, then they are the only players over there.

If you look at places like Kanpur or Pune, I mean, every place is underserved, right? Even in a place like Mumbai, no new hospital has come up in 20 years. I don't think in India, we have a problem of oversupply anywhere.

Amey Chalke: Right. But in generally, speaking about the Mumbai, like we generally don't see except -- I'm talking about the private hospital, we don't see hospitals being overrun or something like that or there is a like we have not seen many hospitals getting added, but we have not seen capacity getting over --

Abhay Soi: No, no. You haven't seen hospitals being added because to make a 400-bed hospital, you need four acres of contiguous land. Firstly, you can't find four acres of contiguous land in Mumbai.

And even if you find it, you'd rather make a residential complex or a commercial building there .

I mean no new hospital has been made for 20 years . It's the same hospital in Hinduja, the same in Breach Candy . They haven't added one square inch because where is the space. Even H.N., they brought down the earlier hospital and made a new hospital . Bombay Hospital is the same one, Jaslok is the same one. Nanavati is the only one which has the land , so we're building on it.

Moderator: We have a next question from the line of Kunal Dhamesha from Macquarie Capital.

Kunal Dhamesha : So just kind of continuing on the previous question. So mature hospitals, we are at 75%. And we are seeing that there is not enough capacity for us. But we have done quarters with 77%, 78% occupancy, right? So , what is it that -- is it a mix of hospitals where our good hospitals are already full and some of the hospitals are below that 75 % run rate ? And secondly, if it's kind of across the hospitals, we are facing difficulty in terms of capacity, then why our peer mix improvement is not accelerating in terms of reducing the institutional patients?

Abhay Soi: Because it is a seasonal business . There are some quarters which are surgical quarters and some quarters which are medical quarters . In a medical quarter, you require rooms while in

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a surgical quarter, you require more ICU beds. And every kind of capacity is not fungible across every hospital.

In fact, some hospitals may have certain constraints that they're operating at 75% because maybe they have lesser single rooms where there's demand for it and more ward beds where you don't have demand for , etc. But whereas in a dengue season, even t

he lower category or

multi-share rooms get absorbed. But that doesn't happen in the surgical season. That's the reason that we are constrained by seasonality as well as the kind of infrastructure that we have.

Now as far as the pay or mix is concerned, the pay or mix is a slow churn now. If I look at ex - Shalimar Bagh, ex-new capacity that we've come up, it's come down from 29% to 27%, but there's been a 6% reduction as far as the OBDs are concerned. What we are seeing is – a turn perhaps more so on the OBDs than anything else.

Kunal Dhamesha : Sure. But let's say, when you say the 122 beds added in the Shalimar Bagh capacity or hospital. And we say that it's been EBITDA positive in 10 days. Is it more on a contribution margin? Because if I back calculate our pay or mix for that 122 beds, there'll be roughly 46%, 47% of those beds have been occupied by institutional patients, right? And we all know that the ARPOB roughly 50% of what you get in other channels.

So, is it on a contribution margin we are seeing that the EBITDA turned positive in 10 days ? Was it loaded with the management cost? Or how should we think about that?

Yogesh Sareen: Kunal, it is on EBITDA. It's after the incremental cost that you have to incur for those beds.

So, it's not contribution , it's EBITDA. We had two options. We had an option to open only 50 beds to start with, and not all of the 122 beds . But what we did is that we opened all the beds and filled up the balance also through the institutional patients.

There were two options and we chose to open all 122 beds and then try and fill up and then filter , then distil -- that's where we are today. But when we say the number, 40% is EBITDA margin, after incremental cost. If there are 122 beds being opened, there is more manpower being recruited, right? There are nurses required. There will be GDAs required, maybe less doctors, but obviously there will be extra costs, etc . So, the number reported is after all those costs.

Kunal Dhamesha : So even with 46% kind of institutional payor mix, we are making 40% EBITDA there. Is that the correct...

Yogesh Sareen: Because the incremental cost of running those beds, which we opened, is very less .

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Abhay Soi: Let me explain this separately. Let's say, if you look at bed -by-bed, unit wise, right? Or let's say, all 100% of the beds over there, you were operating only for institutional business. It will still be EBITDA positive.

It will be EBITDA positive simply because you have operating leverage. The only thing you're getting over there necessarily is the nurses, which is not very expensive, is the resident doctors, again, not very expensive, and allocated to those beds alone, which are operational. What you're not doing is you're not fitting out beds and you're not sort of staffing beds where you don't have capacity.

You basically open floor by floor, whatever you have demand for. There's no fixed cost associated or fixed variable costs associated with beds which are not operational. And the ones which are operational, we have only the low-end cost. It does not have high -end sort of clinicians, etc., or their minimum guarantees, which are being taken on board . Because those clinicians already exist, their minimum guarantee is already being paid. Utilities are common.

The general set of management costs has already been incurred. You don't operate with a separate CEO, separate Medical Director, separate HR, separate Marketing, nothing for that. Kunal Dhamesha : And if I may, just one more on the Supreme Court matter. Now it seems that the next date for that hearing of PIL seems to be somewhere around September. So, what's your internal assessment? Is it kicking the can down or would you say that the risk of adverse outcome has reduced meaningfully in your view? How do you view that? Abhay Soi: We would not like to comment on

it. It's a Supreme Court matter and currently sub-judice. We've seen the rap IMA got, so we definitely don't want to be speaking about it. But in the last hearing, the comments, the observations of the judge, etc. were for everybody to hear and make their own inferences.

Kunal Dhamesha : Sure. Thank you, all the best.

Moderator: We'll take our next question from the line of Shubham Harne from Purnartha Investment Advisors.

Shubham Harne: Sir, on Dwarka, I want to ask questions. So that is getting added in tranches or it will be on one go?

Abhay Soi: In tranches. Because our occupancy will also have been in tranches only.

Shubham Harne: Okay. So initially, earlier, you have said 160 beds will be added and then 140 like -- something like that only?

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Abhay Soi: Yes, since occupancy also ramp -ups in that manner. No point staffing all the 300 beds when we don't have occupancy on day one for all 300 beds.

Shubham Harne: Got it. And on Gurugram sector 56 hospital, by when it will get commissioned or something from -- around that?

Abhay Soi: Gurugram Sector 56, we are delayed by 6 months. So, it should be second quarter of FY26 now.

Keshav Gupta : Yes, Q2 FY26.

Shubham Harne: Okay, thank you sir.

Moderator: We have our next question from the line of Rishabh Tiwari from Allegro Capital Advisors.

Rishabh Tiwari: Sir, just one follow-up on the lease impact. So Rs. 70 crore is what you mentioned for the year, is this taking into account the recent acquisition? Could you please help us with the like-to-like number?

Yogesh Sareen: The recent acquisition does not have any lease rentals. The land is getting converted into free hold. We have already put in our application for same. Rs. 70 crore is on the existing hospitals. Rs. 71 crore is the total impact since there are one or two more entries which come below the line. So, I'm taking all those -- there's also the liability for donations that we have. So, Rs. 71 crore is total impact of the Ind-AS movement. If I was to do IGAAP accounting, then Rs. 71 crore will come before the EBITDA.

Abhay Soi: Just an important point to note. I don't know if you picked up my statement earlier or not that during the fiscal year, we generated Rs. 1,336 crore of free cash flows from operations. This is after interest, tax, working capital changes and routine capex also, let alone leases. Our translation of post Ind AS EBITDA to free cash flows, after the pre Ind AS impact, post interest tax, working capital changes and routine capex, is 70%. This is a significantly high number.

Moderator: Thank you. We'll take our next question from the line of Senthilkumar Natarajan from Joindre Capital Services.

Senthilkumar Natarajan : My question is what is the management policy on writing off goodwill and other intangible assets over a certain period of time as a matter of prudent accounting policy?

Yogesh Sareen: Under the Ind AS method, Goodwill is only tested for impairment. Intangible assets represent some of these contracts that we are writing off over the period of the respective contracts.

But there is an element of Brand in the intangibles, which is again tested for impairment. Ind AS accounting standards required to only test for impairment for Goodwill and Brand. And

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for the other intangibles, you will find that they are coming down progressively year after year because we're putting them in the amortization charge.

Senthilkumar Natarajan : Yes. OK, understood. Thank you. That's it from my side.

Moderator: Thank you. We'll take our next question from the line of CA Vipul Makwana from Makwana.

CA Vipul Makwana: Apart from all the financial queries which my peers have asked, I just want to ask on the qualitative aspects like what is Max doing separately or concisely different than the others were seeing a good number set, like on

ARPOB and everything. So, Abhay if you could just throw some light on it.

Abhay Soi: First and foremost, the fact that we have significantly higher occupancy levels than any of

our peers means that we are doing well on qualitative aspects as well as the value proposition that we are able to provide to our patients .

And it's not only in Delhi or Mumbai . Our highest occupancies also continue to be in Tier 2, Tier 3 cities. Yes, it's for the patients to perceive more than what we can say in terms of what are the qualities that they appreciate . And we continue to cater to a lot of free patients – we treated ~35,000 patients in OPD and ~1,200 patients in IPD in the last quarter itself.

All our hospitals are teaching hospitals. We have 500+ MBBS doctors as part of our DNB program – these are post-graduate students. We write a lot of -- you've seen the number of publications that we're doing, which is the highest ever that we did last year, both for international and domestic journals. We continue to do academics and research as well besides, of course, the financial parameters that we offer.

CA Vipul Makwana: That helps. Thank you so much.

Moderator: Thank you. Ladies and gentlemen, that was the last question for today. I would now like to hand the conference over to management for closing comments. Over to you.

Abhay Soi: I would like to thank each one of you for either covering the company or having invested in the company. We appreciate that. And we believe that the next two years will be years of exponential growth for us, particularly FY26 , where a lot of things are going to be coming on stream. So , we look forward to an extremely lucrative journey going forward. Thank you.

Disclaimer: This is a transcription and may contain transcription errors. The transcript has been edited for clarity. The Company takes no responsibility for such errors, although an effort has been made to ensure a high level of accuracy.

Call: Aug 2024

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August 8, 2024

Listing Department ,
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Symbol: MAXHEALTH Listing Department,
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Scrip Code: 543220

Sub.: Transcript of Earnings Call held on August 2 , 202 4
Ref.: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Sir / Madam,

Please find enclosed copy of transcript of earnings conference call, organised on August 2 , 202 4, on financial results of the Company for the quarter ended June 30 , 202 4.

The said transcript is also available on the website of the Company at [www.maxhealthcare.in/investors/investor -resources](http://www.maxhealthcare.in/investors/investor-resources) .

Kindly take the same on recor d.

Thanking you

Yours truly,
For Max Healthcare Institute Limited

Dhiraj Aroraa
SVP - Company Secretary and Compliance Officer

Encl.: As above

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Max Healthcare Institute Limited
Q1 FY25 Earnings Conference Call
August 02 , 2024

Moderator : Ladies and gentlemen, good day and welcome to Max Healthcare Institute Limited Earnings Conference Call.

Please note that this conference is being recorded.

I now hand the conference over to Mr. Suraj Digawalekar from CDR India. Thank you and over to you, sir.

Suraj Digawalekar: Thank you, Neerav. Good morning, everyone and thank you for joining us on Max Healthcare's Q1 FY'25 Earnings Conference Call.

We have with us Mr. Abhay Soi – Chairman and Managing Director, Mr. Yogesh Sareen – Senior Director and Chief Financial Officer and Mr. Keshav Gupta – Senior Director, Growth, M&A and Business Planning.

We will begin the call with opening remarks from the management following which we will have the forum open for an interactive Q&A Session.

Before we start, I would like to point out that some statements made in today's call may be forward-looking in nature and a disclaimer to this effect has been included in the Earnings Presentation shared with you earlier.

I would now like to invite Abhay to make his opening remarks.

Abhay Soi: A very good morning to all, and a warm welcome to Max Healthcare's Earnings Call for the Q1 FY 25.

It's been a very promising start to this financial year. I am pleased to share that we have operationalized Max Super Speciality Hospital, Dwarka on July 2nd. Located in the heart of South Delhi, it is a 303-bed hospital equipped with the latest high-end technology and currently has 125 doctors and ~480 nurses, paramedics and support staff.

The hospital is seeing good traction since its launch. Presently, the hospital teams are engaged on various fronts including patient outreach, empanelment with insurance companies, clinical accreditations, etc.

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Yesterday, we announced a built-to-suit transaction for setting up a 250-bed hospital in Mohali. This 2.75 lakh sq. ft. building with attendant parking will be constructed by the developer as per our requirement and specifications. This is a long-term lease, extendable up to 50 years at our option, and we expect to commission the facility by FY28. The hospital will cater to surrounding areas of Himachal Pradesh, Punjab and Haryana. This is in line with our asset-light strategy, which allows us to expand while insulating against development risks.

Further, in a strong demonstration of our turnaround capabilities, both Lucknow and Nagpur hospitals that we acquired at end of Q4 last year are ramping up well, with an average occupancy hovering around 60%. For Q1, these hospitals contributed Rs. 99 crore to the revenue and Rs. 18 crore to the operating EBITDA, reflecting a year-on-year growth of 21% and 64% respectively. And more importantly, this was done within the first three months of us acquiring these facilities. Collectively, their ARPOB stood at Rs. 45,300 and annualized EBITDA per bed at approximately Rs. 30 lakhs. A large part of the post-merger integration activities has been completed now, the results of which will be more visible in the quarters to come and further augmented as new clinical teams join and infrastructure is upgraded there. Now, coming to the Q1 performance. This is our 15th consecutive quarter of year-on-year growth.

Overall network gross revenue stood at Rs. 2,028 crore, registering a growth of 18% year-on-year and 7% quarter-on-quarter.

Network operating EBITDA was Rs. 499 crore, net of one-time charge of Rs. 6 crore towards pre-launch costs at Max Dwarka. This reflects a growth of 14% year-on-year and a marginal decline of 1% quarter-on-quarter.

Profit after tax (PAT) stood at Rs. 295 crore compared to Rs. 291 crore in Q1 last year and Rs. 311 crore in the previous quarter. While we expect substantial improvements during the course of this year, we are also infusing equity in the Lucknow subsidiary to bring down the finance costs.

All numbers from here on are going to be now shared for a like-to-like basis, excluding new hospitals.

1. Our average occupancy for the network increased to 77% from 74% in Q1 last year and 75% in the trailing quarter. While occupied bed days grew by around 5% year-on-year and 2% quarter-on-quarter.

2. Average Revenue Per Occupied Bed (ARPOB) for the quarter improved to Rs. 80,100, growing by 7% year-on-year and 3% quarter-on-quarter. Year-on-year improvement in ARPOB was largely due to growth in oncology, orthopedics and

renal sciences as well as increased number of robotic procedures. This was coupled with tariff revisions for self-pay, insurance and institutional segments.

3. Network gross revenue was Rs. 1,929 crore compared to 1,719 crore in Q1 last year and Rs. 1,847 crore in the previous quarter. This reflects an increase of 12% year-on-year and 4% versus the trailing quarter.

4. The international patient revenue stood at Rs. 158 crore. There was a temporary impact on patient flows from some countries due to political situations and credit-risk management related actions taken by us, which accounted for the muted growth in this segment.

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5. Network operating EBITDA stood at Rs. 487 crore, reflecting a growth of 12% year-on-year while declining by 2% quarter-on-quarter. Operating EBITDA margin stood at 26.5% for the quarter. Growth in EBITDA was impacted due to annual merit increase, GST on variable management fee and drop in footfalls for immigration health checks owing to change in visa regulations by some countries, particularly Canada and UK.

6. Annualized EBITDA per bed stood at Rs. 74.7 lakhs, clocking a growth of 6% year-on-year.

7. PAT was Rs. 316 crore versus Rs. 291 crore in Q1 last year and Rs. 322 crore in the previous quarter.

8. Free cash flow from operations was Rs. 258 crore. Of this, Rs. 213 crore was deployed towards the ongoing capacity expansion projects and upgradation of facilities at new hospitals. Consequently, net cash position stood at Rs. 66 crore at the end of June 2024.

9. Continuing our efforts to support the local communities, we treated approximately 36,800 out-patients and 1,150 in-patients from economically weaker sections of society entirely free of charge.

10. Both our strategic business units (SBUs) continued to report significant growth in the revenue and profitability:

- Max@Home reported a top line of Rs. 49 crore, reflecting a strong growth of 23% year-on-year and 6% quarter-on-quarter. This SBU offers 14 service lines over 10 cities and continues to experience a very high rate of repeat transactions.

- Max Lab, the non-captive pathology vertical, now offers its services in nearly 50 cities and has a network of over 1,100 collection centers & active partners. This SBU reported a gross revenue of Rs. 41 crore, reflecting a strong growth of 21% year-on-year and 6% quarter-on-quarter.

11. Now, coming to the status of expansion projects:

- For 590 beds at Lucknow hospital, the work on refurbishing the existing facility is underway. Finishing work for operationalizing additional 140 beds has started. We have also received the environmental clearance (EC) approval for setting up a new 450-bed tower on this site and we expect to complete the development of the new tower within 24 months.

- For 140 beds at Nagpur hospital, work has been initiated to add around 25 beds through internal reconfiguration by Q3 FY25 and environmental clearance (EC) application for adding 115 beds on two additional floors has been filed. We expect approval over the next 2-3 months and the project completion within 24 months.

- For 241 beds in Nanavati (Phase I), the hospital structure will be up in this month and the project completion is expected by end of this fiscal year, as

communicated earlier . The project continues to be on schedule.

- For 375 beds at Max Smart (Saket Complex), the project was fast-tracked in Q4 and we expect its completion by Q1 FY26. This is a steel structure building and the installation of columns has already started. The site is fully mobilized and we don't expect any further delays.

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- For 155 beds at Mohali , work on the 2nd floor slab is underway , while the work on ramp area side is reaching ground level. All high-side equipment has been ordered and the project is on schedule. We expect the completion by Q1 FY26.

- For 300 beds at Sector 56, Gurgaon (Phase I) , basement slabs are nearing completion and the project is progressing as per plan. Orders for all high lead-time items have been placed. We expect completion of the first phase by Q2 FY26.

- For 250 beds at Patparganj (Phase I), post issuance of the no objection certificate (NOC) by the fire and water departments , we have submitted the drawings for approval and the tendering process for this project is on.

- For 415 beds at Max Vikrant (Saket Complex) (Phase I) , the environmental clearance (EC) and consent to establish (CTE) have been received. The barricading and D-walls , etc. has started. We have applied for forest approval and are still awaiting the same. We expect some delay due to ongoing litigation involving DDA and Delhi Government regarding cutting out trees in the eco -sensitive zone near Asola Bhati Wildlife Sanctuary and the Ridge areas without approval.

With this, we open the floor for Q&A.

Moderator: We will now begin the question -and-answer session. The first question is from the line of Amey Chalke from JM Financial.

Amey Chalke: The first question I have is in Lucknow Sahara unit. I understand that we have a plan to expand the unit to 550 beds and then there i

s another tower which could come up.

But what is the near -term action plan for the existing units of the 250 bedded unit in terms of the speciality addition , etc. ARPOB , I believe , is substantially low, so what would be the driver s going ahead for this unit to improve the ARPOB?

Abhay Soi: Clinical programs is clearly one, adding doctors is the other and that's how you achieve growth in clinical programs , and improvement in infrastructure. We expect to refurbish the present infrastructure as well as add 140 beds by end of this calendar year. As far as clinical programs are concerned – oncology represents less than 2% of the total revenue in Max Lucknow. So, that's a major driver for us. As you know, it is a high ARPOB business and contributes more than 24 -25% to our overall revenue. So, if you look at implying that to Max Lucknow , you will see a major fillip in ARPOB. Like I mentioned, we've seen that in the first three months literally . We acquired it in end of March , and in April, May, June compared to April, May, June last year, there's already been a ~58% increase in EBITDA, and this has largely come through all the low hanging fruits, etc. , which were there.

The real changes that we may have made, you're going to have all of those pay dividends in the quarters to come . Also, typically we have a higher proportion of surgical business in our hospitals and a lower proportion of medical business, whereas in both Nagpur and Lucknow, the situation is the other way round. So, when your surgical mix moves up with respect to the medical mix , you see higher ARPOBs in any case. So, there are plenty of levers. We are not sort of concerned about that. And frankly, the kind of traction that we are getting and the early wins that we have been able to demonstrate over there, gives us a lot of confidence that we should be able to ramp up both ARPOB as well as profitability there.

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Amey Chalke: So, this oncology addition, etc. should we expect it in the existing unit or it will happen once the expansion for the bed addition happens?

Abhay Soi: No, no. In the existing unit. We already have bunkers for radiation, etc., which are non-operational. We will just operationalize this. The 140 beds that we are adding as well as refurbishment that we are doing is going to happen by the end of the year.

Actually, even the new tower that we are building is going to be done over the next 24 months because it is 27 acres of land and we don't have to build so many basements, similar to what we are doing at Max Smart. But yes, the oncology business should ramp up in the current year itself. It may not ramp up to 25%, but you will see ramp ups in the quarters to come immediately, as well as in the surgical business. Even after we take over these, when we start adding doctors, when we issue letter of intents and they sign up to join us, they still have 2-3 months' notice period. So, it takes them time to join us and as the infrastructure, the new equipment that has been ordered, etc. comes through, you will see more and more teams coming up. Right now, all the technology, all the equipment, etc. is being replaced, right?

Amey Chalke: The second question I have is on the Dwarka unit, which has been recently commissioned. If you can provide some timeline over there, how the ramp up will happen? And in terms of the Speciality, is it at par at present with the other units or do you think the improvement will happen over a period of time?

Abhay Soi: So, in terms of the infrastructure, it is at par. I think the bunker is not ready as yet. That is going to be ready in due course. But other than that, I think as far as the infrastructure and technology is concerned, not only it is at par, but it's also the new generation. I would say it's perhaps one or two steps ahead of the existing infrastructure. We have again seen very good traction

over there. It is a massive facility. It's the best infrastructure of its kind in the region of Dwarka. There's a huge amount of population. Again, fantastic traction is what we have seen in the first month itself. And I think that should continue. We should have a break even within 6 to 9 months.

Moderator: Next question is from the line of Rishi Mody from Marcellus Investment Managers.

Rishi Mody: Abhay, just on the Mohali hospital that you are planning to add four years out, right, is this going to be firstly an O&M agreement the way we have with Devki Devi and those kinds of hospitals or is it an outright rental sort of a model that we see in retail where we don't incur any CAPEX, we have to just put in a security deposit, and we start operating on it?

Abhay Soi: It is the latter, we're leasing it. It's an outright rental / lease model where somebody else is putting the infrastructure and has invested in the land making a built-to-suit. It's a 50-year lease agreement. The only difference is normally you have commercial leases for 9 or 15 years, but here there's a 50-year lock in.

Rishi Mody: So, just from a broader conceptual point of view, right, are we seeing incremental interest from, say, the builder or real estate community towards having this sort of models or like if you could just share your observations from across north India or rest of the country?

Abhay Soi: It depends on the tenant. Today, say you tell a builder to make a built-to-suit and we will give you 8-8.5% yield, with 3-4% increment every year, which is what the market is, as far as commercial real estate is concerned. As far as the builders or developers are concerned, they are happy to do it for commercial real estate, for retail real estate or for hospitals. The question is how many hospital chains can offer this. Your own

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ROCs have to be significantly higher than what you are paying out over there. In our situation, we are able to take up these hospitals because our ROCs are significantly higher. And so, this built-to-suit model works for us. And it certainly works for the developers. They are taking a punt on our balance sheet effectively and that is where they're getting the confidence that we will be around for 50 years, and they'll be able to, even after making that particular hospital, dedicate it to us for 50 years. Because do remember, this is a built-to-suit, so it's as per our design. If somebody is building something under certain specs and leasing it out to you for 50 years, he has to be very confident that you will be around for 50 years and your balance sheet will be able to hold it. I think that's the punt a developer needs to take because eventually you're a tenant, but here you're a tenant for 50 years. It's not as if you can move this hospital and you will start using it for a mall or give it for some other purpose or hotel or whatever else it is.

Rishi Mody: I think you went through the capacity addition plans and updates pretty quickly, but just on an overall basis versus Q4, any delays or any preponement, if you could highlight that piece?

Abhay Soi: Not really. So, we're not expecting any delays or preponements. I think, except for Vikrant, which is the last one in any case and is a 2027-28 generation hospital where we're expecting maybe a couple of quarters of delay. Other than that, we are not expecting any delays. So, you should have Mumbai, Mohali, as well as Smart up by

Q4 FY25 , Q1 FY26 .

Rishi Mody: Yogesh, I had a question for you on the tax rate piece. So, I am seeing a tax rate for this quarter has come up to 26%. Just wanted to understand because I am under the impression that we have a tax benefit in the BLK and the Nanavati and now with both, the Dwarka unit. So, is my understanding correct and this is just a factor of the profitability for these two charitable hospitals, if you could

just help me understand the tax piece for you all?

Yogesh Sareen: That's right. So, overall rate is 22.8% . The rate has gone up this quarter because we have losses in the new entities, mainly the Sahara hospital entity. Sahara hospital transaction was Rs. 993 crore. We funded this by Rs. 840 crore of loan into the entity. And as Abhay alluded to in his speech, we are changing the cost and capital structure in the company. So, since we have losses in the company, we don't do the DTA as per our policy till such time the company is profitable. That means that you have these losses, but you have tax on the other entities. That is the reason why the rate has gone up, but otherwise the rate was same as in Q4 . Q4 was 21.8% and we do think that going forward it will be around 22%. Also, for BLK, the agreement is that whatever is the net profit , 98% of the profit comes to us. So, the moment we withdraw that money from there, there are two taxes on that : one is that there is GST and also other is that when the money comes on this side, then we have to pay tax on it , right? So, to that extent, BLK doesn't allow me any tax shields.

Yogesh Sareen: We don't trap money in trust. We take it and we pay full tax on it.

Rishi Mody: For example, now in BLK, there's no capacity expansion plan, so you are taking the money, repatriating it to the holding company, but Nanavati, we have a plan of expansion, hence we are not repatriating, but once that expansion is done, it will also become like BLK, is my understanding correct?

Yogesh Sareen : That's right, but also this formula is on cumulative basis . So, what happened is that BLK wiped out their losses on a cumulative basis in Q2 last year. So, the withdrawal has happened from Q3 onwards . Similar thing will happen for Nanavati, the

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withdrawal will happen but that will happen once the cumulative profits are made in that trust.

Rishi Mody: So, maybe 1 -2 years down the line, it will start paying off?

Yogesh Sareen : Yes.

Moderator: Next question is from the line of Damayanti Kerai from HSBC Securities and Capital Markets (India) Private Limited .

Damayanti Kerai: My first question is on the margin contraction which we have seen in 1st Quarter . Can we attribute most of that for the new facility cost or between 4Q and June quarter, have you seen any major change in the business mix say in terms of medical and surgical volumes which are coming to your facility?

Abhay Soi : We have clearly seen a change in the clinical mix. It's moved to a higher -end clinical mix. And like I've mentioned in the past, when you do higher -end surgeries, you do more robotics and more oncology, etc., they typically have lower margins in percentage terms, but higher margins in value terms. Whilst you see a flattening of EBITDA margins, you see an increase in EBITDA per bed on the existing hospitals as well. The takeaway should be that what is the increase in EBITDA per bed because eventually we have got two things - we have got inventory and we have got number of days in a year. You would rather do a 20% margin on a Rs. 10 lakhs surgery than do a 50% margin on a Rs. 2 lakhs surgery, right? So, the percentage is irrelevant. Other thing is that we had a reduction in OP footfalls, like I mentioned , in the current year and these OP footfalls were largely from immigration because of disruption in visa from Canada and from UK and a little bit in Australia. That has also had a couple of percentage points impact.

Damayanti Kerai: My second question is a couple of your hospitals are scheduled to start in another 6 to 12 months if I may say, Nanavati, Smart . So, just want to understand when you are about to start a new facility, when do you start your pre -launch activities and what kind of cost you are anticipating for these upcoming units?

Abhay Soi : Unlike Dwarka, which is a new facility and you need to actually

have a minimum

number of doctors and staff, i.e., you pretty much need to staff the entire hospital. In that case, before you apply for even a nursing home license, you are sitting fully staffed, then you make the application for the nursing home license and then you need to promote the facility by advertising it, etc. by onboarding doctors and getting all the insurance companies and TPAs onboarded , by getting institutional

empanelment , by getting corporate tie -ups, etc. You are really starting from ground zero. All of the facilities that you have alluded to right now, be it Nanavati, Mohali as well as the Max Smart , they're all existing facilities. So, we don't need to promote the existing facilities. There are enough people waiting in the ER over there, waiting for beds . In fact, it's going to be very similar to what happened in Shalimar Bagh. In Shalimar Bagh , we didn't have to incur any pre -op expenses or promote the facility or even wait to get the contracts, etc . All of the contracts were already signed up at the existing facility. So, you won't see any pressure on margins over there. I think almost from day one you are good to go.

Damayanti Kerai: My last question is a clarity on your Mohali planned hospital. So, you said it's a 50 - year long lease and it's a built to suit premise which you will be getting. So, there will be no CAPEX once you get the facility lead, just say, equipment and then people cost which will be going towards this?

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Abhay Soi : That's right. We will be providing the medical equipment , the security deposits, which is interest bearing , and loose furniture . The entire project cost will be incurred by the developer, including the land and building, etc. More importantly, any delays, any cost and time overruns, everything will be on to their account. This is very similar to what we have done in Dwarka .

Damayanti Kerai: The way like Dwarka, it's O&M facility which you got into your network, but -

Abhay Soi : This is not an O&M; it is a lease . But the same arrangement like Dwarka, in the sense that developer will develop it and incur the project cost . We only bring in the equipment and fit-outs. Whether you look at O&M or a lease, the commercials are pretty much the same and the arrangement is asset -light.

Moderator: Next question is from the line of Kunal Randeria from Axis Capital.

Kunal Randeria: Abhay, in the last few quarters, the number of your institutional bed is stagnant around 29%. Do you see headroom for some improvement over here?

Abhay Soi: We do see headroom for improvement here, but usually Q1 and Q3 are weaker quarters. So, compared to a Q4, you take your foot off the accelerator since that's not the time because your occupancy levels subside at that point in time and you are not going to churn your patients. Typically, you will see churn happening in the Q2 and Q4 .

Kunal Randeria: Because even I think in last quarter it was somewhere around 29%, right in the last five or six quarters somewhere in this range.

Abhay Soi: But that's a Q4, right, the fourth quarter is a strong quarter. So, if you see Q3 versus Q4, there may have been some change. But if you look at Q4 versus Q1, you won't have a change. So, typically now again from Q1 to Q2, you will see a change, but Q2 to Q3 you won't see a change.

Yogesh Sareen: I would also say there's a change that you don't see. That change is in the ARPOB of the business, right? We do the PSU business, but while you may see the same beds, but the throughput of those beds has gone up. If I see the improvement in the ARPOB year-on-year, it's a 10% improvement in the ARPOB of the PSU. This means that within those beds we are trying to do higher-end work.

Kunal Randeria: Second question is on ARPOB. This quarter saw around 7% growth on your existing units. So, is this the new normal that we should expect going forward?

Abhay Soi: No, this is because of immigration and visas, that's had maybe a 2-3% impact on ARPOB. That is a business we had last year. But this year there's been some disruption because of Canada and UK not issuing immigration visas, and we have seen the same with Australia as well. We believe it's a temporary phenomenon and that business is a little subdued . But if it doesn't, then we kind of repurpose that space to put more dialysis beds , etc. We will catch that up. I think 7% would be a little muted compared to perhaps what we have had in the past.

Kunal Randeria: When you mention that your growth should be higher than 7%, does it include all the beds or just the existing units?

Abhay Soi: New units will be a significantly higher ARPOB growth . New units are starting at a low base. So, if you combine the two, then your growth should be more. You had a revenue growth of 21% in the new units .

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Kunal Randeria: So, you are saying that maybe Lucknow and Nagpur whose ARPOBs are lower should grow at a much faster pace than your existing hospitals?

Abhay Soi: And Dwarka . It's the new hospital, right. Since your percentage growth, the absolute base is lower, they're starting on a lower ARPOB. If you add a 21% and 64% growth in Lucknow and Nagpur on revenue and EBITDA alone so you can imagine huge

amount of growth coming from that.

Kunal Randeria: I think Lucknow seems to be like a go to spot for a lot of your peers also. Do you think that so many beds coming up in that area, there is enough medical talent available both from the doctors as well as from the nursing perspective?

Abhay Soi: Actually, I don't know of any other hospital coming up there. And besides Medanta, which has been there for a while and a smaller Apollo facility which has again been there for 5-6 years, and Max Lucknow (erstwhile Sahara hospital) was also an existing facility and other than that we have only bought the land in Shaheed Path. But I haven't heard of any other hospitals really coming up there.

Yogesh Sareen: But nevertheless, we see an abundant supply of talent there. In fact, the doctors that we employ in Delhi, a lot of them are from SGPGIMS, Lucknow. So, to that extent, we haven't really –

Abhay Soi: Historically, Lucknow has been the place where we have got most doctors. Around 25-30% of our Delhi doctors are from Lucknow. Historically, Lucknow has had medical colleges and government medical hospitals, etc. and therefore a big base of doctors there.

Moderator: Next question is from Dheeresh Pathak from Whiteoak Capital Management.

Dheeresh Pathak: Sir, which quarter do we take the annual increments -- is it this quarter or -?

Abhay Soi: 1st of April.

Dheeresh Pathak: For the Mohali asset that we mentioned, would it be possible for you to share how much the real estate partner is investing in the land and building for the 250 beds?

Abhay Soi: I can't share that. He owns the land. We have given our specifications and our BOQs, but they don't share what they're spending. But the building alone will cost about 80% of the total project cost if we were to do it.

Moderator: Next question is from the line of Adrit Chaturvedi from Nomura.

Adrit Chaturvedi: So, my question is specifically regarding the inpatient business. So, you have reported this quarter around Rs. 1,600 crore of IP revenue. So, that's around 21% year-on-year growth. And also, the volumes have grown by roughly 17%. So, that translates to about like 3.5% growth in realization. Now, this 3.5% is sort of one of the lowest in the preceding quarters, but at the same time, the volumes are like highest in the preceding quarters. So, is this the sort of business profiling that we should come to expect as you grow and you expand that your realization per patient on the IP business is going to be around 3-4% but you are going to focus more on volumes?

Yogesh

Sareen: I think you are looking at the overall number, which includes the new facilities. You should compare the number on year-on-year basis for the existing hospitals separately because the new hospitals' ARPOB is obviously lower. To that extent,

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you will find a volume increase but not an increase in the revenues because of the ARPOB issue. As Abhay mentioned that as we change the mix of the business in those new hospitals, we also put in oncology there and have more talent there, the ARPOBs will start inching up slowly. So, there's a difference of Rs. 80,000 to Rs. 45,000. The new hospitals have ARPOB of Rs. 45,000 and our existing hospitals have an ARPOB of Rs. 80,000. We think that Rs. 45,000 will surely come up fast. So, it should be anything between Rs. 60,000-65,000 let's say in a year's time and that's how this will catch up. Then you will find that the volume increase and the overall revenue increase will start to have lesser differences.

Adrit Chaturvedi: So, the backdrop also on the levers for these realizations, right? So, like if we look at the sum of shares of oncology, cardiac sciences, orthopedics, neuroscience and renal, the share has come down this quarter on year-on-year basis and that's the first in a while and -?

Abhay Soi: Because you included the new hospitals, which has less than 2% oncology share.

Yogesh Sareen: Because the new hospitals share is very different from what we have in the existing hospitals.

Adrit Chaturvedi: When do you suspect like, as the expansion ramps up like we would have a sort of a resting rate where as you said ARPOB grows at roughly 9-10%. So, I guess the realization grows at 8%.

Abhay Soi: No, it doesn't work like that. Please understand what it is that we focus on? We focus on acquiring facilities, turning them around, and getting higher ROCEs over a period of time, but we look at it on an incremental basis, right? If we bought something with a lower ARPOB and we are demonstrating an increase in that ARPOB, it may or may not get to the same ARPOB levels as the Network average. For instance, Nagpur hospital will not have the same ARPOB, eventually, that the Delhi hospital does nor will it have the same EBITDA per bed. What you have to see is vis-à-vis what you

paid for it, what is the sort of return you are going to get. What we are seeing is that levers for doing all of that is going to be better clinical mix, doing more surgical work, higher oncology business, etc. Now, you may get to that mix of 25% or 24% oncology, okay, but you may not get to the same ARPOB as Delhi because the rates in Nagpur are cheaper. I am not even chasing the Delhi ARPOB for it, right?

Adrit Chaturvedi: Lastly, so a lot of your peers have mentioned that they're sort of translating a lot of the lower ALOS businesses to like day care. And right now, your IP business is roughly 80% of overall. So, you don't give any separate numbers there. So, do you have like any kind of qualitative color regarding what's the OPD margin profiling versus daycare and how it helps your margins, if you sort of translate a lot of those lower ALOS to daycare and non-census beds?

Abhay Soi: Look, the lower ALOS business has its own sort of tenets with it, because currently insurance companies don't cover procedures where you don't spend the night in the hospital. So, you need to spend the night for insurance companies to settle the claim.

Daycare may not be covered by the insurance companies. That itself is kind of a hindrance. The longer you stay in hospital, the hospital loses money since the surgical part is what makes money. So, if you can translate anything into a daycare, get a person in the morning, do the procedure and have him out by the evening, your yield is significantly higher. The question is, why is it that you can't do it, because for most of the procedures, you need to stay in the

evening night in the hospital to get to claim insurance.

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Adrit Chaturvedi: So, on the cash pay patients is there a targeted strategy there to like to move a lot of these lower ALOS to daycare or is it something that's not the focus right now in terms of your non-IP business?

Abhay Soi: So, we do that as hygiene, right? I'm not sure exactly what you are seeking. If you are able to articulate a little better, then perhaps I can answer your question.

Yogesh Sareen: For us, ARPOB is the one that we pursue. The daycare procedures will be all run-of-the-mill procedures. Our focus is to go up the value chain. So, we will be pursuing liver transplants, oncology, neuro, cardiology, cardiac surgeries, etc. I think those are the sort of procedures that we pursue. So, while obviously there'll be shift of some procedures that you do in one day stay to daycare. Yes, we can and we do, but that's not the focus of the hospital administration to really pursue.

Abhay Soi: It really depends on what your proportion is. For everybody, the ALOS is different because the clinical programs are different. Now, if you do a liver transplant, your ARPOB is higher, your EBITDA per bed is higher. But the fact of the matter is that the patient stays much longer. Now, would I replace that with a dengue patient, which has maybe a lower ALOS and higher percentage margin but lower sort of EBITDA per bed and ARPOB? No.

Moderator: Next question is from the line of Prashant Nair from Ambit Capital.

Prashant Nair: As your new projects now start coming onstream over the next few years, if you take some of your larger brownfield projects like say in Saket or Nanavati, when the new beds come online, do they have similar case mix, payor mix, ARPOB, etc., as the older base, or would we start at slightly different levels and then match up to how the existing hospital beds does?

Abhay Soi: So, you are going to have a different mix to start with, because first and foremost you try to ramp up occupancy over there in the new beds. And when you do that, then you are not kind of very strict on the payor mix as well as the clinical mix. People who may not be getting priority today because that's a lower end payor mix or a lower end clinical mix, you open your doors to it. So, your ARPOB from the incremental beds is lower. Having said that, the EBITDA per bed is higher. So, your profitability is higher because you get huge amount of operating leverage over there. Your main costs, associated costs, etc. are already being incurred by the existing hospital.

Prashant Nair: Second question is in these projects, would the investment in the beds follow a similar pattern for say a new Greenfield project where you front load everything, right? So, in brownfield beds, is there a period maybe after the first year, second year when you have to step up either investment in people or equipment for a ramp up, or are they all done at the beginning?

Abhay Soi: This is all done in the beginning. So, you don't have to step up anything. For brownfield, you are operating with the same set of people, the same majority of the equipment is already there.

Moderator: Next question is from the line of Tushar Manudhane from Motilal Oswal.

Tushar Manudhane: So, on international patient ARPOB if you could just help us understand like how much decline in the ARPOB that has happened because of these issues related to

visa's over there year-over-year or quarter-on-quarter?

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Abhay Soi : International business has got nothing to do with immigration business. The se are two separate streams altogether. The immigration business is where people come for visas, they have to get thems elves tested , etc. in our immigration departments. International business is the incoming business that you have. I mean, immigration business is if you have to relocate to Canada, then you h

ave to get your medical

tests and all done. It's outpatient business. The international medical tourism is people who are coming to India. So, these are two separate business streams.

Tushar Manudhane: So, as far as international business, just to understand what kind of ARPOB growth we have seen in the last two or three quarters?

Yogesh Sareen: Typically, the ARPOB in the international business is one and half times of the overall ARPOB. International business growth on ARPOB would be 7% to 10% if you see last year. But it also changes from quarter to quarter , depe nding on what is the procedure configuration that you are getting . So, it will be overall growth. These tariffs are linked to the self-pay tariff.

Tushar Manudhane: So, largely similar to that of the domestic patient in terms of growth, not in terms of value, but in terms of growth, we are largely similar in both international as well as domestic patient, is that the right way to look at?

Yogesh Sareen: Pricing is linked to the self-pay tariff, right? We charge a premium on that, but it's linked to self-pay.

Tushar Manudhane: How much is the OPEX at Dwarka like operational costs when you say break even within six to eight months, so if you could share how much OPEX and how much of that is already into the P&L? You said that we need to get the licenses and also you need to have doctors and nurses already onboard. So, does it mean that it is already in 1Q?

Yogesh Sareen: In this quarter , we have Rs. 6 crore of pre -launch costs. These are the cost s of the doctors and the staff that we had hired. Before you get the nursing home license, you are supposed to have people on your payroll . You need to demonstrate to the Delhi Health Department that we have so many people available for treating patients. So, this is that cost. We haven't had any revenue from the hospital in Q1. This cost will obviously build up . This is the cost which is for the quarter. But obviously if you take June month, the cost will be higher than the average . Rs. 6 crore is not for the full quarter. If I take July, you will have further increase in the cost because we are adding more people. So, we opened 94 beds at this point of time and I can't give you a P&L statement in ter ms of what the cost would be, but we do think that by the 8th to 9th month we should be in a breakeven state, which means that in Q4 , we should start to see breakeven numbers at EBITDA level for Dwarka hospital.

Tushar Manudhane: Trying to understand till that time how much drag will be there on the existing EBITDA of say 500 crore, so from that perspective just wanted to understand?

Yogesh Sareen: We said Q4 will breakeven, right? So, that means the drag will happen till Q4. So, in Q4 you will see that loss becoming kind of breakeven for Dwarka.

Tushar Manudhane: And how much further investment would be required in Lucknow for these 450 beds once the EC approval is there?

Yogesh Sareen : So, I think this will be around Rs. 700 crore plus for the beds.

Moderator: Next question is from the line of Amit Kadam from Canara Robeco Mutual Fund.

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Amit Kadam: Sir, my question is like I am just reading the Slide #10 and I am trying to refer the outpatient growth what we have seen, the fourth chart, where it shows that 4.5% has been the growth for the like -to-like the existing hospital. Whereas when I compare it for the Q1 FY'24 a year back, this run rate was like almost 13%, 14% growth rate. How do I read this particular thing 14% versus 4.5% right now?

Yogesh Sareen : OP consults have come down and Abhay alluded to it in his speech as well. We have immigration business, which is down by around 48%. We have three centers for immigration: two in Punjab and one in Delhi. Now, the footfalls have come down by 48% there. So, obviously that has an impact on these numbers because that's OP business. So, we do think that

this imp act will go on till Q3 . Q4 was the normal quarter

because we have this change in the visa regulations from 1st of January this year.

So, there will be some impact that you will have.

Abhay Soi: But the good part is this is not the OP business which leads to in-patient admissions, right? I mean, for immigration business, OP patients come do the consult and go out

the same day . So, they don't get translated. Normally reduction in OP business or in outpatient footfalls would be concerning because what that means is you are going to have lower inpatients going forward, but this is not the category of people which convert to inpatients. Also , last year , I think we opened Shalimar Bagh and so 3% of that was because of the capacity expansion , as Keshav pointed out . It was new capacity , which had opened in this quarter last year.

Amit Kadam: So, like the kind of capacities what we are going to see in next one odd year, what is the metrics or what number you would like to see because for a kind of a run rate in terms of volume what we want to see and partly driven by ARPOB, but partly driven by volume, this number should be high single digit, that's what could be a number for maybe a steady state growth?

Abhay Soi: Are you talking about the existing hospitals or the new hospitals or combined?

Amit Kadam: Existing, sir, because combined may not be correct reference but like when I just see that inpatient and outpatient both and you say that partly would be the international but then I am just like little confused that international patient coming off may not have a so much -

Abhay Soi: So, the material part is the inpatient.

Amit Kadam: Yes, so inpatient is 5.5 %. That's what is the second part of the question is then what is the run rate I need to see in this particular number to maintain our steady state growth rate as a tracking point?

Yogesh Sareen : So, I will just give you an example of what happened in Shalimar Bagh. This is the hospital where we added 43% bed capacity, and the moment you had the capacity available, the capacity got filled up, the inpatient volumes and the occupancies went up by 34 -35%. That means the outpatient and inpatient footfalls went up by 34 -35%. That's the same thing we expect here to the extent that if there is a brownfield expansion and let's say in a 300-bed hospital, we are adding 150 beds, and we plan to open all those 150 beds in one go , so to that extent you will find that the volume will expand and within 2 -3 quarters we should be reach normal occupancy levels as well –

Abhay Soi : If you want to do apples -to-apples , like you added Shalimar Bagh capacity last year same time, this year you will add Lucknow and Nagpur and then see what the overall growth is. That's why I asked you the specific question, because you are not doing

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apples -to-apples, right. If you are looking in existing hospitals last year, then you have to look at existing hospitals without the Shalimar Bagh expansion.

Amit Kadam: But sir, this is a volume count, right? So, it is irrespective of what hospitals I am trying to count because even I give that benefit of Shalimar Bagh also now part where in the existing part, and if it is 5.5.

Abhay Soi : I just look at it differently. So, let's say last year, you had 3,400 beds and then you add let's say 200 beds to it because you have done a brownfield . Now, you have moved to 3,600 beds. You are not comparing the 3,400 to the existing beds today, you are comparing the 3,600 beds to the existing today, but you had that jump of 200 beds last year. Likewise, if you take a jump of 900 beds this year, then you need to look at overall footfalls.

Yogesh Sareen : Let me convey it the other way around and explain this. So, basically, we have the highest occupancy among our peer group , which means that we cannot have more in-patients. So, what needs to co

me first? You need to have capacity to have more

in-patients. So, to that extent we are limited by the number of beds that we have.

And the example that I gave to you was Shalimar Bagh. The moment we added beds there , the volumes went up. So, as the capacities will get added, we do think the growth in the volume will materially change because we don't have beds right now.

Abhay Soi : If you don't come up with brownfield , if you don't come up with capacity addition, we're already operating at 77% occupancy. I don't have room for my beds ' occupancy to go up. It can only be marginal .

Amit Kadam: This explains that particular thing because maybe on the flip side, I was maybe trying to see is there a shortfall in terms of whatever the demand expectation is.

Abhay Soi : No. I mean, there's a shortfall of supply and that's why we are doing all these brownfields.

Yogesh Sareen : And that's where the Shalimar Bagh fitted .

Amit Kadam: And you mentioned that inpatient and outpatient doesn't have a high correlation with that particular thing?

Abhay Soi : No, it has a very high correlation, but the outpatient business that has reduced is the visa immigration business. So, let's say if you immigrate to Canada, you will come to our hospital and get the medical certificate from us because we were accredited with

the Canadian Embassy. Now that person was not going into inpatient, he was not getting admitted for a procedure. Since Canadian government is not issuing visas right now, so people are not coming for that medical certificate.

Amit Kadam: If you would have added that missing part, what run rate this number 4.5 would have been ?

Abhay Soi : My inpatient would be the same.

Amit Kadam: Not inpatient. Outpatient, then, sir. That 4.5 would have jumped up?

Abhay Soi : Yes. Then that would have gone up.

Moderator: Next question is from the line of Sumit Gupta from Centrum Broking.

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Sumit Gupta: So, just want to understand on the bed share mix. So, basically from the existing universe bed share has remained majorly between 29% to 30% for the institutional segment, however overall institutional bed share mix has come down to 27%. So, how do you see it going forward over the next two to three years, once all the facilities get integrated and they are in full stream?

Abhay Soi: There are two aspects. Like we said that Q1 is a weaker quarter, so you have less demand. Therefore, you don't put your foot on the accelerator and start driving out institutional business, you accept and accommodate because you want to fill your beds. Normally, you see this happening in Q2 and Q4 where you have reduction in institutional business. Having said that, like I mentioned, when you come up with new brownfield capacity, then occupancy is the criteria, not the caliber of clinical mix as well as the payor mix. So, at that stage, we will be taking more institutional. But having said that, even for institutional business, the EBITDA per bed which we're able to eke out is significantly higher than what it is for existing beds, and that has been our experience in Shalimar Bagh also. So, when we opened the new beds there, we saw the incremental ARPOB of the new beds come down because you were taking institutional as well as lower payor mix, etc. but the EBITDA per bed pertaining to those incremental beds was higher than the EBITDA per bed of the existing hospital.

Sumit Gupta: And for the international patients also, you see bed share mix going upwards or remaining at the same level and like how do you see this international business going forward?

Abhay Soi: The growth in that business has been outstripping growth in other segments, so percentage share is incrementally going up.

Yogesh Sareen : So, I think Q1 is an aberration there, but growth has come back as we speak and there was some temporary blip because of situation in Bangladesh, Yemen, Somalia,

India,
etc. –

Abhay Soi: Nevertheless, as long as your growth in international business is more than growth in your other business, you will see the bed share only go up.

Moderator: Next question is from the line of Alankar Garude from Kotak Institutional Equities.

Alankar Garude: We have added two new cities to our network in the past six months. Other than these two, our focus seems to be on growing in the existing market at least over the next four to five years. The question is beyond what we have already announced so far on the expansion front, is it fair to expect a relatively higher expansion announcement coming through towards the newer markets?

Abhay Soi: I think growth in our existing hospitals is not a focus, growth in our existing hospitals is a necessity, because we have run out of capacity. We are already operating at 77% and we need to increase bed strength in our existing hospitals. That's why you are seeing this incremental expansion happening and this is something we embarked on 2-3 years ago. So, they've been under construction and maybe between the next 8 to 15, 20 months you are going to see a lot of that capacity come through. Now, having said that, we still have our balance sheet. You know all the expansion is being incurred through internal accruals. So, we are intending to go to other markets. They are high ROCE markets, high opportunity markets, and frankly, you can imagine whatever multiple we have paid for these two hospitals, has already come down by 60-odd% simply because we have moved the EBITDA up in the first three months. So, we see opportunities in other markets as well and we are focusing on them. That's where the focus is going to be. This doesn't mean that we don't look

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at metros. The point is that in metros, it's going to be that much more difficult to be able to acquire hospitals or to acquire land because it's not available.

Alankar Garude: So, maybe a link to that one, how are you looking at this expansion from an operational standpoint, I mean any need to beef up the senior management team,

can senior management bandwidth be increasingly an issue going forward?

Abhay Soi: On the contrary, I think if you actually see the number of locations that we have added in the last 5-6 years, is essentially three; one is Dwarka, which is opened, one is Lucknow and one is Nagpur. So, from maybe 17 hospitals, we have gone to 20. Everybody wants career progression, right? We have got close to 30,000 people working for us, and if it's one unit head, he also wants, over 5-6 years, his area of influence to increase. He's handling one hospital, now he wants to handle two, the person who is handling two, wants to handle three or four. So, I think providing that growth, providing that segue for people's career path, is a necessity today. And essentially, a lot of our expansion is brownfield. Now if I increase 800 beds by another 400 beds in Saket, it's the same location, you are not really operating another facility. So, from 200 beds in Mohali, you go to 355 beds, you don't require management bandwidth for it. I mean, you are just doing more of the same effectively. And 85% of our growth coming through going forward is coming through brownfield. So, it's your existing capacity that you are increasing. From a leadership standpoint or down the line, with three locations getting added on top of 17, there is not much of a bandwidth issue. On the project side, of course we have augmented the teams, because when we embarked upon all of these expansions, we needed to fortify those teams. So, that we focused on. Other than that, not really. And of course, we have strengthened the teams, etc. but that happens through due course. I don't see bandwidth issues because frankly the expansion hasn't been that much in terms of locations.

Alankar Garude: So, maybe that question becomes more relevant

as we go beyond these 20

locations. So, the second one then, Abhay is we have been increasingly hearing about resistance from insurance companies on rising healthcare tariffs. Over a period of time, do you expect any change in the balance of power when it comes to negotiations between corporate hospital chains like us and the insurance companies?

Abhay Soi: Insurance companies by the end of the day piggyback on what your schedule of charges is, what your rate list is for cash paying patients. And frankly, I think you have the most democratic system over here because no hospital has a monopoly. Patients are choosing to go where they want basis the value proposition that they are able to get. It's not as if people are not kind of reducing their prices to get more patients. Everybody does that. None of the big chains are price makers, we are all price takers. Eventually, 90% of all healthcare is provided by smaller nursing homes, etc. Every year they know what the inflation is, they come up with a particular price, and when we seek price data, that is the basis on which we increase our rates. From our standpoint, do keep in mind that our EBITDA per bed is still 55% better than the second-best player in the industry. Our ARPOBs are maybe 20% better than the second-best player in the industry. So even if there is price negotiation, that sort of comes down for us. I frankly think that the insurance companies are a lot more fragmented.

Moderator: Next question is from the line of Shubham Dugad from Purnartha Investments.

Shubham Dugad: I want to know how many nursing seats you have across hospitals?

Abhay Soi: You mean nursing colleges?

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Shubham Dugad: You have some nursing facility in Lucknow, I think.

Abhay Soi: We have one nursing college in Lucknow and that's around 100 seats.

Shubham Dugad: And do you want to increase that or something like planning there?

Abhay Soi: No, we have tie-ups with many nursing colleges.

Shubham Dugad: How many beds being operationalized in Dwarka?

Yogesh Sareen: So, it's a 303-bedded hospital, and so far, 94 beds are operational.

Moderator: Ladies and gentlemen, I now hand the conference over to the management for closing comments.

Abhay Soi: So, thank you everyone for joining us for the Q1 FY25 Earnings Call. We look forward to catching up again next quarter. Thank you.

Disclaimer: This is a transcription and may contain transcription errors. The transcript has been edited for clarity. The Company takes no responsibility for such errors, although an effort has been made to ensure a high level of accuracy.

Call: Nov 2024

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November 13, 2024

Listing Department ,
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Symbol: MAXHEALTH Listing Department,
BSE Limited
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Scrip Code: 543220

Sub.: Transcript of Earnings Call held on November 7 , 2024
Ref.: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Sir / Madam,

Please find enclosed copy of transcript of earnings conference call, organised on November 7 , 2024, on financial results of the Company for the quarter and half year ended September 30, 2024.

The said transcript is also available on the website of the Company at www.maxhealthcare.in/financials#earnings-call.

Kindly take the same on record.

Thanking you

Yours truly,
For Max Healthcare Institute Limited

Dhiraj Arora
SVP - Company Secretary and Compliance Officer

Encl.: As above

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Max Healthcare Institute Limited
Q2 & H1 FY25 Earnings Conference Call
November 07 , 2024

Moderator: Ladies and gentlemen , good day and welcome to the Max Healthcare Institute Limited Earnings Conference Call. Please note that this conference is being recorded.

I now hand the conference over to Mr. Anoop Pujari from CDR India. Thank you, and over to you, sir.

Anoop Pujari: Thank you. Good afternoon, everyone and thank you for joining us on Max Healthcare's Q2 and H1 FY25 Earnings Conference Call .

We have with us Mr. Abhay Soi – Chairman and Managing Director , Mr. Yogesh Sareen – Senior Director and Chief Financial Officer and Mr. Keshav Gupta – Senior Director , Growth , M&A and Business Planning of the Company.

We will begin the call with opening remarks from the management , following which we will have the forum open for an interactive question and answer session. Before we start , I would like to point out that some statements made in today's call may be forward looking in nature and a disclaimer to this effect has been included in the earnings presentation shared with you earlier.

I would now like to invite Abhay to make his opening remarks.

Abhay Soi : A very good afternoon to everyone and a warm welcome to Max Healthcare 's Q2 & H1FY25 earnings call.

This financial year was projected to be one of limited growth. Our brownfield capacity expansions were scheduled for completion at year -end and our greenfield hospital in Dwarka would still be in its early stages of operation. However , the first half of the year has seen stellar growth fuelled by recent acquisitions and their successful integration into our Network . By expanding into new , but proven geographies , we have supplemented our growth momentum.

Max Lucknow and Max Nagpur have been fully integrated into the Network and have significantly improved their performance. Combined revenue reached Rs.130 crore for these two hospitals in Q2 , representing a growth of 40% year -on-year and 32% quarter -on-quarter. Compared to the previous year , the combined EBITDA nearly doubled to Rs. 33 crore in Q2 with a margin of 26%.

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The newly operational Max Dwarka hospital reported a revenue of Rs. 33 crore and EBITDA loss of Rs. 18 crore , with occupancy of 41% and Average Revenue Per Occupied Bed (ARPOB) of Rs. 80,000. This hospital has been performing very well and we believe that it will break even much before the end of the year and perhaps will be the fastest breakeven in our history.

We are excited about the recent acquisition of Jaypee Hospital , Noida. It is a marquee asset and will strengthen our presence in the National Capital Region. The enterprise value for this acquisition was Rs . 1,660 crore and we expect to integrate the hospital into the Network quicker than others . In the near term , our plan is to increase the operational bed capacity from 376 beds currently to 430 beds by March 2025 and upwards of 480 beds by December , 2025. We are simultaneously strengthening the medical programs in urology , medical oncology , gastroenterology and neurosciences through a combination of external hires , infrastructure enhancement and technology upgrades.

Now coming to Q2 performance highlights ; this is our 16th consecutive quarter of year-on-year growth.

Overall network revenue stood at Rs. 2,228 crore , registering a growth of 22% year -on-year and 10% quarter -on-quarter .

Network operating EBITDA was Rs. 566 crore , but more importantly , excluding the extraordinary items comprising of Rs. 18 crore of start-up losses at Max Dwarka and Rs. 7 crore of transaction costs for the Jaypee deal, the operating EBITDA stood at Rs. 591 crore , reflecting a growth of 19% year -on-year and 17% quarter -on-quarter. Profit after tax (PAT) stood at Rs. 349 crore compared to Rs. 338 crore in Q2 last year and Rs. 295 crore in the previous quarter. Yet again , excluding the extraordinary items as mentioned above , PAT stood at Rs. 383 crore , reflecting a growth of 13% year-on-year.

From here on , I will mention all numbers and percentages excluding the extraordinary items.

1) Our average occupancy for the Network increased to 81% from 77% in Q2 last year and 75% in the trailing quarter , while the occupied bed days (OBDs) grew by 18% year -on-year and 8% quarter -on-quarter .

2) ARPOB for the quarter stood at Rs. 76,100, growing by 2% year -on-year and remaining flat quarter -on-quarter . Like -for-like ARPOB grew by 7% for the

existing hospitals .

3) Network gross revenue was Rs. 2,194 crore compared to Rs. 1,827 crore in Q2 last year and Rs. 2,028 crore in the previous quarter. This reflects an increase of 20% year -on-year and 8% versus the trailing quarter.

4) The international patient revenue stood at Rs. 178 crore . This reflected a growth of 12% both year -on-year and quarter -on-quarter , despite contraction in patient footfalls from Bangladesh and Yemen due to political unrest .

5) Network operating EBITDA stood at Rs. 591 crore , reflecting a growth of 19% year-on-year and 17% quarter -on-quarter . Operating EBITDA margin stood at 28.2% for the quarter.

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6) Annualized EBITDA per bed stood at Rs. 75.5 lakhs , remaining flat year -on-year and increasing by 6% versus the trailing quarter. Like-for-like EBITDA per bed grew by 4% for the existing hospitals.

7) Profit after tax was Rs. 383 crore versus Rs. 338 crore in Q2 last year and Rs. 295 crore in the previous quarter.

8) Overall free cash flow from operations was Rs. 464 crore , of which Rs. 217 crore was deployed towards ongoing capacity expansion projects and upgradation of facilities at acquired hospitals. Consequently , net cash position for the Network stood at Rs. 313 crore at the end of September 2024 compared to Rs. 66 crore at the end of March , 2024.

9) Continuing our effort to support the local communities we treated approximately 39,600 outpatients and 1,300 inpatients from economically weaker sections of society entirely free of charge.

10) Both our strategic business units continued to report significant growth in the revenue and profitability.

- Max@Home reported a top line of Rs. 53 crore , reflecting a robust growth of 24% year -on-year. It offers 14 specialized service lines across 12 cities , with over 50% repeat transactions.

- Max Lab reported a gross revenue of Rs. 47 crore , reflecting a strong growth of 21% year -on-year. It provides services in 50 cities through its network of over 1,100 collection centres and active partners.

11) Now coming to the status of our expansion projects :

- For capacity augmentation at Max Lucknow , finishing work for additional 140 beds is in progress and will be completed by December 2024 , as communicated earlier . Further , environmental clearance (EC), consent to establish (CTE) and fire no-objection certificate (NOC), etc. have already been received for the additional 450 -bed tower at Max Lucknow . We are completing the drawings for the same and will be appointing the contractors shortly .

- For 140 beds at Nagpur , 12 beds have been added in October 2024 . For the balance beds on additional floors , applications for EC, etc. are in progress .

- For 268 beds at Nanavati in Phase 1, we have completed most of the structural work. The project continues to be on schedule and we expect completion by end of the financial year , as communicated previously .

- For 400 beds at Max Smart , this project is on track. We expect its completion within Q1 FY26 .

- For 155 beds at Mohali , most of the structural work is complete and we

expect its completion again by Q1 FY26 .

- For 501 beds at Sector 56 Gurgaon , the structural work is complete up to the plinth and above. We expect completion of the first phase of 300 beds by end of Q3 FY26.
- All of these are on schedule and we will see, over the next 12 to 15 months , significant ramp up in our capacity , most of which via Nanavati , Mohali , as well

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as Max Smart (Saket) should be coming through in the months of April-June 2025 .

- For 367 beds at Patparganj , post issuance of NOCs by fire departments and water departments etc. , we have already submitted our plans and we have received the environmental clearance . This project is largely on schedule as well.

- For 550 additional beds at Max Vikrant (Saket) , EC, CTE, etc. have already been received and barricading work is on. The forest approval over here is delayed due to Supreme Court proceedings in relation to tree felling involving DDA and the Lieutenant Governor of Delhi. For the past 6 months , they have not permitted anybody to remove any trees. But we think that this should get resolved fairly soon.

And finally moving on to the overview of the company performance for the first half of financial year 2025:

12) Network gross revenue stood at Rs. 4,222 crore , reflecting a growth of 19% year -on-year.

13) Overall Network operating EBITDA grew by 17% year -on-year to Rs. 1,089 crore , reflecting a margin of 27.2%. While EBITDA per bed stood at Rs. 72.8 lakhs per bed .

14) In the first half , we generated Rs. 722 crore of free cash flow from operations , after interest tax , working capital changes and routine CAPEX , of which Rs. 430 crore was deployed towards ongoing expansion projects.

With this , we open the floor for any Q &A.

Moderator: Thank you. The first question is from the line of Amey Chalke from JM Financial Institutional Securities Limited .

Amey Chalke : The first question I have on Nagpur and Lucknow units , both have done well during the quarter. I understand that Nagpur might be going through the seasonal peak , but both the units are now delivering 20% + margins. Is it possible for us to give some colour – what would be the potential margins for optimally utilized unit both at the Nagpur and the Lucknow unit? Specifically , Lucknow , can it

match the profitability of

our existing hospitals in Saket , etc.?

Abhay Soi: Firstly, we do not give forward looking statements . But intrinsically , margins and percentage margins are only one part . They are still operating at lower ARPO Bs compared to the rest of the Network and compared to the potential in the market over there as well . In the coming quarters , we will see improvement in that. There will be improvement towards better quality programs , that means higher -end surgical mix, more robotics , more complicated surgeries , as the new teams that we have hired come in and kick in, some of them being very recent hires . We will see all the benefits coming through. For Lucknow so far , we have a bunker over there but oncology has not been a big play . It accounts for only 7-8% of revenue as against 26-27% across the Network. As we get the bunker ready and the new LINAC machine comes in place , you will see this number move up . And so, you will automatically see an increase in ARPOB . Our preoccupation , as you are aware , is never with margins in percentage terms , it's with EBITDA per bed . And we think there is a significant amount of juice over there with respect to EBITDA per bed.

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Also, I just want to add that as we are going to get 140 more beds over there , which is almost 50% more capacity , our operating leverage on the incremental beds is much more , so the EBITDA per bed for incremental beds will be significantly more.

You will see the margins expand because of that as well.

Amey Chalke : So, do you see the Rs. 70 lakhs per bed kind of EBITDA to be achieved in Lucknow ?

Abhay Soi: Well, we will not give you a forward -looking statements on that.

Amey Chalke : The second question I have is on the Sake t Complex . So, here we are adding almost 1000 beds or maybe 800 to 900 beds o ver the next 2 to 3 years. So , should we consider this as a brownfield expansion? Because considering this much number of beds are getting added , I believe that everything like utility , doctors , everything should be added as well. So , the delta in the margins will not be significant. Is my understanding correct or you think that this is like a plain brownfield expansion which is happening in this locality?

Abhay Soi: No, it works like a plain brownfield expansion. When you look at u tilities , you are looking at capital cost. As far as the operating cost is concerned , there will not be a significant increase in operating cost. If you look at the P &L, about 15% of our costs are related to doctors. And out of the 33% of indirect cost s, 23% is non-medical personnel – most of it is being management cost. About 25 -30% of the operating cost may increase on incremental basis , but we will still have signifi ca nt amount of room . In each one of the brownfield s that we do , the utilities are the same. We have to increase our utilities , whether it is chillers for air conditioning or sewage treatment plants (STPs), etc . but it is all in the same complex. Yet you have these benefits of a brownfield . Also, please note that it is not 800 -900 beds over the next 3 to 4 years. You will have to look at it slightly differently. We will have 375 beds coming up in the month of April -May 2025 . We expect a very quick ramp up of th ese 375 beds , as we have witnessed in our other brownfield expansion s. The next set of 550 beds will come up after 3.5 years. Basically , we will have a run on these 375 beds for the next 3.5 years. Our belief is that these beds are going to get occupied in very quick time and we a re going to run out of capacity way before our new beds , which is another 550 beds in Vikrant , are going to come out.

Amey Chalke : And the third question if I can squeeze in , our profitability for the core business is still healthy at 20% + margin but it has dropped slightly year -on-year. If you

have to call out anything there.

Abhay Soi: Again , the point I would like to highlight is that you need to change your view o n the margins. When one starts doing high -end surgeries , or high-end payor mix, margin in percentage term come s down but it goes up in absolute terms. We would rather do a Rs. 10 lakh -surgery with a 20% margin than a Rs. 2 lakh -surgery with a 50% margin. If you look at medical patients , medical patients give you more margin in percentage terms but less in value terms. Whereas a surgical patient , say a liver transplant or a heart transpl ant patient , gives you less in percentage terms but more in value terms. But that's the patient you would want . You will need to see it in context of three aspects . First, you need to look at EBITDA per bed . Secondly , you have to compare it to the same time last year rather than on a quarter -on-quarter basis. Q2 is always a medical quarter and ARPOB s are lower in Q2 , but it is always also characterized by higher occupancy. And the third aspect is that if you are going to look at EBITDA per bed , you have to take out at least the newer acquisitions that we have done because be it Dwarka, which has started its operations only right now , or Lucknow and Nagpur , which have been acquired and are still in the building stage. So, they enjoy lesser EBITDA per bed. Other than that, if you look at the EBITDA per bed , it's fairly healthy.

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Yogesh Sareen: EBIT DA per bed has g rown by 7% YoY and this is despite the fact that our immigration business , which we mentioned last quarter also , has dropped by 44%. And it was very high EBITDA margin business for us . So, this has had some impact but we think that it will get normalize d from Q4 onwa rd because we had the drop in busines s from Q4 last year. So, we think that once that gets normalize d, you will see that even EBITDA per bed growth would be much better than what you see today , which is 6% YoY.

Ameya Chalke : So, going ahead for the existing business , the EBITDA per bed improvement would be largely inflationary or you think there is there would be improvement in terms of the productivity as well.

Yogesh Sareen: ARPOB is growing by 7% and if we really adjusted for the drop in immigration business, it would be 8-9% growth in APROB , of which only 2% is because of price . Rest all is efficiency in terms of process es, as well as payor mix and case mix. If this is what you are referring to as efficiency, then this will continue. And we do not think that we will see any big change in these numbers in terms of growth in ARPOB going forward.

Moderator: The next question is from the line of Neha Manpuria from Bank of America Securities .

Neha Manpuria: On the J aypee asset acquisition , I just wanted to get a sense on do you think if I look at the numbers E BITDA per b ed obviously can be better but in terms of occupancy ,

ARPOB, it seems pretty decent. Obviously, it can be higher. But what do you think is required here from either do we think we need more medical equipment, more doctor teams, does it need upgradation? How are you thinking about ramping up the Noida asset?

Abhay Soi: I think all of the above. It has been a company, which has been in liquidation for a significant period of time. So, most of the equipment is the end of life. As far as the clinical programs are concerned, again these have been operating with just (bare minimum) gravity. So, there is a huge amount of upside over there. Separately, it has been operating as a 376-bed hospital and we believe that we can move it to a 500-bed hospital very quickly, within the current structure, without making any significant changes. And this can be done in matter of a few quarters itself. So, if we ramp up the capacity, we move up the clinical programs, we move up ARPOB and payor mix, etc., we think we will be able

to get there soon. To us, given the location, the access and the quality of infrastructure, it is a marquee asset. So when you have this kind of advantage, it becomes easy. We have no doubt that this will play out the same way as Lucknow, Nagpur or even better.

Neha Manpuria: And are there any litigation etc. that we need to be worried about which needs resolving or which could be a liability in the future that we need to be mindful of?

Abhay Soi: None whatsoever. I mean there are no pending litigations. That's the benefit of buying through an NCLT process that if there are any creditors, if there are any claimants, etc., there is a time-bound manner in which they have to raise their hand and put in their claims, be it any department, even a government regulated department. So, the NCLAT process allows claimants to step up and state what their claim is. And after the expiry of time mentioned by NCLAT, those claims are not entertained. In this case, all the claims that were entertained at the NCLAT proceedings, all of them have been satisfied and more importantly, even post the proceedings there are no further claims that have been raised. When you get something through a High Court order, i.e., NCLAT is a High Court, it cannot be questioned.

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Neha Manpuria: So, it's a fairly clean, from a litigation perspective, there's no big overhang that we need to be worried about then?

Abhay Soi: Since we are getting it through the High Court, the title is clear. And there were no creditors which were dissatisfied creditors either. All the claims have been fulfilled.

Neha Manpuria: What about the two, the Bulandshahr and the Anoopshahr and Bulandshahr is commercial. So, would that be meaningful enough? Should we be considering that when we are looking at the math?

Abhay Soi: No, they're not meaningful enough. And we did not even attribute any value to it. We are trying to determine whether we should focus and spend CSR time and money on this or just wrap it up.

Neha Manpuria: And last on Jaypee, we have acquired part stake. The rest of it what is the process for that? By when do we see the full acquisition?

Abhay Soi: I think in the next few days we should have that wrapped up.

Yogesh Sareen: There is a call and put option, and they have exercised the put. We think that hopefully by next week we should be wrapping it up.

Moderator: The next question is from the line of Damayanti Kerai from HSBC Securities and Capital Markets (India) Private Limited.

Damayanti Kerai: My question is on Dwarka, so Abhay in your opening remark you mentioned Dwarka will still be in ramp up phase for most of this FY25. So, just want to understand like on TPA and insurance empanelment I believe discussions are underway. So, when do you expect this 18% revenue share will likely move up to the Network level 30% when you empanel the required number of channels?

Yogesh Sareen: So, presently the TPA empanelment is underway. We are also waiting for the NABH certification as some of the TPAs have a requirement of NABH certification. We do think that by end of December 2024, we should have all of them in place. And January 2025 onward we think we should see more than 30% revenue coming from TPAs.

Damayanti Kerai: So, by December most of the contracts will be signed and then you will build up from them?

Abhay Soi: Yes.

Damayanti Kerai: And the ARPOB which you mentioned in your presentation Rs.80,000. That is because we understand most of the patients so far are with the cash channel patients. And maybe ARPOB will settle down more at your Network level for this hospital also?

Yogesh Sareen: Yes, there are two aspects . One is that we have very high outpatient consults and since we do not have TP A empanelment , the outpatient consults were happening in Max Dwarka but in-patient

t admission was happening in some other Max hospital.

These doctors did have some attachment in other Max hospitals and were admitting the patients in the other hospital . For example , the neurosurgeon was attached with Max Gurgaon , so we admitted the patient there . Hopefully when we have everything in place , then the ARP OB will moderate a bit. But it will not be a very big change because it will be closer to the national average .

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Abhay Soi: But it's also a play between occupancy and ARPOB . Our belief is , like I said earlier, that this will probably be the fastest greenfield breakeven that we have witnessed .

We have seen very good traction and it is only a matter of time when all the TPAs signed up . We started end of July , so basically beginning of August. It does take 3 to 6 months to sign all the contracts . I mean a lot of it is in play already , and like Yogesh mentioned , by January 2025 we expect all of the empanelment to be done.

Dam ayanti Kerai: My next question is on your Network level revenue contribution from TPA and corporate channel. So , it's around I guess 38% in the first half. So , looking ahead I guess when we are adding a lot of capacities into the Network where I guess this payor mix etc . will take some time to build up. So , say in next 2 to 3 years , what kind of contribution we can assume from this particular channel , from 38% can we assume it can comfortably go up to 45% or so or how do you see this pie moving up?

Abhay Soi: So, we cannot give you any forward -looking guidance but we can tell you what our experience has been . When you open a new brownfield capacity , you essentially start taking in a larger amount of lower -end payor mix as well as lower -end clinical mix. One of the characteristics of a brownfield is that you are sort of finished with distilling your payor mix and you are doing very little or none of institutional business in the existing hospital . Like in Saket , in the main hospital , we have stopped doing it , in Nanavati , we have stopped doing it. The minute you put up additional brownfield , one of the things you are going to do is that you are going to switch on the tab for institutional over there , which is a lower -end payor mix. But because of operating leverage , even with the lower -end payor mix, which means with lower ARPOB, the EBITDA per bed is high. That's what we witnessed in Shalimar Bagh when we opened the brownfield . If you actually see what has worked for us in Nagpur – it was operating at maybe 55 % or 60% occupancy , and the first order of business after taking over operations was to sign up the institutional business , which is again lower ARPOB but provides higher occupancy. Now , the result is that ARPOB got somewhat muted , occupancy went up to 90 %+, but the EBITDA moved up by more than double . So, I think that is what we have seen is when you occupy incremental beds , the cost of which is only incremental , that even with a lower -end payor mix, it comes down to the bottom line and your margin improves. To what extent that I will not guide you.

Dam ayanti Kerai: And my last question is in some of your near-term upcoming capacities. So , Lucknow and then I guess Nanavati, those are coming in near term. So, for these facilities when do you start putting up a new set of doctors , medical team etc. or it happens very near to the commencement of these units.

Abhay Soi: We have new capacities coming up , firstly in Nanavati , in Max Smart (Saket) and Mohali as well as Lucknow is coming by December. But the first three are coming up at the same time in the month of March -May 2025 . These are brownfields. Unlike Dwarka , which is a greenfield , where you are going to get doctors , nurses , other staff, and they will be on your books for about two months even before you commence operations , and even once you commence

operations , it takes you time

to sign up TPAs and there's a ramp up required . So, there are some limitations in ramping up , which are not limitations of the operational level , but there are limitations of enabling that occupancy . However, in a brownfield you do not have any of these limitations . It is the same existing contracts , the same licenses , etc. And most of your doctors are the same. The more expensive doctors are the chair persons or heads of the clinical programs . It is not the junior consultants or resident doctors . In the first month itself , you get these junior doctors and absorb them. So, we do not see a major impact because of it, unlike Dwarka .

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Dam ayanti Kerai: So, any hiring in these Brownfield s as you said maybe the department heads or the really senior position but most of the team is already in place ? That's why like there won't be any meaningful delta in terms of cost?

Abhay Soi: That's right. Even on the management side , it will be t he same heads for facility , F&B, nursing , engineering , medical superintendent , etc. It's not a significant amount and particularly in Smart, where we are just expanding beds. The team there is already in place .

Moderator: Next question is from the line of Sumit Gupta from Centrum Broking Limited .

Sumit Gupta: Couple of questions , Firstly, can you tell what is the international patient's bed share and what kind of trend we can expect?

Yogesh Sareen: International bed share is 5.5% a nd it constitutes around 9% on the revenue side . But the bed share is 5.5% as the ARPO B is higher than the national average when it comes to international patients.

Sumit Gupta: Second is on the J aypee hospital. So , can you explain the ca se and the payor mix of this hospital in Noida?

Keshav Gupta : For Jaypee hospital patient case m ix, about 25% of revenue comes from institutional business and about 10 -14% of business month -on-month comes from international patients .

Sumit Gupta: And just want to understand on the broader aspect like how the competitive scenario in Noida just like so there are hospitals which are also going to expand in this geography , so how do you see that panning out? Will there be any impact that you see on the volume side?

Abhay Soi: I think there is still some time before the other hospitals come up . But Jaypee is a AAA+ location and you cannot get a location better than this in Noida or Greater Noida. It is an 18-acre complex , with 9.5 lakh square feet of built -up area. H onestly, we think none of us would ever build a hospital like that. We would not want to. The size and scale of it is something which is first class . So, from that standpoint the benefit is that you get something with that size , that scale , that location , that infrastructure , which nobody is going to operate. It's like you can make a new hotel in Jaipur , but there is only one Rambag h Palace . So Jaypee is that Ramba gh Palace in Noida . So, it's an easy sell , easy access , easy everything from that standpoint . It already has a start , even with a lot of limitations that it operated in, under liquidation for over 10 years , very little medical equipment , etc. yet you have seen halfway decent results coming out of this hospital. The reason for that is just fundamen tally structural. I mean it's like getting 7-star's treatment at maybe 4-star, 5-star prices or quality of service s.

Sumit Gupta: Lastly on the Dwarka hospital , just like in the presentation you have mentioned that the hospital is expected to open nearly 200 beds by end of FY25. We just want to understand on this , was like you're going to add like 200 b eds or what 1 41 beds are going to be operational capacity to get to 200 b eds?

Yogesh Sareen: No, it's a 303 -bedded hospital. So , that in a greenfield situation we don't open all the beds , because when you open

more beds that means there's a cost attached to it.

So, we open beds as are we able to fill it up. So, I think what we s aid is we have 141 beds open as of now and we think by March 2025 we will open 200 beds in total .

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Abhay Soi: What happens is that you have to get a license . If we want to get a license for 300 beds, then we need to staff all 300 beds. Then you have to prove it to the licensing authority that you have staffed all the 300 beds. But if you have occupancy over 150 beds , then you'll just say , give me a license for 150 beds because you will staff accordingly then. There's no point carrying staff of nurses and resident doctors for 300 beds , when your occupancy only 120 -150 beds because you are waiting for all the TPA empanelment, etc. to happen.

Moderator: The next question is from the line of Prashant Nair from Ambit Capital Pvt Ltd .

Prashant Nair: So just a follow up on the J aypee asset . When you acquired it , you had mentioned that there is scope to raise bed capacity meaningfully here. I think you also alluded to that now. Have you worked out over what period of time you will be looking to expand the bed capacity here? Also, second part , for the current hospital how much do you think you would need to invest in order to upgrade medical equipment , clinical capabilities etc. over the next couple of years?

Abhay Soi: On the first part of the question , we will look at expanding the capacity at the earliest. We will be limited by the actual time it takes to construct. So, right now we are looking at moving up to 480 odd beds from the current capacity and thereafter it should take us 2-2.5 years to build another 450 -500 beds , which we will embark on fairly soon because we see the current capacity being filled out in a matter of months and way

before the new capacity that we are looking at because it takes time to construct .

Yogesh Sareen: As far as the BME medical equipment upgrade is concerned , we already have some plans for that. There will be Rs.150- Rs.200 crore spend that we are planning to expand first to 480 beds and as Abhay mentioned , after that we will start the work to build additional beds.

Abhay Soi: But this includes the medical equipment over Rs.150 crore.

Yogesh Sareen: Yes.

Abhay Soi: Because you have to appreciate that almost all of the medical equipment is end of life. It requires replacement.

Moderator: Next question is from the line of Nitin Agarwal from D AM Capital.

Nitin Agarwal: We've had a pretty busy last few quarters in terms of these acquisitions that we've done. And with a bunch of new assets coming on stream next year as you mentioned , are we looking to take a pause on the inorganic growth part of plans or we still continue with inorganic growth activity for the next few quarters ?

Abhay Soi: We have a pipeline . If we keep getting opportunities to add marquee assets , we will continue to do that. We certainly have the balance sheet and we certainly have the bandwidth.

Nitin Agarwal: Secondly on Jaypee , so we mentioned a Rs.1,660 crore if I remember as the EV for the business acquisition. This includes the entire pay-out for the entire equity and whatever debt we've taken on for the asset.

Abhay Soi: That's right. That's the enterprise value.

Nitin Agarwal: And beyond this we mentioned there is going to be some registration and transfer charges. So, how much of that will really account to?

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Abhay Soi: It has already been paid.

Yogesh Sareen: Rs. 62 crore , which is over and above. Yamuna Expressway Industrial Development Authority (YEIDA) has a rule where even if there's a change in shareholding , there's an amount we have to pay for the transfer , which is Rs. 62 crore that we paid.

Abhay Soi: That's already paid out , no further transfer charges.

Nitin Agarwal: So, I mean how should we think about the assets? So , w

e paid about Rs1,700+

including the upgrades and all about Rs.1,800 odd crore for buying out an asset for about 500 bed asset. So, is that the way we should think about it? Apart from obviously it's a running asset that gives us immediate EBITDA versus a greenfield investment that we would have done. But in your mind how should you evaluate this versus assuming you were to make a same greenfield investment assuming there was an opportunity like this in Noida to versus acquiring this asset for the price if you paid?

Abhay Soi: If you look at it from a return on capital employed (ROCE) standpoint . Essentially what you are doing is you are looking at 500 -bed asset , which is renovated with new equipment etc. and capacity to add another 1 ,500 to 1 ,700 beds on the same land . So, very clearly if you want to look at it over a 4 to 5-year plan, you look at 500 beds plus another 450 beds that you would be able to add since you have already got the land for it. And then you look at what is the EBITDA and what sort of ROCE you are able to get. If you're able to generate more than 20-25% ROCE over that period of time, then it is a good acquisition. That's how we look at it. It so happens that this one is the easiest yet perhaps because it is a marquee asset with that location. But no matter how you cut it , no matter how you look at it , even if you look at it sans any expansion , a 500-bed hospital in this location should give you about Rs. 300 odd crore of EBITDA.

Yogesh Sareen: So, let me just tell you how we look at it . Basically , we have a hospital in Vaishali , which is a 387-bed hospital and Jaypee is going to be 480 beds . Now, 480 beds with all the money that we are going to spend is going to be roughly around Rs. 1,900 crore . And the hospital in Vaishali does roughly around Rs.380 crore of EBITDA every year and it is a 387-bed hospital.

Abhay Soi: So, this is actually a better location , better access , bigger , better infrastructure than Vaishali.

Yogesh Sareen: More beds and on 18 acres , while Vaishali hospital is on a 2.5-acre piece of land. So, you can obviously can see the kind of numbers that we are expecting from Jaypee, given that we have a benchmark in the same market already giving that level of EBITDA and margins .

Abhay Soi: Our normal bed size average is about 1 ,100 square feet per bed. This is close to 2,000 square feet per bed , already constructed.

Nitin Agarwal: Abhay when you are looking at , obviously you are looking at consistently evaluating a pipeline of various sort of assets. So , the kind of transaction that you have gotten

in the Jaypee Hospital, is more of a one-off transaction or there are similar such transactions which are there for taking in the market?

Abhay Soi: Well, if Jaypee is a one-off then I guess Sahara was a two-off and then Nagpur is a three-off. So, I think we keep doing these 'one-offs', which we like to do every once in a while. I think Sahara Lucknow was very similar, and so was Nagpur.

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Nitin Agarwal: No, I got to appreciate that. And lastly on Dwarka you talked, it's going to be our fastest probably breakeven for a greenfield. So, what is the time horizon looking at for the breakeven here?

Abhay Soi: We are looking at it before the end of this financial year.

Moderator: Next question is from the line of Andrey Purushottam from Cogito Advisors.

Andrey Purushottam: My question is really a follow up from one of the earlier questions, in terms of the levers that you have to increase EBITDA per bed. If you could just expand a little bit on that and also answer two specific questions. One is, has your proportion of your low margin CGHS kind of business actually diminished over the last year or two?

And how do you intend scaling up your international business per se and also perhaps to compensate for the possible reduction in volumes from

places like

Bangladesh and how important is Bangladesh to your total share of patients of the international business per se?

Abhay Soi: I'll take the last one first. Bangladesh used to be less than 1% of our total business.

So, it really has not impacted us that much. And we have been able to compensate that through other markets and all our initiatives everywhere else. So, Bangladesh is not affecting us. What did affect us in the past was Afghanistan which was 12% of our international business. But we have not had that business for many quarters now. So, you don't see the impact of that. Hopefully as and when it comes through, there will be a positive impact because of it. Having said that, levers for EBITDA per bed, let's start from the top with revenue lever. And perhaps most of it are revenue levers, which include adding clinical teams, adding occupancy, increasing occupancy, increasing more beds. I mean every revenue lever that you can think of will play into the bottom line and increase your EBITDA per bed. Because I think as far as costs are concerned, we are pretty much in line with where we want to be, any benefits over there will be only incremental.

Institutional business has diminished marginally over the last few quarters. But the ARPOB of the institutional business has almost doubled. The rates are becoming closer to our cash rates. More importantly, in a lot of our metro locations where we already have very high capacity, we have already distilled away from that. So, if you look at Mumbai, we do not do any institutional business. But once we start the new capacity, we will start the institutional business, and we just occupy the beds with it till such time that we have idle capacity. Once that kind of runs out, we start distilling it again, and go back to cash business. At Saket, in the main facility, we do not do any institutional business. At some stage, once we come up with new capacity at a Max Smart, we will start taking on more institutional patients. But again, over a period of time, we will distill it. But then, in the third phase 3-4 years later, when Vikrant comes again, we will start doing taking institutional business. So, you keep playing that game. But more importantly, as you start running out of capacity, you start distilling payors and within those payor groups also you start distilling the clinical mix.

Andrey Purushottam: And going back to the international business, you said that you have a 5.5% bed share and a 9% revenue share. Now if you're looking to increase this share, what was the approach that you're following? Is it largely walk-in business or are you making a fair amount of proactive efforts and other geographies to attract patients there?

Abhay Soi: We have set up offices over last 2-3 years in about 16 countries. We have our own offices as well as partner offices. And that is the reason that we have seen an outsized sort of growth in our international business. It may be 9-10% of our revenue, but our revenue has also been growing significantly over the last 4-5 years. This

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business has been growing a 20% CAGR for us. It sort of outstrips are overall growth and the reason is the investments initiatives that we have taken in terms of setting up these direct-to-fly offices overseas.

Andrey Purushottam: One hears about lots of problems that people face with the NHS in England and similarly in places like Canada etc. So, given the fact that we can provide outstanding medical care at a fraction of the cost, do you see more traction coming from developed countries like UK and Canada etc. where it has a lot of bureaucratic

problems about getting service in their own countries?

Abhay Soi: It's a question of whether you want bad healthcare for free or you want to pay for good and upfront healthcare and you want to come to India for it . We are

setting up

some beachhead operations in the UK as we speak , because of the same reasons that you mentioned. But we are not expecting some floodgates to open . It is more a long-term thing that we need to have some sort of presence over there to at least get people of Indian diaspora to India to be able to treat them . But we do not see people coming in drones because there is waiting time in NHS . There is waiting time but ultimately it is free.

Moderator: Next question is from the line of Madhav Marda from Fidelity International .

Madhav Marda: I just had one question , on the Dwarka unit it's quite encouraging that a greenfield asset can breakeven in a very short period of time. Could you just give some colour in terms of what guides us because I guess a general understanding for everyone has been that greenfield assets take let's say 2-2.5 years to reach EBITDA breakeven ? But since we can do it at a very good pace , just what are things that are working in our favour , just for my understanding ?

Abhay Soi: I do not think it is 2-2.5 years. Typically , it should take about 18 months. That's par for the course. But having said that , Dwarka is again an excellent location. The first factor is always a location. Second is the infrastructure. The third is how much underserved that market is. And fourth , I tend to believe that our brands are NCR centric . We already have around 14 facilities over there , which makes a big difference . There is a very strong brand recall . And our belief is that we would have already broken even if we had all the institutional tie -ups, NABH certification, TPA empanelment, etc . Typically , you need 6 months of data to get NABH. Once you get NABH, you get higher rates from institutions. So , a lot of it is also process related.

But otherwise, for us again , once the location is good , once the infrastructure is good , access is good , then it should not take much time to ramp up. Also, the team of doctors we have been able to access and get very good teams of doctors . But that's the Max advantage , particularly in NCR, that we are able to attract quality talent.

Moderator: Next question is from the line of Amit Thawani , an individual investor .

Amit Thawani : I particularly enjoyed the response to the question whether Jaypee was a one off. But jokes apart , I also enjoyed how we've broken up the business into existing and new units. So, at 81% kind of capacity utilization, we are little bit maybe constrained for volume growth in the existing units. A little bit of a forward -looking statement , what would the growth that you estimate in the existing units in volume and ARPOB going forward? I mean just a ballpark even a range is fine and even I'm looking for more of a longer -term number rather than a short -term number.

Abhay Soi: Firstly , 81% is a seasonal number , because of seasonal flu , etc. Normally , Q2 has higher occupancy. You will see in Q3 perhaps the occupancies will get a little more muted than that and that's traditionally been the case. Whatever is the occupancy in Q2, Q3 is lower , Q1 is lower and then Q4 again you ramp up. So, yes, the other

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characteristic of having a capacity constraint is that you have a higher move up in your ARPOB simply because when you have the capacity constraint , the lower -end clinical mix and payor mix are perhaps not given priority. And when you are unable to give the mix priority , you post those surgeries and procedures at a later date , they are likely to evaporate. When you have capacity , all of that lower -end payor and clinical mix comes through. But like I said , it's also characterized by higher EBITDA per bed. So , it works out better to have higher occupancy than a ramp up in ARPOB . The fact of the matter is that we have been operating at these kind of capacity levels for the last couple of years with the 1 -2 percenta

ge points here or there as far as

occupancy is concerned , and that is always because at the margin there will be some elasticity but it is diminishing returns. And we need to be cognizant of the fact that we have capacity constraints. We are cognizant of the fact that we have capacity constraints and therefore, we embarked on this massive journey of brownfield expansion . Some people will ask me the question like you are asking me that look you're at full capacity , where do you go from here? So , the answer to that is that's what the brownfields are for and some other people tend to ask that look there's so much capacity coming in the country , why are you putting up more capacity? Well , this is the reason we are putting it up. Because we have run out of capacity.

Amit Thawani : But any quantitative guidance. Would you want to venture in that?

Abhay Soi: No, we stay away from providing any forward -looking guidance.

Moderator: Next question is from the line of Alankar Garude from Kotak Institutional Equities.

Alankar Garude: Just one question for Yogesh. With Dwarka coming online , what will be the annual lease payment at the Network level?

Yogesh Sareen: I think the overall number for the year would be around Rs. 95 crore . So, specifically for Dwarka, this number is around Rs. 28-29 crore .

Alankar Garude: And the number for Dwarka when you say Rs. 95 crore , would be for nine months , right?

Yogesh Sareen: No, I'm saying Rs. 28 crore is Dwarka, total is Rs. 95 crore for the overall Network.

Abhay Soi: And Rs. 28 crore is for the year.

Yogesh Sarin: I'm talking all annual numbers.

Alankar Garude: So, basically for FY25 the number would be Rs. 7-8 crore lower. Got your point.

Moderator: Thank you. As there are no further questions , I would now like to hand the conference over to the management for the closing comments.

Abhay Soi: Thank you everyone for joining us on the Q2 results. We look forward to interacting with you again next quarter. Well appreciated , thank you.

Call: Feb 2025

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(CIN: L72200MH2001PLC322854)

February 6, 2025

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Sub.: Transcript of Earnings Call held on January 31 , 202 5
Ref.: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Sir / Madam,

Please find enclosed copy of transcript of earnings conference call, organised on January 31 , 2025, on financial results of the Company for the quarter and nine months ended December 31 , 202 4.

The said transcript is also available on the website of the Company at www.maxhealthcare.in/financials#earnings-call.

Kindly take the same on record.

Thanking you

Yours truly,
For Max Healthcare Institute Limited

Dhiraj Arora
SVP - Company Secretary and Compliance Officer

Encl.: As above

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Max Healthcare Institute Limited
Q3 FY25 Earnings Conference Call Transcript
January 31 , 2025

Moderator : Ladies and gentlemen , good morning, and welcome to the Max Healthcare Institute Limited 's earnings conference call.

Please note that this conference is being recorded.

I now hand the conference over to Mr. Suraj Digawalekar from CDR India. Thank you, and over to you , sir.

Suraj Digawalekar : Thank you, Ryan . Good morning , everyone, and thank you for joining us on Max Healthcare Q3 and 9 M FY '2 5 Earnings Conference Call.

We have with us Mr. Abhay Soi – Chairman and Managing Director, Mr. Yogesh Sareen – Senior Director and Chief Financial Officer and Mr. Keshav Gupta – Senior Director, Growth, M&A and Business Planning.

We will begin the call with opening remarks from the management , following which we will have the forum open for an interactive Q&A session.

Before we begin , I would like to point out that some statements made in today's call may be looking forward in nature and a disclaimer to this effect has been included in the earnings presentation shared with you earlier.

I would now like to invite Abhay to make his opening remarks . Thank you, and over to you Abhay .

Abhay Soi: A very good morning to everyone and warm welcome to Max Healthcare's Q3 FY25 Earnings Call .

We are considerably pleased by our performance this quarter with over 30% year -on-year growth across parameters such as revenue, EBITDA and occupied bed days , notably supplemented by the growth momentum of our recent acquisitions. We are happy to share th at we achieved EBITDA breakeven in December 2024 , within a record period of 6 months from the launch of our greenfield hospital in Dwarka . This hospital reported a revenue of Rs. 59 crore s and an EBITDA loss of Rs. 5 crore s in Q3.

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Max Lucknow demonstrated year -on-year growth of 58% in revenue and 94% in EBITDA, while Max Na gpur reported year -on-year growth of 22% in revenue and 50% in EBITDA in the third quarter.

Additionally, Jaypee Healthcare Limited became a wholly -owned subsidiary of the Company during the quarter. Jaypee Noida is presently being integrated into our Network and reported a gross revenue of Rs . 112 crore s with an operating EBITDA of Rs. 23 crore s at a margin of 21% in the third quarter.

We have now been able to demonstrate remarkable operating efficiencies across all formats of inorganic growth, namely greenfields, acquisitions and brownfields. This fortifies our confidence for the upcoming phase of accelerated growth driven by significant brownfield additions wi thin the next six months.

Consequently, we continue to strategically pursue inorganic opportunities. We are expanding our footprint in the Mumbai Metropolitan Region through a foray into the attractive Thane micro-market , given its rapid urban growth and proximity to Mumbai. Our Board has accorded its approval to enter into an asset -light 'built-to-suit' agreement for a 500 -bed hospital at a prime location in Thane , to be set up by the partner as per our specifications on a built -up area of appr oximately 6 lakh square feet. The hospital is expected to be commissioned in 2028. This marks our third asset -light transaction , designed to drive future growth and maximize potential return on capital employed (ROCE) with minimal investment.

The Board has also provided i ts approval for enhancing the capacity of our upcoming asset -light 'built-to-suit' hospital in Mohali (Zirakpur) to 400 beds from 250 beds planned previously.

Now, coming to the Q3 performance highlights, which is our 17th consecutive quarter of year -on-year growth.

1) Our average occupancy for the Network stood at 75% versus 73% in Q3 last year and 79% in the trailing quarter, while the occupied bed days (OBDs) grew by 36% year -on-year and 8% quarter -on-quarter.

2) Average revenue per occupied bed (ARPOB) for the quarter stood at Rs. 75,900, remaining relatively flat both year -on-year and quarter -on-quarter. Like-for-like ARPO B for Existing Units, however , grew by 7% year-on-year and 3% quarter -on-quarter .

3) Network gross revenue was Rs. 2,381 crore s compared to Rs. 1,779 crore s in Q3 last year and Rs. 2,228 crore s in the previous quarter. This reflects an increase of 34% year-on-year and 7% versus the trailing quarter.

New Units reported a gross revenue of Rs. 323 crore s. While Existing Units registered a year -on-year growth of 16% in revenue, driven by 8% growth in OBD s and 7% growth in ARPO B.

4) The international patient revenue stood at Rs. 201 crore s, registering a growth of 28% year -on-year and 8% quarter -on-quarter , despite contraction in patient footfalls from Bangladesh and Yemen due to political unrest.

5) Network operating EBITDA stood at Rs. 622 crore s, reflecting a growth of 32% year-on-year and 10% quarter -on-quarter. This includes Rs . 60 crore EBITDA contribution from New Units.

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6) Network operating EBITDA margins stood at 27.3% for the quarter. Existing Units improved their EBITDA margin by 70 basis points to 28.6%.

7) Annualized EBITDA per bed for the network stood at Rs. 73 lakhs . Like -for-like EBITDA per bed for Existing Units stood at Rs . 82.6 lakhs, reflecting a growth of 9% year-on-year.

8) Profit after tax before exceptional item was Rs . 390 crore s versus Rs.338 crore s in Q3 last year and Rs.349 crore in the previous quar ter, reflecting a growth of 15% year -on-year. The exceptional item of Rs . 74 crore was towards charges paid to Yamuna Expressway Industrial Development Authority (YEIDA) for securing permission for a change in shareholding of Jaypee Healthcare Limited prior to acquisition.

9) Overall

free cash flow from operations was Rs. 303 crore s. During the quarter, Rs. 362 crore was deployed towards ongoing capacity expansion projects and upgradation of facil ities at acquired hospitals , while Rs. 146 crore was distributed as dividend and Rs. 1 ,716 crore (net of cash at Jaypee Healthcare Limited) was used for Jaypee acquisition. Consequently, ne t debt for the Network stood at Rs . 1,608 crore s at the end of December 2024.

10) Continuing our effort to support the local communities, we treated approximately 37,500 outpatients and 1 ,300 inpatients from economically weaker sections of society entirely free of charge (worth Rs . 52 crore s at hospital tariff).

11) Both our strategic business units continued to report significant growth in revenue and profitability.

- Max@Home reported a top line of Rs . 55 crore s, reflecting a robust growth of 24% year-on-year. It now offers 15 specialized service lines across 14 cities , with over 50% repeat transactions.

- Max Lab reported a gross revenue of Rs. 41 crore s, reflecting a strong growth of 22% year -on-year. It provides services in 48 cities through its network of more than 1,200 col lection centres and active partners.

12) Now, coming to the status of our expansion projects :

- 128 beds at Max Lucknow – 64 beds have been commissioned in January 2025 , and the balance 64 beds will be added in February 2025. Further, we are awaiting in -principal approval for the existing 13th to 17th floors for hospital use, which will add another 140 beds almost immediately.

- 127 beds at Max Na gpur – 12 beds have been added in October 2024. For the balance beds on additional floors, we are expecting the environmental clearance (EC) to come by March 2025. Project completion should take another 24 months thereafter.

- 268 beds at Nanavati in Phase 1 – Interior fit -out work s are in progress

currently . The project continues to be on schedule and we expect completion within the next 3 to 4 months.

- 400 beds at Max Smart (Saket Complex) – Majority of the structural work is complete. The project is on track and we expect its completion within Q1 FY '26.

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- 155 beds at Mohali – Interior work is in progress and we expect its completion again by Q1 FY '26.

- 500 beds at Sector -56 Gurgaon – Structural work is in progress. We expect completion of the first phase of 300 beds by the end of Q3 FY '26. All of these are on schedule, and we will see a significant ramp up in our capacity over the next 12 months , a large part of which is coming through Nanavati , Mohali and Max Smart within 6 months.
- Thereafter, 367 beds at Patparganj – Post receipt of environmental clearance (EC), tendering work is in progress currently . This project is largely on schedule.

- 550 beds at Max Vikrant (Saket Complex) – Forest approval is delayed due to Supreme Court proceedings in relation to tree felling involving DD A and the Lieutenant Governor of Delhi for the past 6 months. They have not permitted anybody to remove any trees in Delhi , but we think that this should get fairly resolved soon. All other statutory approvals are in place.

- 400 beds at Zirakpur, Mohali – No objection certificate (NOC) from fire department has been received. Project is expected to be completed within 30 months.

And finally, moving on to the overview of company 's performance for nine months ending December 2024 :

13) Network gross revenues stood at Rs . 6,636 crore s, reflecting a growth of 25% year-on-year. New Units contributed Rs . 585 crore s to the gross revenue.

14) Overall network operating EBITDA grew by 20% year-on-year to Rs. 1,687 crore s, reflecting a margin of 26.6% , while EBITDA per bed stood at Rs. 71.5 lakhs. Existing Units reported an EBITDA margin of 27.7% and EBITDA per bed of Rs . 78.5 lakh s.

15) Max Lucknow demonstrated year -on-year

revenue growth of 41% in revenue and 67% in EBITDA , while Max Nagpur reported a year -on-year growth of 26% in revenue and 118% in EBITDA within 9 months of acquisition.

16) Since becoming operational in July, Max Dwarka clocked a revenue of Rs . 92 crore s and EBITDA loss of Rs . 29 crore s. This greenfield hospital achieved EBITDA breakeven in December 2024, a record 6 months from its launch as highlighted previously.

17) During the 9 months, we generated Rs. 1,025 crore s of free cash flow from operations after interest, tax, working capital changes and routine CAPEX . Rs. 793 crore was deployed towards ongoing expansion projects and upgradation of facilities at acquired hospitals , Rs. 146 crore was distributed as dividend and Rs . 1,716 crore was used for Jaypee acquisition.

With this, we open the floor for any Q&A.

Moderator: Thank you. Ladies and gentlemen, we will now begin the Q&A session. The first question comes from the line of Amey Chalke from JM Financial Institutional Securities Limited . Please go ahead.

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Amey Chalke: Hello, thank you so much and congrats to the management on good set of numbers. So, the first question I have is on the revenue from the existing units. It seems that quarter -on-quarter from Q2 to Q3 the revenue has remained largely flat despite being the weak quarter of Quarter 3 . Is it possible to give an explanation on the front where we have seen the improvement or performance improvement during this quarter for

the existing units?

Yogesh Sareen : Amey , as you would know , Q3 is typically a weak quarter because you have festivals in this quarter , and if you see the history , you will find that typically the revenue s come down by 2 -3% and EBITDA also drops by 3-4% in this quarter. Despite the history, this time it is flat and in fact the overall EBITDA has improved over Q2.

That way , the performance has been much better and it is mainly because of the fact that in the Diwali month, typically you will see the occupancy drop to around 65 -70% range , but this time we had very healthy occupancy even during Diwali , and that is what has made all the difference in this quarter.

Abhay Soi : You must look at it on a year -on-year basis because of seasonality. Every quarter must be seen on a year -on-year basis rather than sequentially quarter -on-quarter .

Amey Chalke: So, no, because I was expecting a drop this quarter considering it is a seasonally weak quarter. That's why the question was.

Yogesh Sareen : Occupancy levels in the Diwali month made a lot of difference. Typically , the occupancies do drop, but this time it did not happen. It is also because of the fact that Diwali was at the very end of the month, which also helped in a way. If it is in the middle of the month, then you will have more impact.

Amey Chalke: Sure, and second question I have, we were expecting a price increase for some of the insurance schemes. So , have that been taken place or do you expect in the next one year any price increase to happen on the insurance side?

Abhay Soi: We are not expecting a price increase on the insurance side , we were looking at a price increase on the institutional side. We are still expecting that as it is long overdue. Our belief is that it should come within a month or two. But let's see what happens on that.

On the insurance side, it happens on a rolling basis. That means whichever insurance contracts come offline every two years, you get the new rates over there.

So, that's happening as course of hygiene.

Amey Chalke: Sure. And if you see our therapy mix, it is continuing to improve. The oncology mix has also improved from Quarter 3 of last year to this year. Considering the new bed additions which have happened would have a little bit lower oncology proportion , I believe for existing hospital, the mix would have moved up sharply. So , where should

we see the optimized mix for the oncology revenues going ahead?

Abhay Soi: In the new hospitals , it should be increasing considerably in fact. In Lucknow , for example , the new bunker is yet to get ready and is expected to be operational by July. Thereafter, oncology revenue should increase . What happens is that without radiation oncology , even the other programs suffer in as far as oncology is concerned. So , you will see a major uptick over there.

We are looking at a new bunker coming on stream even in Dwarka. The facility is without a bunker right now, so there is no radiation oncology there. It needs to kick

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in over there as well. Besides that , even for Jaypee and others, we are going to see further increase in oncology.

You are absolutely right. The current pie although takes into account improvement or increase in oncology business , but that number is kind of subdued or pulled down by the new hospitals, new acquisitions where we are looking at perhaps significant increase in oncology business.

Amey Chalke: Right. Just to add more , like, which would be the hospital which would have highest oncology mix and what would be that number so that we would know the upper limit for the oncology ?

Yogesh Sareen : We can't give you hospital -wise numbers, but the very fact that we have 25% + as percentage share of oncology in the overall setup, obviously there will be hospitals that will be in the range of 29 -30% as well . The new hospitals will be lower . For example, Dwarka would be around 12% . These are the kind of ranges that we have.

Abhay Soi : Even Lucknow will be in single digits again ?

Yogesh Sareen : Yes, Lucknow will also be around 10 -11%, so it is half the network average.

Amey Chalke: Just last question . The PHF profitability has pulled down a bit this quarter. Any reason for the same?

Yogesh Sareen : No, you will have to see it in the overall aspect. If you read the notes given , some of them have donated money to the other trusts . Also, we have revised the fee structure for two of the PHFs, which means that there is more upstreaming happening in the first column , that is we are getting more fees into MH IL, especially from Balaji and DDF.

Amey Chalke: Thank you so much. I will join back the queue .

Moderator: Thank you. The next question comes from the line of Sumit Gupta from Centrum

Broking Limited . Please go ahead.

Sumit Gupta: Sir, on the Lucknow performance specifically, I just want to understand how the market is panning out and what kind of trend s to expect in the overall profitability going forward?

Abhay Soi: We are not going to give any forward -looking guidance, but the market is panning out very well. Like we said, we are launching another 140 beds. The reason we are doing that is because the occupancy requires that. 64 beds have already been commissioned in the month of January. These are new beds and another 64 beds will be commissioned in February, that is in the current month. Thereafter , we are looking forward to another 140 beds , which can immediately come online post approval.

We have the requirement for these beds and, that is what we are anticipating.

Therefore , we are getting these beds online.

We are also looking at the new bunker to come on stream over there. So , with the new bunker, the radiation oncology business, the day care business, etc. increases.

And we are seeing very good traction with clinicians, bringing in new clinicians and so on. So , I think , Lucknow has a significant amount of meat over there.

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Sumit Gupta: Okay . So, what are the competitive scenarios there on the addition al intensity being panning out like it is going up. What is the trend of the int ensity?

Keshav Gupta : It's the same as earlier, so there is an Apollo and there is a M edanta. The same hospitals are still there.

Abhay Soi: And there

are some other smaller nursing homes and hospitals.

Sumit Gupta: Okay. And s ir, of the Nagpur facility , the Q-o-Q, there is a decline in the overall profitability . What has led to that decline in the growth ?

Abhay Soi: Quarter -on-quarter , like we said, is a seasonal business . You must look at it on year - on-year basis .

Sumit Gupta: Okay , understood. Thank you , sir.

Moderator: Thank you. The next question comes from the line of Damayanti Kerai from HSBC Securities and Capital Markets (India) Private Limited . Please go ahead.

Damayanti Kerai: Hi, thank you for the opportunity. My first question is on your debt side, so Rs.1,600 crore net d ebt after payment to the Jaypee etc. So , now in view of multiple projects coming in like coming quarters or years, how should we look at the funding side ? And then maybe you can just like give your upper limit for net debt to EBITDA , like what will be your upper tolerance level there?

Abhay Soi: So, our upper limit is 2.5 x net de bt-to-EBITDA , but we are far from that. The new ones that we have announced such as Mohali (Zirakpur) as well as Thane, they are both asset -light models. The developer incurs the cost and we are essentially leasing these spaces from them thereafter. Dwarka expansion again is on a similar line, which is an asset -light model. As you are aware that Dwarka itself is an asset -light model.

Now other than that , we are looking at Rs. 500-600 crore over the next 3 -4 months of CAPEX towards the brownfields and then thereafter. But yes , our overall cap is 2.5x net debt-to-EBITDA. This would include not only current CAPEX but also any further inorganic growth or whatever else we may do , including on and off-balance sheet de bt.

Yogesh Sareen : We are at 0.65 x at this point in time after the Jaypee acquisition.

Damayanti Kerai: Okay, so comfortable headroom .

Abhay Soi: Yes, and we are very conservative on the de bt side.

Damayanti Kerai: Okay , and just if you can remind us like what kind of cash is currently generated from the existing business.

Abhay Soi: This quarter we generated Rs. 303 crore s of free cash flows. Thi s is after tax, working capital increase, maintenance CAPEX and any interest.

Damayanti Kerai: So, on an average, we can assume like Rs.1,200 crore of cash per year is getting generated against your all growth needs.

Abhay Soi: Hopefully more , given the year-on-year growth that we have.

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Damayanti Kerai: Okay . My second question is on price increase which you mentioned on the institutional channels. Can you bit elaborate, are you expecting something to come up on the CGHS rate or what is it regarding ?

Abhay Soi: That's right . We are expecting some revisions to come up in CGHS rates that will also impact the other PSU business. The gap earlier used to be 44% betw een our cash rates and our institutional , and now it has come down to 36%. The delta has

reduced. On top of that, we are expecting better rates coming through on CGHS now. But we have been expecting this for some time. Our hope is that this comes in this quarter.

Damayanti Kerai: Have you heard any announcement from the Government? You mentioned next two to three months, right? You are hoping to hear something?

Abhay Soi: We will just have to wait until it comes through.

Damayanti Kerai: Okay, great. And then my last question will be on institutional bed share, which is around 30% for the quarter. So, how should we look at, because earlier you mentioned your endeavour is to bring it down, right? But eventually, I understand you will take up scheme patients even when you have beds to ramp up. But any like guidance or any target in your mind for this part of the business?

Abhay Soi: See, what happens is that as we add new hospitals or new capacities, your institutional business is going to go up with that. Our endeavour is not

to reduce

institutional business. Our endeavour is to accommodate growth in our preferred channels of business. But if we can do both, we do not have a problem doing it. You distort it when you have a capacity constraint. If you are able to add more and more capacity to it, then it is not a problem because even the institutional business is contributing towards your fixed costs. The idea is not to, unless it was a loss-making business, then you wouldn't be doing it in the first place. The first choice is to create more capacity to accommodate that business as well as any growth in your preferred channel business. And wherever you cannot do that, you start distilling that business.

Damayanti Kerai: Okay. So, you have the flexibility to play around with this mix, right, to optimize the asset utilization?

Abhay Soi: We have done that previously. You see plenty of facilities where we have brought it down to zero, but you will see that some facilities that we open up now, let's say Mumbai, we will start institutional business because we are coming up with new capacity. The first idea is to fill the beds, because this business contributes towards fixed costs.

You have seen it in the Shalimar Bagh brown field as well. But having said that, even with the lower rates, your EBITDA per bed is higher simply because you have got operating leverage when you are adding brown field capacity on existing hospitals. So, if you take an example of Nagpur, it used to operate at 55-60% occupancy. But we have been able to ramp up the occupancy by taking in institutional business. The hospital never used to do so before our acquisition. And the immediate fallout of that is that it all percolates down to your EBITDA. So, you are better off taking where you have idle capacity. So, if we can create idle capacity, then it is great. You take institutional business, it percolates down to EBITDA and you have higher EBITDA per bed from the incremental institutional business.

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Damayanti Kerai: Okay, thank you. That's very clear.

Moderator: Thank you. The next question comes from the line of Prashant Nair from Ambit Capital. Please go ahead.

Prashant Nair: So, my first question was on the Mumbai project and the additional beds that you intend to add in Mohali. As both are built-to-suit projects, what would your investment outlay be for these two assets?

Abhay Soi: Essentially, it's going to be medical equipment, which will be let's say about Rs. 150-200 crore. But that only happens at the end when it's constructed – the last 3-6 months or whatever.

Abhay Soi: Yes, it's about Rs. 30 lakh per bed, but it's all back-ended. Medical equipment comes after the entire project is almost complete.

Prashant Nair: And Mumbai you intend to operationalize in fiscal '28, is that right?

Abhay Soi: End of '28, that is right. So, it takes about 3-3.5 years to build, including permissions.

Prashant Nair: And Yogesh, for your business, would say 65% cash conversion be kind of a reasonable number to work with, assumption to work with, or can this change, say, at any point over the next few years?

Yogesh Sareen: Yes, I think 65% is the right number. I mean, we only hope that the tax numbers will come down going forward in terms of cash outflow. So, this should be okay.

Prashant Nair: All right, great. That's it from me. Thank you.

Moderator: Thank you. The next question comes from the line of Tushar Manudhane from Motilal Oswal Financial Services. Please go ahead.

Tushar Manudhane: Thanks for the opportunity. Sir, first one on the hospitals which are coming up over the, say, next six months. So, with respect to those, what kind of operational cost addition can be factored for FY '26?

Abhay Soi: Is this for the brown fields ?

Tushar Manudhane : Yes sir, the ones which are coming up in say Q1 FY '26, Mohali , Max

Smart , Saket ?

Abhay Soi: Marginal operational cost increase because these are all brownfields. We already are incurring all the major costs in the existing hospitals.

Tushar Manudhane : So, secondly, with respect to this build-to-suit where the investment is lower, but accordingly, I mean, subsequently as and when let us say the steady -state occupancy of say 55 -60% as and when that happens, what kind of margins, you know, we are sort of assuming this build -to-suit because there would be rent share or there could be some revenue share with the partner unlike, you know, own green field, brownfield expansion ? So, you know, basically trying to understand what kind of margin dilution happens?

Abhay Soi: You have to look at what we get vis-à-vis what we invest. So , you have to look at it on a ROCE basis . Now, if 80% of the capital cost , which is land and building in this case , is incurred by the partner and you are locking it at 8 -9% yield , and we enjoy a

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35% ROCE in a stable state on the whole hospital , so you can imagine what it does to our 20% contribution. Our ROCE goes beyond 100% effectively . And most importantly, you are insulated against any cost and time overruns because that is to the partner's account and not to your account.

Tushar Manudhane : Understood. And what kind of ARPOBs , you know, do you think this location can drive it compared to say metros or tier 1s where currently MHIL is at Rs.75,000-76,000? What sort of assumption for this location ?

Abhay Soi: I don't think there is any difference between Thane and what any of the Mumbai hospitals will be operating. And you have some listed players like Jupiter , which are operating in Thane.

Rs. 75-80,000 is the ARPOB at present. Three years later, hopefully, it will be more. In Mohali, it should be no different from our existing hospital in Mohali, which is at an ARPOB of about Rs. 55,000 at this point of time. Three years later, it should be more, as you are aware that ARPOB is growing by 7 -8% every year.

Tushar Manudhane : Got it , sir. And just one more on Lucknow . Given the current occupancy and current profitability at least at the existing centres , so the growth will be more driven only by beds now, given that the operational efficiency is largely in place or you think there is still some more efficiency which can drive the profitability while beds will drive the volume growth ?

Abhay Soi: So, when you have higher occupancy, the cost also defrays over a larger number of beds. So, automatically, you have operational efficiency coming through. Other than that, like we mentioned, that there is no bunker over there right now, which should come on stream shortly and improve day care, radiation, etc.

It is coming on stream in July. And thereafter, you will have a higher amount of ARPOB emanating from that particular facility. There are other clinical programs' equipment which are online and coming onstream. We have ordered the equipment , but it takes time for it to come. All of this will contribute to higher ARPOB, better operating efficiencies , etc.

Yogesh Sareen: Also, some of the doctors have joined during Q3. So, there will be a full quarter impact going forward.

Tushar Manudhane : Understood. And just one more, if I may, on Nagpur, given the kind of size of revenue, in fact, it's relatively smaller in the overall scheme of things, but just to understand here, in terms of, you know, the current, the occupancy, which is sort of dragging down and subsequently having an impact on profitability, even if I leave aside its seasonal impact, still, just to understand, you know, the potential to add another 115 beds here.

Abhay Soi: The occupancy on a year -on-year basis has increased.

Yogesh Sareen: Q2 was 91% , and this quarter is 79%. They had higher incidence of vector borne diseases last quarter. This does happen

seasonally.

Abhay Soi: 79% is by no means a low occupancy in a slow season. 79% is midnight occupancy and like we said, it's going to take us 24 months to build the additional beds. We have no doubt that even if you take a 5% increase in occupancy over the next two years, we are going to be fully occupied out. And do keep in mind that you create

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infrastructure for your peaks, not for your troughs. So, our peak occupancy was 91% last quarter.

Tushar Manudhane : No, I meant to say that at 79% occupancy, we are at say Rs. 54 crore revenue, Rs.11 crore EBITDA, which is like roughly 20%. So, from a profitability point of view, we are more or less there and then given the kind of profitability here, we intend to still add 150 beds is what I am trying to ask. Or is the EBITDA margin still possible to get better at this site?

Abhay Soi: No, of course, it is possible to get higher EBITDA margins. Also, if you say occupancy is not an issue, then as far as the higher occupancy is concerned, you also get huge amount of operating leverage, because of the brownfield. EBITDA per bed is significantly higher then. You also have to visualize this basis the entire ROCE that we are shooting for, which is 20 -25% within 4 years.

Yogesh Sareen: The other important aspect is that we still have to get to the respectable ROCE there. Our target is 20 -25% range, and we are at 10 -11% range as of now. So, we have to add those additional beds to get to that ROCE level. And that's how it was all planned when we acquired the hospital.

Abhay Soi: Keep in mind that this is only the 9th month, right?

Tushar Manudhane : Got it. Thanks. That's it from my side.

Moderator: Thank you. The next question comes from the line of Rishi Mody from Marcellus Investment Managers. Please go ahead.

Rishi Mody: Yes, so Abhay, just wanted to understand this partner healthcare facility (PHF). So, we have used the profitability there to fund the new Vikrant and the other which is again a PHF facility. So, wanted to understand like do we just control the cash flow, or do we have ownership over that free cash flow that these Balaji Society and all of these guys create? Like can you give out that money as dividends to the shareholders or you are not allowed to do that?

Yogesh Sareen : No, there is no dividend that can be declared for any society. But one society can contribute to the other society if their objectives are same. For example, Balaji Society and Modi Society objectives are the same. So, if they have ample cash, they can donate to other societies to use it for their construction and other purposes and that's what they did. But what can be given as dividend is basically you have to upstream to the main company. But the moment you mainstream it to the main company, then you get only 75% of it because you have to pay tax on it.

So, our endeavour is always to, if you need cash in a society, you move it from one society to the other society. But you can upstream it, you have the ability to upstream.

Abhay Soi : We have the ability. Your question is can it be dividend? Yes, it can be upstreamed and then given as dividend, but of course we will be paying a 25% tax.

Rishi Mody: Okay, got it. And second, you mentioned on the PHF the fee has been revised.

What's the change like today, I am guessing we are getting around 25% to 28% of that revenue out as a fee which gets recognized in our Max Healthcare revenue books. So, just what's the change?

Yogesh Sareen : Every two years the fees get revised for each of these PHFs. There is a method to the madness. We review the cost and their cash flow, and based on that the fees are

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revised. For Balaji, annual impact will be around Rs. 25 crores for the fee revision.

Now I don't know the percentage, but that's the absolute amount of PBT, which

will

be upstreamed into MHIL through the MSAs that we have.

Rishi Mody: Okay, so we will get an extra Rs. 25 crores going forward from Balaji.

Yogesh Sareen : Yes.

Rishi Mody: Okay. Second on the Thane Hospital. So, I read that the lease is only for five years, the initial lease, and then you have two renewals versus when I look at the Zirakpur one, we have around a 20-year lease with some 20-year renewal. So, just wanted to understand like is there a risk of that property post, you know, we getting it being taken away by the developer and being given to someone else for a higher rental? Or like why did we get into only a five-year lease? And secondly, the two renewals post those five years, how many years are they for and who has the right for that renewal?

Abhay Soi: There are two things. It's not five years or three years. It's actually 15 years.

Keshav Gupta : 5 + 5 + 5, so 15 years lease.

Abhay Soi: Second is that after 12 months of operations, we have a call option on it. We can acquire it at any point of time. We can acquire it and sell it down to a REIT. We can acquire it and nominate somebody else or whatever else it is.

Yogesh Sareen: Otherwise, you have to understand the commercial logic of it, because if we do a 15-year lease in that state, then we have to pay higher stamp duty. Since we have the option to buy, we said we will take a call later and then renew it. It's at our option.

Rishi Mody: Okay, so like five plus five plus five. Okay. All right, just fine. Second, the last

question from my end is the IRDAI regulations for the guidelines that have come through.

Abhay Soi: Sorry, I just want to go back to that point. It's 15 years with a call option at our choice.

Rishi Mody: Yes, to buy the property within 12 months. I heard that. Yes.

Abhay Soi: Yes. So, don't look at it as only 15 years. We can always, if the 15 years is not extended, buy it or we can actually nominate somebody else to buy it on the same lease.

Rishi Mody: And what would be that amount if you exercise the call option, like how much do you have to end up paying to the developer?

Abhay Soi: It's at cost.

Keshav Gupta : It's at cost and a very nominal yield on the real money spent on the project.

Rishi Mody: Okay, all right . So, 1 crore per bed would, effectively Rs.1, Rs.1.5 crore per bed would become the cost if you end up acquiring it.

Keshav Gupta : Yes, that's the broad range. The real cost for putting a project up is about Rs.1.9-2 crore per bed for a greenfield.

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Rishi Mody: Okay . Last on the IRDAI guidance that's come out today or yesterday where they decided to cap the increment on senior citizen 's insurance policy price hikes and they have asked these insurance companies to get back together and negotiate with hospitals and bring it closer to the PM-JAY rates. So , just wanted your view on , firstly , how much of our revenue within the insurance pool comes from senior citizen . And secondly, do you see any rate negotiation impact from this instruction?

Abhay Soi: No, there is no rate negotiation impact. In fact, we see a benefit from it because the number of people who may be going out of the insurance net because their premium goes up, there is a step jump in that premium , now won't go out of the insurance net because of this . It's more palatable . As far as premium is concerned, there is no rate negotiation for different age groups. It's not cut that way. I mean, the cost is cost right, whether it's for a senior citizen or a junior citizen from our standpoint .

Keshav Gupta : And they are giving a price hike of 10% per year. What we negotiate with insurance companies is around 10 -12% every two years. If the underlying logic was to continue through and through , the reflective increase that the hospital seeks and most of the hospitals seek is lesser than what they

are allowing anyway.

Abhay Soi: The price change on insurance every two years is about 12 -13% at best. So, that comes to a medical inflation of about 6% per year effectively, at best, 5.5 -6% a year.

Rishi Mody: Right, yes, I got that. So, that 's the secure part just like these guys collectively, the insurance companies collectively come in and bargaining, does that impact?

Abhay Soi: That would be a Competition Commission issue, right? I don't think you can cartelise . But no, if that was to happen, then they should be kind of getting together and negotiating overall current rates? Why would they take only one segment? Like Keshav said, if our increase is 6% per year and they are being permitted 10% per year, so where is the problem?

Rishi Mody: Okay. All right. Understood. Yes, that's it from my end. Thank you for taking my questions.

Moderator: Thank you. The next question comes from the line of Andrey Purushottam from Cogito Advisors. Please go ahead.

Andrey Purushottam : Hello , this is Andrey. I had one question regarding the payor mix. How much of the payor mix is in your control? And to the extent that is in your control, what are the steps that you are taking next year or so to hopefully in favour of higher profitability?

Abhay Soi: No, it is not . It's the burden of disease, right? Effectively, we start adopting and align ourselves with what the market demand is. You move towards higher technology and perhaps more robotics and higher end programs. That is what drives the clinical mix.

Andrey Purushottam : No, I am talking about the payor mix.

Abhay Soi: Sorry, as far as the payor mix is concerned, we have seen there is an increase of 28% in international business. So, this is because we have engaged deeper with various geographies that we get patients from as far as the domestic patients are concerned. Upcountry is 40% of our NCR business . All of that is growing at a faster pace and so we want to have deeper engagements with upcountry. And other activities that we do to engage with the communities that should enhance the preferred channels for us.

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Andrey Purushottam : Okay, thank you.

Moderator: Thank you. The next question comes from the line of Vaibhav Saboo from Nippon AIF. Please go ahead.

Vaibhav Saboo: Yes, thanks for giving me the opportunity and congrats for a good set of numbers. It was asked previously also, but just want to understand, you know, that on a consolidated basis, for example, our EBITDA margin is somewhere around, like, 20-25 percent, but you told to assume for mature hospitals, it would be around 29-30%. I just wanted to understand for the asset-light model, while I understand completely that ROCE would be very much attractive, I just wanted to understand what would be a similar, what would be the EBITDA number for the mature business for asset-light models. It could be around 20-25%, just wanted to understand that range.

Abhay Soi: This is post Ind AS, so the rental line comes below EBITDA.

Vaibhav Saboo: But for pre Ind AS basis, what would be the EBITDA margin?

Yogesh Sareen: It should depend on what the maturity of the hospital is, but I think it should be lesser by 5-6%.

Vaibhav Saboo: That's it from my end. Thanks, and all the best.

Moderator: Thank you. The next question comes from the line of Neha Manpuria from Bank of America. Please go ahead.

Neha Manpuria: Yes, thanks for taking my question. Abhay, just one clarification on the Mumbai expansion. I think once the new tower is commissioned, we are planning to phase out some of the older ones and rebuild that. So, would the net addition still be the 268 beds that we have talked about or should I also adjust the beds that will be going off for some time till we get it back? How should that phasing happen?

Abhay Soi: No, so it is going to be a net reduction there. When

we start Phase 2, which should

be almost immediately, you will have a reduction of 160 beds.

Keshav Gupta: Yes, we are currently adding 268 beds. After that is commissioned, we have to tear down 160 beds. The next we will be adding 271 beds. That's a phase of about 1.5-2 years period. So, net reduction eventually will be 111 beds in Phase 2.

Yogesh Sareen: So, for the time being, you should take 268 beds. There is no reduction there. We have to take a call on reduction. Once we do the Phase 2, then the reduction will happen.

Neha Manpuria: So, the Phase 2 is not happening immediately?

Abhay Soi: Yes, at that stage you will bring down about 160 beds.

Neha Manpuria: Okay, and this is over the next two years. This entire process will get completed in the next two years.

Abhay Soi: That's right. But the beds which are being pulled down are more ward style. One of the things we have been able to do through this expansion is right-size the type of beds. Because the type of beds that we have a demand for and occupancy for are single beds, deluxe beds and so on. Whilst the present facility, one of the reasons that the occupancy levels typically have been lower at Nanavati compared to the rest

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of the network, is because a lot of the beds were old style, ward style, nightingale ward beds. Those are the beds that are getting pulled down.

So, we are right sizing the beds. So, the net impact shouldn't be that much. You know, you are getting what is required where we have almost a 90% occupancy on single beds, ICUs, etc. Those will continue. While the ward style, nightingale wards, etc., come down, which in any case weren't in demand.

Neha Manpuria: Okay. So, there won't necessarily be a financial impact from it because of the right sizing beds, but then how should I think about the improvement because I think one of the areas that we wanted to improve was also the margin of the Nanavati Hospital. So, does that start happening once we have commissioned all of the when you completed Phase 2 fully?

Abhay Soi: Absolutely, and it happens on two counts, because one is your number of beds increases. Let's say if your operational beds are 280, and even if you take an increase of about 140-150 beds on top of that, net increase, you are looking at 50-60% more capacity addition. So, you will have the same cost structure defrayed over more number of beds and even the higher cost structure defrayed over the more beds and the right size beds. So, you will get the operating leverage, and your margins will certainly go up.

Neha Manpuria: Understood. Okay, that's helpful. My second question is just an extension of the previous question. I think there has been some chatter among insurance companies about getting together and trying to negotiate pricing, even using IRDAI as one of the agencies that does that. I know you mentioned Competition Commission, but do you see that as a risk when we are thinking about pricing with insurance? Could that 10% over two years be much lower as we think about the next, let's say, three to four years depending on how this process progresses?

Abhay Soi: Not at all. We haven't seen any approaches, any discussions, anything other than

that. IRDAI is actually the regulator for insurance companies. So, I don't think insurance companies can get into the umbrella and have IRDAI negotiate that or get into the discussion. Because I think it is far beyond their mandate, right? Other than a conversation like this, where you are mentioning a rumour, and that also far and few that have come our way, there has never been any approach on that. Secondly, and most importantly, the moot point is that medical inflation is 6-7%. That's the increase we get every year. You just get it every two years, so it shows up at a 12% increase every two years. But the fact is, it's for two years. O

n an annual basis, it's

still 5-6%. Any increment that you see is on real growth, which is new technology, robotics coming in. Of course, in terms of percentage margins, you get less margins. But in terms of value, you get more.

Because do understand, eventually all the innovation that happens in the healthcare sector eventually has to pass through the doors of hospitals. The same hospitals 30 years back who used to do conventional surgeries are now doing robotics. The same hospitals which used to do CABGs at best are doing transplants now. We used to have general surgery surgeons working on oncology. Now you have organ specific oncology. You have got radiation, chemo, etc. You have got other robotics doing those surgeries. When you do all of that you go up the value chain as well. So, there is real growth.

Yogesh Sareen: Neha, Also, if the insurance companies get organized, on the other side, the hospitals can also get organized, right? There can also be a collective bargaining on the hospital side. That's easier to do in fact. Hospitals are far lesser in number if you take organized chains.

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Neha Manpuria: Yes, the demand supply equation, I guess, is in your favour.

Abhay Soi: Not only that, also the density of beds, right? We are dominant players in Delhi NCR.

And similarly, some of our peers are in their own markets. If you are kind of dissipated across the country, then it's a different matter. Today, we have 15 facilities in Delhi NCR, which are twice the number of facilities of the next three listed players put together. So, literally three players put together and the next 20 players are not even the same size. Your bargaining power is slightly different in that case. But it's not even a question of bargaining power. I think it's a question of what is right and what isn't. 5-6% growth in pricing every two years is not something that you can dispute, right? How much lower do you want to be is the question.

Neha Manpuria: Yes, fair enough. Thanks so much, Abhay.

Moderator: Thank you. We take the next question from the line of Kunal Dhamesha from Macquarie. Please go ahead.

Kunal Dhamesha: Hi, thank you for the opportunity. The first one on the D warka. While we have done a very good job of breaking even, based on our reported number, if I look at the indirect cost in the D warka Hospital for the 140 beds, it seems to be roughly around Rs.150-Rs.160 crore a year, assuming that our direct costs are similar in line with the network average. So, my question is given that we are going to add almost around 1,400-1,500 beds next year, how should we think about the indirect cost related to that? I know there are some brownfields etc as well, but again these brownfields are separate towers versus the current towers and all that, so there might still be higher kind of indirect cost there. So, let's say, all in all these 1,500 beds, what is the indirect cost that we should assume whether it's Rs.1,000 crore or Rs.800 crore or how should we think about that?

Abhay Soi: No, I think the way you need to think about is that indirect cost is not linear. Right?

Kunal Dhamesha: It would come primarily in the first year?

Abhay Soi: It's already there. Whatever indirect cost you have been incurring up till breakeven, we have incurred the indirect cost. It is that indirect cost on 140 beds that we have broken even on. Thereafter, every bed that you add, your indirect cost is not going to be linear. It's only the direct cost which is going to be linear. Now indirect cost will be just some nurses and some resident doctors etc., which is a marginal cost.

What is going to happen is that every bed that you add, the revenue from that bed will give you a lot more leverage onto your EBITDA. A lot more which means almost your entire contribution margin. Let's say if you are operating at a

60% contribution

margin, 50% of your top line, or that means almost your entire contribution margin will flow to your EBITDA, because your indirect cost is not linear. And that is the basic principle of any brownfield or opening any hospital. And frankly, over the next few quarters, you are going to see that being demonstrated.

Keshav Gupta : And those beds are not coming in new tower. The existing structure already has 300 beds .

Abhay Soi: It already has the beds. The beds need to be fitted out.

Keshav Gupta : We only opened 140 beds.

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Kunal Dhamesha: Correct. So , once you operationalize another 160 beds, there would still be some incremental cost related to it, right?

Abhay Soi: If you have a hospital which has 8 floors, and you have operationalized 4 floors. Now you need to operationalize another 4 floors. All your costs of your clinicians, management, utilities or common areas, all the support functions, kitchens, everything is already incurred.

When you open up another floor, what do you do? You get nurses and resident doctors , which don't cost us much . Effectively, you are looking at maybe 8 -9% of your revenues attributed to this cost. If your contribution is 60%, you take out 8% from that. So, 52% of your top line is going straight to your bottom line.

Kunal Dhamesha: When you ramp up.

Abhay Soi: Obviously when you ramp up, right? So, every incremental bed will give you more and more.

Yogesh Sareen: Post the breakeven stage.

Abhay Soi: Post the breakeven stage. And that's not only a hospital , any other venture works like that . That's basic economics.

Kunal Dhamesha: Sure. Another one on the institutional mix and the payor mix or bed share and the payor mix, if I look at the nine-month s payor revenue from the institutional patients have grown at roughly around 34% year-on-year, and the bed share has only grown at around 20 %. So, my view is that there is a decent amount of kind of rebasing of the pricing of some sort was happened , and I also see there was some order passed in February '24 from the Delhi Government advising the C GHS rate. So , what are we looking forward in the next one or two months incrementally?

Abhay Soi: Delhi government has nothing to do with CGHS rates. CGHS rates are under Central Government .

Yogesh Sareen: Yes, and Kunal, Abhay already mentioned that the gap between the cash, the self - pay and the institutional segment has come down in terms of ARPO B, right? Earlier, it used to be 44%, now it's only 36%. That means there is an 8% improvement in terms of the ARPO B. Basically, we are doing more oncology . So, as a result, you find that the bed growth is not there, but there is a revenue growth. That obviously means that there is an ARPOB growth which is happening, and it's not because of rates. There is nothing happening on the rates. It's only because of the change in the mix of the patients.

Kunal Dhamesha: Sure, I have some document . Probably I will share it in more detail on that. I have just one more question , if I can.

Yogesh Sareen : Yes.

Kunal Dhamesha: Sure . So, sir, this Rs.40 crore donation that we have done from Balaji and Devki Devi Society , and that kind of , you know we have taken it pre -EBITDA , which kind of reduces our profitability. So , does this kind of help with maintaining the tax-exempt status for this trust hospital?

Yogesh Sareen : Kunal, first of all, it's not before EBITDA, after EBITDA. It's in EBITDA only. It's basically from one society to the other society. The other societies are not having

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operating income, so we don't show them as a separate column , right? We put that under the elimination column. Then you see that the overall number matches. So , it's not a major movement. It's the only movement of one society donating to the other one , and it has nothing to do with tax status.

It is b

basically, under tax status, you are supposed to use the money that you have for the objectives of the society and that part of the objective is also to help other societies . If one society has surplus cash, then it can obviously donate to the other societies. It's within the ambit of their 'objective ' clause and within the ambit of the approvals that they have in terms of exemptions from the Income Tax department to donate money to other societies for furthering the objectives of the society.

Moderator: Thank you. As there are no further questions, I now hand the conference over to the management for their closing comments.

Abhay Soi: Thank you everyone , for taking time out to join us for third quarter results. We look forward to interacting with you again next quarter.

Disclaimer: This is a transcription and may contain errors. While the transcript has been edited for clarity and efforts have been made to ensure a high level of accuracy, the Company assumes no responsibility for any errors.

Call: May 2025

May 27, 2025

Listing Department ,
National Stock Exchange of India Limited
Exchange Plaza, Plot C -1, Block G,
Bandra Kurla Complex, Bandra (E),
Mumbai – 400 051

Symbol: MAXHEALTH Listing Department,
BSE Limited
Phiroze Jeejeebhoy Towers,
Dalal Street,
Mumbai – 400 001

Scrip Code: 543220

Sub.: Transcript of Earnings Call held on May 21, 2025

Ref.: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Sir / Madam,

Please find enclosed copy of transcript of earnings conference call, organised on May 21, 2025, on financial results of the Company for the quarter and financial year ended March 31, 2025.

The said transcript is also available on the website of the Company at www.maxhealthcare.in/financials#earnings-call.

Kindly take the same on record.

Thanking you

Yours truly,
For Max Healthcare Institute Limited

Dhiraj Arora
SVP - Company Secretary and Compliance Officer

Encl.: As above
Page 1 of 18

Max Healthcare Institute Limited
Q4 FY25 Earnings Conference Call Transcript
May 21, 2025

Moderator : Ladies and gentlemen, good morning, and welcome to the Max Healthcare Institute Limited Earnings Conference Call.

Please note that this conference is being recorded.

I now hand the conference over to Suraj Digawalekar from CDR India for opening remarks. Thank you, and over to you.

Suraj Digawalekar: Thank you, Ryan. Good morning, everyone, and thank you for joining us on Max Healthcare's Q4 and FY '25 Earnings Conference Call. We have with us Mr. Abhay Soi – Chairman and Managing Director; Mr. Yogesh Sareen – Senior Director and Chief Financial Officer; and Mr. Keshav Gupta – Senior Director, Growth, M&A and Business Planning.

We will begin the call with opening remarks from the Management , following which, we will have the forum open for an interactive Q&A session. Before we begin, I would like to point out that some statements made in today's call may be forward -looking in nature and a disclaimer to this effect has been included in the earnings presentation shared with you earlier.

I would now like to invite Abhay to make his opening remarks. Thank you, and over

to you, Abhay.

Abhay Soi: Good morning, everyone, and thank you for joining us on Max Healthcare's 4th Quarter and full year FY '25 Earnings Call.

This year has been a pivotal one for us, fuelled by focused strategic decisions, disciplined execution on ground, and significant milestones that have set new benchmarks.

After acquiring hospitals in Nagpur and Lucknow in the last quarter of the previous year (FY '24), we acquired the 500 -bed marquee Jaypee Hospital in Delhi NCR this year. As part of our asset -light expansion strategy, we commissioned Max Dwarka and signed up contracts for built-to-suit hospitals to be set up by our partners in Mohali, Thane and Pitampura (Delhi).

During the year, in what could have been a year of moderate growth otherwise, we initiated multiple long -term growth plans, including the announcement made last week regarding acquisition of approximately 1 acre land parcel , adjoining our fully occupied 400 -bed hospital in Vaishali.

Page 2 of 18 Our recent acquisitions, Max Lucknow, Max Nagpur and Max Noida , played a key role in accelerating top line and EBITDA growth. Our overall financial performance for FY '25 reflected this momentum , with a year -on-year growth of 26% in revenue and 22% in EBITDA.

Notably, Max Lucknow demonstrated year -on-year growth of 56% in revenue and 102% in EBITDA, while Max Nagpur reported a year -on-year growth of 23% in revenue and 86% in EBITDA in their first year since acquisition. Max Noida is being integrated into our network and reported a gross revenue of INR 228 crore with an operating EBITDA of INR 48 crore at a margin of 21% post -acquisition , since October '24.

Additionally, our newly operationalized asset -light hospital in Dwarka achieved EBITDA breakeven in 6 months – a new record. The hospital clocked a revenue of INR 171 crore and an EBITDA loss of INR 29 crore for the entire year FY '25 , since becoming operational in July '24. It exited the year with a revenue of approximately INR 30 crore per month and 73% occupancy on 235 beds in March '25, with balance 68 beds yet to be opened. We are already looking forward to embark on the next phase of expansion of 200 beds at this facility.

These results demonstrate the resilience of our operating model and the calibre of our teams. It reinforces our confidence as we prepare to commission three new brownfield towers at Max Smart, Nanavati -Max and Max Mohali hospitals within the next three months , and complete our greenfield facility in Gurgaon by the end of this year, adding approximately 1,500 beds in total.

Among the year's other standout achievements, we are proud to have been ranked amongst the Top 20 companies in S&P BSE 100 Index and recognized under the "Next Leaders " category for corporate governance excellence by Institutional Investor Advisory Services (IIAS) , India's largest proxy advisory. This recognition underscores our unwavering commitment to the highest standards of transparency, accountability and integrity in corporate governance.

On that note, we would like to highlight that we continue to undertake corporate actions to simplify the holding structure, improve governance and optimize cash flows. To that effect, we have concluded merger of two wholly -owned subsidiaries Alps Hospital Limited and Max Hospitals & Allied Services Limited. We have also filed an application with the NCLT for merger of Crosslay Remedies Limited and Jaypee Healthcare Limited, which will, in effect, reduce our outflow on the acquisition by INR 200 -225 crore.

Now coming to the performance highlights of the fourth quarter , which is our 18th consecutive quarter of year -on-year growth :

1) Our average occupancy for the Network stood at 75% versus 74% in Q4 last year, and at similar levels in the trailing quarter . While the occupied bed days (OBDs) grew by 30% year -on-year and 2% quarter -on-quarter . Do note that the Network occupancy stood at 78%, if we exclude Max Noida which was acquired in Nov'24 and is still being integrated into the Network .

2) Average Revenue Per Occupied Bed (ARPOB) for the quarter stood at INR 77,100, remaining relatively flat year -on-year and growing 2% quarter -on-quarter. Like -for-like ARPOB for the Existing Units grew by 7% year -on-year and 2% quarter -on-quarter.

3) Network gross revenue was INR 2,429 crore compared to INR 1,888 crore in Q4 last year and INR 2,381 crore in the previous quarter. This reflects an increase of 29% year -on-year and 2% versus the trailing quarter. Of this, New Units

reported a gross revenue of INR 353 crore . While Existing Units registered a year-on-year growth of 12% in revenue, driven by 6% growth in occupied bed days and 7% growth in ARPOB .

4) International patient revenue stood at INR 202 crore, registering a growth of 28% year-on-year, despite contraction in patient footfalls

from Bangladesh and Yemen due to continuing political unrest.

5) Network operating EBITDA stood at INR 632 crore, reflecting a growth of 26% year-on-year and 2% quarter -on-quarter. This includes INR 67 crore EBITDA contribution from New Units .

6) Network operating EBITDA margin stood at 27.2% for the quarter. Existing Units reported an EBITDA margin of 28.5%.

7) Annualized EBITDA per bed for the Network stood at INR 74 lakhs . Like-for-like EBITDA per bed for Existing Units stood at INR 84 lakhs, reflecting a growth of 7% year -on-year.

8) Profit after tax, excluding exceptional items and one -off tax gains, was INR 376 crore versus INR 311 crore in the Q4 last year and INR 372 crore in the previous quarter, reflecting a growth of 21% year -on-year. There was an exceptional item of INR 74 crore towards CIS charges paid to YEIDA for seeking permission for change in shareholding of Jaypee Healthcare Limited prior to acquisition and one-off gain in tax costs of INR 18 crore consequent to voluntary liquidation of a wholly -owned step -down subsidiary in Q3 FY '25.

9) Overall free cash flows were INR 422 crore . During the quarter , INR 390 crore was deployed towards ongoing capacity expansion projects and upgradation of facilities at acquired hospitals. Consequently, net debt for the Network came down by INR 32 crore to INR 1,576 crore at the end of March '25.

10) Continuing our efforts to support the local communities, we treated approximately 36,500 outpatients and 1,200 inpatients from economically weaker sections of society entirely free of charge , worth INR 53 crore as hospital tariff.

11) Both our strategic business units (SBUs) continued to report significant growth in the revenue and profitability.

- Max@Home reported a top line of INR 56 crore, reflecting a robust growth of 22% year -on-year. It offers 15 specialized service lines across 15 cities with over 50% repeat transactions.

- Max Lab reported a revenue of INR 46 crore, reflecting a strong growth of 19% year -on-year. It provides services in over 50 cities through its network of more than 1,200 collection centres and active partners.

12) Now coming to the status of our expansion projects :

- 268 beds at Nanavati in Phase I: Interior work is in progress and we expect to commission this facility within 90 days.

- 155 beds at Mohali : Finishing work is underway and we expect to commission this facility within 90 days as well. Plans to add 45 more beds through internal reconfiguration will be initiated once the new tower is completed.

Page 4 of 18 • 400 beds at Max Smart (Saket Complex): Interior and MEP -fit-out works are ongoing . We expect to commission this facility latest by Q2 FY '26.

- At Max Lucknow : We have added 128 beds on floors 9 to 12th, as communicated previously. Through internal reconfigurations, we have added 35 beds in May and plan to add 39 more beds in the next 12 months. We expect to complete the onco block here by Q2 this year.

- 500 beds at Sector 56 Gurgaon : Structural work is in progress. We expect to commission the facility by end of this calendar year.

- At Dwarka : The onco block is expected to be commissioned by Q3 this year.

All of these are on schedule, and we will see significant ramp -up in our capacity over the next 12 months.

- 127 beds at Max Nagpur : 12 beds have been added in October 2024. For the balance beds on additional floors, we are awaiting the Environment Clearance (EC) approval , while the bill of quantity (BoQ) detailing has started . We expect to complete this project within 24 months.

- 397 beds at Patparganj : We have received the environmental clearance and the tendering work is in progress. The project continues to be largely on schedule.

- 550 beds at Max Vikrant (Saket Complex) : We are still awaiting the clearance from forest department for tree transplanta

n. All other statutory approvals already in place, and we expect to complete the project by 2028.

- 400 beds at Zirakpur (Mohali) : Our partner is currently awaiting the EC approval and has initiated the tendering process, BoQ detailing , contractors, etc. The project is expected to be completed within the next 30 months.

- 140 beds at Vaishali : As announced earlier, we have acquired the land adjoining to Max Vaishali and will be initiating drawings, detailing, etc. in the next couple of months. We expect to complete this brownfield project in the next 30 months as well. Presently, Max Vaishali is operating at 83% occupancy .

And finally, moving on to the overview of the Company 's performance for the full year ending March '25:

13) Network gross revenue stood at INR 9,065 crore, reflecting a growth of 26% year-on-year. New Units contributed INR 938 crore to the gross revenue.

14) Overall network operating EBITDA grew by 22% year -on-year to INR 2,319 crore, reflecting a margin of 26.8%, while EBITDA per bed stood at INR 70 lakhs. Existing Units reported an EBITDA margin of 27.9% and EBITDA per bed of INR 80 lakhs.

15) During the full year, we generated INR 1,447 crore of free cash flows from operations after interest, tax, working capital changes and routine capex. INR 1,182 crore was deployed towards ongoing expansion projects and upgradation of facilities at acquired hospitals , INR 146 crore was distributed as dividend and INR 1,716 crore (net of cash at Jaypee Healthcare Limited) was used for the Jaypee acquisition.

Page 5 of 18 With this, we would like to open the floor for any question and answers.

Moderator: Thank you. The first question comes from the line of Amey Chalke from JM Financial.

Amey Chalke: Thank you so much for taking my question. Congrats to the Management on good set of numbers. First question I have, if we can give information on the profitability of the acquired units like Nagpur, Noida, Lucknow for the quarter?

Abhay Soi: As far as Lucknow hospital is concerned, we had 56% growth in revenue and 102% growth in EBITDA. Nagpur has reported a 23% growth in revenue, 86% growth in EBITDA. The Noida hospital , of which we completed the acquisition in Nov'24, reported a gross revenue of INR 228 crore and EBITDA of INR 48 crore at 21% margin. This is for full year FY '25.

Yogesh Sareen: Given the fact that the 2 of these hospitals were acquired in Q4 last year , Alexis (Nagpur) unit was acquired in February '24 and Lucknow unit was acquired in March '24, it is difficult now to segregate the last year numbers into 2 parts ; that's why we have not put the numbers for these from this quarter . But nevertheless, you have the Overall and Existing Unit s' numbers with you. We had INR 67 crore of EBITDA from New Units, which is 19.4% margin .

Now if I take out Dwarka, which just broke even in Dec'24, and even in this quarter, there is nominal EBITDA from this hospital , our EBITDA margin is 24.9% in the New

Units .

Amey Chalke: Sir, the reason I wanted to ask because how much margin expansion still can happen in these units, except Dwarka, obviously, which is recently commissioned. But the other 3 units, how much margin expansion scope is there for next 1 to 2 years .

Abhay Soi: Significant . This is only the first year of acquisition. By the time you put the building blocks in place, which you must appreciate takes some time. Just to give you an example, in Lucknow, you had expansion of EBITDA by 102%, yet, there is no radiation oncology there. There is no bunker . And that bunker is going to come into play at the end of H1 this year.

And once that happens, your oncology business , which even currently is not anywhere near the oncology business of the rest of the hospitals, there is a major move up over there. If you look at Nagpur, for example, we are already now approaching very high -capacity

utilization. The teams are coming in to play. We are already looking at the next phase of expansion , which is adding another 100 beds over there. So once that happens, that gives you a major fillip, because you don't have any capacity left over there.

As far as Max Noida is concerned , we bought a unit, which is operating at less than 50% occupancy. So , you have 50% more occupancy, higher sort of ARPOBs as well as all the clinical programs, etc. coming. Typically, when you do these acquisitions, the first year you will have less of an improvement curve in terms of absolute value, maybe in percentage terms as you are operating off a smaller base. The second year is when you are going to have that improvement kicking in . And each one of these, whether it's Jaypee, Nagpur or Lucknow, holds significant further brownfield expansion. So , you will have yields coming out in years to come.

Yogesh Sareen: The important question is to say that we have very respectable margins. For example, Lucknow margin is more than 30% , Nagpur margin is around 22% , and Jaypee will also be in similar range as Nagpur . The main thing is that we want Page 6 of 18 EBITDA per bed to improve now, which means that we have to start medical programs that are higher -end.

For the New Units , overall EBITDA per bed is 43% of the rest of the network, which means we have to really grow that number. And that's one of the reasons why you will find that the EBITDA growth is a bit tepid compared to revenue growth , because the share of new hospitals is going up and their EBITDA per bed is lower than the rest of the network.

Abhay Soi : But going forward, you are going to see that the EBITDA growth will outstrip revenue growth there. Not that the revenue growth is slowing down, there's more than enough fuel over there too.

Amey Chalke : Sir, second question I have is on that itself, because we have said that 1,500 beds Brownfield expansion will happen in next year. So this year, we have seen a lot of acquisitions. So what are the objective next year? Is it a Brownfield focus here? Or you think the M&A would still be there?

Abhay Soi : It's not one at the cost of the other. The brownfield expansion is already under construction. It's already being built. If you look at Mohali Hospital, it's operating 80 - 85% plus kind of occupancy levels . And the number of beds that are coming in should get taken up very quickly. That's the whole purpose of a brownfield expansion .

Now similarly, if I look at Mumbai, in the next 90 days again, you are going to have some beds come up that solves the problem of the Mumbai hospital . Then Max Saket again, is at a very high occupancy, and that's our main hub. We are really crying for space and more beds over there. These are the three brownfields we are doing and the balance sheet is something which is completely different.

Gurgaon Sector 56 - Gurgaon for us is the highest ARPOB, the highest EBITDA per bed market. Again, with this new hospital, which is very well located in Sector 56, we are looking to adopt the same approach , will have the same results that we have had in Dwarka.

That has nothing to do with our balance sheet and our capabilities of doing other things. We will continue to seek acquisition opportunities also. But like I said, we have to be able to touch base or clear 2 of our filters. One is the 20 -25% ROCE within 4 to 5 years , and in markets where we have at least 1 or 2 of our peers. And if these 2 conditions are met, we are happy to look at acquisitions , and we will continue to pursue them.

Amey Chalke: Sure. Thank you so much, I will join the queue.

Moderator: Thank you, the next question comes from the line of Damayanti Kerai from HSBC.

Damayanti Kerai : Hi, good morning , and thank you for the opportunity. My first question on your payor situation at new hospitals. So if I look at your institutional bed shar

e, you mentioned

it went up to, say, 33% in 4Q. So is it because you are putting more of these patients to really like move up in the occupancy, cover up the fixed cost? And then can you please comment on it?

Abhay Soi: That's absolutely right. Typically, when you open a new hospital, Dwarka, for example, you kind of fill it up with all payor groups, because the first focus is to get the occupancy up and then you start churning it.

Page 7 of 18 If I look at Nagpur, for example, it was operating at 50 -odd percent occupancy prior to acquisition, but we took up institutional business over there in order to ramp up the occupancy and the occupancy now is almost full up.

What that does is, it helps you occupy your idle beds, and yet cover fixed costs or at least a part of it, and it all trickles down to EBITDA. From our standpoint, so long as contribution is positive from any payor group and there is idle capacity, it makes sense to do that. We continue to do that, and that's what is paying us the yields / dividends / returns as well. There is no sense in keeping the beds idle. If you have any idle beds, you must do institutional business over there. And we continue with that strategy.

Damayanti Kerai: Sure. So at what occupancy you choose to optimize between institutional or TPA patient? So say you ramp up to 40%, 45% or what level you take that decision?

Abhay Soi: We can go up to, let's say, about 80% occupancy. Till we get to 80% occupancy effectively, we can take institutional patients. Suppose we have 60% occupancy other than institutional, then for the balance 20%, we will take institutional. We will not have an idle bed.

Yogesh Sareen: Look at it the other way round. As long as we are not refusing any cash or insurance patients, we would like to accommodate the institutional patients.

Abhay Soi: That's right. Any growth in cash and insurance patients will be accommodated first.

Damayanti Kerai: Okay. Got it. My second question is on international patient revenue. So I remember a few quarters back, you were talking about initiatives or incentive provided by the government to ease flow of the medical international tourists in India. On back of that, have you seen any meaningful pickup in your international patient business? Because if I see, I think it's still like 9% of your hospital revenue, although on a bigger base, but any significant or notable push, which you might have seen from these initiatives?

Abhay Soi: It will be 9%, but please understand we have increased capacity by 30%. And the growth in that business this quarter has been 28%, which is a significant increase. I would not entirely put it down to Government of India, because that's a slow move. But the image of the country has improved.

But then you have certain setbacks. You have geopolitical unrest. We have had that issue with Bangladesh, Yemen, etc. We most recently had this with the Pakistan issue, where the airspace has been closed. But you will always be two steps forward, one step back. If you were to draw a line, we have had more than 25% growth in this business for a long period of time, and 28% is an acceleration in spite of these setbacks.

Damayanti Kerai: Got it. And my last question is, if I look at your ARPOB on a network basis, including new and existing units. So it's somewhere around, say, INR 75,000 for FY '25. So what kind of we should look at this parameter, given now going ahead also, we will be having mix of existing plus new beds on a consistent basis?

Abhay Soi: It's irrelevant what the overall ARPOB is. Today, if we acquire something for \$100 and it gives us \$25 of EBIT, it's a 25% ROCE. We do not worry about what it is producing is lower ARPOB or higher ARPOB. What we have to look at is what is happening to overall EBITDA and EBITDA per bed, what is happening to ROCE vis-à-vis what we are deploying.

Page 8 of 18 Damayanti Kerai:

Got it. Okay. Thank you.

Abhay Soi: If I look at the ARPOB growth of existing facilities, it has gone up by 7.4%. But when we buy something with lower ARPOB, that drags it down. But the fact is, should we not be acquiring something with lower ARPOB, even if it is very high on ROCE, even if we are getting it very cheap. Answer to that can never be 'do not acquire'. Answer to that always has to be 'yes, you must'. So long as we can deploy money at a 20-25% ROCE. Our FY '25 ROCE is pretty high, it's 26% overall.

Damayanti Kerai: Yes. That's clear. Thank you.

Abhay Soi: Thank you.

Moderator: Thank you. The next question comes from the line of Neha Manpuria from Bank of America.

Neha Manpuria: Yes. Thanks for taking my question. I think in the presentation, we mentioned that the IP growth at 3.5% was impacted due to lower footfall in Internal Medicine, Paediatrics. Is there something to read into this? Because it isn't as if March is a

seasonal quarter. So just trying to understand the reason for this weakness .

Yogesh Sareen: No. Typically, when you compare quarter -on-quarter, Q3 happens to have more Internal Medicine patients and Q4 happens to have less Internal Medicine patients.

There's that impact of seasonality which comes in, that's what we are trying to explain there, because the share of Internal Medicine has come down.

Neha Manpuria: Okay, so the drop you are talking is on a quarter -on-quarter. But even if I were look at a year -over-year, it's just 3% IP volume growth. So hence, I was wondering if there is anything one-off in the quarter?

Yogesh Sareen: A lot of dengue patients, etc. comes up through quarter . Sometimes it happens in October, sometimes it goes into Q4. That's probably the change that we are referring to here.

Neha Manpuria: Okay. Got it.

Abhay Soi: Last year, if you recall, October had significant amount of dengue patients, particularly in places like Mohali and Dehradun, where we had massive dengue. For some strange reason, it went into October. This year, there wasn't.

Neha Manpuria: Yes. But I was thinking about March versus March, but I understand that there could be...

Abhay Soi: Yes, but nothing to read into it. It could also be due to some of the new facilities that we acquired, they weren't there last year, now they are . They may have a lower level of Peds and Internal Medicine . None of our signs are showing any weakness . If I look at the number of OPDs overall, there's been a significant increase .

Yogesh Sareen: In the 4th Quarter , surgical business comes up, so IP volum es goes up.

Abhay Soi: Year -on-year.

Yogesh Sareen: It will be basically configuration of the new hospital versus the old hospital.

Page 9 of 18 Neha Manpuria: Okay. Got it. The second question is on Dwarka. I think in the presentation, we mentioned that the Dwarka achieved breakeven in December quarter. And based on the loss number, if I were to calculate, I still see there is a loss in Dwarka in the quarter. It's sti ll making EBITDA loss. Is that correct or am I missing something here?

Abhay Soi: No, there is no loss. After breakeven, we have not made a loss. It's obviously lower profit. It's not as if in we broke even in December and then January, February or March, any of the months that we have lost money, we have not. Since then, till date, we have not lost money. Only occupancy has moved up. Your flow through to the bottom line has improved since then.

Neha Manpuria: Okay. And have we ramped up more beds in Dwarka from the 140 that we commissioned and what's the plan for the ramp -up for the rest of the beds?

Abhay Soi: We already are at 235 beds . We had an occupancy of 73% on these beds in March. We had INR 30 crore of revenue coming from that single hospital in the month of March. We are expecting now any time to be opening the 68 additional beds. These beds are ready , we just need

to staff these beds as the occupancy increases .

Neha Manpuria: Understood. Yes, yes. Makes sense. Thank you.

Abhay Soi: Thank you.

Moderator: Thank you. The next question comes from the line of Prashant Nair from Ambit Capital.

Prashant Nair: Yes. Thanks. Good morning, everyone. Just had a question on the expansion plan, since there is no slide on that in this quarter's presentation. So the 1,400 -odd beds that you were planning to add in Financial Year '2 6, that remains on course, would it roughly be in the same range?

Abhay Soi: That's right. We usually have it in the investor presentation, not in the quarterly presentation. If you see the investor presentation, it continues to be the same. We are expecting within 90 days, both Nanavati 268 beds and Mohali 155 beds.

By second quar ter, you will have Max Smart. By end of calendar year, you will have the Gurgaon Sec-56 facility, and in the midst , we are also adding some beds in Lucknow and other places. We'll be hitting around 1,500 beds, not 1,400 beds.

Prashant Nair: Okay. And of these, how many would you operationalize this year, if you can just give an approximate range? I imagine the bigger project s would have not all beds operational from the beginning. How do you calibrate that?

Abhay Soi: There will be a ramp up. At Dwarka, for example, we didn't commission all 300 beds, although they're ready . What we commission ed is as per occupancy. We started the hospital in July '24, and by Marc h '25 we are already at 73% occupancy .

By April -May, it should theoretically be further. When you see 75% -plus occupancy in any set of beds, that's the time we open the next lot. Now we are opening the final lot in Dwarka. And so, roughly within the year, we would have been able to occupy all the beds.

Keshav Gupta : And that was a greenfield . Majority of the beds that are coming up are brownfield .

Abhay Soi: Gurgaon should have the same trajectory as Dwarka, whereas the brownfields would

be much faster, right? The uptake is almost immediate.

Page 10 of 18 Prashant Nair: Right. So that's what I was trying to figure out. So for the greenfield, would the rule of thumb be that you initially operationalize, say, 50%, and then once that hits 75% occupancy, you add the next block? Is that how one should think about it when...

Abhay Soi: Well, you can think of it like that, but the only difference is that in a greenfield, you take 6 months to break even. By the time you get to 75% occupancy, it probably takes you a year. In a brownfield, you do it almost in a few months. Like in couple of months, you would just be ramping up capacity. Your take-up is much faster.

Prashant Nair: Understood, thanks. And when you categorize into existing and new units, so new units would include all the acquired hospitals plus just the Greenfield ones? Or would you also include some of the Brownfield ...

Abhay Soi: All acquired units plus Dwarka.

Prashant Nair: Yes. And going forward also, it is the greenfield plus the acquired...

Abhay Soi: That's right.

Prashant Nair: Thank you very much. That's it from me.

Moderator: Thank you. The next question comes from the line of Vivek Agrawal from Citigroup.

Vivek Agrawal: Thanks for the question. If you look at EBITDA, this year the Company has done quite well, we have seen expansion in EBITDA margin in the existing units, and even the new units acquired in that division was quite well. Now we are seeing another 1,400-1,500 beds coming up this year. So it would be great, actually, if you can just give some qualitative colour on how to look at the overall EBITDA growth margins like for the next couple of years? Thank you.

Abhay Soi: Brownfields normally give you higher EBITDA margins, in percentage terms as well, because fixed cost is already incurred. Primarily, with the brownfields coming in, which is close to 1,000 beds, they

should throw out significantly higher EBITDA

margins. And I am not going to give you any forward-looking statements, but theoretically, they should give you higher EBITDA margins compared to your existing business.

Even if you are occupying those beds with institutional business, they give you higher EBITDA margins. That's what we have analysed from all our previous brownfield that we have done. That should work out well for us.

And again, we increased capacity by 30% last year. Dwarka, of course, for the first 6 months was operating at a loss and after that it has been profitable. You will see the full strength of Dwarka coming into the current year. You will look at the momentum of both Lucknow and Jaypee sort of snowball in the current year.

On top of that, you have got three brownfields that are coming. And the one greenfield, which comes at the end of the year will produce some operating loss, but like Dwarka did, it should more than get set off or absorbed by what we are doing on the brownfields and the momentum of others.

That's where we are. We are probably going to be entering, if I was to look at it, the strongest year in the last 5 years that we have had.

Vivek Agrawal: Thank you sir, this is comprehensive. Just one more question. As I think there is a significant capacity expansion by you as well as the peers, so how you should look

Page 11 of 18 at the situation as far as supply of doctors or medical tellers, nurses, et cetera? Thank you.

Abhay Soi: The brownfield capacity additions do not typically require increase in senior-level clinicians. It does not, because the existing doctors increase their footprints within those hospitals, since they are the ones who are seeking these beds to start with. Other than that, we do not see massive capacity expansion happening other than one more hospital other than ours in the current financial year in Delhi NCR. Nobody else is coming in the current financial year and thereafter.

We have a very strong presence in Delhi NCR. We have 14 facilities there. We tend to pick the best of what is available. And if I look at Mohali, it is not as if any new hospitals are coming, we are just expanding our capacity. In Mumbai, no new hospitals are coming right now, we are just expanding our brownfield capacity. But again, you have to keep in mind that in India, there is a need for quality healthcare and new greenfields, brownfields are required.

And all the senior clinicians over the next few years or a decade are going to be emanating from current set of MBBS doctors, etc. that you have. Even today, the cost of MBBS, i.e., the salary is about INR 45,000-50,000 per month. The demand-supply gap shows up in the salary standards as well. So, we do not foresee any shortage in senior clinicians.

Vivek Agrawal: Thank you, sir. That is from my side.

Moderator: Thank you. The next question comes from the line of Piyush Kumar from Magnus Hathaway Investment.

Piyush Kumar : Yes. So sir, my question is can we see the average revenue per occupied bed go into the territory of 80,000 plus in the coming quarters? And if yes, what specialty could be the main contributing factor?

Abhay Soi: All the specialties , which have been growing , should take you there. We had a 7% growth in ARPOB for the existing hospitals.

Yogesh Sareen: Yes, there is a gap between new units and existing units. We do not have radiation oncology in Dwarka and Lucknow; once we start that, it will add to the ARPOBs. We obviously want to shorten the gap but there will still be a gap between a Nagpur ARPOB and a Delhi ARPOB .

Keshav Gupta : And the second part is that all the expansions coming up are in Mohali, Saket Complex (Smart), Gurgaon , Mumbai . These hospitals have a higher ARPOB in the current mix.

Piyush Kumar: And sir, my second question is regarding the stock price. So sir, in the last 6 months, the st

ock prices range bound over 1,200 level. So any comment on that, sir?

Abhay Soi: You have to tell me. We are happy to discuss operations , financials and balance sheet, but markets have their own territory.

Piyush Kumar: Thank you, sir.

Moderator: Thank you. The next question comes from the line of Tushar Manudhane from Motilal Oswal Financial Services .

Page 12 of 18 Tushar Manudhane : Yes, t hanks for the opportunity , sir. Sir, just if you could share like for FY '25, ARPOB, maybe like payor -wise, institutional ARPOB, international patient ARPOB , and let's say, cash ARPOB.

Yogesh Sareen: We typically do not share payor -wise ARPOB separately, but we do mention the various parameter s. Typically, the international ARPOB is 1.3x of the cash , in this quarter , institutional is around 40% lower than the cash. So that's the parameter s coming into this quarter . It changes from quarter to quarter depending on what is the contribution of patients that we got from international and institutional channels . Institutional ARPOB has de -grown a bi t, by around 3% to 4% in this quarter , compared to previous quarter. But that's temporary , I would say there's nothing changed directionally in any of these ARPOBs.

Tushar Manudhane : And on a full year basis, you have to think about in terms of either growth or decrease, if you could share that?

Yogesh Sareen: No, I would say similar trends as we see. The recent trends are more important than the full year trends. Let's say, the cash is 100, the international will be anything between 1 .3 to 1.5 x, depend ing on which quarter you talk about . And the Institutional would be 52% of that 100 % in some quarters or it will be 57% of that 100 % . And Insurance will be 8 -9% lower than the cash. That's the range it all works out to.

Tushar Manudhane: Got it, sir. Got it. Thank you. That's it from my side.

Moderator: Thank you. The next question comes from the line of Kunal Lakhan from CLSA.

Kunal Lakhan : Hi, good morning. Abhay, you said earlier during the call that because of the Brownfield expansion, the margins should improve or rather it's accretive to the margins. So our existing units are operating at about 28.5% margin and EBITDA of INR 84 lakh per bed. There is an upside potential to this going ahead as per you then?

Abhay Soi: That's right. We have always guided to that. Brownfield s have higher EBITDA per bed and have higher EBITDA margins , because the management cost s and senior clinician cost s are already incurred by the existing hospital . Any incremental beds that you have, even if you were to fill them with lower ARPOB businesses like institutional, they will still give you a higher EBITDA per bed as well as EBITDA margins.

Kunal Lakhan : Sure. As and when this incremental Brownfield capacity stabilizes, right, where do you think these ARPOBs or rather like EBITDA margins or EBITDA per bed would settle?

Abhay Soi: We are not going to give you a forward -looking statement.

Kunal Lakhan : Okay. No worries. Okay. Secondly, on the free cash flow side that we are generating some serious free cash flow from operations. And going by your newer assets are also like reaching EBITDA breakeven faster , any upfronting of your long -term guidance of, like, say, adding or doubling your capacity over the next 4 years, any upfronting of that guidance?

Abhay Soi: No. Any guidance that we give, is based on us already having acquired the asset, breaking ground or permissions , etc. What we do not tell you is that look, our belief is that over the next 5 years, we will generate, let's say, INR 15,000 crore . So we would look at tripling our capacity. No, that's not how we sort of guide.

Page 13 of 18 What we can tell you is that, we have guided that we are happy going up to 2.5x net debt to EBITDA. We have very little leverage on our balance sheet. We are going to

be funding all of our expansion

mostly through internal accruals . And we have more than spare room to acquire. Just last year, we did three acquisitions . What is very difficult for us to do is to give any guidance till the transaction is closed. Now there are other places that we are looking at expanding, putting up facilities, etc. But till we have that signed, we cannot give any guidance . We are cognizant of the fact that at 2.5x net debt to EBITDA, we can get money at 8 -8.25% and we are able to deploy it at 20-25% ROCE . But as and when the deals happen, we will announce and that number on a rolling basis will only go up. So today, we are guiding you that by 2028, we may have 9,000 beds, but I am pretty sure one year down the line, that number would have moved up.

Kunal Lakhan : Understood.

Yogesh Sareen : Availability of cash is not a constraint for the capacity additions.

Abhay Soi: It's not only cash . We have enough room in the balance sheet too.

Kunal Lakhan: Correct. Understood. Lastly, a bookkeeping question. Can you give me occupancy numbers in Q4 for the newer units individually, Nagpur, Lucknow, Noida?

Abhay Soi: So Dwarka, I've told you is 73% in March . The older units were 79%. The Chitta and Jaypee Noida acquisition, which we completed in November '24, that's 46%.

Yogesh Sareen: 46% on average basis . 52% in Jaypee Noida and 26% in Chitta.

Abhay Soi: And Chitta is a really small unit. We obviously did not pay much where we did not have any EBITDA to start with.

Kunal Lakhan : Sorry. And how much you said for Lucknow and Nagpur?

Yogesh Sareen: Lucknow is 65% , with the additional beds that we opened . Lucknow 's operating capacity has gone up. And in fact, in this quarter, again, it will go up to 413 beds. It was 343 beds in last quarter. So, including the additional beds that we have added , the overall occupancy was 65%.

Abhay Soi: No, in the operating capacity that we bought was what?

Yogesh Sareen: That will be 209 beds on 234 beds that we started with . So it was around 90%.

Abhay Soi: It was 90% occupancy on the beds that we bought. After that, we keep ramping up the beds. If you look at the end number of beds today, then it is 65%, but this is basis the beds that we have added over there .

Abhay Soi: Nagpur is 81%.

Kunal Lakhan: Okay. Very helpful, sir. Thank you so much and all the best.

Moderator: Thank you. The next question comes from the line of Rajit Aggarwal from Nilgiri Investment Managers Private Limited .

Rajit Aggarwal: Good morning, sir. First off, congratulations on a good set of results and the overall growth achieved during the year. I just have 2 very quick questions. And this is not a Page 14 of 18 concern exactly. The interest cost has been going up quarter -on-quarter. And just for how to model it, should we assume the current quarter as the run rate going forward?

Or is it expected to go up even further?

Yogesh Sareen: No. We do not expect any material change in the number. The interest cost went up since the October '24 quarter, because we borrowed money for the Jaypee acquisition of INR 1,000 crore , and then we had taken INR 600 crore loan for the Sahara acquisition in March '24 . The net debt we have is INR 1,572 crore at the end of March '25, which means that primarily these are the two term loans that we have taken which is actually affecting the net debt. Unless we really do any major acquisitions, we do not think any major change is expected in the interest cost as such.

Rajit Aggarwal: So similar run rate for the remaining quarters.

Abhay Soi: If we do an acquisition, then yes there will be change .

Rajit Aggarwal: Yes, absolutely. Second question, while Mr. Abhay touched upon on the supply side, on the competition side, but specifically, given that a peer of yours is coming up with the same number of beds in Gurgaon and at the similar premium location. Do you think that is

going to impact your numbers or the overall scenario for the next 1 to 2 years?

Abhay Soi: No. We are not seeing any impact of this. We have a very large network in Delhi NCR. Like I said, we have got 14 facilities . Today, in terms of number of locations, we have twice the number of locations that our next three peers have put together. In terms of number of beds, we are equal to or more than the number of beds that all of them have put together in NCR. We have got 35,000 healthcare workers, of which 20,000 live in Delhi NCR. Out of our 6,500 senior clinicians, about 4,000 live in Delhi NCR. We are the largest home care business, and only profitable one in the country, almost entirely focused on Delhi NCR. Then we have third largest lab over there.

In terms of brand, it's much bigger . And like I said, today we have many facilities in Delhi NCR but the performance of each of them is far better than the flagship of any other hospital chain that you heard of over there, in terms of occupancy, ARPOB, EBITDA per bed , etc.

Rajit Aggarwal: Yes, absolutely, I agree.

Abhay Soi : Yes. For us, we are solving our own needs. I don't think the other hospitals are the same size either, probably half the size.

Rajit Aggarwal: Okay. They're in the same number, but yes, you would know much better. Anyway, thanks for your feedback.

Abhay Soi : Thank you.

Moderator: Thank you. The next question comes from the line of Dheeresh K. Pathak from WhiteOak Capital.

Dheeresh Pathak : Sir, thank you for the opportunity. Sir, for the Noida asset, what is the total FSI potential and how much has been used currently?

Page 15 of 18 Abhay Soi: It's 18 acres of land for Noida. And we can add another 1,000 + beds. The present facility is operating at 50% occupancy , so there is tremendous potential over there.

And we keep adding. I don't think we have a problem for the next decade at least.

Dheeresh Pathak: Okay. Understood. Thank you.

Moderator: Thank you. The next question comes from the line of Tarun Bhatnagar from Tribeca Investment Partners.

Tarun Bhatnagar : Yes. Hi. Thank you for your time. My question is on Gurgaon specifically. Now Gurgaon gets a lot of international patients. So should we assume that once Gurgaon comes up, then your international numbers will increase? And you mentioned on the competition that you have some strength. Any particular competitor who you think can be like very tough competitor for you in Gurgaon? Or you think that you have much better, like right to win in that region? Thank you.

Abhay Soi: Every hospital of ours, in its micro market , is the best -performing hospital, with respect to perhaps every line item, including significantly higher occupancy. We operate at maybe 75% and the next close competitor is at 65% . It's a function of the value proposition we put together.

Now Delhi NCR also, coincidentally , happens to be our backyard. So , I am quite confident about what we are going to be doing over there. So, nothing much is moving our competitive intensity or what we believe would have been the outcome. That's where we are as far as Gurgaon is concerned.

Tarun Bhatnagar : Okay. And international numbers would see an increase once Gurgaon comes up?

Abhay Soi: It is destination for international business, but it is very similar to Delhi. We do not give it a significantly higher weightage . But in the overall scheme of things, we do not think it is going to really move the needle for us in terms of international business .

At the enterprise level, will it significantly increase medical tourism? Answer is 'No'.

In any case, in the first year or so, we are going to be taking in all sorts of occupancies, not only international business, but institutional , etc. like we have done in Dwarka. And we do not see day one majority of the beds being filled up with international patients. It also takes time to do that, to mature

that business for that

particular hospital. So no, I don't think it will be a major move.

Tarun Bhatnagar: Thank you.

Moderator: Thank you. The next question comes from the line of Rishi Mody from Marcellus Investment Managers.

Rishi Mody: Yes. So Abhay, I am just looking at your capacity beds, it's around 5,100 beds. And our operational bed count is around 4,654 beds , which comes to a ratio of 90% operationalization. This has been the ratio for the last couple of quarters. Just wanted to understand, historically, we used to do around 95% of our capacity beds were operationalized. Are we going to get to that 95%? Or is it like there's a theoretical capacity, but we are using the space for something else and hence the operational bed count on the existing infrastructure will remain the same?

Abhay Soi: No. If I take the example of the last two acquisitions that we did, we acquired Jaypee Noida Hospital. And along with it , there was a hospital each in Chittoor and Bulandshahr , which have come. They have a 200-bed capacity and it's operating at 26%. So that leaves out about 150 beds there and then.

Page 16 of 18 Yogesh Sareen: 26% on 100 beds. We haven't opened out 100 beds yet.

Abhay Soi: Okay. So that's one. But nevertheless, it's 150 beds over there out of your 5,100.

Then you have the Max Jaypee Hospital, which was acquired, which is a capacity of 500 beds.

Yogesh Sareen: We opened only 377. Your question is about the operational capacity and available capacity. There are 3-4 hospitals where we haven't opened all the beds. For example, Jaypee Noida, the hospital has the capacity to go up to 500 beds , we have

opened only 377 beds. There is some spend to be done to build up that 377 capacity to 500. We are planning for the spend, but knowing that the occupancy on the 377 beds is 52% only, we are not really fast tracking that. Similarly, Chitta Hospital is a 200-bed hospital, but only 100 beds have been opened. And on those 100 beds, we have only 26% occupancy. So, there is no plan to open the balance 100 beds because that will also require some spend.

Dwarka Hospital, as Abhay mentioned, that we opened only 235 beds. The capacity is around 303 beds. We'll open another 68 beds soon, some of them have already been opened as we speak. And then we have 200 beds at Bathinda Hospital, where we have opened only 100 beds. So, this is the capacity. At Bathinda and probably Chitta, leaving these 2, the others will open in due course of time. At Bathinda, we have not seen the demand, so we do not plan to really open the other 100 beds. But rest all will be onboard, I would say, in another 8-9 months' time.

Keshav Gupta: Also, to the point, all the capacities are real. They are an outcome of the business plan. We do not make them live because the business in some sites may not require so at that point.

Rishi Mody: Okay. Got it. So like in practicality, if the demand is there, you can open up 5,100 beds?

Abhay Soi: That's right, yes. That's right.

Rishi Mody: Lastly, I wanted to understand ALOS, right? So I am comparing existing to existing, which is around 4.2 days. Like I just wanted to understand, are we taking any structural efforts to bring ALOS further down on at least the existing infrastructure? Or it's more like now we have optimized to a level, and hence the fluctuations will remain on account of seasonality of mix?

Abhay Soi: It is seasonal, and it is ongoing and it also depends on your clinical programs. You can't even compare any two hospitals across the board. You have to do it on an absolute basis, because no two hospitals have the same clinical programs.

ALOS by itself is not a bad thing. It depends on which programs you are running. Just to give you an example, higher-end business, which is international business and higher-end surgical program, which is, let's say, transplant, will always have a higher

ALOS, but will have a higher EBITDA per bed per day as well.

That is what we should be focusing on rather than purely ALOS, unless you have a huge capacity constraint. At this point in time, we have that and the teams do work on discharge processes, etc. to turn around the beds faster.

But if you have capacity, then it becomes theoretical, simply because let's say, I have one bed for 365 days, now whether I have 365 patients on one bed per day or one patient staying for the whole year, what matters to me is what is the EBITDA per bed, purely from a financial perspective.

Page 17 of 18 Rishi Mody: Okay. All right. Finally, I think on the Nanavati piece, you mentioned we have got one block upcoming. And post that, I think we are going to demolish some 260 beds block, and then build up a new one to replace it, which net-net we'll have a higher bed amount. So just wanted to understand till what period like on those 260 beds at Nanavati, which will be demolished will we be losing revenue?

Abhay Soi: No, we are not demolishing 260 beds, we are demolishing 160 beds. Also, of these 160 beds, a large amount of them are the ones which are not occupied. We are adding 268 right now. For a period of 2 years, these 160 will not be available and then we come back with another 270 beds or so.

Rishi Mody: Okay. So if I understand it correctly, the current 160 beds, which will go under demolition 2 years out, which means in FY '28. Is that correct?

Keshav Gupta: Currently, we have 328-bed hospital. We are activating 268 beds approximately now as Phase -1.

Abhay Soi: After that 580, minus 160...

Keshav Gupta: Minus 160, and we'll be activating another 275-280 beds as Phase -2.

Rishi Mody: So the 160 deactivation is happening in FY '26 or FY '27?

Abhay Soi: These will go, then you have new beds coming, another 280 beds coming. For 2 years, you will have 400 beds when we deactivate 160.

Rishi Mody: But these 160 you are saying are not that occupied, like I am just trying to understand the hit on revenue will be commensurate to the bed or it will be lower?

Keshav Gupta: Most of these 160 beds are ward structures and lower occupancy beds. They are the old economy beds. The idea was to redevelop them any way. We have got a chance to develop them in a much better fashion, to add a new tower altogether on that footprint.

Rishi Mody: Okay. Great. Thank you.

Moderator: Thank you. The next question comes from the line of Alankar Garude from Kotak Institutional Equities.

Alankar Garude : Just one small clarification. Had you booked some cost in Dwarka in the first quarter before commencing operations, which are contributing to the full year EBITDA loss of INR 29 crore ?

Yogesh Sareen : Yes, we did. You know that the opening of this hospital was delayed . We were earlier planning for its opening in quarter 1 of last year . We had hired manpower and that's the reason we could ramp it up faster. There was some loss in the quarter 1, it's around INR 6 crore , which was also reported, we actually mentioned about it also in the earnings update.

Alankar Garude: Understood. Yes. That's it from my side. Thank you.

Moderator: Thank you. Ladies and gentlemen, as there are no further questions, I would now hand the conference over to the Management for their closing comments.

Page 18 of 18 Abhay Soi: Thank you, once again. We will hear from you next quarter. We appreciate all your time.

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Call: Aug 2025

August 21, 2025

Listing Department ,
National Stock Exchange of India Limited
Exchange Plaza, Plot C -1, Block G,
Bandra Kurla Complex, Bandra (E),
Mumbai – 400 051

Symbol: MAXHEALTH Listing Department,
BSE Limited
Phiroze Jeejeebhoy Towers,
Dalal Street,
Mumbai – 400 001

Scrip Code: 543220

Sub.: Transcript of Earnings Call held on August 14 , 2025

Ref.: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Sir / Madam,

Please find enclosed copy of transcript of earnings conference call, organised on August 14 , 2025, on financial results of the Company for the quarter ended June 30 , 2025.

The said transcript is also available on the website of the Company at www.maxhealthcare.in/financials#earnings-call.

Kindly take the same on record.

Thanking you

Yours truly,
For Max Healthcare Institute Limited

Dhiraj Aroraa
SVP - Company Secretary and Compliance Officer

Encl.: As above

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Max Healthcare Institute Limited
Q1 FY26 Earnings Conference Call Transcript
August 14 , 2025

Moderator: Ladies and gentlemen, good day, and welcome to Max Healthcare Institute Limited's Earnings Conference Call.

Please note that this conference is being recorded.

I now hand the conference over to Mr. Suraj from CDR India. Thank you, and over to you, sir.

Suraj Digawalekar: Thank you, Nirav. Good afternoon, everyone . And thank you for joining us on Max Healthcare's Q1 FY '26 Earnings Conference Call.

We have with us Mr. Abhay Soi – Chairman and Managing Director; Mr. Yogesh Sareen – Senior Director and Chief Financial Officer; and Mr. Keshav Gupta – Senior Director, Growth, M&A and Business Planning.

We will begin the call with opening remarks from the management , following which we will have the forum open for an interactive Q&A session.

Before we begin, I would like to point out that some statements made in today's discussion may be forward-looking in nature, and a disclaimer to this effect has been included in the earnings presentation shared with you earlier.

I would now like to invite Abhay to make his opening remarks. Thank you, and over to you, Abhay.

Abhay Soi: Good morning, everyone, and thank you for joining us on Max Healthcare's Q1 FY '26 Earnings Call.

We are pleased to report a strong start to the financial year, with year-on-year growth of 27% in revenue and 23% in operating EBITDA for the Network. This marks the 19th consecutive quarter of year-on-year growth, underscoring the success of our strategy and strong execution capabilities of the team. This performance is driven, in part, by the successful integration of the hospital acquisitions made in recent years, which have significantly bolstered both our top and bottom-line results.

Looking ahead, our focus remains on commissioning the new bed capacities scheduled to come on stream during the course of the year. We expect to add approximately 1,000 brownfield and 500 greenfield beds. Trial runs have already been initiated at the new 160-bed brownfield tower at Max Mohali.

In addition, our Board has recently approved the execution of an agreement-to-lease for a "built-to-suit" 130-bed hospital in Dehradun. The site is located approximately

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100 meters from our existing 220-bed Max Dehradun facility, which reported over 80% occupancy in Q1 FY '26. The new facility is expected to be commissioned by the end of 2028 and will primarily focus on advanced oncology services, including radiation therapy, which is presently missing at our Max Dehradun facility.

As part of our strategy to focus on super-specialty care in selected geographies, we have executed a binding term sheet to divest Chitta and Anoopshahr hospitals for Rs. 40 crores, with completion expected by September '25. Of the two, only Chitta hospital was operational and reported approximately Rs. 5 crores in revenue and Rs. 1 crore of EBITDA loss in Q1 FY '26. These assets were part of the overall Jaypee Healthcare acquisition and had limited strategic alignment.

Now coming to the performance highlights of the first quarter. Please note that the term "Existing Units" hereafter refers to the Network facilities that were operational prior to Q1 FY '25, while Max Noida and Max Dwarka are categorized as the "New Units".

1) Average occupancy for the Network stood at 76% compared to 75% in both Q1 last year and the trailing quarter. However, Existing Units achieved occupancy levels of over 78%.

2) Occupied bed days increased by 26% year-on-year and 4% quarter-on-quarter.

3) Average revenue per occupied bed for the quarter was Rs. 78,000, growing nominally by 1% year-on-year as well as quarter-on-quarter. For the Existing Units, like-for-like ARPOB grew by 5% year-on-year and 2% quarter-on-quarter.

4) Network gross revenue was Rs. 2,574 crores, up from Rs. 2,028 crores in Q1 last year and Rs. 2,429 crores in the previous quarter. This reflects an increase of 27% year-on-year and 6% versus the trailing quarter. Of this, New Units reported a gross revenue of Rs. 231 crores. Like-for-like revenue growth in Existing Units was 16%, driven by an increase of 10% in occupied bed days and 5% in ARPOB.

5) Digital revenue from online marketing activities, web-based appointments and digital lead management was Rs. 744 crores, accounting for approximately 29% of the overall revenue. Website traffic witnessed over 69 lakh sessions during the quarter, growing by 61% year-on-year and 7% quarter-on-quarter.

6) International patient revenue reached Rs. 208 crores, registering a growth of 32% year-on-year and 3% quarter-on-quarter despite airspace restrictions and geopolitical volatility in certain regions.

7) Network operating EBITDA stood at Rs. 613 crores, reflecting a growth of 23% year-on-year and a 3% dip quarter-on-quarter, primarily due to the impact of annual increments and additional manpower hired for new capacities. New Units contributed Rs. 27 crores to the Network EBITDA, representing a growth of 19% compared to trailing quarter.

8) Network operating EBITDA margin was 24.9% for the quarter. Existing Units reported an EBITDA margin of 26.2%, which is 44 basis points higher than Q1 last year. Also, adjusted for the one-time donation of Rs. 12 crores, the margin increased to 26.7% for Existing Units.

9) Annualized EBITDA per bed for the Network stood at Rs. 68 lakhs. Like -for-like EBITDA per bed for Existing Units was Rs. 75 lakhs, reflecting a 7% growth year -on-year.

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10) Profit after tax for the Network was Rs. 345 crores versus Rs. 295 crores in Q1 last year and Rs. 376 crores in the previous quarter, reflecting a growth of 17% year-on-year.

11) Free cash flows for the quarter were Rs. 389 crores . We deployed Rs. 435 crores towards ongoing capacity expansion projects and facility upgrades at newer units, while Rs. 131 crore was spent towards land purchase for brownfield expansion at Max Vaishali. As a result, net debt for the Network stood at Rs. 1,755 crores compared to Rs. 1,576 crores at the end of March 2025.

12) Continuing our efforts

to support the local communities, we provided free treatment to approximately 40,000 patients from economically weaker sections of the society , worth Rs. 62 crores at hospital tariff.

13) Both our strategic business units continued to report steady growth in the revenue and profitability.

- Max@Home reported a top line of Rs. 60 crores , reflecting a robust growth of 22% year -on-year. It offers 15 specialized service lines across 15 cities, with over 50% repeat transactions.
- On the other hand, Max Lab reported a revenue of Rs. 48 crores , reflecting a growth of 19% year -on-year. It provides services in over 55 cities through its network of more than 1,300 collection centres and active partners.

14) Now coming to the status of our expansion projects :

- 268 beds at Nanavati in Phase 1 – We are in the advanced stages of commissioning three basements, ground to third floors, along with the seventh floor, over the next few weeks.
- 400 beds of Max Smart at Saket Complex – Interior and MEP fit -out works are progressing as planned, with phased commissioning expected to start towards the end of Q2 FY '26.
- Max Lucknow – The current capacity of the hospital stands at 413 beds, and we expect this to increase to 520 beds by end of the financial year. Of the 107 additional beds, 32 beds are ready for commissioning. The oncology bunkers are also in advanced stages of completion , with LINAC installation expected to start in early September of this year.
- 500 beds at Sector 56 Gurgaon – Structural MEP work is currently in progress , with high side equipment under installation . We expect to commission the facility by the end of this financial year.
- 100 beds at Max Nagpur – We are currently awaiting formal environmental clearance , while the project has been otherwise approved . Civil contract has been awarded and we expect to complete the project within 24 months.
- 397 beds at Patparganj – Post receipt of environmental clearance, barricading and tendering work is in progress. The D-wall designs, etc., have been done and work is now being awarded accordingly. This is also in line with what we had projected.
- 550 beds at Max Vikrant at Saket Complex – We are still awaiting clearance from the Forest Department for tree transplantation. A fresh application has been submitted in line with discussions with the department. In any case, this project is to start upon commissioning of the 400 beds at Max Smart by the end of this year.

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- 400 beds at Zirakpur, Mohali – Partner has received approvals for the drawing and construction, and site work is progressing at a very fast pace. The project is expected to be completed within the next 24 months.
- 140 beds at land adjoining Max Vaishali – Demolition of the existing structure is underway, and the building plans are being filed with the authorities. We expect to complete this project within the next 30 months.
- 500 beds at Thane – Partner is in the process of finalizing the master plan for

the larger site . Detailed drawings for the hospital are being prepared, and we expect submissions to begin by the end of this quarter.

- 250 beds at Pitampura, Delhi – Partner has submitted drawings for approval, while site clean -up and barricading works have been completed . Construction work to commence upon receipt of necessary approvals.

With this, we open the floor for any questions you may have.

Moderator: Thank you very much . The first question is from the line of Tushar Manudhane from Motilal Oswal Financial Services Ltd .

Tushar Manudhane: Sir, just in terms of the operational beds have increased by almost 25%, and that is how the revenue growth it seems to be given the stable ARPOB at the headline level.

So, if I exclude these additional operational beds , which would have happened over last one year, then the remaining beds performance, how it has been, if

you could
throw some light.

Abhay Soi: Like we mentioned, Dwarka opened in July '24, and Jaypee Noida was acquired in Oct-November '24. If we just take these two facilities out, then the growth in revenue has been 16%.

Tushar Manudhane: And that is driven again by further sort of optimizing payor mix? Or is it a combination of --?

Yogesh Sareen: Basically, for the Existing Units , that means including the Lucknow and Nagpur hospitals , the growth has been 16% and 18% in revenue and operating EBITDA , as Abhay mentioned . If we take out the se two ne w hospitals, i.e., Lucknow and Nagpur, the acquisition s of which happened in the Q4 FY '24 , in that case, the growth would be around 13% in revenue and around 15% in operating EBITDA , excluding one -time donation. So that means even the base leve l of existing hospital s, after taking out Lucknow and Nagpur, has seen quite a robust growth.

Also, I want to mention that in that case, the growth in ARPOB will be 7% instead of 5% that Abhay mentioned over the Existing Units . The base level existing hospitals growth is 7% in ARPOB, 5% in the OBDs and overall growth in the revenue is 13%.

Tushar Ma nudhane: And of course, the occupancy would be, any which ways , going sort of 80% plus, right? I mean , because the headline occupancy itself is at 75%, 76%.

Yogesh Sareen: Occupancy in the base level hospital s will be around 79.9% in this quarter.

Abhay So i: Almost 80% in the quarter for the base hospitals. But if you add Lucknow and Nagpur to it, then it comes down to 78%. And then if you add the other two (Dwarka and Jaypee) which we did last year, then it comes down to that 76%, which is similar to what it was last year.

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Tushar Manudhane: Got it. Yes, fair. I mean , the further extension to this is that if I exclude this new whatever that got added over last year, then the base hospitals , given that we have already grown in this year as well, and we are already at 80% occupancy --

Abhay Soi: That's right. But I just want to tell you that we are calling the two hospitals that were added in the last 12 months, as existing hospital s, right? But if we extend thi s argument to 15 months, then it becomes four hospitals. So therefore, we are just telling you f or the existing hospitals that we are only counting the 12 months. In spite of that, the occupancy has gone up from 75% to 78%. But if we take out the two, which were added in last March, then it goes up to about 80%.

Tushar Manudhane: Got it. So subsequently, new growth because of these new additions is very much intact as we for FY '26 to '28 or '29. Just trying to understand if I exclude all these and only on the base hospitals, probably now the base, it's been dynamic in nature. So, I cannot do much about it. But let's say, if I exclude last 12 months additions, then subsequently whatever there's a recent comment and we are reaching at 80% occupancy , in the next two to three years is there still scope to grow and further increase the occupanc y? Or that becomes more or less the base, and so the addition of beds is what is going to drive the growth going forward?

Abhay Soi: No. See, for your base hospital s itself, the nomenclature changes. You see whatever has been added really over the last 12 months or the 15 months has been acquisition s and greenfield , right?

Tushar Manudhane: Yes.

Abhay Soi: For example, what we are going to be adding in the next 30 -60 days are the three brownfield s that are coming through. So, they add on to the existing facilit ies. Then it's difficult to say that, look, in a brownfield , the earlier beds are growing by this much and the newer beds are growing by that much. And since it's brownfield , the take up should be very quick.

Tushar Manu dhane: Understood. And sir, just secondly on this international patient flow, which all other geographies we have been sort of engaged with

to get this kind of high growth?

Abhay Soi: So, I do not want to go into details of specific countries. There have been newer geographies, including some of the developed countries as well. But of course, some time back, we had guided you to the fact that we were opening direct -to-fly offices in other countries. There was a focus towards international marketing, and that business is something we had called out much earlier even a few years back. And I think what this has led to is that we have got increased traction through that strategy. And we have been doing more of that and being able to drive more patients.

Tushar Manudhane: Got it. And just lastly from my side in terms of net debt, given the projects at hand, without considering any inorganic opportunity at this point of time because that will be as and when it comes, how should one sort of think of net debt at the end of FY '26?

Yogesh Sareen: Yes, the net debt will go up to some extent because we have tied up some funds for the projects underway. By the end of FY '26, we may probably have addition in the net debt to the extent of around Rs. 400-500 crores.

Abhay Soi: Rs. 400-500 crores. It will translate into Net debt -to-EBITDA of still less than 1.0x I believe.

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Moderator: Thank you. Next question is from the line of Damayanti Kerai from HSBC Securities and Capital Markets (India) Private Limited.

Damayanti Kerai: My first question is on your focus on the oncology segment. So, this is largest for you, and year-on-year also I think it has gone up. So, it's already 25%, 26% of your total hospital revenue. And I understand in newer hospitals also you are trying to offer these services. So, in terms of further growth from current level, what kind of headroom do you think is available here?

Abhay Soi: First and foremost, some of the newer hospitals, particularly Dwarka and Lucknow, do not offer radiation oncology at this point of time, because the bunkers are not there. But in the third quarter, we are expecting the bunkers to come through in both of these hospitals. Therefore, you will see a larger share of oncology certainly in these hospitals, and that will impact the overall share as well. We do not believe that there is any reason for the growth in oncology to abate. These are structural factors and we are essentially catering to the market. Essentially, the market is growing at what it is, and that is what is leading to increase in our share of oncology.

Damayanti Kerai: Okay. And do you think the kind of demand out there in the market and your leading position here, this therapy could be upwards of 30% in, say a few years from now. And why I am asking because I guess what we are hearing from your peers also, they are focusing a lot more on oncology compared to a few years back, from that perspective?

Abhay Soi: I think to a large extent we can say we are focusing on it, but we are also all reacting to what is happening in the market. And yes, we have seen growth in oncology numbers, perhaps more than other specialties. And if you compound it at that, then certainly you will start increasing the share of oncology, and it will be a higher market share. It has been growing, and it's not necessarily that the new facilities that we acquired that has led to this growth in oncology alone. Because the newer facilities, like I mentioned, have a lower component of oncology at this point of time since the radiation -oncology is not there yet.

For example, Nagpur does not have radiation -oncology. Lucknow did not have radiation -oncology. Dwarka does not have a radiation -oncology. Jaypee does have, so that's really only one out of the four new hospitals. And like I said, when we are able to expand our suite of services, which is in Q3, in at least two out of these four hospitals, then the share should go up.

Damayanti Kerai: Got it. My second

question is on your newer hospitals, which will open soon. Say, Nanavati will start in next few weeks or so, and then Smart is also coming up by end of this current quarter. So, what is the status right now on the doctor hiring part? Like when do you onboard most of the doctors, like before start of the unit or you will add on gradually as you offer more and more services? And what kind of numbers you are looking here?

Abhay Soi: All the brownfield expansions largely offer the entire suite of services. Like I mentioned earlier, some amount of hiring that was required for augmentation of these newer hospitals has already happened, particularly at Nanavati, Mohali and Smart (Saket). For instance, when we open 400 beds, we are not opening 400 beds on day one. As and when you get the floors, you open it up but we have more than adequate clinicians already onboarded to be able to support the operations. Then incrementally you will keep growing thereafter as well. But that will only be incremental, so you would not feel it. So, whatever has been absorbed is absorbed and you will not see any impact because of that.

Damayanti Kerai: Got it. That's helpful. Thank you and I will get back in the queue.

Moderator: Thank you. Next question is from the line of Shaleen from UBS Securities India Private Ltd .

Shaleen Kumar: Hi, guys. Thank you so much for the opportunity. So, one question which I want to understand, you made a comment Abhay also that direct cost has gone up, and it's largely because of the New Units . But is this going to be stable from here? For example, I can see that your direct cost is Rs. 1,000 crores right now from Rs. 917 crores to Rs. 1,015 crores . So now the growth of this unit would be inflationary or there will be more additions will be that? And the subsequent impact on the margin. So, from Q4 to Q1, I understand this is seasonally weak. But if direct cost shouldn't go up, then one should expect margin to expand from here onwards meaningfully?

Abhay Soi: As far as direct cost is concerned, the significant majority of it is manpower cost. And on 1st of April we have increased salaries , so you get that bump up from 31st March to 1st April. We have always maintained that brownfield s do not really have an impact , but 30 days before or a few weeks before when you are doing trial runs, etc., you have staffed the hospital . It may not necessarily only be clinicians, but also people down the line. You staff for the hospital before fully commissioning and you are kind of carrying that cost. So yes, I think that little bit of cost may not be generating extra revenues, but it will do so over the next few weeks, once these brownfield s kick in. Coming to your point, theoretically, it should increase.

Keshav Gupta : On the direct cost specifically going to about Rs. 1,000 crores. Yes, a bunch of doctors have joined, which are part of the system that was basically a prebuilt up for the new capacities coming in. But a lot of them will also join alongside when the capacities get commissioned phase by phase. They will go in tandem to the revenues kicking in as well.

Abhay Soi: Yes. But there's no negative impact of that.

Yogesh Sareen : Shaleen, also as we progress into the coming quarters, the share of the institutional beds will come down as far as these hospitals are concerned . Because you know that our strategy is to fill up the beds and then distil the payor mix. So, this ratio should improve . The institutional share has gone up because of the new beds, etc. and as we speak, the occupancies in hospital like Dwarka, which started only 12 months back, is already 81-82%. It's because we said let's fill it up with whatever demand we have . Once the payor mix ratio improves, the direct cost percentage will also come down.

Shaleen Kumar: Sure, sir. Because typically when we move from Q1 to Q2,

there is a sequential reasonable jump of 5%, 6% happens on the revenue. It may be more or less depending upon ramp -up of the new facility. I hope it's more. But we should not expect direct cost to also move up in the same line, right? It may not move up, right .

Yogesh Sareen: Yes, as a percentage, this should be moderate.

Abhay Soi: You are right, your hypothesis is correct.

Shaleen Kumar: Correct. All right, sir. So, we should expect a decent margin expansion then. This Rs. 12-crore donation , is it part of your CSR expense? Or is it outside of that?

Yogesh Sareen: No. CSR expense is generally incurred by the companies and not by trusts . This is a spend, which has been incurred by the Devki Devi Foundation (DDF) and done on

the recommendation of its governing body . Nevertheless, it's a one -time donation that has been made from the DDF society.

Shaleen Kumar: So, this is a part of our indirect cost?

Yogesh Sareen: It's part of the indirect overhead s, yes, at this point of time.

Abhay Soi: It's one -time.

Shaleen Kumar: One-time. So, net-net, yes, so I just wanted to ask because it is CSR then it's not one-time. But it's ex of CSR, then it's one time .

Yogesh Sareen: In a charitable society, there is no CSR. CSR only applies to companies . That money has been spent by the DDF society. So that's the reason why it is a donation and not CSR.

Shaleen Kumar: Basically, when I think about it, let's say, everything remains same next quarter, then I should add back this Rs. 12 crores to the revenue, right, to the EBITDA.

Abhay Soi: That's right. Rs. 12 crores will never happen again, yes.

Shaleen Kumar: Yes. So yes, so that's my Rs. 12 crore , which I can add back to my EBITDA, right, next quarter?

Abhay Soi: That's right.

Shaleen Kumar: And the last bit, any colour if you can give us on the 2Q? How has it been going for

you guys, right? How is the season on the vector -borne, etc? Typically, it's a strong season for us.

Abhay Soi: I am going to avoid giving any forward -looking statements. I will just tell you that normally the vector season happens towards the end of monsoon. This year the monsoon has been long. So only at the end of monsoon, we will start seeing it. There has to be stagnation of water for vector -borne diseases to kind of set in. It normally happens when the monsoon starts receding. But right now, the monsoon has been sort of continuing.

I think the important thing, to look at, is that at the existing hospitals, excluding any additions that we did over the last 15 months, there's a 7% growth in ARPOB and there is a significant increase in revenue and EBITDA. And occupancy has certainly gone up to 80% for the base hospitals.

Second thing to look at is the two hospitals (Lucknow and Nagpur), which were added 15 months ago, both of these will have a better sense of maturity about them because they have been with us for longer. Both of these have also contributed to increase in the occupancies. And in spite of both of them, you still see ARPOB move up by maybe 5% and occupancies go up by a couple of percentage points.

And then if I add the two hospitals, one we really acquired in October -November '24 (Noida), the other was a Greenfield (Dwarka) in July '24 that we started. In spite of those, we see flat occupancies on an overall basis. So, even the newer facilities haven't really dragged down our operations or our numbers significantly. And yet they provide us an opportunity as they mature.

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Shaleen Kumar: Got it. Absolutely, I can see that. Thank you so much, guys. I will get back in the queue. Thank you.

Moderator: Thank you. Next question is from the line of Raj at Shah from PGIM India Asset Management Private Limited.

Rajat Shah: Yes. So, I just wanted to know regarding the growth trajectory of the acquired hospital on revenue and EBITD

A basis. How has it been post the acquisition?

Yogesh Sareen: You are talking about Nagpur and Lucknow?

Rajat Shah: Including Lucknow, Nagpur and also Noida, Dwarka.

Yogesh Sareen: Yes. So, Lucknow has been doing very well. On a Y-o-Y basis, the revenue growth is 97% and EBITDA growth is 191%. At Nagpur, we have grown the revenues by 27% Y-o-Y and EBITDA growth is also in the same range. Noida has been a bit subdued, with around 14% Y-o-Y growth in revenue and EBITDA growth is 32% Y-o-Y. Again, this is based on the numbers that we have of the last year. These are not our numbers, but basically the numbers that we could get from the erstwhile management.

Rajat Shah: Okay, perfect. That's it from my side.

Moderator: Thank you. Next question is from the line of Neha Manpuria from Bank of America Securities India Limited.

Neha Manpuria: First question, Abhay, when does Nanavati's new block fully become operational? You said that we are currently operationalizing one of the floors and I think the basement. When can we see the unit becoming, let's say, fully operational from a bed perspective?

Abhay Soi: The basement is also used for onco services, nuclear medicine, etc. So, we will be starting over the next few weeks with a couple of floors. And then by October - November '25, we should be starting the rest of the beds, including operation theatres, etc.

Neha Manpuria: Okay. Understood. And my second question is, if I look at the New Units numbers that you have given for fourth quarter versus this quarter, it seems like it's broadly similar to what we did in the fourth quarter. I thought that Dwarka had achieved breakeven last quarter, and that should be positive. So, is it just a seasonality thing? Because I thought the seasonality isn't as stark quarter-on-quarter from fourth to first. And I would have expected that, or should I assume till the time we do not see onco come in, in Dwarka, you would not really see that big ramp-up happening?

Abhay Soi: No, on the contrary. You are right that there is seasonality. But for a new unit, there is no seasonality because your growth momentum itself takes care of the seasonality and beyond. We have seen very high ramp-up of occupancies and everything else at Dwarka. So no, I do not think we are waiting for anything to come in. In fact, we are already planning brownfield expansion at Dwarka because we are running out of capacity over there.

Yogesh Sareen: Dwarka's revenue growth is 24% quarter-on-quarter, which means that we have grown the revenues from the Q4 FY '25 to Q1 FY '26 by 24%. EBITDA is also up by Rs. 7-8 crores from Q4.

Abhay Soi: Yes. And I think we have passed the breakeven and any seasonality questions over there. We are already at capacity run out status.

Neha Manpuria: Okay. And so, in which case, then I am wondering if Noida could have done better this quarter, but because I would have assumed that you would see a much larger improvement given the ramp-up in Dwarka and also Noida starting to improve performance. So just trying to get a sense, is Noida on track with what we are expecting? Or should I expect an acceleration as we go through the rest of this year?

Abhay Soi: You will certainly see acceleration in Noida as well. Like I had mentioned, we acquired it in October-November '24. The first couple of quarters are always the most crucial because you are kind of integrating it into your IT systems, etc. We changed the name of the company when we bought it from the NCLT. In this particular case, all the licenses, including transplant licenses, blood bank licenses, etc., had to be reapplied.

So those have been coming through gradually.

So, literally, we have been operating a little bit with our hands tied behind our back.

And then again, this was a company which has been in liquidation for many years.

The equipment

is out of life. We have ordered all the equipment, which have been coming in phases. All the building blocks have been put in place.

So typically, in any acquisition that you do of this nature, the second year is better than the first year. The first quarter will always be the weaker quarter, right? It will be the weakest quarter because that's perhaps the time you have been able to implement the least number of changes that you wish to because whether it's equipment, management, doctors, etc., the onboarding does take 3 to 6 months.

And like I said, in this particular case, even though it's an acquisition that we did, but somewhere it was a greenfield because we lost all the licenses. And then we had to get them all renewed in the new company. But yes, most certainly, what's going to happen is that going forward, you are going to have a snowballing effect. In the quarters to come, you should see significant upside there.

Neha Manpuria: So, it's fair to assume that all of the ground work that needed to be done for Noida in terms of licensing, equipment, etc., will be done. I mean you should start seeing that from the subsequent quarters.

Abhay Soi: It is already done. But when you order the equipment, the supply chain itself comes from overseas. There is a 6-month process from the day you indent it for you to get some of these equipment in place.

Neha Manpuria: Understood. Yogesh, sir, what did you mention the Noida EBITDA at? Sorry, I missed that number.

Yogesh Sareen: For Noida, Y-o-Y growth in operating EBITDA is 32% and in revenue is 14%.

Neha Manpuria: And would you have the absolute number, sir, for the EBITDA, sorry?

Yogesh Sareen: It will be around Rs. 24 crore for the quarter.

Neha Manpuria: Okay. Thank you very much. That's helpful, sir. Thank you.

Moderator: Thank you. Next question is from the line of Kunal Dhamesha from Macquarie Capital Securities (India) Pvt. Ltd.

Kunal Dhamesha: Just continuing on the Dwarka and Noida thing. I think the total EBITDA contribution is around Rs. 23 crore or something?

Yogesh Sareen: It is Rs. 37 crore.

Kunal Dhamesha: Total contribution for Dwarka and Noida, which we have mentioned, the new hospitals, it is Rs. 27 crore, of which we are saying Rs. 24 crore is Noida, right?

Yogesh Sareen: No. You will also need to consider that the unit EBITDA will be 2.5% higher because we also cross-charge the HO cost when we report numbers to the investors. But when you see a unit performance, then it will be 2.5% higher in terms of EBITDA margin. 2.5% of the revenue is allocated to units as HO cost.

Kunal Dhamesha: So, this Rs. 27 crore is unallocated?

Yogesh Sareen: Rs. 24 crore that I mentioned is the unit EBITDA.

Kunal Dhamesha: And which would come down to what, around Rs. 20 crores, because Dwarka is...

Yogesh Sareen: It will come down by Rs. 2 crores.

Kunal Dhamesha: Because Dwarka, you said Rs. 7-8 crore, right?

Yogesh Sareen: That's right. Rs. 27 crore EBITDA is after 2.5% of Rs. 231 crore being allocated to these units. If you take out this HO cost, you will add Rs. 5 crore to this number. So, Rs. 32 crore is the combined unit EBITDA for Dwarka and Noida.

Kunal Dhamesha: Sure. Second question on the ARPOB. You suggested that including Lucknow and Nagpur, the ARPOB is around, what, 4.9% growth, right, excluding Noida and Dwarka. And then you are saying that the 3,500 bed that we originally had at 7%, that

translates to around 7% ARPOB degrowth for Lucknow and Nagpur. Is that a correct math?

Yogesh Sareen: No. I think there's some confusion. So first of all, let's take the existing hospital s, excluding even Nagpur and Lucknow, the ARPOB growth is 7% . These are the hospital s which were operation al in December '23.

Kunal Dhamesha: Correct.

Yogesh Sareen: So, factoring in Nagpur and Lucknow, which are the hospital s t

hat we acquired in

February '24 and March '24, then the ARPOB growth is 5% . If I factor in Dwarka as

well as Noida, then the ARPOB growth is 1.2%.

Kunal Dhamesha: Correct. So, I am saying that for 3,500 -bed to d o 7% ARPOB growth, the ARPOB should have been degrown in Lucknow and Nagpur, right? Then only 7% comes down to 5%.

Yogesh Sareen: No, I am not sure what you are saying because there's no Y -o-Y number for the Dwarka . There's no number for last quarter in Dwarka.

Kunal Dhamesha: No, no. Just exclude Dwarka and this thing .

Abhay Soi: There is no degrowth in ARPOB in any of th e hospitals . There's only increase in ARPOB.

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Kunal Dhamesha: But then numbers are not basically connecting. How was that 7% ? So, 3,500 beds, you are saying 7% ARPOB, right? We include Lucknow and Nagpur, which is around 600 beds, our presentation says that 4.9% growth or 5%, let's say, right? So then 7% is coming down to 5% by addition of Lucknow and Nagpur in the mix, which means -

Abhay Soi: It is a verage revenue per "occupied bed ".

Yogesh Sareen: You are getting confused. Let me just give you the numbers. In the hospitals, which were operating until December '23, the occupied beds were 2,73 2. If we factor in the two hospitals, which is Nagpur and Lucknow, the occupied beds were 3,117 . And if we take Dwarka and No ida, the occupied beds were 3,556 . Now you can do the math .

Kunal Dhamesha: I will maybe connect offline on this.

Yogesh Sareen: The math will work out.

Abhay Soi: Please get your formula right. I have seen your formula and in spite of us saying that occup ied bed days have gone up and ALOS has gone down, I believe th at statements have been made in your formula that footfalls have come down for IPD. Theoretically, that's not even possible.

Kunal Dhamesha: Sure. I will do that, sir. Second question is, we have seen significant increase in institutional business, right? Even, let's say, because last year same quarter, we would not have Noida or Dwarka, right? But there is a significant increase. And I assume that Lucknow and Nagpur are not the centre s for a lot of institutional patients, right?

Abhay Soi: No, we started institution al in Bombay also in view of the new beds coming in. We started it in Nagpur as well , where it was not there prior to our takeover. It was started last year , and so it is in Lucknow.

Kunal Dhamesha: Okay. But then existing beds, on the 3,500 beds, what would be your view? Is it more or less stable in terms of bed days from institutional or ?

Yogesh Sareen: For example, Mohali, we are adding 160 beds. You cannot take institutional at that point of time . And so, we have taken institutional in the last quarter and the occupancy has already gone up . We are basically preparing for the new beds coming in .

Abhay Soi : So, it i s in Mumbai, because it takes you six months to empanel yourself . Mumbai, for example, erstwhile there was no institutional business. Now we have e mpanelled for institutional.

Kunal Dhamesha: So, wherever the new expansion is coming, it is more strategic?

Abhay Soi: That's right.

Kunal Dhamesha: Great, sir. Thank you and all the best.

Moderator: Thank you. Next question is from the line of Mohammed Patel from Edelweiss Financial Services Ltd .

Mohammed Patel: I have a question on ARPOB growth. It has been flat overall. So how should we think of overall ARPOB in the near term?

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Abhay Soi: Well, it has grown . For existing hospitals , it has grown by 7%. If you add the two hospitals that we acquired 15 months ago, then it has grown by 5%. If you look at all of the hospitals, then it looks flat. If you are going to start a new hospital or you acquire a new hospital, the ARPOB of that particular hospital is going to be lower . Therefore, it is going to b ring the average down. Otherwise, for the existing hospital s, ARPOB

and occupancy have gone up significantly .

Mohammed Patel: So, the overall ARPOB growth will take some time to reflect, right?

Abhay Soi: Yes. When you acquire a new hospital, you acquire it because it has lower ARPOB, and you acquire it at that price. And the whole idea is to increase the occupancy and the ARPOB of that hospital. Going forward, you will see ARPOB move up.

Mohammed Patel: Okay. So eventually, after a few quarters, the overall ARPOB growth will again start inching towards the high single digit?

Abhay Soi: That's right.

Mohammed Patel: Okay. And my second question is should we...

Abhay Soi: Having said that, it also depends on where you are acquiring. We buy for ROCE. We may acquire something in a tier-2 city, where the ARPOB is, let's say, half of the present ARPOB . The ARPOB of that place may never go up to our existing cluster ARPOB. The fact is , we have to figure out what ROCE are we buying it at. If we are buying it at a 20 -25% ROCE, or if we believe we can get it to a 20-25% ROCE in spite of the hospital having a lower impact on our overall ARPOB, we will still acquire it, right? I think the cadence you are looking at is not correct. Looking at ARPOB by itself or margin by itself, of any capacity that you are acquiring or you are starting today, is not correct. What we have to see is do we have superior return on capital profile over there or not.

Let's say, if we get an opportunity to buy a chain of hospitals, which operates at less than half of the overall ARPOB we are operating at, and we are able to buy it at 25% ROCE, should we or shouldn't we acquire it ? We should, right? What it would do is bring down our overall ARPOB and then we may never be able to climb up to the same present levels of ARPOB, so be it.

Mohammed Patel: Yes, yes, I get your point. My second question is, so should we expect EBITDA margin improvements in FY '26?

Abhay Soi: We do expect improvement. But having said that, EBITDA margin is not the correct metric to assess . Our international business is growing by 32% . It is supposedly higher margin business in value terms, but it's a lower margin business in percentage terms . When you do robotics , transplants or any of the high -complexity surgical work , it gives you less margin s in percentage terms, but it gives you more in value terms. Please consider EBITDA per bed as the right cadence and not EBITDA margin. On one end, because of better occupancy, higher utilization , etc. your margins will increase. On the other end, because you are going for a superior payor mix and a superior clinical mix, your margins should decrease . But your EBITDA per bed and ROCE will increase.

Moderator: Thank you. Next question is from the line of Sujit Shah from SK Enterprises.

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Sujit Shah: Yes. Firstly, I want to appreciate the entire Max Healthcare team. At this scale expanding so rapidly while maintaining profitability is not easy task. And my all question is already asked and answered. So not any question I have to ask.

Abhay Soi: Thank you.

Moderator: Thank you very much. Ladies and gentlemen, next question is from the line of Rajesh Gajra from Informist Media Pvt. Ltd. Please go ahead.

Rajesh Gajra: Yes. Earlier in the call, you mentioned that at the end of FY '26, the net debt will be and you gave some figure for that. I was not able to catch that number. Can you please repeat that, the net debt level at the end of FY '26?

Abhay Soi: What Yogesh mentioned is that between now and end of the year, your total debt for project purposes may go up by another Rs. 400-500 crores.

Rajesh Gajra: Okay. Alright. That's it. Thank you very much.

Moderator: Thank you. As there are no further questions, I will now hand the conference over to the management for closing comments.

Abhay Soi: Thank you once again. We appreciate all your time and look forward to connecting

with you next quarter. Thank you so much.

Disclaimer: This is a transcription and may contain transcription errors. The transcript has been edited for clarity. The Company takes no responsibility for such errors, although an effort has been made to ensure a high level of accuracy.

Call: Nov 2025

November 21, 2025

Listing Department ,
National Stock Exchange of India Limited
Exchange Plaza, Plot C -1, Block G,
Bandra Kurla Complex, Bandra (E),
Mumbai – 400 051

Symbol: MAXHEALTH Listing Department,
BSE Limited
Phiroze Jeejeebhoy Towers,
Dalal Street,
Mumbai – 400 001

Scrip Code: 543220

Sub.: Transcript of Earnings Call held on November 17, 2025

Ref.: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Sir / Madam,

Please find enclosed copy of transcript of earnings conference call, organised on November 17, 2025, on financial results of the Company for the quarter and half year ended September 30, 2025.

The said transcript is also available on the website of the Company at www.maxhealthcare.in/financials#earnings-call.

Kindly take the same on record.

Thanking you

Yours truly,
For Max Healthcare Institute Limited

Dhiraj Aroraa
SVP - Company Secretary and Compliance Officer

Encl.: As above

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Max Health care Institute Limited
Q2 & H1 FY'26 Earnings Conference Call
November 17, 2025

Moderator: Ladies and gentlemen, good day, and welcome to Max Healthcare Institute Limited's Earnings Conference Call. Please note that this conference is being recorded.

I now hand the conference over to Mr. Suraj Digawalekar from CDR India. Thank you, and over to you, Suraj.

Suraj Digawalekar : Thank you, Michelle. Good morning, everyone, and thank you for joining us on Max Healthcare's Q2 and H1 FY'26 Earnings Conference Call. We have with us Mr.

Abhay Soi, Chairman and Managing Director ; Mr. Yogesh Sareen, Senior Director and Chief Financial Officer; and Mr. Keshav Gupta, Senior Director – Growth, M&A and Business Planning. We will begin the call with opening remarks from the management, following which, we will have the forum open for an interactive Q&A session.

Before we begin, I would like to point out that some statements made in today's discussion may be forward-looking in nature and a disclaimer to this effect has been included in the earnings presentation shared with you earlier.

I would now like to invite Abhay to make his opening remarks. Thank you, and over to you, Abhay.

Abhay Soi : Good morning, everyone, and thank you for joining us on Max Healthcare's Q2 and H1 FY' 26 Earnings Call.

We are pleased to report that the Network has maintained its strong growth momentum through the first half of FY'26 . For the second quarter, revenue grew by 21% year -on-year, while operating EBITDA increased by 23%, thereby extending our track record to 20 consecutive quarters of consistent growth. Importantly, this performance reflects the continued strength of our core operations , with Existing Units achieving revenue growth of 14% and EBITDA growth of 19%.

Building on this strong performance, we continue d to enhance our capacity and service offerings through brownfield expansions. At Max Mohali, the new 160 -bed

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brownfield tower has been commissioned, including the additional radiation oncology program. At Nanavati -Max, the new 268 -bed brownfield tower is set to be commissioned this week , and at Max Smart, the new 400 -bed brownfield tower will be ready for commissioning within the next 30 days.

Alongside our expansion efforts, we also streamlined our corporate structure. Divestment of hospitals in Chittha and Anoopshahr was completed in September 2025. Further, the merger of our two wholly owned subsidiaries was approved by the Hon'ble NCLT, Chandigarh Bench with an appointed date of October 5, 2024, which has resulted in a one-time benefit of INR 149 crore during the quarter.

After long last, CGHS has revised the prices effective October 13th. While some of it is yet to kick in, we expect a favourable impact of over INR 200 crore once fully implemented. Further, with regard to insurance renewals, there was an impasse with certain players leading to temporary stoppage of cashless facilities. The same has been resolved.

Now coming to the Q2 performance highlights. Please note that the term "Existing Units" hereafter refers to the Network facilities that were operational prior to Q3 FY'25 , while Jaypee Hospital s are categorized as "New Units".

1. Average occupancy for the Network stood at 77% compared to 79% in Q2 last year and 76% in the trailing quarter. Existing Units reported occupancy levels of over 79% .
2. Occupied bed days (OBDs) were up by 19% year -on-year and 4% quarter -on-quarter.
3. Average Revenue Per Occupied Bed (ARPOB) for the quarter was INR 77,300, registering 1% growth year -on-year. For the Existing Units, like -for-like ARPOB grew by 3% year -on-year and remained stable sequentially.
4. Network gross revenue stood at INR 2,692 crore compared to INR 2,228 crore in Q2 last year and INR 2,574 crore in the previous quarter. This reflects an increase of 21% year -on-year and 5% versus trailing quarter. Of this, New Units contributed INR 144 crore , while the Existing Units delivered a 14% like -for-like revenue growth, driven by an increase of 11% in OBDs and 3% in ARPOB.
5. Digital revenue from online marketing activities, web -based appointments and digital lead management rose to INR 803 crore , accounting for approximately

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30% of the overall revenue. Website traffic crossed 79 lakh sessions during the quarter, growing by 53% year -on-year and 15% quarter -on-quarter.

6. International patient revenue reached INR 231 crore , registering a growth of 25% year-on-year and 11% quarter -on-quarter.
7. Network operating EBITDA stood at INR 694 crore , reflecting a growth of 23% year-on-year and 13% quarter -on-quarter. Of this , New Units contributed INR 23 crore , while the Existing Units delivered 19% like -for-like growth in EBITDA.
8. Network operating EBITDA margin was 26.9% for the quarter. Existing Units reported an EBITDA margin of 27.5%, higher by 90 basis points year -on-year and 200 basis points sequentially.
9. Annualized EBITDA per bed for the Network stood at INR 73 lakhs . Like-for-like EBITDA per bed for Existing Units was INR 76 lakhs, reflecting a 7% year -on-year growth.
10. Profit after tax (PAT) for the Network was INR 554 crore , against INR 349 crore in Q2 last year and INR 345 crore in the previous quarter. This includes one-time favourable tax impact of INR 149 crore pursuant to the merger of Crosslay Remedies Ltd. and Jaypee Healthcare Ltd. Excluding this impact, PAT stood at INR 406 crore , reflecting 16% year -on-year growth.
11. The Network generated free cash flows of INR 291 crore during the quarter.

There was a buildup of accounts receivable in the institutional segment, which impacted the cash flow from operations. INR 456 crore was deployed towards ongoing capacity expansion projects and facility upgrades at new units, while INR 146 crore was distributed

as dividend. As a result, Net Debt for the Network stood at INR 2,067 crore compared to INR 1,755 crore at the end of June 2025.

12. Continuing our effort to support the local communities, we provided free treatment to approximately 44,000 patients from economically weaker sections of the society, worth INR 61 crore at hospital tariff.

13. Both our strategic business units continued to deliver steady growth in revenue and profitability:

- Max@Home reported revenue of INR 63 crore, reflecting a robust 20% year-on-year growth. It offers 15 specialized service lines across 15 cities with over 60% repeat transactions.

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- Max Lab reported a revenue of INR 54 crore, reflecting a 16% year-on-year growth. It provides services in over 60 cities and served more than 6 lakh patients during the quarter.

14. Now coming to the status of our expansion projects.

- Max Lucknow – The current capacity of the hospital stands at 413 beds, and we expect this to increase to 550 beds by end of this financial year. The onco radiation program, including PET-CT, will be launched in the next two weeks.

- 501 beds at Sector 56, Gurgaon – The project is progressing well at site, while GRAP had some impact on the pace of work.

- 100 beds at Max Nagpur – We have received environmental clearance and are awaiting the Consent-to-Establish (CTE). We expect to complete the project within 24 months, post receipt of the CTE.

- 397 beds at Patparganj – All approvals have been received and barricading work is complete. The project is expected to be completed by FY '28.

- 550 beds at Max Vikrant (Saket) – The project work will start immediately upon commissioning of the 400 beds at Max Smart. We expect to complete this project within 40 months.

- 400 beds at Zirakpur, Mohali – Project is continuing faster than anticipated and is now expected to be completed by end of calendar year 2027.

- 140 beds at Max Vaishali – Demolition of the existing structure is complete and approvals are in process. We expect to complete this project in 24 months, post receipt of the requisite approvals.

- 500 beds at Thane - Master plans for the site have been finalized by the partner. The project is expected to be delivered in 42 months.

- 250 beds at Pitampura, Delhi – The partner is expecting the building plan approvals during the quarter, post which the project is expected to take 36 months.

And finally, moving on to the overview of the company's performance for the first half of financial year 2026:

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15. Network gross revenue stood at INR 5,266 crore, reflecting a growth of 24% year-on-year. Of this, New Units contributed INR 278 crore, while the Existing Units delivered a 17% like-for-like revenue growth.

16. Overall Network operating EBITDA grew by 23% year-on-year to INR 1,308 crore, translating to a margin of 25.9% and EBITDA per bed of INR 71 lakhs. While Existing Units delivered an EBITDA margin of 26.5% with EBITDA per bed of INR 74 lakhs.

17. In the first half, we generated INR 679 crore of free cash flow from operations, after interest, tax, working capital changes and routine capex. INR 891 crore was deployed towards ongoing expansion projects and facility upgrades at new units, INR 131 crore was spent towards land purchase for brownfield expansion at Max Vaishali, and INR 146 crore was distributed as dividend.

With this, we open the floor for any questions that you may have. Thank you.

Moderator

Moderator: Thank you very much sir. We will now begin the question-and-answer session. The

first question is from the line of Damayanti Kerai from HSBC and Capital Markets (India) Private Limited .

Damayanti Kerai: Hi. Thank you for the opportunity. My first question is to Abhay. So what we have heard, there has been some senior doctor departure from your team in NCR hospitals. So first, have you rehired

the required doctors? And what kind of impact was there, if any?

Abhay Soi: Yes, that level of iteration is normal. You have certain teams always sort of move out, and new teams come in almost immediately. And that is what has happened.

So, we do not expect any impact.

Damayanti Kerai: So approximately, like what is the attrition rate in your doctor's team, which you said is in line with the normal trend?

Abhay Soi: Less than 1% of the doctors.

Keshav Gupta : When the new facility comes , another 0.5% happens.

Abhay Soi: Yes, so you could say, between 1% to 1.5% at best. I mean anecdotally, if two or three doctors' leave or certain doctors leave, it does not have any major impact . We have got close to 6,000 doctors , right?

Damayanti Kerai: Okay. Okay. So no impact as such and then it is a normal trend. Okay.

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Abhay Soi: Absolutely.

Damayanti Kerai: Okay. My second question is on your settlement with issues on the insurance company. So has like all the matters now been put to rest and do you have any further contracts which are coming up for negotiations and there might be some updates there?

Yogesh Sareen: So Damayanti, first of all, whatever issues we had in terms of these three insurance companies , that we have already sorted out. And not only sorted out for now, but we are also sorted out for future . In the sense that when we concluded this arrangement , we also agreed the next revision with them . There are other companies , which are also up for renewal this time, including GIPSA . Those negotiations are underway, I would say.

Abhay Soi: But this sort of impasse happens once every few years, and we do not expect a repeat of this.

Damayanti Kerai: Okay. So -- but we have not heard such instances in recent past. So that is what I was a bit curious whether things are now put in place for permanently and then we might not see similar situations coming up.

Abhay Soi: We have had suspension of cashless facilities in the past too. It happened a few years ago as well.

Yogesh Sareen: We have had issues with GIPSA also, at one point of time.

Keshav Gupta : And everyone has it. These are resolved and cashless is active.

Damayanti Kerai: Okay. Okay. And one last question. You said CGHS revision when it happens fully like for your portfolio exposure, that will be additional INR 200 crore kind of revenue which will come in?

Abhay Soi: That is right.

Damayanti Kerai: Okay. And approximately, like in your portfolio, what has been the tariff increase after this division comes into effect very broadly?

Yogesh Sareen: So Damayanti, we cannot give you the numbers of what the increase is because obviously, it is a negotiation with the insurance companies. But I think the CGHS part is clear, which is that around 10% of the CGHS revenue will increase . We have around INR 2,000 crore of revenue coming in from CGHS, and so we will have a 10% jump there with this additional revenue.

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Abhay Soi: But this is a net increase because for certain medicines, etc. they have also capped the rates.

Damayanti Kerai: Thank you. I will get back in the queue.

Moderator: Thank you. The next question is from the line of Tushar Manudhane from Motilal Oswal.

Tushar Manudhane: So just on the international patient side where the growth has been quite robust even this quarter, almost 25%. So if you could just sort of help break down into volume and realization growth?

Yogesh Sareen: Most of it was volume growth. I do not think there is any increase in the ARPOB in

that segment. So, you can assume that 25% is entirely volume growth.

Tushar Manudhane: And more so, sort of -- if you could also elaborate in terms of the geographies, like in terms of diversification.

Abhay Soi: Broadly, the geographies remain the same. Like we mentioned in the past, it is Middle East, Eastern Europe, African cluster, etc.

We do not want to give any specific countries because it is privileged information.

Tushar Manudhane: Got it, sir. And lastly on this clarity, the MSSH Saket 400 bed hospital in terms of the time line for the commissioning? Or is -- I missed -- I probably missed it on the opening remarks, if you have already highlighted.

Abhay Soi: Currently, there is deep cleaning happening at Nanavati -Max, and we are expecting it to be commissioned within this week. Max Smart will be commissioned within the next 30 days.

Tushar Manudhane: Got it, sir. And so how do you see the scale up now that post commissioning, not literally in terms of numbers, but in terms of the time line, in terms of ramp-up of occupancy or maybe the EBITDA breakeven?

Abhay Soi: EBITDA breakeven is almost immediate. We have also stated that in the past, for brownfields, we do not have any EBITDA drags necessarily.

Tushar Manudhane: Understood. Thanks, that is it from my side.

Moderator: Thank you. We take the next question from the line of Andrey Purushottam from Cogito Advisors LLP.

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Andrey Purushottam: Hi. Congratulations for the consistent set of good results. I had just 2 questions. One is that your profit growth after correcting is 16% year-on-year which lags a little behind EBITDA and revenue growth. Is there any insight you can provide to us on this?

Yogesh Sareen: You would have seen that the finance costs have gone up, and depreciation has gone up as well because we have added new hospitals in Q3 last year. And their EBITDA margins are lower. We have also taken borrowings to fund these acquisitions. As their EBITDA growth comes in level with the other hospitals, you will find that the PAT growth number will also start to follow the same pattern.

Abhay Soi: Yes. Last year, Jaypee was not there, right? So that is about INR 2,000 crore of acquisition that we have done.

Andrey Purushottam: Okay. My second question was, I wanted your view on ARPOB. Are you getting a feeling on what is the internal discussion in the company that the ARPOB is kind of -- the rate of growth of ARPOB is becoming slower, at least that is what optically it looks like? I first want to know whether this assessment from our side is something that you share. And if so, is there any discussion on what to do about ARPOB going forward?

Yogesh Sareen: ARPOB growth is 3% y-o-y. But it also includes the impact of the new hospitals that we acquired and started last year. If we take that out and consider all the existing hospitals -- existing hospitals mean the one that were operating before Q4 FY24 -- then the ARPOB growth is around 7% y-o-y. It is basically the impact of the mix that is bringing this number down since the share of the new hospitals (Lucknow, Nagpur and Dwarka) has gone up in this quarter compared to Q2 last year.

Andrey Purushottam: So, thank you once again and congratulations for a good set of results. And I also like the disclosures that you make in your presentation, they are very comprehensive. So, we appreciate that as investors.

Abhay Soi: Thank you.

Moderator: Thank you. The next question is from the line of Amit Thawani from Clear Blue Capital Advisors LLP.

Amit Thawani: My first question is on insurance. I think you have spoken about the issues in insurance that we had this quarter. And because of that, our growth in insurance has actually been one of the lowest amongst all our peers. And then you have mentioned that the cashless has been reinstated. Can you confirm that for December quarter,

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like we have already -- we have done with half of November and October, are we back on track? I mean, with industry level growth in insurance business?

Abhay Soi: I think you need to look at it on an overall basis. Insurance growth may have come down, but your self-pay growth has gone up. If you are not pr

avoiding cashless, it is

not necessary that you lose the patient. The patient gets converted to self-pay, because he can get it reimbursed later. He does not necessarily change the hospital. That's why you need to look at what is the overall occupancy between cash and insurance, and what was the impact? You will not see any major impact on the occupancy of the hospital.

Amit Thawani: Right. Just by -- I mean, my only worry is that are we doing more institutional business because the insurance patient is not coming, but that does not seem to be the case.

Abhay Soi: Yes, that's not the case. You have seen that the self-pay business share has moved up, and you will see that the occupancy has moved between cash and insurance, because that is fungible, right?

Amit Thawani: Right. That is fungible.

Yogesh Sareen: If you come to me as a doctor, you will use your insurance while you can. If you cannot, then you will just sort of pay cash.

Amit Thawani: Correct.

Abhay Soi: And if you do believe there was an impact, then the point is just that going forward, you will see the benefit of it coming back now that it is all back to normal.

Amit Thawani: Got it. Got it. Our cash flows were weak. And as you rightly mentioned, it is because of our institution al business. Just wondering because now we are coming up with Nanavati and Saket, do we expect some kind of further deterioration because of this?

Abhay Soi: No, no. Institutional payments have historically been lumpy. They do not necessarily come on time. There are certain months when there is a build-up because various PSUs, whether it is ECHS or CGHS, run out of budgets. Then they write to the ministries, get their budget replenished and pay. They have been lumpy in the past as well. So, there is nothing to be concerned about.

Amit Thawani: So we do not expect further deterioration in the cash flows?

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Abhay Soi: No, not at all. I mean, even this is not a deterioration. It is a one-time thing. In the next quarter, you will probably see a large amount of cash flows coming through.

Amit Thawani: Got it.

Abhay Soi: If you put a line through the year, it will pretty much be the same number of days. At a given point of time, lumpiness happens in institutional payments.

Amit Thawani: Got it. I understood. My last question is on Mohali. So we added 160 beds, but the presentation shows an operational increase of 53 beds only. So is it that the next 107 beds will move into operational in Q3? And the Mohali is an O&M agreement with the government. So -- I mean, because in O&M, the entire revenue does not get recognized. So does that mean that these beds really do not have a material impact on our P&L?

Abhay Soi: No, no, two things. One is, for Mohali, it is a revenue share arrangement. So, let us say, 5% of our revenues are given to the government.

Amit Thawani: Okay.

Abhay Soi: Second is, as far as the beds are concerned, the entire tower has 160 beds. Now you do not wait for the last bed and the last light to be put to commission the whole facility. For example, of 160 beds, if the first 4 floors are ready, then we start the first 4 floors. And the rest of the floors as and when they come online, you start them.

Amit Thawani: So in Q3, how much will get operational in Mohali?

Yogesh Sareen: As we speak, 90 beds are operational. Out of the 160 beds, 53 were made operational from August 1st. That means the impact in the quarter is around 35 beds because you will have to consider the weighted average. And that is the reason why the operational capacity will be lower. We are obviously not short of beds there. As we see the occupancy ramp up, we will open more beds. But in quarter 3, we will have all the beds ready. We will open them as we see the demand there.

Amit Thawani: Okay, got it. Got it. And in Lucknow and Noida, we are

seeing a lot of interest from a

lot of our peers as well. They are coming out with some big capacities. Any comments on the competitive intensity out there?

Abhay Soi: Not really. The new Noida capacity, I mean, it will not come overnight. It has been under construction for the last 4 years. Any competitor coming up with any capacity has been constructing for the last 4 to 5 years. In fact, we purchased Jaypee later.

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That was like it fell from the sky and we purchased it, right? So, I mean that was a cold start.

Amit Thawani: Okay. My last question, on the ARPOB growth, Yogesh ji mentioned we are trending at about 6% now this quarter. Is that a fair comment for our existing units?

Yogesh Sareen: We are around 7% ARPOB increase if I take the existing units. When I say existing units, I mean units which were operating in Dec'24 or before.

Amit Thawani: So this December quarter is clocking a 7% on our pre-existing units?

Yogesh Sareen: Not December quarter, I am talking September quarter.

Abhay Soi: We have not given any guidance for Q3.

Yogesh Sareen: In the September quarter, the ARPOB increase was 3.3%. If I take out the impact of the new hospitals that we added in Q4 FY'24, then the ARPOB increase in the existing units is 6.8%.

Amit Thawani: Understood. Thank you so much.

Moderator: Thank you. The next question is from the line of Kritika Damani from Prospera Financial Solutions.

Kritika Damani: Thank you. Congratulations on a strong quarter. My first question is that you have an occupancy of over 75% and a consistent trend on the ARPOB because of the strategic push in your high-acuity care like the new oncology-focused hospital. Since the case mix naturally increases the average length of stay, how are you balancing its trade-off to maintain patient throughput at your already busy hospitals?

Abhay Soi: You do it by opening new capacities, improving patient services, reducing discharge times and such. Having said that, you have to keep in mind one thing. Unlike a hotel, when a hospital is full and there are people sitting in the emergency, I can not send them away by saying, my hotel is fully booked, please go somewhere else, right?

There will be a point of diminishing returns where patients will wait in emergency, wait for a bed sometimes for 6 hours or even 1-1.5 days. Then the complaints increase. You try to do your best, but we are also in the job of serving every patient who is coming over there. So, I think that is a very important differentiation.

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Kritika Damani: Okay. My second question is that for the first half of your FY '26, can you quantify what percentage of your total hospital revenue came from the international patients?

And what is the yield profile of this business? Like how does the ARPOB and the average length of stay for an international patient compared to that of the domestic patient portfolio?

Abhay Soi: It is about 9% of the revenues. And I think typical ARPOB is about 30% higher for international patients compared to our average.

Yogesh Sareen: ALOS is 1.2x.

Abhay Soi: 1.2x, because people who come for international visiting are coming from more acute diseases, right?

Kritika Damani: Right. And last question is that your asset light divisions like the Max Lab and Max@Home are posting excellent revenue growth. So my question is about the profitability and capital efficiency. For the first half FY '26, what is the consolidated EBITDA margin? And what is the long-term margin target for these segments as it scales?

Yogesh Sareen: That is already there in the investor release, right? So Max Lab reports an EBITDA margin of around 16%, right? And for the Max@Home business, the arm's length EBITDA would be around 20%. But if I take the global EBITDA, that will be probably more than the hospital EBITDA.

Abhay Soi: These are also sub-

scaled right now because, as you mentioned, we are growing at a fast pace. It is because we are investing for the growth.

Kritika Damani: All right. Thank you. I will get back in the queue.

Moderator: Thank you. The next question is from the line of Neha Manpuria from Bank of America.

Neha Manpuria: Yes. Thanks for taking my question. Abhay, last call, you had mentioned that in probably 6 months' time, we should start seeing traction in Noida with insurance empanelment. So is that on track? Should we start seeing that improvement in Noida, let us say, in the December, March quarter? And at what point do you see Noida getting to, let us say, mid 25% margins?

Yogesh Sareen: Noida is currently around 18%.

Abhay Soi: Insurance empanelment has already happened as far as Noida is concerned.

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Yogesh Sareen: Well, on Q-on-Q, you see that there is 8% growth in Noida, right? And the EBITDA margin has also grown compared to last quarter. We do expect that to continue. And if we pick up the revenue of this hospital since January of this year, every month has been higher than the previous month. So, we are seeing the traction. We hope that, you know, this will also come up to Network level.

Abhay Soi: I think it was not insurance empanelment. You know, it was licenses for various clinical programs, which had to come through.

Neha Manpuria: Yes, yes.

Abhay Soi: Such as transplant and so on and so forth. All that has come through. So now we are seeing traction with that.

Neha Manpuria: What is the occupancy in Noida currently then?

YS So it is around 64%.

Neha Manpuria: Okay. So this, therefore, should improve as we see these, I mean, as you have seen these new clinical programs getting added, right?

Abhay Soi: That is right.

Neha Manpuria: Okay. Understood. My second question is on Dwarka. What is the occupancy in Dwarka on our operational bed and what margins are Dwarka at currently?

Yogesh Sareen: Dwarka is 81% plus occupancy in the quarter. We have 285 beds operational now. It is a 300-bedded hospital, once we get the oncology block, then we will be able to vacate some onco day-care beds and move them to the onco block and have this hospital's capacity restored to 300 beds. EBITDA (Unit) margin will be around 15%, currently.

Neha Manpuria: Okay. Okay. Okay. Understood. So, I was just -- sorry.

Abhay Soi: The typical trajectory for a greenfield would be that you sort of ramp up the occupancy first, right?

Neha Manpuria: That is correct.

Abhay Soi: And then after that, you see expansion of margins.

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Neha Manpuria: Yes. So it would then -- in that case, given that we are at 80% occupancy already for Dwarka, Dwarka should -- I mean by next year, it should be in line with corporate average in terms of margins. Would that be a fair assumption?

Abhay Soi: Well, hopefully, that should be the case, yes. But do keep in mind, even oncology bunker is not operational over there right now, right?

Neha Manpuria: Okay. Okay.

Yogesh Sareen: Yes. It also has a very high level of institutional occupancy. So, we have to bring that down. Basically, the idea was first to fill up the hospital, and then start to distill the payor mix. I think we are at a stage where we have started to distill that now.

Abhay Soi: Yes. So, you are going to see higher ARPOBs and expansion of margins. You are going to see that happening through both patient mix and clinical mix, including higher share of oncology. Once the radiation oncology starts over there, you are also going to see us planning and implementing another 200 beds on that site. I think the idea is to do the brownfield. Like I said, we have already run out of capacity there.

Neha Manpuria: Yes. Understood. Okay. Because I was just wondering about the ARPOB growth of 3%, including all hospitals, except for Noida, right? That is your

existing ARPOB

growth that you mentioned. Given that, you know, I am assuming Lucknow and Nagpur should have also seen very strong ARPOB growth. The 3% number and the 7% that you mentioned for existing network, how should I read those two numbers?

Yogesh Sareen: By the way, Dwarka's ARPOB has come down. When we started it, we did not have any institutional business, right? The institutional business started in Q3 FY'25. So actually, the ARPOB has come down.

Neha Manpuria: All right. Okay. Understood. Okay, okay. That is helpful.

Abhay Soi: Because typically, when you do a greenfield, for you to get any institutional business, it takes you about 6 months to get empanelled.

Neha Manpuria: Okay. Yes, understood.

Abhay Soi: You start off immediately with high ARPOB, low occupancy since it is only self-pay business. You do not even have any insurance contracts in the first six months.

Neha Manpuria: Okay. I would assume that we will probably do more emergency cases, which then move to higher acuity?

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Abhay Soi : That is right. We started in July last year , so till the end of the year, it was high ARPOB simply because we did not have institutional or insurance empanelment .

The self-pay segment is high ARPOB but low occupancy, and the payor mix starts changing thereafter which drags your ARPOB down eventually .

Neha Manpuria: Okay. And the CGHS, you said INR 200 crore upon completion -- so once fully completed. So what is the duration by which this will fully get reflected? Would it be a phased manner in which we will see the realization of these price increases?

Abhay Soi: They have introduced a new category called super specialty hospitals. On the portal, they have not completed the codes for that. We are expecting that in maybe the next 15-20 days .

Neha Manpuria: Okay. Okay. So that is the only thing that is pending for this to be implemented?

Yogesh Sareen : Yes, it is partially implemented -- the super specialty will be implemented once they update their portal.

Neha Manpuria: Got it. Okay. Thank you so much.

Moderator: Thank you. The next question is from the line of Sumit Gupta from Centrum Broking.

Sumit Gupta: Hi, good afternoon. Thanks for taking my question. The first is like what was the growth on the ARPOB growth of the segments, be it international or institutional -- for the ARPOB growth of the segment basically be it cash or institutional mix, what was the ARPOB growth?

Yogesh Sareen : The walk-in business, both insurance and the self -pay, grew by around 8% to 10%, on a like -to-like basis. It is basically the impact of the new hospital s that got added into the existing hospital s, which has brought down the overall growth to 3 %.

Sumit Gupta: Yes. So, no, so with respect to institutional mix like have we witnessed any ARPOB growth?

Yogesh Sareen : No, ARPOB growth is flattish in the institutional.

Abhay Soi : We will have growth coming in , going forward, right?

Yogesh Sareen : And because the rate got revised only in October, so in Q2, there is no growth in the institutional ARPOB. But in the self -pay and insurance , the growth is in the range of 8% to 10% .

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Sumit Gupta: Understood. And sir, second question is like what was the performance of Lucknow and Nagpur unit in this quarter?

Yogesh Sareen : We do not share the hospital -wise performance, but suffice to say that Lucknow did grow over the previous quarter. On a quarter -on-quarter, we reported 17-18% growth in revenue and the EBITDA has also grown by more than 30%.

Sumit Gupta: Okay. Okay. So you are maintaining the 30% plus margins in Lucknow. So we can expect this to sustain?

Yogesh Sareen : Yes.

Sumit Gupta: Okay. And for Nagpur?

Abhay Soi: Again, we do not give specific hospital by hospital numbers.

Sumit Gupta: Okay, understood. Thank you, sir. All the best.

Moderator: Thank you

. We will take the next question from the line of Prashant Kshirsagar from Unived Corporate Research Private Limited.

Prashant Kshirsagar: My question is on the Noida hospital facility. In the -- one of the earlier presentations, you had mentioned you have land parcels with further bed potentials in Greater Noida, 400 beds and Sector 128 Noida, 700 beds. I just wanted to ask you which of the -- is the land adjacent to the existing hospital facility or it is at a dif ferent location?

Abhay Soi: The present one (Sector 128) we have in Noida is 18 acres of land . You can imagine we can keep building over there. It is a contiguous land parcel. Similarly, in Lucknow, it is 27 acres of land. So we can go for another 2,000 - 2,500 beds there . We also have another land parcel in Lucknow. At Noida , in addition to that, we have land in Greater Noida.

Prashant Kshirsagar: So that is what I am seeing. So the location is different from the existing...

Keshav Gupta : Greater Noida and Noida are separate locations. So , in your question, the 700 -bed site, which is a very faraway project , is adjacent site to the recently acquired Noida Hospital, the Jaypee Noida Hospital. And what you mentioned as a 500 -bed site, that is a separate site in Greater Noida.

Prashant Kshirsagar: So this sector 128 is adjacent...

Yogesh Sareen : It is the same complex, yes. It is the 18-acre complex that we mentioned .

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Prashant Kshirsagar: Yes, that is the 18 -acre complex. Yes. Okay. That is what I wanted to clarify. Thanks a lot, sir.

Moderator: Thank you. The next question is from the line of Kunal Dhamesha from Macquarie.

Kunal Dhamesha: Hi. Thank you for the opportunity. One clarification. Can you please share the international patient bed share for this quarter?

Abhay Soi: Yes, so similar to what it used to be earlier -- yes, it would be around 5-5.5%.

Kunal Dhamesha: And sir, for the other channels, for self-pay and TPA? Can you also share for those?

Yogesh Sareen: Self-pay is around 26 -27%, insurance is around 34 -35%. And international, as we mentioned, is 5.5%. The rest is institutional.

Kunal Dhamesha: Right. Okay. Second one for Mr. Soi on the sorting of issues with the insurance. So sir, how should we think now with whatever negotiation which would have gone between you and the insurance companies for the next 1 or 2 years, for us, the procedure rates, do we keep getting similar hikes with what we had taken in historically in the last 2, 3 years? Is it going to be higher, lower? If you can share, some colour would be great.

Abhay Soi: Well, I am not going to give you specifics, but yes, it is in line with what has been happening historically -- within the band.

Kunal Dhamesha: And then, sir, let us say, the 4 insurance companies, which at least media suggested, had suspended cashless, all 4 of them are now on like where they are doing cashless?

Abhay Soi: That is right.

Kunal Dhamesha: And then for us, are they material in terms of, let us say, out of a 35% insurance revenue, are they material in terms of contribution to our top line?

Abhay Soi: Yes. I mean they are material. But like I said, you need to look at cash and TPA as a homogeneous body, right? I mean when you cannot do cashless for patients, the question is do the patients not come or do the patients come and pay out of pocket and then claim later from the insurance companies. Their cashless facility gets suspended, not their insurance policy.

Kunal Dhamesha: Sure. So patient did come. It is just the change of the channel for now. And then now it will be went back...

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Abhay Soi: Yes, we saw a spike in our cash patients.

Yogesh Sareen: So Kunal, the share of the insurance segment went down and the share of self-pay went up in terms of bed share, right? And the overall occupied bed days also grew quarter-on-quarter. So, the

is is not a major impact, right?

Kunal Dhamesha: Sure, sure. And sir, this 7% ARPOB growth on a like-to-like basis, I know you shared that Dwarka has gone down a bit and hence, the overall is 3%. But would not the specialty mix also helped us given the medical patients are down quite a bit in this quarter on a year-on-year basis?

Abhay Soi: Yes, it takes into account that, right?

Kunal Dhamesha: Right? But then with Medical going down, should not the growth be higher for our existing hospital is like my question because oncology has also gone up in terms of the contribution. So shall we continue to maintain this around 7% plus or minus 1% going forward for existing hospitals?

Abhay Soi: But that has been historical as well, right? I do not see any changes there.

Kunal Dhamesha: Sure, sir. Sure.

Yogesh Sareen: So, if you think that internal medicine is basically the impact that you generally used to have in quarter 2. But for the new hospitals, the share of internal medicine is low. So, while there is some impact, and you can see that in the earnings update, but it is not that big of an impact.

Kunal Dhamesha: You are saying new hospital it grew and existing hospital it was not that?

Abhay Soi: Yes.

Kunal Dhamesha: And lastly, sir, on the operationalized bed, because we are opening the capacity at Nanavati and Max Saket, how should we look at operational beds in Q3 and Q4 like what the potential number from where we are in Q2?

Yogesh Sareen: It is tough to really give you the exact number because we will also have to see for how many days we open those beds, when do we get the approvals, NOC, etc.

Abhay Soi: And I want to avoid any forward-looking statements on that.

Kunal Dhamesha: Okay, but let us say, is the trajectory should be in line with more like Dwarka? Is it something like in one year we are almost up with all beds?

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Abhay Soi: I mean, its greenfield (Dwarka) versus a brown field (Nanavati & Max Smart) , right? It is very different .

Kunal Dhamesha: Sure.

Abhay Soi: Because the EBITDA ramp up for these two segments is very different. Normally it is immediately positive in a brownfield . I am going to avoid giving any forward -looking statements.

Kunal Dhamesha: Sure, sir. Thank you for all the clarity and all the best.

Moderator: Thank you. The next question is from the line of Neha Manpuria from Bank of America.

Neha Manpuria: Yes, thanks so much, sir. Just a quick question. What was the network capex intensity in the first half?

Yogesh Sareen : We spent around INR 900 crore.

Neha Manpuria: Okay. And second half would be a similar number?

Yogesh Sareen : A bit higher probably. Hopefully around INR 1,100 crore .

Neha Manpuria: All right. Okay. Got it. Thank you so much.

Moderator: Thank you. We will take the next question from the line of Gourav Bhamra from JM Financial.

Gourav Bhamra: Yes. Hi, sir. Good morning. First of all, congratulations on a good set of numbers. I just wanted clarification on two fronts. The first being that when we say that ARPOB of this quarter was subdued, that was largely due to the insurance issues and the impact of the new hospitals, right?

Abhay Soi: Not so much the insurance, I would say. Like I said, any reduction in insurance perhaps was made up because we had that many people shifting to self-pay. But I think, it was perhaps because of the new hospitals.

Gourav Bhamra: Understood. And the second question regarding the impact of CGHS, the INR 200 crore positive impact that we are expecting. I know you answered it before, but there were some disturbances. Just wanted to clarify again, what is the estimated timeline we are expecting for that to reflect in our numbers?

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Yogesh Sareen : I think some of it has already started from 13th October. But I think there is a super speciality rate that will also kick in , I think

by the end of this month. I mean, we do not know how much time it will take, but I think the expectation is that by the end of November, that rate should also be up and working .

Gourav Bhamra: Understood. Thank you, sir. That is all from my end. Thank you for the opportunity.

Moderator: Thank you. Ladies and gentlemen, we will take the next question from the line of Nitin Agarwal from DAM Capital.

Nitin Agarwal: Thanks for taking the question. Just following up on the CGHS question, sir, the CGHS impact you talked about, it is largely for CGHS or you have also included possible, I mean, the expectation is similar rate revisions will happen for all other central government agencies also. So, is that over and above this?

Abhay Soi: All central government agencies are linked to CGHS. So, this number we mentioned is for CGHS and CGHS -like accounts. So, new rates , including super speciality rates, etc. are also expected on ECHS & others accounts because they are normally in-line with CGHS . However, they have to go through the paperwork.

Nitin Agarwal: Okay. So, that essentially, our estimate pretty much includes that inflation also?

Abhay Soi: That is right. So, this is CGHS and CGHS -linked accounts.

Nitin Agarwal: Okay. Thank you so much.

Moderator: Thank you. The next question is from the line of Shaleen Kumar from UBS.

Shaleen Kumar : Yes. Hi. So, just continuing regarding the past participant. Is it fair to assume that the benefits start kicking in from FY27? What are the benefits we should talk about of around INR 200 crore?

Abhay Soi: Of what?

Shaleen Kumar : The CGHS pricing increased benefits. You know, you said in next 15 -20 days we should resolve it. And then beyond CGHS also the similar CGHS -like institutions will follow it. So, FY27 is the year where we can expect the total benefit to come in?

Abhay Soi: Yes.

Shaleen Kumar : So, then what kind of a flow -through will it have on your operating profit EBITDA?

Abhay Soi: Pretty much 85 %.

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Shaleen Kumar : So, if you are looking at roughly INR 200 crore of top line, something like INR 150 crore, 160 crore of EBITDA addition can happen through this?

Abhay Soi: We are looking at INR 200 crore and hopefully good numbers should flow through.

Shaleen Kumar : Got it, sir. Got it. Regarding Smart and Nanavati, I understand both are Brownfield, but there is a bit of a cost. Is it fair to assume that a large part of cost is already in our P&L reflecting in 2Q or can we expect some more increase in 3Q as well?

Abhay Soi: I mean, you are talking about fixed cost or variable cost?

Shaleen Kumar : Variable cost.

Abhay Soi: Variable cost, of course. Variable cost would be related to the...

Shaleen Kumar : Sorry, sorry, sorry fixed cost. Fixed cost sorry, I made a mistake.

Abhay Soi: No, I mean, these are Brownfields, right? So, your fixed cost is largely absorbed by the existing hospitals.

Shaleen Kumar : Yes. So, my point is right, because we have been in a process of keep on adding new beds, right? So, our top line goes, then our EBITDA, and then our PAT. And I think the point, we will be there where, you know, most of our hospitals reach a stage where our EBITDA start growing faster. I think we are reaching that stage. So, is it a fair assumption that even in 3Q, we should see the same kind of phenomena, that margin expansion despite adding to or despite starting to Brownfield?

Keshav Gupta: You are right. There would be a bit of operating leverage that will kick in once those brownfields start getting full up, that may be 15 days here or there. But, yes, operating leverage should come in. That is the whole concept of brownfields .

Shaleen Kumar : Yes, yes.

Abhay Soi: So, you should have expansion of margins with brownfields because they give you higher margins than existing hospital beds.

Shaleen Kumar : Yes, there are two thi

ngs happening, right, in our growth journey, right? One, our new hospitals, which we have added in the past, they are significantly improving in their profitability. But at the same time, also, we are adding new hospitals, right, which are coming in at a lower margin. So, they are two balancing acts. So, given that we have added a substantial number of beds over the past 18 months, where

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the flow -through of the profitability can be higher and will negate any negative impact of these new beds. That is what I am trying to understand?

Abhay Soi: That is right. My point is something even better, right? As a new bed, because largely what is coming is brownfield, which does not necessarily have a drag on EBITDA that you are trying to subsidize via your previous expansion.

Shaleen Kumar : For sure .

Abhay Soi: It would be not only will you have expansion or, you know, benefits coming out of what you expanded over capacity addition over the last 15 -17 months , that is about 30% capacity, but majority of the capacity coming up now, another 30 %, is brownfield . And that brownfield capacity should , soon after commissioning, get into higher EBITDA margin business to start with.

Shaleen Kumar : Got it. That is it from my side and thank you so much.

Abhay Soi: Thank you.

Moderator: Thank you. As there are no further questions, I would now like to hand the conference over to the management for closing comments. Thank you and over to you, sir.

Abhay Soi: Thank you everyone for joining us today. We appreciate all your time and look forward to interacting with you again next quarter. Thank you.

Disclaimer: This is a transcription and may contain transcription errors. The transcript has been edited for clarity. The Company takes no responsibility for such errors, although an effort has been made to ensure a high level of accuracy.

Call: Feb 2026

February 12, 2026

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Ref.: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Sir / Madam,

Please find enclosed copy of transcript of earnings conference call, organised on February 6, 2026, on financial results of the Company for the quarter and nine months ended December 31, 2025.

The said transcript is also available on the website of the Company at www.maxhealthcare.in/financials#earnings-call.

Kindly take the same on record.

Thanking you

Yours truly,
For Max Healthcare Institute Limited

Dhiraj Arora
SVP - Company Secretary and Compliance Officer

Encl.: As above

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Max Healthcare Institute Limited
Q3 & 9M FY '26 Earnings Conference Call
February 06, 2026

Moderator: Ladies and gentlemen, good day, and welcome to Max Healthcare Institute Limited Earnings Conference Call. Please note that this conference is being recorded. I now hand the conference over to Mr. Suraj Digawalekar from CDR India. Thank you, and over to you, Mr. Suraj.

Suraj Digawalekar : Thank you. Good morning, everyone, and thank you for joining us on Max Healthcare's Q3 and 9M FY '26 Earnings Conference Call. We have with us Mr. Abhay Soi, Chairman and Managing Director; Mr. Yogesh Sareen, Senior Director and Chief Financial Officer; and Mr. Keshav Gupta, Senior Director – Growth, M&A and Business Planning. We will begin the call with opening remarks from the management, following which, we will have the forum open for an interactive Q&A session. Before we begin, I would like to point out that some statements made in today's discussion may be forward-looking in nature and a disclaimer to this effect has included in the earnings presentation shared with you earlier. I would now like to invite Abhay to make his opening remarks. Thank you, and over to you, Abhay.

Abhay Soi: Good morning, everyone, and thank you for joining us on Max Healthcare Earnings Call for the third quarter and nine months ended December 2025.

We are pleased to share that the Network delivered its 21st consecutive quarter of year-on-year growth in Q3, despite excessive unanticipated seasonal softness due to lack of vector-borne diseases and transitory external factors. Revenue increased by 10% year-on-year, while operating EBITDA grew by 4%. Overall occupancies remained strong, and ramp-up of the new brownfield beds progressed in line with our expectations.

During the quarter, we commissioned 63 brownfield beds at Nanavati Max, of which 45 beds are currently occupied. At Max Mohali, 53 brownfield beds were commissioned in second quarter, of which 46 beds are currently occupied. The remaining beds at both hospitals are expected to be commissioned during the Q4

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FY '26. The incremental bed capacity at both these locations is already EBITDA and margin accretive. At Max Smart, infrastructure for around 200 beds, along with operation theaters and OPDs is ready for commissioning, and we are currently awaiting the occupancy certificate, which is expected by the end of February. We also took an important step to expand our geographic presence in Western India to develop a 450-bed hospital on a prime piece of land in Pune by 2030. In addition, driven by the exceptional ramp-up in operations at Max Dwarka, the Board has approved the addition of another 260 beds at the existing site, taking the hospital's total capacity to 560 beds.

On a sequential basis, revenue and EBITDA were impacted prima

rily due to a

temporary shift towards institutional patients following the disruption in cashless services with stand-alone health insurers, which was fully restored towards the end of the quarter. Performance was also affected by the discontinuation of select high-value patented chemotherapy drugs in light of the revised CGHS pricing guidelines and by the reduction in GST on drugs and consumables. Further, the results reflected pre-commissioning expenses related to brownfield bed additions and other non-recurring costs.

Looking ahead, with cashless services now fully restored, upward revision in CGHS tariffs expected to fully kick in by April 2026 and margin accretive incremental capacity coming on stream, we believe the Network is well positioned to continue delivering sustained growth.

Now coming to the Q3 performance highlights :

1. Average occupancy for the Network stood at 74%, compared to 75% in Q3 last year and 77% in the trailing quarter, despite an 8% year-on-year increase in operational bed capacity.
2. Occupied bed days were up by 7% year-on-year, but dipped by 4% quarter-on-quarter due to seasonality.
3. Average Revenue Per Occupied Bed (ARPOB) for the quarter was INR 77,900, registering a 3% growth year-on-year and 1% sequentially.
4. Network gross revenue stood at INR 2,608 crore, compared to INR 2,381 crore in Q3 last year and INR 2,692 crore in the previous quarter. This reflects an increase of 10% year-on-year.

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5. Digital revenue from online marketing activities, web-based appointments and digital lead management was INR 803 crore, accounting for approximately 31% of overall revenue. Website traffic crossed 71 lakhs sessions during the quarter, growing by 44% year-on-year.

6. International patient revenue was INR 230 crore, registering a growth of 14% year-on-year and accounting for 9% of the revenue from hospitals.

7. Network operating EBITDA stood at INR 648 crore, reflecting a growth of 4% year-on-year.

8. Network operating EBITDA margin was 26.1% for the quarter, compared to 27.3% in the third quarter FY '25 and 26.9% in the trailing quarter. Margin was largely impacted by payor mix change, pre-commissioning expenses for brownfield beds and GST rate changes.

9. Annualized EBITDA per bed for the network stood at INR 71 lakhs versus INR 73 lakhs in both Q3 FY '25 and the previous quarter.

10. Profit after tax (PAT) for the Network, after exceptional items, was INR 344 crore against INR 316 crore in Q3 last year and INR 554 crore in the previous quarter. During the quarter, there were exceptional items aggregating to INR

55 crore relating to impact of the Code on Wages, 2019 and provision for stamp duty on the merger of two of our subsidiaries.

11. The Network generated free cash flows of INR 281 crore during the quarter. INR 408 crore was deployed towards ongoing capacity expansion projects, facility upgrades at newer units.

12. Net debt for the Network stood at INR 2,166 crore compared to INR 2,067 crore at the end of September 2025, while the net debt-to-EBITDA ratio continued to be less than 1.

13. Continuing our efforts to support the local communities, we provided free treatment to approximately 40,000 patients from economically weaker sections of the society, worth INR 61 crore at hospital tariff.

14. Both our strategic business units continue to deliver steady growth in revenue and profitability:

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- Max@Home reported a revenue of INR 68 crore, reflecting a robust 23% year-on-year growth. It offers 16 specialized service lines across 15 cities with over 56% repeat transactions.

- Max Lab reported a revenue of INR 47 crore, reflecting 13% year-on-year growth. It provides services in over 60 cities and served more than 5 lakh patients during the quarter.

15. Now moving on to the status of our expansion projects coming

on stream in the

next 2 to 3 years:

- Max Lucknow – The current capacity of the hospital stands at 413 beds, and we expect this to increase to around 500 beds by the end of this financial year. Radiation bunker and nuclear medicine services have now commenced at the hospital.

- 500 beds at Sector -56, Gurgaon – The pace of work at site has picked up post GRAP-related disruptions. We now expect to commission the first phase by end of H1 FY '27.

- 100 beds at Nagpur – We have received Consent-to-Establish (CTE) and civil work has started. We expect to complete this project within 24 months, as communicated earlier.

- 400 beds at Zirakpur (Mohali) – Project work continues to be on track, and we are scheduled to commission the hospital in FY '28.

- 200 beds at Max Vaishali – We are awaiting environmental clearance and approval of building plans to commence the work on site. We expect to complete this project in 24 months post receipt of these approvals.

- 397 beds at Patparganj – All approvals have been received and barricading work is complete. New design for D-wall has been firmed up and the project is now expected to be completed by FY '29.

And finally, coming to the overview of the company performance for the nine months ended December 2025:

16. Network gross revenue stood at INR 7,874 crore, reflecting a strong growth of 19% year-on-year.

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17. Overall Network operating EBITDA grew by 16% year-on-year to INR 1,956 crore, translating to a margin of 26% and EBITDA per bed of INR 71 lakhs.

18. In the nine months, we generated INR 960 crore of free cash flow from operations, after interest, tax, working capital changes and routine capex. Further, INR 1,299 crore was deployed towards ongoing expansion projects and facility upgrades at newer units, INR 131 crore towards land purchase at Vaishali and INR 146 crore distributed as dividend.

With this, we open the floor for any questions you may have.

Moderator: Thank you. We will now begin the Q&A session.

The first question comes from the line of Damayanti Kerai from HSBC Securities and Capital Market (India) Private Limited.

Damayanti Kerai: Hi. Thank you for the opportunity. My first question is on your oncology contribution. So during the quarter, obviously, we saw some softness, which you attributed to discontinuation of patented chemo drugs for institutional patients. So do you think you can go back to the prior level of contribution from oncology? And what could drive it back?

Abhay Soi: Well, when you say contribution, this is high-value drugs. So, the pricing was high in terms of the revenue, but the margins were not substantive, compared to the rest

of the business. Do you mean in terms of revenue, or do you mean in terms of margins?

Damayanti Kerai: In terms of revenue. Yes, when we look at the contribution from oncology, I think which is part of the presentation, it is around 24% or so compared to 26%, 27% last year. So, I was referring from that context?

Abhay Soi: This impacts the institutional patients only. These are high-value drugs, which were low-margin, and were being used actually for the institutional patients. What they have asked now is to sell them below your purchase cost. So obviously, everybody has discontinued it.

Keshav Gupta: And oncology's contribution to the revenue mix may increase, but it is a future addition. It's just that it was increasing in the past as well for last 4-5 years as an outcome of what was happening in the society. So directionally, will that number increase? It should follow the same trajectory but let's see.

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Damayanti Kerai: Okay, sir. So we see this as like a one-time adjustment and then going ahead, the growth will be...

Yogesh Sareen: So we continue to talk to CGHS, right? There is a lot of noise among the institution

al patients. For example, CGHS has their own dispensaries to supply these medicines. They said that they will supply these medicines directly to the patients. That means when the doctor writes the prescription, the patient pays to the CGHS dispensary and brings the medicine from there.

Similar tariff has been applied to ECHS patients also, but ECHS does not have any dispensaries. So obviously, there is a lot of noise out there. And let's see how the things shape out, but we are in continued discussion with CGHS. Obviously, they prefer to supply it themselves.

Secondly, they are saying otherwise, we will give it to you at 70% of the MRP, but our margins are less than 20% in these drugs. So, there is no question about supplying these drugs. We cannot be cashed out of these drugs while supplying these drugs. I think that is where the discussion is on and we are asking CGHS to give us some top-up on cost-plus basis. So, if our cost is, let's say, INR 80, then we are saying give us some margin, say 10%. We will be okay to supply at INR 88 rather than supply at INR 70, because we are buying at INR 80.

Abhay Soi: We believe it is the error on their part when they revised the CGHS rates. This is something that does not make sense because these are branded drugs and if your margins are already, say less than 20%, and you are being asked to supply them at a 30% discount to MRP, then obviously, nobody is going to supply and we are going to have a problem. So, we have been talking to them.

Damayanti Kerai: Sure. And in oncology, again, I guess, we heard about some doctors' team departure, etc. So has team fully back in strength? And what kind of further pickup we can see in the oncology space, leaving aside the CGHS issue, which will, I think, be cleared in some quarters to come?

Abhay Soi: Typically, what happens is that for organizations like us, if there is a departure of a certain clinician, then there are almost immediate replacements from equivalent institutions. In this particular case, while you may have heard the noise of departure, there has already been an addition and there were big advertisements in the papers about it. You may have seen it. We hired a very large team who has just joined us from one of our peers, who sort of replaced that particular team that has gone to that peer. It has been a swap effectively.

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Damayanti Kerai: Yes, okay. My last question is on your regulation with the insurance partners. So have you like done renewing all the insurance contract for this cycle and you have nothing like pending on that part?

Abhay Soi: I mean, of course, this was disruptive in some manner to us in this quarter. It did not happen in the previous quarter. And you may have also read in the papers today that number of complaints of some insurance companies actually moved up by over 40% in this quarter. So obviously, there was a lot of noise because of this sort of disruption.

But all of it has been restored. We have got an increment and a mechanism has also been put in place that there will be annual increments rather than having the sunset periods. Typically, your insurance contracts expire and those negotiations take time. So, now, a process has been put in place where at least with these insurance companies, there is automatic renewal on pre-agreed sort of increments.

Damayanti Kerai: So these annual increments, it is already like pre-agreed or how this mechanism

will work out?

Abhay Soi: That is right, pre-agreed.

Damayanti Kerai: It is annual revision for all the contracts now instead of, say, 2 to 3 years cycle earlier?

Yogesh Sareen: So, you know that this whole issue started from the tariff revision. We were asking for price increase, they were asking for price reduction and that is where this whole stalemate started. So eventually, we

got a price increase. Also, while we got the price increase this time, we also sorted that out for the next year. There is a mechanism in place now. So hopefully, they will live up to it and we should not have the same kind of stalemate coming up. But this is only with the companies that we had a problem, in fact, four companies basically.

Damayanti Kerai: Okay. Thank you.

Abhay Soi: So now with those four, like Yogesh has said, this was the issue at the end of the cycle. Now mechanism has been put in place that you have automatic renewal of the contracts. So, we should not have these issues coming into the next cycle or anything like that now.

Damayanti Kerai: Got it. Thank you, Abhay. Thank you.

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Moderator: Thank you. Next question comes from the Shaleen Kumar with UBS Securities India Private Ltd.

Shaleen Kumar: Hi, sir. Continuing from some of the question asked by the previous participant, possible to get the quantum of increment -- and like -- or at least can we compare like what kind of increment will be getting compared to the past year for insurance company?

Yogesh Sareen: Shaleen, tough to really give actual numbers. We got an increment, that is for sure. It is a moderate one, but then we will not be able to give you a number.

Shaleen Kumar: Okay, but is it in the ballpark of kind of increment?

Abhay Soi: Yes. I would say it is not adverse.

Shaleen Kumar: It is not adverse, right.

Abhay Soi: It is in the ballpark of what we were seeing earlier. It is an upward revision and it is in the same ballpark.

Shaleen Kumar: Got it. Sir, one concern which we kind of -- there is a debate this happened with these four companies, can it happen with others as well? So do you think that can happen? Second, do you think that this kind of mechanism can smoothen out? Are you trying to do this?

Abhay Soi: I think clearly there are learnings when something like this happens, right? It has also led to a lot of noise both ways and inconvenience to patients, not only our patients, but also to insurance companies. And you would have read the article in ET today about the number of complaints, which have increased for these insurance companies. So hopefully, everybody has learned from it.

By the end of the day, and I keep saying this, that medical inflation is in very low single digits. If your ARPOB growth has only been 8% or 9 historically, which has included growth in oncology by around 25%, growth in robotics by around 40%, growth in international patients, etc. and you actually back it all off, what is the real growth in medical cost, apples-to-apples?

And then you apply that to 70% of the billing, because 30% of the billing is for drugs, which are at MRP, in any case. I do not think there is much sort of play there. You keep hearing anecdotally of various issues. But the fact of the matter is, that the apples-to-apples inflation has only been so much, right?

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Shaleen Kumar: Sure. Fair enough. Moving on, I heard partly of your initial commentary maybe I missed something. I heard that you talked about the EBITDA contribution from new facilities in Nanavati and Saket has already accretive. So is it possible to understand that? Obviously, when you started these things there will be some incremental cost which would have hit you in the quarter? Is it possible to quantify that kind of cost?

Abhay Soi: I think what happens is that, and this is something we have guided to in the past as well when people have had concerns on capacity expansion, etc., essentially what you are doing is that you are moving cash from your balance sheet and you are creating an asset.

Because of the operating leverage, you do not have any suppression of margins. In fact, the breakeven is almost immediate and is accretive very quickly. This is

s what

we have seen in the past and this is what we have demonstrated by both of these. Even with whatever little additional cost there is for every incremental bed, which gets commissioned now, your margins will only expand.

And we have had 39% and 30%, respectively, margins from both these units, where we have only started about 60 to 70 beds, right? As we keep adding beds, and all of those beds are coming through now as we speak, and will be all through before end of March, so you should see us continue on this accretive journey.

Now we did have GRAP-related delays but we did build close to 2.5 million square feet over the last 3 to 3.5 years. But the fact of the matter is that there has been a delay of 3 to 4 months, collectively, if you look at it. And nothing has actually moved out more than 6 months from a time line standpoint.

But at one point of time, yes, this is what we had guided to in terms of time lines, etc. On the other standpoint, if you look at it, 3-year projects will take another 4 to 5 months, but it is perhaps faster than what most developers deliver from that standpoint. And we, of course, continue to ask more of ourselves and there has been a lot of learnings in this.

Yes, there are pre-commissioning costs sitting, but it is not as if they are only sitting and not contributing. If you bring doctors along, they perhaps do not contribute as much, but they do contribute to the existing facility. So, it is a little difficult to dissect the costs from that standpoint because there is no management cost as I already explained.

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Largely, the clinician cost is the same. So, you bring more housekeeping people, more nurses, more front-end staff and so on. But they are also training in the current facilities. You're overstuffed in the current facility and then you are going to start to move them into the new one. So, it is a little difficult to thread that cost out because it is not like a typical greenfield.

Shaleen Kumar: Sure. Basically, the reason to ask you a lot of things have happened in the third quarter and that has kind of hurt our profitability. We understand that there is going to be a step jump from third quarter or fourth quarter in terms of profitability because a lot of things have been corrected. And even new facilities are coming, it will just help us to understand what kind of a step jump because one you are ...

Abhay Soi: So, I think these are the smaller factors that have affected the profitability. If you ask me, the two big factors, perhaps, which have affected it would be, first and foremost, the seasonality. Last year, right up till Diwali, we had a very strong vector-borne season, and high occupancies because of dengue, which has happened historically as well.

This year, the rainy period continued straight into winter. There was really no stagnation of water from that standpoint. Normally, what happens is that rainy season gets over, there is a humid period in between and then winter starts. So, in this humid period is when you have the vector-borne diseases. But this year, the rains continued right into the foot of winter.

So, we had very bad seasonality. You may have seen results of other companies as well, particularly North-based companies, and you will see this effect. And the second was, of course, the insurance disruption. Now when the disruption happened with the SAHI companies, we replaced all of that with institutional patients because that is the easier part to replace it with. So, you see our institutional business has moved up. As a result, occupancy did not get impacted, but our quality of revenues got impacted and therefore, our profitability.

Shaleen Kumar: Got it, sir. Got it. So, Abhay, any sense on this quarter, like fourth quarter fiscal like almost end of February, right?

Abhay Soi

i: We are unable to give you any forward-looking guidance. We have never given guidance on forward-looking numbers. We typically only give it in terms of the new capacities that will come in and what the current run rate is, etc.

Shaleen Kumar: Sure. It is okay. Last question from my side if I can. And this is more on the industry level. So a lot of debate again on this as well, it is like too much capacities

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are coming, too many hospitals are coming, and in certain micro markets for example Gurgaon right? And the hospital is, you know, it is not just the local market, it is also the intercity market, where I also come from north, right? So I know that.

But what is your take on it, right? Do you think that a lot of hospitals coming in Gurgaon can impact Saket or customer types are different? Do you think there will

be enough demand because there is enough intercity travel happening . So I just want to hear your thoughts ?

Abhay Soi : I think there are two or three things over here. If you look at it on a holistic level, I think over the next 4 to 5 years, there are about 20,000 beds , which are coming up across the country . It does not really move things . Over 5 years, the capacity increment CAGR is, I think, 5% per year.

Let us look at Gurgaon specifically . Now if a couple of hospitals are coming out of Gurgaon and we happen to be actually one of them . So, I think we are less likely to take a hit at Saket or anywhere else because if any of our doctors want to move to Gurgaon, then they will choose our hospital, right? That is sort of easier , if that location advantage is valuable to somebody.

Shaleen Kumar : Sure .

Abhay Soi : What happens after these 2 or 3 hospitals have come up? There is no visibility of another hospital coming up in the next 5 years , because if there was one coming up, then some plans would have already been passed and something would have been built, which is not the situation, right?

Shaleen Kumar : Yes.

Abhay Soi : Now when that happens, there may be some temporary disruption of costs . So, your clinician cost at that point of time may go up because, one, you are attracting in your new hospital, say, when we put up Gurgaon , so we are going to be approaching doctors and bring them, and paying them more than what they are perhaps earning at the peers.

If somebody is trying to take your doctors again, you know, they are going to offer something more. So temporarily for some time, the cost goes up. But that is also a temporary factor, right? Eventually, if there is any cost inflation, it does get passed off to the customer, and that is what we have seen historically.

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It is not the first time that you are seeing all of this capacity come up . If you look at the numbers, now we are building another 400 beds in Saket, which is the center of the city. Now it is very different from Gurgaon. Gurgaon typically does not affect Delhi but Gurgaon will affect other hospitals in Gurgaon.

Normally, it would have affected our small hospital in Gurgaon, but we are coming up with a big hospital in Gurgaon . If there were hospitals coming up in Mumbai , they may affect our hospital, but a brownfield will always be de-risked.

Keshav Gupta : And directionally what you were asking earlier, right from our own sites, right? So , you can see that the capacity we created in Mohali is already occupied, Nanavati is already occupied. Gurgaon converges entire Haryana and Rajasthan also directionally comes here. So, there is a significantly large pool of demand that gets absorbed by Gurgaon.

Moderator: We will take our next and is from the line of Karan Vora with Goldman Sachs. Please go ahead.

Karan Vora : Yes. Thank you for taking my question. The first question is with respect to insurance. So just wanted to get a sense what would

be the rough share of, say,

top 5 insurers for us as a percentage of hospitals revenue? And when we set up a new hospital, two ways. One is a greenfield and the other one is when we do a new tower in the existing setup, how easy or difficult is the empanelment ? So do we get the same rates as our other hospital in the same city or we have to negotiate from scratch? How does that work ? That is my first question.

Abhay Soi : So, on a brownfield, you do not need to negotiate because the same hospital license continues into the new . So, in case of brownfield, there is no re-discussion or re-empanelment . In case of a new hospital, that means a new hospital license, right? If you do not have the empanelment terms already agreed, then you need to empanel . That means you will make those applications for a new hospital.

Karan Vora : Okay. Got it.

Yogesh Sareen : Yes. Karan, basically most of the insurance companies, we have agreed a category of the hospital. It is CAT 1, CAT 2, CAT 3, CAT 4, right? So , whenever a new hospital comes up, the discussion is always around which category it will fall in. For example, Dwarka now is in CAT 2, Saket is CAT 1 . It is basically that discussion, and then the rates automatically apply. We don't need to get into a negotiation for each hospital separately.

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We already have a set of rates. And obviously, we agree, say, for CAT 1, we know CAT 1 to CAT 2 the difference is so much , and so on . That is how the tariff gets

drive n to the insurance company. Typically, it is not that we have to get into discussion with each hospital , but the discussion is which CAT it will fall in.

Keshav Gupta : And the top 4 insurance companies would contribute about 24 %-25% of the gro up revenue coming from insurance segment .

Karan Vora: And just one clarific ation here. So when you say we have categories determined so how long does it take? So when , say, for example, Gurgaon, whenever it comes online in the next 1 or 2 quarters, what is the expectation of the full empanelment across insurer? Like does it take 3 months, 6 mon ths, 12 months? Any r ough sense there?

Yogesh Sareen : Within 6 months.

Abhay Soi: Yes, because typically, you require NABH and to get NABH, you typically require 6 months of data. Even the institutional patients, take about 6 months to onboard.

And you have differe nt rates for NABH and non -NABH.

So, I think the first gating item for a new hospital would be to get NABH. Also , concurrently you need various licenses in a greenf ield, such a s transplant , blood bank , etc. So, it is not as if on a brownfield, where all existing licenses continue. In a greenfield, you need to apply for e ach one of those license s. You can not just start doing liver transplant or kidney transpla nt or whatever it is. So , the full range of services does not start.

Karan Vora: So basically, broadly 6 to 9 months is where you can get the NABH accreditation as we ll as the insurance empanelment . That i s the rough sense we sho uld have?

Abhay Soi: That's right. But I mean, it depends on hospital to hospital. Now of course, this was the same in Dwarka as well for us. But as you are aware, we started Dwarka last July. By now, we have already been fully occupied on the 300 beds for a while .

And we are alread y planning a brownfield for another 2 60.

We had a breakeven within 6 months ove r there, because it was cash patients, et c.

So, it is about how you also sequence it . I think the very important part over here is how do you preserve cash flow and how do you sequence your bed s. Now we could have started all 250 beds over there . But we have to forecast the number of beds, we have to plan acco rdingly and we have to staff ac cordingly.

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If you are going to staff all the beds, then you are going to lose mon ey, be i t with a brownfield or a greenfield . I

think that somewhere you need to be a l little more tactful ab out it. Our total loss for Dwarka in 6 months till breakeven was about INR 30 crore – in a greenfield in a brand -new micro market, where we d id not have any presence.

Karan Vora: And my second questi on is with respect to -- I think, Abhay, you menti oned on the - in the opening commentary that we should be back with respe ct to growth from Q4 onwards. So just wanted to get a sense. So do we foresee a step -- like a stepped or phase d manner of recovery growth that Q4, you might be partially back and Q1, you should be fully back? Or from Q4, it is a c omplete clean quarter and there should be no one -offs or no deterrent from Q4 itself?

Abhay Soi: I think the big de terrent on our growth has been capacity. I mean, essentially, if we go beyond the seasonality of it and whatever one-time disruptions were , which are back to normal . I mean everything that we have acquired over the last couple of months or the new capacitie s that we set up have b een ramping up very w ell. Two years back, the issue w as where is growth going to come from because the fact is that we were operating at very high capacity and the growth was going to be coming in essent ially from the capacity a ddition. And there has been a delay of a quarter or two. So now that it is online, then we should be back to trajectory.

Karan Vora: And the last question is with respec t to Gurgaon. Sorry, if I missed in the ope ning remarks, like do we expect it to commissio n like by Q4 end or Q1? And w hat would be the impact like maybe a quarter or 2 impact of losses from Gurgaon since it i s like a large Greenfield? Any color there will be helpful ? Thank you.

Abhay Soi: I think towards the end of H1 FY '27 is when we should be able to sort of commis sion the first phase over there. We are not going to give you a ny guidance on the losses there, but you have the history in front of you.

And I will tell you why we are not doing that . Because it is a function of the clinicians yo u are able to get and what sort of programs you are able to start. We are happy to make a bigger loss o ver a shorter pe riod of time and to have a deeper trough . That essential ly means that we have been able to get the clinicians on day 1 in a greenfield .

Karan Vora: Got it. Thanks. That i s helpful.

Moderator: Thank you. Next question comes f rom the line o f Vivek Agrawal with Citigroup.

Please go ahead.

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Vivek Agrawal: So one question on CGHS, ECHS rate revision. So you earlier talked about approximately INR 200 crore kind of a positive impact on revenues, but that is including the impact of discontinuation of some patented drugs, etc., right? So is it possible for you to split it like what is the absolute impact of rate revision? And how much of that is likely to be netted off from a discontinuation of patented drugs etc.?

Yogesh Sareen : Vivek, we already said that the net impact is INR 200 crore . We have netted off the onco impact , because onco is part of that MoU when the price got revised. So, I would say it will be probably INR 280 crore minus INR 80 crore. But I must also mention to you that the whole price increase has not happened in Q3. ECHS has revised the prices only in December. Some of the PSU are asking for new budgets, etc., to increase the prices to the CGHS levels and so that is also yet to come in . Also, the super specialty rates within that category will be available from 1st of April. So, to my mind, the full impact of this will start to come from Q1 FY27. But I think a large part will start to flow from Q4 FY26 .

Vivek, I must also mention that there is a negative impact of GST both on the revenue side and the margin side . Then you have to net off another INR 60 crore out of INR 200 crore number. So, it will be INR 140 crore sustained positive impact in the margin.

margin.

Vivek Agrawal: So actually, is it right to understand like the impact of the discontinuation of patented drug, etcetera, or the negative impact as well as including the GST, etcetera. So that is largely in this particular quarter. Basically, the GST plus discontinuation of patented drug is the net in the quarter and that the positive impact, let us say, around INR 280 crore only on the rate revision that is likely to come from...

Yogesh Sareen: No, the INR 200 crore is sustained impact of CGHS prices. So, the onco effect will continue because under their MOU, they are saying you will have to give discounts on the chemotherapy drugs of 30%. So, some of the drugs where the margin was less than 30%, we discontinued to supply and where the margin is more than 30%, we still continue to supply, but still at a lower revenue .

So, there is impact -- there are two main impacts on the CGHS price increase of the oncology drugs. One is that we discontinued some drugs and second is that some we are giving 30% discounts. So, total of both impacts will be, as I said, INR 80 crore .

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INR 80 crore is sustained impact , so that means net is INR 200 crore . And then you have to reduce out of that the GST impact. So INR 140 crore is the net impact if you ask me on an ongoing basis. So, it is not one time, I am saying sustained impact, that means you will have both going forward.

Vivek Agrawal: Understood. Thanks. This is helpful. And just one more question again on insurance side. It looks like that it has been targeted towards Max, while we are not seeing this kind of impact and for the other -- or some of your other peers, etc., in Delhi NCR. So, any specific reason for this, basically why it has been targeted towards Max? Separately, what kind of the safeguards that you have that the other insurance companies, let us say, does not do it again in future?

Abhay Soi: I think, hopefully, everybody has learned from it. Because I do not think the disruption is only for Max. I think the disruption would also be for other insurance companies. And there is a reason that other insurance companies did not join in. I think it just made some more media this time. And it has been disruptive to patients, and these patients are also customers of the insurance companies. And therefore, you have seen higher number of complaints .

Vivek Agrawal: Understood. And last question on institutional patient share . So, it has gone up quite a bit, 36% and part of this disruption. So, any color how to look at this number, let us say, 1 year down the line or 2 years down the line?

Abhay Soi: Well, I think the important thing is that when you are coming up with capacity, the more capacity you come up with, you will have institutional, exactly to what extent is another matter . But the fact of the matter is still the EBITDA per bed is higher -- even with the lower rates for the brownfield capacity additions. That is what we are kind of demonstrating right now.

Vivek Agrawal: Understood, sir. Thanks, that is it from my side.

Moderator: Thank you. Next question comes from the line of Bansil Desai with J.P. Morgan India Private Limited . Please go ahead.

Bansi Desai : So, my first question is on the growth of our existing hospital beds. So traditionally, if we see barring any seasonality impact, you still managed to see good low teens kind of growth for our existing beds despite the fact that they have been operating at like 75% plus occupancy levels, partially, it could be because of higher onco share or robotics, etc., which you mentioned?

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But as we go forward, do you think theoretically, this has to normalize at some point in time? And for us, therefore, what could be the levers which can keep the growth momentum high over the next 2, 3 years?

Abhay Soi: Look, I think seasonality, if

you look at the past 10 years has been a reality. The big difference is that last year we had big seasonality, and this year, we had actually no seasonality. So, it is a little bit of a double whammy. If you look at the numbers last year, we had some 30% -35% growth as a group year-on-year basis in Q3. So, it was a very high growth. A lot of it was also due to a little higher part on the seasonality. And whereas this time, it is lower. Now having said that, the big jump in the current year was going to be coming through capacity additions. And clearly, there has been a quarter or two of delay as far as that is concerned. So that is something which should have contributed to increase in revenue. And I think as and when we start getting that on stream, we are going to see that.

Bansi Desai : Yes. So Abhay, actually, I was just trying to say that if I actually look at this quarter and if I map it over a 2-year CAGR, this number still suggests a 20% CAGR over 2 years. So, if I remove that seasonality impact, and this is beside the fact that we have not added as many beds?

So, what I was trying to understand is that it does mean that our existing beds are still growing well in that low-teens rate. And we have seen that peers that most mature beds beyond a point once you reached your optimal occupancy levels, will probably come down to high single-digit kind of growth rates, etc. So, I was just trying to understand that from our side, if seasonality were to remain favorable, then do we continue to see the kind of growth that we have seen?

Abhay Soi: There are two aspects to it. One is we are not looking at peaks and troughs as far as seasonality is concerned, and we are just looking at it on a static basis, saying that if you were to put a line straight through to it, then what would the mean be. So, we are not looking at extensive seasonality or it happening or not happening. I think secondly, in terms of where the existing beds are concerned, we have always sort of guided that there are levers. Some of those levers get obfuscated, because we are opening new beds.

But the fact of the matter is the real higher institutional business, which will be getting distilled through it. But it gets obfuscated by the fact that we are opening new beds. So, there are levers in the current capacity, it is not as if they are not. I mean it depends how you look at it, how you bifurcate this.

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Yogesh Sareen: I think also as management our job is to make sure that even the existing hospitals grow. So, on one hand, they can grow on the ARPOB side, the case mix side, etc. and on the other we are also adding more beds in these hospitals. We are trying to do brownfield expansion, where in the same campus, when you add more beds, then the existing hospital will also grow. So, there is no existing bed versus new; it is the same one hospital that we are trying to grow.

For example, wherever we have higher occupancies, we try to get brownfield beds there. So, you know we are now adding 260 beds to the existing Dwarka hospital. That will make the existing hospital grow.

Abhay Soi: In Dwarka, while we are putting the additional beds over there, our current mix has almost got 50% institutional patients over there. And in the first year, as you are going to ramp up capacity, you are going to take all sorts of business. And as you go along, you are going to see that distilling.

Bansi Desai: And my second question is more clarificatory in nature. You mentioned that the GST rate has also had a bearing on our margin. So just wanted to understand this would have not impacted our absolute EBITDA, right? It was just impacted our margins because your realizations go down?

Yogesh Sareen: No, it does because when you bill to the patients, the bill is at MRP, but you do not pay the

GST on the margin because these goods are used for delivery of medical services. So, there is an impact on the margin as well.

Abhay Soi: So that is how Yogesh led to INR 200 crore net of onco drug impact and then minus another INR 60 crore for GST. You are coming to about INR 140 crore as a result of this entire CGHS revision as well as GST.

Bansi Desai: Because I would assume that GST realization or EBITDA would have been net of that tax amount, right?

Yogesh Sareen: Probably we will have to get online separately. I will walk you through as to why it impacts the margins.

Bansi Desai: Okay, okay, got it. Thank you.

Moderator: Thank you. Next question comes from the line of Tushar Manudhane, Motilal Oswal Financial Services Limited. Please go ahead.

Tushar Manudhane: Thanks for the opportunity. Sir, first, clarification, the top 4-5 insurance companies from the 25% of the insurance business, right?

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Abhay Soi: Yes.

Tushar Manudhane: Yes, secondly, we have been doing roughly INR 400 crore -- INR 420 crore plus/minus sort of a capex per quarter. But FY '26, I guess, the target is up to INR 1,900 crore. So are we on track to do that kind of capex?

Yogesh Sareen: Yes.

Abhay Soi: What typically happens is that a lot of the capex is back ended. You don't necessarily relate work done to capex. I do not think it works in tandem necessarily.

Yogesh Sareen: Also, when you do a cash flow projection, you will always do a conservative projection, right? That is how we plan for it. We do not want the project to suffer because of financial closure, etc. So, you will always find that we will be spending less than what we are projecting because of the fact that we project a conservative number.

Tushar Manudhane: Even if I take roughly INR 400 - 450 crore per quarter sort of a capex, subsequently for FY '27, also, it will be higher capex, right? I am not referring to bed addition. I am not even connecting that to bed addition. I am just referring to the amount that would spend for the new hospitals effectively?

Yogesh Sareen: I think, you have the numbers already in the slide. We will revisit the numbers at the end of March '25 in any case, and then probably we will float the new numbers, but as of now you have the numbers available in our presentation.

Abhay Soi: They are fairly conservative numbers. So, we would certainly not be doing more than that.

Tushar Manudhane: And given the current sort of cash flow, which is like roughly INR 300 crore + cash flow from operations, I am referring to. So does it mean that you still have some more debt coming on balance sheet?

Yogesh Sareen: I think the incremental beds that we are getting on stream now, they will start to give us operating margin. And we mentioned that all these beds are EBITDA as well as margin accretive. So, we obviously expect better cash flows from these operations now that the new beds are getting operationalized.

Tushar Manudhane: So, which means effectively INR 2,100 crore net debt is that the number in FY '27 as well. Does that -- I mean is that the safe assumption?

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Yogesh Sareen: I think last time also the question was asked, it will go up by around INR 500-600 crore in terms of net debt, but it will be still less than 1 unless we do any M&A, etc.

Tushar Manudhane: And just lastly, Smart, there has been like almost 5, 6 months delay. So -- and still like the regulatory approval is to come through, so February '26, is that sort of now largely certain or that might get pushed for this?

Abhay Soi: So as far as Max Smart is concerned, the original time given was FY '28. This project has taken us about 24 to 25 months, in its entirety. We expect approvals by end of Feb '25. It has just got ten ready now, and we have just applied for approval. So, there will be no delay in ap

provals. I want to put that also in place. So, it is not as if the project has been lying ready and sort of approvals is what is delaying it. The project has been delivered now and approvals have been applied for and we are expecting it by end of Feb '25. In fact, whatever attendant approvals are, they have been coming through very quickly and very smoothly. Actually, in this one, we have been ahead of schedule. This one and Zirakpur, we seem to be ahead of schedule.

Moderator: Thank you. The last question comes from the line of Nitin Agarwal with DAM Capital. Please go ahead.

Nitin Agarwal: Thanks for taking my question. Abhay, you have talked a couple of times about the

Dwarka hospital and the fact that we have seen encouraging progress on it to go in for expansion. If you can just give us some more colour on what's been the progress -- financial progress of the hospital in terms of where it is reached right now, which prompted you to go for expansion at this early stage?

Abhay Soi: We are already operating at close to 75% capacity, although almost half of the business continues to be institutional. Like I said, early -- the life cycle of a hospital, you would first think about ramping up occupancy. And you ramp up occupancy with all sorts of business, and then you start distilling it. So we are already doing 20% margins at Dwarka while 50% of the business is institutional. When we put up capacity going forward, it will take us at least 2 years to put up that capacity. That means in the better part of two years, I would not have enough beds, easy to surmise that from 75%, we will probably go to 85%, so we will be distilling our beds. We are already seeing month-on-month, there are more cash and insurance patients coming through and the institutional patients are reducing. And therefore, you will see the margins move up. That is the encouraging part.

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Nitin Agarwal: And secondly, on the Jaypee Hospital, can you give us any update on the progress on that?

Yogesh Sareen: So Jaypee Hospital is doing well now. I think the occupancy has improved over the period in the hospital. So, are you looking for some specific numbers for that?

Nitin Agarwal: I am just curious in terms of where -- from where you acquired to where it is come to, what the distance we have covered in terms of performance improvement in the business?

Yogesh Sareen: So I think if I consider year-on-year, it will be more than 30% growth. We acquired this in same quarter last year. The revenue is up by 30%-35% and EBITDA has also improved. Obviously, we cannot give you specific numbers in terms of revenue but I think that is the status. And when I say 30% increase in revenue, that obviously means that the revenue has come down first because when we acquired the hospital, we stopped all the referrals, etc., the revenue tanked a bit and then we brought it back.

So, this 30% growth Y-o-Y same quarter is basically after that dip, which happened in the first quarter after acquisition and then we built it up. The real growth would actually be around 40%.

Nitin Agarwal: And in terms of profitability, is it now closer to your network profitability? How far is it from there?

Yogesh Sareen: No, it is not. It is less. So, I think the first endeavour was to stabilize the operations. We had a lot of complaints in that hospital, so we were working on that. I think the margin is probably 3-4% lower than the overall margin that we have in the Network.

Nitin Agarwal: And secondly, when we look at our business for the next couple of years, the EBITDA per bed for us has been a pretty dramatic journey, which we have had over the years, around 7.5 million to 8 million is where we are at. I mean, does the network EBITDA stabilize around that? Or do you see opportunities for us to significantly increment

it from these levels? And how should we think about EBITDA per bed take a 2-3 year view from here?

Keshav Gupta: No. We do not go towards the direction for chasing EBITDA per bed also as a base. Our trajectory is to deploy capital efficiently to have yields on the capital.

Abhay Soi: We may have a lower EBITDA per bed, but you have to focus on ROCE over there. I mean we are not focusing on making EBITDA per bed or ARPOB accretive or whatever. We are focusing on ROCE.

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Nitin Agarwal: And last one. Over the next 2 years, from where do we see our operational beds sort of fitting out, we are about 4600 operational beds right now. Where do we end up in the next 2 years?

Abhay Soi: We are around 4,800 right now. And we are adding another 1,200 beds, including Gurgaon. So that is around 6,000 and by FY '28, we should be hitting 8,000. It is available in the presentation.

Nitin Agarwal: About 8,500 beds should we look at that...

Abhay Soi: Please look at the presentation for the exact year-wise listing. The presentation has beds mentioned location wise and how many brownfields, how many greenfields, etc.

Nitin Agarwal: With respect to the delays and all that you foresee by the time we finish FY28, you still brought more or less in the same ballpark that we had in the presentation, is what I meant.

Abhay Soi: That is right. So, I mean we have given it out in the presentation, updated numbers. The current quarter numbers will be updated for any delays.

Nitin Agarwal: Okay, perfect. Thank you so much.

Moderator: Thank you. Ladies and gentlemen, we have reached the end of question-and-answer session. I would now like to hand the conference over to the management for closing comments.

Abhay Soi: Thank you, everyone, for joining us today. We appreciate all your time. We look forward to interacting with you again next quarter. Thank you very much.

Disclaimer : This is a transcription and may contain transcription errors. The transcript has been edited for clarity. The Company takes no responsibility for such errors, although an effort has been made to ensure a high level of accuracy .

Call: May 2026

May 29, 2026

Listing Department ,
National Stock Exchange of India Limited
Exchange Plaza, Plot C -1, Block G,
Bandra Kurla Complex, Bandra (E),
Mumbai – 400 051

Symbol: MAXHEALTH Listing Department,
BSE Limited
Phiroze Jeejeebhoy Towers,
Dalal Street,
Mumbai – 400 001

Scrip Code: 543220

Sub.: Transcript of Earnings Call held on May 22, 2026

Ref.: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Sir / Madam,

Please find enclosed copy of transcript of earnings conference call, organised on May 22, 2026, on financial results of the Company for the quarter and financial year ended March 31, 2026.

The said transcript is also available on the website of the Company at www.maxhealthcare.in/financials#earnings-call.

Kindly take the same on record.

Thanking you

Yours truly,
For Max Healthcare Institute Limited

Dhiraj Arora
SVP - Company Secretary and Compliance Officer

Encl.: As above

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Max Healthcare Institute Limited
Q4 & FY '26 Earnings Conference Call
May 22, 2026

Moderator: Ladies and gentlemen, good day, and welcome to Max Healthcare Institute Limited Earnings Conference Call. Please note that this conference is being recorded.

I now hand the conference over to Mr. Suraj Digawalekar from CDR India. Thank you, and over to you, sir.

Suraj Digawalekar : Thank you, Neeerav. Good morning, everyone, and thank you for joining us on Max Healthcare's Q4 and FY '26 Earnings Conference Call. We have with us Mr. Abhay Soi, Chairman and Managing Director; Mr. Yogesh Sareen, Senior Director and Chief Financial Officer; and Mr. Keshav Gupta, Senior Director – Growth, M&A and Business Planning. We will begin the call with opening remarks from the management, following which we will have the forum open for an interactive Q&A session.

Before we begin, I would like to point out that statements made today may be forward-looking in nature, and a disclaimer to this effect has been included in the earnings presentation shared with you earlier.

I would now like to invite Abhay, to make his opening remarks. Thank you, and

over to you, Abhay.

Abhay Soi : Good morning, everyone, and thank you for joining us on Max Healthcare's earnings call for fourth quarter and full year ended March 2026.

Let me begin by highlighting that over the last 2 quarters, we have rolled out phased commissioning of more than 20% additional brownfield capacity across our hospitals in Mohali, Nanavati in Mumbai and Max Smart in Delhi . All the beds will be ready to be operationalized over the next 2 to 3 months. Further, we expect to add another 10% capacity once our 500 -bed greenfield hospital in Gurgaon is commissioned during the year. We have already onboarded clinical and non - clinical talent for these capacities and expect significant operating leverage to come through as operations progressively ramp up.

We are also pleased to share that we have completed the acquisition of a controlling stake in Kalinga Hospital Limited this month. Kalinga owns and operates a 250 -bed hospital on a prime 10 -acre land parcel in the heart of Bhubaneswar . The acquisition marks our entry into Eastern India and provides us a strong platform with established clinical programs and significant potential for future expansion at the existing site. We have firmed up plans to revamp and expand the facility.

Page 2 of 16 Further, the Board has approved an investment of INR 1,400 crore for the construction of a 700 -bed greenfield hospital at Shaheed Path, Lucknow. This investment reflects our continued confidence in the region where we have seen encouraging momentum since the acquisition of our existing facility. The proposed hospital will add meaningful bed capacity and position us to serve the growing demand for high -quality health care services

in one of North India's important health care markets.

With respect to the Q4 performance, the Network delivered its 22nd consecutive quarter of year-on-year growth , with the revenue increasing by 10% and operating EBITDA by 8%.

As we move into FY '27, our priorities remain focused on scaling the recently commissioned capacities, integrating Kalinga Hospital into the Network , and progressing our outlined expansion projects, including the Sector -56 Gurgaon hospital . At the same time, our existing hospital operations continue to provide a steady foundation , supported by strong clinical capabilities and consistent execution across the Network. This positions us well to deliver sustained growth while maintaining capital discipline.

Now , coming to the Q4 performance highlights :

- Average occupancy for the Network continued to be more than 75% despite increase in operational bed capacity , with most of the units operating at near optimal capacity.
- Occupied bed days (OBDs) were up by 8% year -on-year and 4% quarter -on-quarter.
- Average Length of Stay (ALOS) was temporarily higher by 9% compared to Q4 last year, characteristic spurt due to multi -location capacity rollout simultaneously.
- Average Revenue Per Occupied Bed (ARPOB) for the quarter stood at INR 77,900. This was after absorbing the impact of higher ALOS and discontinuation of select high -value chemotherapy drugs for institutional patients.
- Network gross revenue stood at INR 2,664 crore compared to INR 2,429 crore in Q4 last year and INR 2,608 crore in the previous quarter. This reflects an increase of 10% year -on-year and 2% quarter -on-quarter.

Due to discontinuation of select high -value chemotherapy drugs for institutional patients, share of oncology in in-patient revenues dropped to 21% from 26% in Q4 FY '25 and 24% in Q3 FY '26. Excluding oncology, gross revenue grew by 15% year -on-year and 5% quarter -on-quarter.

- International patient revenue was INR 227 crore , registering a growth of 12% year-on-year and accounting for 9% of the revenue from hospitals.
- Digital revenue from online marketing activities, web -based appointments and digital lead management was INR 838 crore, accounting for approximately

Page 3 of 16 31% of the overall revenue. Website traffic crossed 90 lakh sessions during the quarter, growing by 39% year -on-year.

- Network operating EBITDA stood at INR 682 crore, reflecting a growth of 8% year-on-year and 5% quarter -on-quarter.

- Network operating EBITDA margin was 26.8% for the quarter, compared to 27.2% in Q4 FY '25 and 26.1% in the trailing quarter.
- Annualized EBITDA per bed for the Network stood at INR 73 lakh, versus INR 74 lakh in Q4 FY '25 and INR 71 lakh in the previous quarter. This was also reflective of the higher ALOS.
- Profit after Tax (PAT) for the Network was INR 387 crore, against INR 376 crore in Q4 last year and INR 344 crore in the previous quarter.
- The Network generated free cash flows of INR 581 crore during the quarter. INR 328 crore was deployed towards ongoing capacity expansion projects and facility upgrades at newer facilities.
- Net debt for the Network stood at INR 1,908 crore compared to INR 2,166 crore at the end of December '25, and the Net debt-to-EBITDA ratio continued to be less than 1.
- Continuing our efforts to support the local communities, we provided free treatment to approximately 42,000 patients from economically weaker section of society, worth INR 59 crore at hospital tariff.
- Both our strategic business units continue to deliver steady growth in revenue and profitability :
 - o Max@Home reported revenues of INR 73 crore, reflecting a 30% year-on-year growth. It offers 16 specialized service lines across 15 cities, with over 56% repeat transactions.
 - o Max Lab reported a revenue of INR 52 crore, reflecting 14% year-on-year

growth. It provides services in over 60 cities and served nearly 6 lakh patients during the quarter.

- Now, moving on to the status of our expansion projects coming on stream in the next 2 to 3 years :

- Max Lucknow : The current capacity of the hospital stands at 426 beds, and we expect this to increase to 570 beds over the next 2 quarters.
- 500 beds at Sector 56 Gurgaon : Interior and facade works have started. We are targeting to commission this facility by the end of this year.
- 100 beds at Max Nagpur : Project work continues to be on track, and we expect commissioning by FY28.
- 400 beds at Zirakpur, Mohali : Structural work is ongoing, and we are on schedule to commission the hospitals in FY28.

Page 4 of 16 v. 260 beds at Max Dwarka : Building plan submission is underway and the project is expected to take 24 months to complete.

- 200 beds at Max Vaishali : We are waiting building plan approvals, while all other clearances in place. Project is expected to take 24 months post receipt of approvals.

- 400 beds at Max Patparganj : D-wall construction has started, and we expect commissioning by FY29.

And finally, coming to the overview of the company's performance for the full year ended March 2026 :

- During the year, we have initiated phased commissioning of nearly 20% additional brownfield capacity across the Network.
- Network gross revenue stood at INR 10,538 crore, reflecting a growth of 16% year-on-year.
- Overall Network operating EBITDA grew by 14% year-on-year to INR 2,638 crore, translating to a margin of 26.2% and EBITDA per bed of INR 72 lakhs.
- Profit after tax (PAT) for the Network increased to INR 1,631 crore compared to INR 1,336 crore in FY25, registering a growth of 22%.
- During the year, we generated INR 1,541 crore of free cash from operations after interest, tax, working capital changes and routine capex. Further, INR 1,627 crore was deployed towards ongoing expansion projects and facility upgrades at newer units, INR 131 crore towards land purchases at Vaishali, and INR 146 crore was distributed as dividend.

With this, we open the floor for any questions you may have.

Moderator: Thank you very much. The first question is from the line of Neha Manpuria from Bank of America Securities.

Neha Manpuria : My first question is on the brownfield beds that we have added. When do we start seeing them contributing to EBITDA more meaningfully? Did I hear it correctly that all of these brownfield beds will be commissioned in the next two quarters. So, second quarter, third quarter is when they should start showing up more meaningfully on EBITDA?

Abhay Soi : So they are already contributing to EBITDA and it is not any form a negative contribution . But what happens is that you sort of get the better end of operating leverage as you go along because right now, for the total 1,000 -odd beds , we have initiated a phased rollout of less er beds.

Say, if you have 400 beds at Max Smart, which are being rolled o ut in a phased commissioning, you would have started with about 100 beds over there. So , as you open up the balance beds over the next couple of quarters, you will see the ent ire operating leverage as the balance beds get occupied because your costs related to even the brow nfield are not linear effectively.

Page 5 of 16 Neha Manpuria : Okay. And is it fair to assume that the occupancy and these ramping up should not be a problem ? We should g et to a fairly good level of occupancy as soon as we start these beds. That should not be a problem , right? That would be a fair assumption.

Abhay Soi : Yes. So , I mean it is a two-way thing. I mean , we do not open beds if we do not have occupancy, but what we have seen is a very good ramp -up of that occupancy. And therefore, you have seen in spite of ne w beds opening up, occupancy remains high. But having said that,

I must also point out what is embedded within it is also higher ALOS. So , what tends to happ en is you are just a little more efficient when you do not have the beds.

When you open up th e new beds, there is a tendency for the ALOS to increase. It is slightly temporary in nature. We have tightened it again, but you have seen th at ALOS h as gone up by 9%. So, the impact of that is that shows up in both your ARPOB and occ upancy – while the occupancy seems a little higher, your ARPOB comes through a little lower.

Neha Manpuria : Understood. My second question is on Gurug ram. Did I hear you correctly tha t we are now expecting Gurugram commissioning by the end of th is fiscal year? I mean I am not sur e if I picked that up correctly?

Abhay Soi : Yes, we are expecting commissioning by the end of the year.

Neha Manpuria : Okay. And we should not be expecting any further delay on that because that is been pushed out a few times now.

Abhay Soi : That is right.

Neha Manpuria : Okay. And sorry, one last question, if I may. On the Bhubaneswar asset that we have acquired, this will start integrating from first quarter itself? Or is there any approval, etc , that we requi re before closing this?

Abhay Soi : Yes, from t he first quarter . We have already acquired the majority stake so we will be consolidating it.

Neha Manpuria : Okay. I have a f ew more questions, but I will get back in the queue. Thank you.

Abhay Soi : Yes.

Moderator: Thank you. Next question is from the line of Banshi De sai from JP Morgan.

Banshi Desai: Yes, thanks for the opportunity. So just again on Gurugram, how should we th ink about the operationalization of beds, assuming we commission towards the end of fiscal '27? What wi ll be the Phase 1 operationalization? And what is the count that we should expect in fiscal '28?

Abhay Soi : So, I think in fiscal '28, we will be looking at breaking even within the year . I mean , it is a greenfield, as you are aware. Having said that, our experience with the Dwarka greenfield, we operationalized it , we guided to a 1 -year breakeven , but we actually broke even in 6 months. We had an operating loss, consolidated EBITDA loss in the first 6 months of about INR 35 crore. However, by th e end of the 12 - month period , it was less than INR 10 crore . Even if that number is more or less in

Page 6 of 16 this case, it is not a meaningful change to perhaps what the projections are going to be.

Banshi D esai: But in terms o f beds, are we expecting phase -wise manne r of operationalization here, because it's a 500 bed facility ...

Abhay Soi : Yes. You always do it , even tactically, in that manner . So physically, the beds come out in phases, but even so tactica lly because if you h ave 500 beds, day one, you are not going to have occupancy of 500 beds. So , you do not operationalize or staff all the 500 beds. So , if I take the example of Dwar ka, we had 300 b eds, we started with 140 beds. We ramped up occupancy, bro ke even with the 140 . So, the balance beds start to yield as you go along.

You are seeing a similar sort of story play out in our brownfield s right now. And with respect to even the green field at Gurgaon, you are going to start with, let's say, about 200 -odd beds. And once y ou kind of break even within that, then you start rolling out the balance beds.

Bansi Desai: Understood. That is clear. And my second question is on the onco share decline that we have seen in Q4. While clearly, the reason highlighted is a discontinuation of chemo drugs, but it still feels a bit sharp, given we had quantified onco drug impact to be about INR80 -odd crore. So, if you could help us understand what has happened here? And by when do we expect this to reverse?

Abhay Soi: There are two things. You have the onco drugs, that is day care, the margins on which were coming out a little perverse to us. So, discontinu

ation of these high -

value drugs not only impacts your top line, but also impacts your OBDs (occupied bed days), which are related to it because some of the patients, which are coming for day care, are also admitted in the night. So, it has a knock-on effect on that as well. Our OBDs have sort of come down by about 5% to 6% in oncology, which is related to this.

Bansi Desai: So, I mean, do we have a plan in place, how do we replace it? What alternative protocols would you have?

Yogesh Sareen: Basically, some of this is permanent because we know that we will not be able to do this kind of a business on the minus-margin basis. As you have also seen in this quarter, although the OBDs have degrown by 6% in oncology, but we have overall OBDs have grown. That means the other specialties have been able to compensate for it. So, I think that is the plan even going forward, that we do not expect the share of oncology come back to 25 -26% as it was earlier. It will continue to hover around 21 -22%, and we will then have the other specialties fill up that vacuum.

Bansi Desai: Got it. And the CGHS rate revision benefit, has that started to flow through in Q4?

Yogesh Sareen: Yes. All except the super specialty rates. A very large part is already in, but there is a small part, which is left out, which is around INR 25-30 crore per annum. That will be phased out over this year. It has started to come in two hospitals, but balance is still pending. But that is all in this quarter.

Abhay Soi: We are expecting that to come in the next few months.

Bansi Desai: Thank you.

Page 7 of 16 Moderator: Thank you. Next question is from the line of Damayanti Kerai from H SBC Securities and Capital Markets (India) Private Limited.

Damayanti Kerai: My first question is clarification. Abhay, you mentioned you are rolling out beds in a phased manner, even for say facility like Smart. So, help me to understand this better. In the past, whenever you have opened or commissioned brownfield facility, I understand the ramp-up happened much faster than what we are seeing right now. So according to you, anything has changed since then? That is why you are going for more gradual phased way of prescribing beds.

Abhay Soi: No, we have always opened brownfield in a phased manner. Whether brownfield or greenfield, it has always been opened in a phased manner. And it is opened in a phased manner because as soon as any part of a new facility, any floors are ready, there is always a tearing need for those floors. And you have seen that play out in the occupancy as well. So, we try to put it to work as soon as possible. So whichever floors are ready, and it is the same at Nanavati, it is the same at Mohali, it is the same at Smart. It has been same in the past at Shalimar Bagh or Vaishali or every facility, which we have rolled out has been rolled out in this manner.

Keshav Gupta: We mentioned in the speech also, our available capacity of operational beds have gone up by 8%. So, the OBD has gone up 8%. As and when the beds are ready, they are being taken up. In context, the Smart beds have become live within 26-27 months of the project starting. So typically, a project of this size takes about 36 months. But because we are taking our projects to operations within a shorter time, floor by floor, hence, they are also being taken up floor by floor.

Damayanti Kerai: Sure. And when it comes to ramp-up of some of the newer facilities. You mentioned about Dwaraka. Similarly, can you update on the status of the Noida unit, how it is ramping up in terms of occupancy, etc.? Last quarter, if I remember correctly, you mentioned there are some issues which you are trying to resolve. So, if some update you can share on Noida unit?

Yogesh Sareen: So, the Noida has ramped up well now in this quarter. So the occupancies have

been more than around 64 -65%. There is further room to grow. But on the revenue side, it has done well compared to last quarter. And we have also hired doctors in that hospital. I would say we are happy with where we are with respect Noida when it comes to quarter-on-quarter growth.

Abhay Soi: Yes, I think it has pulled around very well. And in terms of EBITDA, also, it is a substantial growth that we have seen over there. It is well on its way.

Damayanti Kerai : It is in line with our expectations, right?

Abhay Soi: Yes, absolutely. We are very encouraged by the last quarter and more.

Damayanti Kerai : Okay. And it can very well go to the Network level occupancy of say 75% or so ?

Abhay Soi: Absolutely. I think maybe shortly, we will need to do a brownfield over there and add more beds as well.

Damayanti Kerai : Okay. Good to hear. And my second question is a clarification on discontinuation of oncology drugs. You mentioned 5 -6% OBD got knocked off because of it. Just wanted to understand, these drugs are like high in terms of ticket size, etc. but were they meaningful contributor at the EBITDA level also?

Page 8 of 16 Yogesh Sareen : Yes, earlier, they were given at MRP, and we had 15-16% margins on those. So that used to flow to the EBITDA .

Abhay Soi: It was a 16% margin . But because now we have to give a 30% discount to MRP , so we sort of discontinued it .

Yogesh Sareen : Earlier they were contributing to EBITDA . So, that is the reason when we gave to the price impact of the CGHS, we netted off that. To that extent it has been absorbed through the price increase .

Abhay Soi: We suggested I think it is a INR 200 crore net benefit...

Yogesh Sareen : INR140 crore after GST.

Abhay Soi: After GST , it was INR140 crore . With the CGHS rate revision, while we have got higher rates, but we got capped out on this high -value drugs. So , from a CGHS standpoint, the net benefit was INR 200 crore . There was also an impact of GST, so net of it the total benefit was INR 140 crore .

Damayanti Kerai : INR200 crore, you mentioned it is a gross number and then when you include the GST impact, etc., INR 140 crore is the number, which was coming from it...

Yogesh Sareen : That is right. Of this INR 30 crore, as I mentioned is super specialty rates, which has not flowed in yet.

Abhay Soi: So that is about INR 100-110 crore has flowed in. About INR30 -40 crore is yet to flow.

Yogesh Sareen: This is all annual numbers .

Damayanti Kerai : Sorry, just to process , another INR30 crore flow out from here?

Abhay Soi: Yes. So out of INR 140 crore, INR 30-40 crore is yet to flow in. About INR100 - 110 crore has already flown in on an annualized basis.

Damayanti Kerai : And I think my last question is on the pipeline projects, which you indicated, which are coming up in '28 or so. Is any facility there where we are seeing some delays, etc., in terms of approvals or it is just like completion of the facility, which should be done as per your indicated timeline? So, any regulatory clearance or any other clearances which are due?

Abhay Soi: No, nothing . No regulatory clearance is pending over and above what has been anticipated. In the past, there have been delays because of GRAP 3. We have had delays because of shortage of manpower due to the LPG crisis .

Forest approval was delayed because there was an issue with respect to Delhi tree transplantation where the Supreme Court had taken cognizance of . There was a matter between the Lieutenant Governor versus the Supreme Court - contempt of court matter because of which it got stalled.

So, these are not typically regulatory approvals, which have been sort of delayed .

But I think we had issues and incidents with respect to pollution and shutdowns of construction or tree transplantation or Iran war causing shortage of LPG and

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herefore, manpower not showing up at site.

Page 9 of 16 Damayanti Kerai : Okay, that is helpful. Thank you. I will get back into the queue .

Mod erator : Thank you. Next question is from the line of Karan Vora from Goldman Sachs.

Karan Vora : The first question is with respect to doctor costs. So , we see that our doctor costs have gone up and we have hired in advance. But just wanted to get a sense on for which all expansions have we hired and like which are the hospitals where further doctor additions are still pending, which might hit the cost line item in the next one to two quarters?

Abhay Soi : That is a very good question. So , there are two things. End of 2024, we essentially added close to 25% to 30% more capacity. This was whether through Dwarka , whether through the Jaypee acquisition, Sahara done some time before that or Alexis in Nagpur. This year we have already started a phased rollout of 20% plus more capacity, which includes Mohali (Punjab), Nanavati (Mumbai), Smart (Saket) as well as we added more beds in Lucknow as well.

So, somebody asked me a question before this about Noida. And we have seen meaningful improvement over there because we have expanded our doctor base.

We have seen the same in Luck now where we are ramping up . We have seen the same in Smart. We have seen the same in Nanavati, Mumbai. We have seen the same in Mohali.

So, all of these new ones which are coming, we have added doctors. Even Dwarka, now it is operating at 80-85% capacity. But through the last quarter, we had added some more people over there as well. So yes, so I think it has been pretty much quite secular across the portfolio because it has been a multi-location addition of capacity . You are going to see the same thing in Bhubaneswar now because we will add manpower over there. Although it should not move the overall needle simply because there is just one hospital. But the minute you are doing it in 4 or 5 multi-locations and at the same point of time, you are going to see a little bit of lumpiness.

Karan Vora : Okay. Got it.

Abhay Soi : The reason I mentioned in end of '24 is that when you do sudden acquisitions, it is not as if you are able to do all your hiring in the next 6 -12 months. It takes time and moves in a phased manner , particularly if there is competition in those areas.

Karan Vora : Okay. Got it. And sir, just to better understand, so this INR 435 crore for doctor fees in Q4, that should not materially change going forward, at least for the next few quarters. Is that the fair way to think about it?

Abhay Soi : Yes, it should actually start getting operating leverage in fact. My belief is that marginally all of this percentage starts coming down

Karan Vora : Got it. And this Kalinga Hospital. So , any start-up losses or like what about breakeven timelines? How should we think about that?

Abhay Soi : It is already profitable. I think there is about INR 10-odd crore of EBITDA. So, you are not starting with a negative .

Karan Vora : INR 10 crore per quarter ?

Page 10 of 16 Abhay Soi : Annual.

Karan Vora : Annual. Okay. Got it. And so , we have like unlike some other places where we had to discontinue businesses, right, because we wanted to streamline practices. All those things have been taken care of. And even after that, we should be able to maintain broadly that EBITDA run rate. So , no chance of it going into negative, that is what I am trying to.

Abhay Soi : Let me just correct you . Even in the other places, where we corrected the business behavior, so to say, it did not go into negative, we just made less profit. None of these acquisitions have been loss-making or even after taking over when we did whatever actions we took, it did not go into a negative territory.

Karan Vora: Okay. Got it. Thanks for the clarification.

Abhay Soi

: Even in this case, it is INR 10 crore positive and we have just done the acquisition. So, you are going to see it ramp up and in case we are taking some actions, you still would not see meaningful negative numbers .

Karan Vora : Got it. And the last question would be with respect to the new units or whatever we have operationalized in the last 12 to 15 months, how have their overall revenue and margin trajectory look like, any color there will be helpful?

Abhay Soi : Basically before rolling out this 20-25% capacity rollout that we have recently done over the last 3 to 5 months , the previous generation for about 12 -13 months, we did not really add any capacity. We added it before that, again, which was a little lumpy. We did Lucknow, Nagpur, Dwarka , Jaypee ; so are those the four you are referring to?

Karan Vora : Yes.

Abhay Soi : Okay. I think all four have done significantly well. Lucknow, for example, is doing pretty much 5x of EBITDA of what we acquired it for. The meaningful addition in Nagpur, and in Noida also we have seen that . Dwarka is not only operating at 80 - 85% capacity utilization, but we are already planning a brownfield over there of another 200 -plus beds. In fact, oncology bunker over there and the oncology center over there is in Dwarka yet to start, which is going to start by next month. So that should reap even further benefits. So, we are seeing those benefits come through.

Karan Vora: Got it. Helpful. Thanks.

Abhay Soi: Yes. But you are going to see a bit of a hockey stick there because you have the benefit. When you move from like, let us say, in Noida, when you move from the 65% to that 75 -80%. You see at 50 -55%, you break even right, most units. At 60 - 65%, you start doing very well, but the real juice starts coming from that 60 -65% up to that 80 %-odd occupancy that we do.

Moderator: Thank you. Next question is from the line of Tushar Manudhane from Motilal Oswal Financial Services Ltd.

Tushar Manudhane : Thanks for the opportunity. This was more on the Lucknow side, while we have a very decent land bank as far as Sahara hospital is concerned. And, of course, in the history you see Shaheed Path land was also acquired more or less at the similar timeline. So how are we evaluating in terms of which land bank to sort of set up? I

Page 11 of 16 understand that the Board has given sort of approval for Shaheed Path. But if you could help understand how are we going to sort of utilize both the land banks and build up Lucknow sort of hospital network.

Abhay Soi : So, I think it is a very good question. We have a land bank of about 27 acres of land at Gomti Nagar, which is right in the heart of Lucknow, and 5-plus acres, which is on Shaheed Path. We are expanding our capacity over the next few months, you will see our capacity of Gomti Nagar go up to about 570 beds. What we intend to do is we intend to start another hospital. We want to have a multi-location strategy of about 700 beds in Shaheed Path, which we will be operating in a phased manner. It should take about 3 years to build it.

Simultaneously, we believe we will be running out of capacity at Lucknow at Gomti Nagar as well. So, we will be adding another 200-300 beds over there. So, we are going to have a multi-pronged strategy. I think one does not have much to do with the other because these are 2 locations, which means we will have two sets of doctors and clinicians. It allows you to do that.

You see, if we have one, in terms of the strength of senior clinicians, you have one Chairman cardiology, one Chairman oncology, etc., so it becomes a choke point for other senior personnel coming in. When you have multiple locations, it allows you to do that.

And we have seen that advantage play out for us in Delhi, in other places, etc. so, we like the whole cluster approach. I mean, you have benefits of

brownfield, but it

is also beneficial to have a cluster approach and then have brownfields emerging at multiple locations.

Tushar Manudhane : Understood. So, this is not so much more sort of a cannibalizing given that it is hardly a 14 kilometers distance, it will be more like a multi-location playing out?

Abhay Soi : Absolutely.

Yogesh Sareen : Yes. Also, on this Shaheed Path, the capacity can go up to roughly 900 beds. So, I mean, there is more we can build there even after these 700 beds.

Tushar Manudhane : So, I mean, it reflects the kind of confidence you have on this Lucknow as a location, building up such a strong almost it will be about 1,000, in fact, more than 1,000 plus sort of a bed size eventually and not like immediately, but over a period of time?

Abhay Soi : Absolutely. Let me put it this way, over the next decade, I see even probably Gomti Nagar going close to perhaps 2,000 beds, in just that one location. We will do it in a phased manner. We have the land bank over there. And the kind of ramp-up we are seeing is because our clinical programs are very strong over there.

Tushar Manudhane : Understood, second, sir, are you seeing the risk of these medicines being taken directly by the, let us say, health scheme or CGHS in other therapies?

Abhay Soi : Firstly, because of being in Delhi, we have a larger amount of institutional business, which was coming to our Delhi hospitals and this impacts that largely.

What they basically said is that we have to provide these drugs at 30% discount to MRP.

Page 12 of 16 And so, they have not selected the drugs. They said all medicines at a 30% discount to MRP or alternatively CGHS, patients can get the medicine from CGHS. So, from a hospital standpoint, if your margins are less than 30%, then you probably discontinue it because it becomes loss-making, but the patient has the option of buying it from the CGHS dispensary.

But our present margins, on a drug-on-drug basis also is reflective of that action. It is not a government action, it is a CGHS price revision. So, you get some and you lose some, I guess. The net benefit is INR 200 crore. After GST impact, which is a separate hit that we got, it becomes INR 140 crore net benefit.

Tushar Manudhane : I got you. Sir, just that clarification, this is onco therapy specific or this is more between CGHS doing it across the therapies?

Abhay Soi : All chemotherapies.

Tushar Manudhane : Yes, this is why I was trying to ask that, is this getting expanded to other therapies where the patient...

Abhay Soi : No. This is the first time they made any changes on the price in 14 years.

Tushar Manudhane : Understood. And practically, if CGHS is able to provide the medicines on a timely basis? Or do you think the patient will come back to the hospital pharmacy or the

hospital ?

Yogesh Sa reen : I think there is a lot of noise among the CGHS pensioners, etc due to delays .

Abhay Soi : We have been given to understand that they are reconsidering it. But again, one does not know . There has been a lot of representation made even by patients on this.

Tushar Manudhane : Got it, sir.

Abhay Soi : One of the problems is that this also applies to ECHS, for example and other PSUs.

But while CGHS has dispensaries to dispense these drugs, ECHS and other PSUs do not have the dispensaries and they cannot access CGHS dispensaries. So, the problem with these drugs are that they are not accessible to them.

Tushar Manudhane : Got it. So effectively , this should sort of start getting reflected back probably in maybe some time in future ?

Abhay Soi : Sorry, it should get reversed you are saying?

Tushar Manudhane : Yes.

Abhay Soi : I mean it stands to reason.

Tushar Manudhane : Understood. Got it.

Abhay Soi : Otherwise, we will have service men who are not able to access these, which is presently the case.

Page 13 of 16 Tushar Manudhane : Got it. And just lastly, with Gurugram, we had to sort of start second half. So , is it like now we are indicating in for a few months, sort of taking some time? Is that the way to think about?

Abhay Soi : No, that is right. We were saying second half and we meant middle of second half , and now it is end of the second half. But there have been two issues over here.

One was a lot of labour went back during elections , particularly Bengal elections. It was a bit of a festive season for them from that standpoint. So yes, we had a lot of reduction in manpower at the sites. Second is also the LPG issue. Most labour , they cook their own food , so we had disruption over there because the labour did not have LPG . We have started serving meals for all labour at our sites now , in order to surpass that .

The labour is not on our books , they are the contractor's labour . So, there is a particular method that you have to follow to even initiate that because literally, we have got about 1,100 -1,200 people working on the Gurugram site . So, our kitchen is supplying meals to the 1,100 -1,200 people . So yes, these are the issues which have caused a delay of a couple of months.

Tushar Manudhane : Okay. Thanks for that clarification. And just lastly, if I may, just one more, if I may squeeze in . With respect to doctor talent cost , given the kind of bed addition and starting of hospitals by multiple corporates probably over the last , maybe couple of years and subsequently over the next 4 to 5 years. Are you seeing this doctor talent cost sort of the negotiating power moving to doctors from corporate?

Abhay Soi : There are two things. Firstly, it is not a new phenomenon that hospitals come up in locations where there are existing hospitals. And it is not as if in any micro market, it is like you have got five hospitals and three more have come up or four more have come up. It is pretty much been one here and one there .

But when that happens, you know your own clinicians tend to negotiate their compensations and it does go up from that standpoint. But having said that, it does even out and it is transferred over a period of time to the patient because eventually, it's still a 10-12% PAT margin business , it is capital intensive, and we are reinvesting . It is a natural phenomenon. It gets a little lumpy sometimes. But it is not as if it is not passed through - you will see this normalize across the board. I am not just saying it for Max, but I believe for the entire industry.

Tushar Manudhane : Got it sir this is helpful. Got it. Thanks a lot.

Moderator: Thank you. Next question is from the line of Aditya Chheda from InCred Asset Management.

Aditya Chheda : Hi, thank you for the opportunity. It seems like confusion regarding the discontinuation of chemotherapy drugs due to MoU conditionalities, this is specific to Max Healthcare in a region or this is industry-wide pan-India. And if I understood it correct, it had a knock of negative impact on revenue to the tune of INR130 crore, and they had around 16 -17% EBITDA margins, if you can help clarify that?

Abhay Soi: Firstly, this is an industry-wide phenomenon. Secondly , as a proportion of its business, Max perhaps does the maximum amount of oncology business. And then within that, if you actually see, we probably do the maximum amount of institutional business as well , which is CGHS , ECHS , etc. So, although it is an

Page 14 of 16 industry-wide phenomenon, the impact would be felt maximum by us for these reasons.

Yogesh Sareen : The overall impact of the discontinuation of these drugs is IN R 200 crore in the drug billing, i.e. top line.

Aditya Chheda: Is this the quarterly impact ?

Yogesh Sareen : This is the annual number. Annual billing of chemotherapy drugs will come down by INR 200 crore because of the discontinuation.

Aditya Chheda: Got it. Thank you. Thanks for the clarification.

Moderator: Thank you. Next question is from the line of Abdulkader Puranwala from I CICI Securities.

Abdulkader Puranwala: Just a follow-up on this CGHS part. So, if I see the contribution from CGHS, it is barely moved the needle. And where our onco revenues are seeing a sizable dip.

And within that, if you could help us understand that what portion of your CGHS revenues actually comes from oncology and how should we look ahead as well?

Yogesh Sareen: Obviously, there's a lesser impact on the beds for this. These chemotherapies are billed in the day care. When you discontinue these drug billings or providing these drugs to the patient, there's an impact on the revenue but there's no impact on the beds. Nevertheless, there are some of these patients who also then avail surgeries in the network and some of these patients are also then admitted in the hospital. To that end, the Occupied Bed Days (OBDs) for oncology patients have come down by 6% YoY. Now your question is that within the overall CGHS business, how much is oncology? That'll be probably around 35-40%.

Keshav Gupta: It used to be 50% as it is reduced to 40% now.

Abdulkader Puranwala: Understood, sir. And sir, then on your existing Network that is prior to any bed additions that you have done, in terms of the steady state revenue and EBITDA growth, sir, how should we look at in terms of your, say, bed network, what you had developed for '25 or for '26. And the levers of growth, would it be more ARPOB driven? There is some element of case mix as well, which can help to at least post a single digit kind of a growth.

Yogesh Sareen : Basically, there are two elements which make the revenue go up. One is the ARPOB and other is the Occupied Bed Days (OBDs). You know that even at the existing hospitals, we are adding more beds. There is brownfield expansion being done in Mohali, Nanavati, Lucknow and Smart. Our role is to make sure that where we have 80-85% occupancies, then we add more beds there and try to ensure that there is OBD growth also.

So, if we do not have OBD growth, then the only growth would be the ARPOB growth rate, which will be, let us say, 6-7% of the revenues. But our role is to ensure that wherever we find this high-occupancy situation, then we add more beds.

Abdulkader Puranwala: Okay understood. And sir, last one from my end. Would it be possible to quantify the EBITDA drag with the new hospitals would have had in Q4 and for the full year '26?

Page 15 of 16 Yogesh Sareen: No, it is very tough to do it now. You can do it in the probably first month or two, but then the patients start to mingle. So, when we put another Brownfield tower, we will not be able to separate the revenues or EBITDA of Tower 1 and Tower 2.

Abhay Soi: Because what you also do, for example, is move certain specialties into A-Tower versus B-Tower.

So, when you move those specialties, then wherever you move it from, those beds get occupied by other specialties and where you move it to, that gets occupied, so, both will have its own trajectory. Let's say, if I move oncology into a new tower, then that becomes an onco-tower. So, in the previous place where it was occupying those beds, those beds get occupied by other specialties. Whereas in the new tower, you will have largely oncology, so you will have different ARPOBs and different EBITDA per bed, etc.

Abdulkader Puranwala: Okay, understood.

Abhay Soi: But on a consolidated basis, what we have seen is that there is no real pressure on margins.

Abdulkader Puranwala: Okay, got it.

Moderator: Thank you. Next question is from the line of Lavanya from UBS Securities India Private Ltd.

Lavanya: Hello. Thank you for the opportunity, sir. Just a clarification on chemo drugs, again, sorry, here, we are losing out on OBD

also. So, if it is only the dispensary

and the drugs to who are -- I mean, to whom we are losing the OBD patients in general, the 5-6% impact on OBD, clarification on that will be great? And Q1 so

far -- I mean until now, you are seeing a full impact of this chemo drugs, at least, right?

Yogesh Sareen: That is correct. Quarter 4 has already had the full impact. We started to discontinue these drugs in Oct '25. So, to that extent, quarter 4 has the full impact of discontinuation of all these chemo drugs. What we have not gotten yet is the full impact of the price gain on the CGHS side, which will come starting this quarter. So, as I said, 2 of the hospitals have already got the super specialty rates. And during the course of the year, we will have more hospital also getting those rates.

Abhay Soi: So, on the other question that you have on the OBDs, whom are we losing to? Well, we do not exactly know. They go, perhaps, to the disorganized sectors or smaller nursing homes.

Lavanya: Okay. Got it. And here do we expect any resolution or this is going to be there for the next couple of quarters and until the base of Q3, like it will start to...

Abhay Soi: Like I said earlier, it does not seem to stand to reason, because it is not as if our margins are any different from the price at which perhaps the government is buying it. These are medicines like Keytruda, which is the largest and the fastest-growing oncology medicine in the world. And they do not make any exceptions for anybody in terms of what the margins are, for example, one is that.

Page 16 of 16 Secondly, the CGHS and CGHS rate applied to the other public sector undertakings and other panels like ECHS, which is ex-servicemen and so on. While CGHS patients have an option of procuring the drugs directly from the CGHS dispensary or their pharmacy. The ECHS and PSUs do not have that option because they do not have those pharmacies.

So of course, a lot of patients are complaining because they are unable to access these drugs, at least Keytruda, and they have to move to other medicines. And it is not a necessity, since every patient is different. For some patients, it is necessary. I guess they are finding it difficult to source this, so we are facing a lot of complaints, and I am sure so is the CGHS. And we are hoping that they will reconsider this. But asking for timelines, it is difficult to pin that down.

Lavanya: Okay. Got it. So just a clarification, ECHS on the PSU patients are seeing disruption to the treatment for the last 6 months or they are moving out to other medicines, whatever is alternate available?

Abhay Soi: Same thing. They are going to behave the same way as the CGHS patients.

Yogesh Sareen: Yes, and there are other alternatives to some of these medicines.

Lavanya: Okay, got it.

Abhay Soi: It is not that there are no alternatives to it, there are alternatives, okay? But they may not be preferred alternatives from a clinical standpoint. The question is the efficacy. If a doctor believes that this particular branded drug has more efficacy for a particular patient may not be for B patient, but for A particular patient. I mean, very simply, each one of us reacts differently to different paracetamol. Reacts differently to even a different antibiotic, let alone chemo.

Moderator: Thank you very much. Ladies and gentlemen, we will take that as the last question. I will now hand the conference over to the management for closing comments.

Abhay Soi: Thank you, everyone, for joining us today. We appreciate all your time and look forward to interacting with you again next quarter.

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